II

VOLUME II · EXAMINATION PRACTICE

Answers & Justifications

1,500 clinical single-best-answer questions

All 1,500 keyed answers and available clinical rationales

Foundation 270 · Intermediate 730 · Advanced 500

Question numbers match the separate Questions PDF

Independent revision material · Specialist review is still required

Dr. Kam O Man, Owen

BDS (HK) · MFDS RCSEd · MFDS RCPS (Glasg)

MClinDent (Fixed and Removable Prosthodontics) (KCL)

MImpDent (RCSEd) · MGDS RCSI

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Using this answer book

What this file includes. The keyed answer to every Q1-Q1500 question, accompanied by the source explanation, competing-option notes when available and the revision unit reference.

How to use it. Work through the separate Questions PDF first. Use the same Q-number to locate its answer in this booklet. Use PDF bookmarks to navigate the Foundation, Intermediate and Advanced answer sections.

Scoring. Correct = 1; incorrect or unanswered = 0. A score of 105/150 represents 70% on a 150-question practice paper, but is not an official examination pass threshold.

Explanation quality. Some entries do not have independent analyses of each distractor, and several original source explanations remain brief. The PDF preserves these limitations rather than inventing additional certainty.

Clinical status. Source-derived teaching points may be superseded by newer guidelines. These AI-assisted educational drafts have not completed independent specialist peer review or psychometric calibration.

Printed companion to the GitHub Pages question bank

- 1,500 question IDs, five answer options each.
- 270 Foundation / 730 Intermediate / 500 Advanced.
- Author: Dr. Kam O Man, Owen.
- Editorial edition: November 2026.

PART II / ANSWER CARDS & RATIONALES

The answer cards follow the same difficulty and topic order as Part I. They reproduce the keyed answers and explanations in the current website data set; the level of supporting detail is not uniform across all items.

Foundation answers · 270 items

BLOCK A · Applied anatomy

Q2 FOUNDATION · Block A

Best answer: D. Temporalis

Justification: Temporalis lies in the temporal fossa and is the muscle most often palpably tender in tension-type headache linked to TMD.

Why not the closest alternatives: Masseter is felt over the ramus; the pterygoids cannot be palpated reliably from the temple.

Revise: Unit 1 §1.1 (exam traps), Unit 9 · LO 1.1

Return to Question 2 ↑

Q3 FOUNDATION · Block A

Best answer: A. Lateral pterygoid (inferior head)

Justification: The inferior head inserts into the pterygoid fovea on the condylar neck, so after a neck fracture it pulls the head forwards and medially.

Why not the closest alternatives: Masseter, temporalis and medial pterygoid act on the ramus and coronoid, below the fracture.

Revise: Unit 1 §1.1 · LO 1.1

Return to Question 3 ↑

Q5 FOUNDATION · Block A

Best answer: B. Posterior temporalis

Justification: The posterior horizontal fibres of temporalis are major retractors of the mandible; masseter and medial pterygoid chiefly elevate.

Why not the closest alternatives: The anterior temporalis can elevate strongly, whereas the inferior lateral pterygoid contributes to opening and protrusion.

Revise: Unit 1, In one minute · LO 1.1

Return to Question 5 ↑

Q11 FOUNDATION · Block A

Best answer: B. Glossopharyngeal afferent, vagus efferent

Justification: IX carries the sensation from the posterior tongue and oropharynx; X drives the pharyngeal response.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.4 · LO 1.1

Return to Question 11 ↑

Q14 FOUNDATION · Block A

Best answer: B. Relieve the distobuccal flange after confirming the functional masseteric notch

Justification: The masseter contracts during clenching and can impinge on an overextended distobuccal border. Functionally localised relief addresses the cause without sacrificing support elsewhere.

Why not the closest alternatives: An occlusal cause should still be considered, but the reproducible local border interference is more discriminating than a generic lack of retention.

Revise: Volume I, Unit 1 · Diploma LO 1.1; 2c.13; 2c.21 · Reference themes: 2015 MRD; 2020 Edinburgh MPros

Return to Question 14 ↑

Q15 FOUNDATION · Block A

Best answer: D. Coronoid process

Justification: On opening and lateral movement the coronoid moves forward into the space beside the tuberosity, limiting the width of the upper distobuccal flange.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 15 ↑

Q16 FOUNDATION · Block A

Best answer: C. Fovea palatini, near the vibrating line

Justification: The post-dam lies between the anterior and posterior vibrating lines; the foveae sit close to this area and help locate it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 16 ↑

Q20 FOUNDATION · Block A

Best answer: C. Parotid duct

Justification: It opens opposite the upper second molar and can be compressed by an over-extended or thick buccal flange.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6, §1.8 · LO 1.1

Return to Question 20 ↑

Q24 FOUNDATION · Block A

Best answer: B. Cavernous sinus thrombosis via the angular and ophthalmic veins

Justification: Upper canine infection above levator anguli oris enters the canine space. It can spread through the angular and ophthalmic veins to the cavernous sinus.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7 · LO 1.1

Return to Question 24 ↑

Q26 FOUNDATION · Block A

Best answer: D. Pterygomandibular space

Justification: It contains the inferior alveolar and lingual nerves and vessels and is the target of the block.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7 · LO 1.1

Return to Question 26 ↑

Q28 FOUNDATION · Block A

Best answer: E. Long buccal nerve, the sensory branch of the anterior division

Justification: The buccal nerve is the only sensory branch of the anterior division of V3; the inferior alveolar block does not reach it, so the buccal gingiva of lower molars needs a separate injection.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.2 · LO 1.1

Return to Question 28 ↑

Q29 FOUNDATION · Block A

Best answer: B. Mylohyoid and anterior belly of digastric

Justification: The nerve to mylohyoid leaves the inferior alveolar nerve and supplies both.

Why not the closest alternatives: The posterior belly of digastric is facial nerve; geniohyoid is C1.

Revise: Unit 1 §1.2, §1.4 · LO 1.1

Return to Question 29 ↑

Q30 FOUNDATION · Block A

Best answer: C. Chorda tympani carrying taste and parasympathetic fibres via the lingual nerve

Justification: Chorda tympani contributes anterior-tongue taste and parasympathetic secretomotor fibres that join the lingual nerve en route to the submandibular ganglion.

Why not the closest alternatives: Lingual injury may also affect sensation, but the characteristic combined loss after ear surgery implicates chorda tympani rather than tongue motor nerves.

Revise: Unit 1 §1.2, §1.8 · LO 1.1

Return to Question 30 ↑

Q31 FOUNDATION · Block A

Best answer: C. Auriculotemporal nerve

Justification: It carries secretomotor fibres to the parotid. After surgery these fibres can regrow into sweat glands (Frey's syndrome).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.2 · LO 1.1

Return to Question 31 ↑

Q32 FOUNDATION · Block A

Best answer: A. Middle meningeal artery

Justification: A branch of the first part of the maxillary artery, it enters the skull through the foramen spinosum. The auriculotemporal nerve embraces it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.3 · LO 1.1

Return to Question 32 ↑

Q34 FOUNDATION · Block A

Best answer: B. Second (pterygoid) part

Justification: The second part, related to the lower head of lateral pterygoid, gives the deep temporal, pterygoid, masseteric and buccal branches.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.3 · LO 1.1

Return to Question 34 ↑

Q35 FOUNDATION · Block A

Best answer: E. It arises from the main trunk of V3, before it divides

Justification: The main trunk gives the nervus spinosus and the nerve to medial pterygoid before dividing. That nerve also passes through the otic ganglion to tensor veli palatini and tensor tympani.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.1 (exam traps), §1.2 · LO 1.1

Return to Question 35 ↑

Q37 FOUNDATION · Block A

Best answer: C. Temporalis

Justification: Temporalis inserts into the apex and medial surface of the coronoid and the anterior border of the ramus, so it pulls a coronoid fragment upwards.

Why not the closest alternatives: Lateral pterygoid inserts into the pterygoid fovea, disc and capsule, never the coronoid.

Revise: Unit 1 §1.1 · LO 1.1

Return to Question 37 ↑

Q38 FOUNDATION · Block A

Best answer: E. Temporalis (posterior, horizontal fibres)

Justification: The near-horizontal posterior fibres are the main retractors; the deep masseter assists.

Why not the closest alternatives: Lateral pterygoid protrudes, the opposite movement.

Revise: Unit 1, In one minute · LO 1.1

Return to Question 38 ↑

Q39 FOUNDATION · Block A

Best answer: D. Right lateral and medial pterygoids

Justification: To move the chin left, the right condyle must move forward and medially; the right pterygoids do this.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.1 · LO 1.1

Return to Question 39 ↑

Q40 FOUNDATION · Block A

Best answer: B. Lateral pterygoid (superior head)

Justification: The superior head attaches to the disc and capsule, so it is the muscle most associated with disc displacement, clicking and locking.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.1 (exam traps), Unit 9 · LO 1.1, 2a.15

Return to Question 40 ↑

Q41 FOUNDATION · Block A

Best answer: E. Vagus via the pharyngeal plexus

Justification: Palatoglossus is the only tongue muscle not supplied by the hypoglossal nerve. Normal protrusion shows XII is intact.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.4 · LO 1.1

Return to Question 41 ↑

Q45 FOUNDATION · Block A

Best answer: B. C1 fibres travelling with the hypoglossal nerve

Justification: Geniohyoid is supplied by C1 fibres that travel with XII; it depresses the mandible and pulls the hyoid forward.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.4 · LO 1.1

Return to Question 45 ↑

Q47 FOUNDATION · Block A

Best answer: C. At least the anterior half to two-thirds

Justification: This gives support and a posterior seal without impinging on the pterygomandibular raphe.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 47 ↑

Q49 FOUNDATION · Block A

Best answer: A. Create selective relief over the torus while preserving support elsewhere

Justification: A palatal torus may require selective relief so that concentrated pressure does not produce pain while other bearing surfaces remain supported.

Why not the closest alternatives: Broad indiscriminate relief weakens support and does not target the local high-pressure area.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 49 ↑

Q51 FOUNDATION · Block A

Best answer: D. Pressure over the superficial mental foramen, with referral if symptoms persist

Justification: Advanced resorption may make the mental foramen vulnerable to denture pressure, producing lower lip paraesthesia; persistent unexplained neurosensory change also warrants investigation.

Why not the closest alternatives: Other border adjustments do not directly explain focal lower-lip sensory symptoms.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 51 ↑

Q52 FOUNDATION · Block A

Best answer: C. Submandibular (about 65–70%)

Justification: The submandibular gland produces most unstimulated (resting) saliva.

Why not the closest alternatives: The parotid dominates stimulated flow.

Revise: Unit 1 §1.8 · LO 1.1

Return to Question 52 ↑

Q53 FOUNDATION · Block A

Best answer: A. Stimulated saliva

Justification: The parotid contributes most of the stimulated saliva, so its loss reduces flow during eating most.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.8, Unit 4 (radiotherapy) · LO 1.1

Return to Question 53 ↑

Q54 FOUNDATION · Block A

Best answer: D. Submandibular (Wharton's) duct, at the sublingual papilla

Justification: It opens at the sublingual papilla beside the lingual frenum, the common site for salivary stones.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.8 · LO 1.1

Return to Question 54 ↑

Q55 FOUNDATION · Block A

Best answer: B. Glossopharyngeal nerve, lesser petrosal nerve, otic ganglion, auriculotemporal nerve

Justification: This is the parasympathetic route to the parotid.

Why not the closest alternatives: A is the submandibular route; E supplies the lacrimal and nasal glands.

Revise: Unit 1 §1.8 · LO 1.1

Return to Question 55 ↑

Q58 FOUNDATION · Block A

Best answer: A. Superficial temporal and maxillary arteries

Justification: The TMJ is supplied by branches of these two vessels.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 58 ↑

Q59 FOUNDATION · Block A

Best answer: D. Synovial ginglymoarthrodial (hinge and gliding) joint

Justification: It hinges (rotation, lower compartment) and glides (translation, upper compartment).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 59 ↑

Q60 FOUNDATION · Block A

Best answer: C. Sphenomandibular ligament

Justification: It runs from the spine of the sphenoid to the lingula, medial to the nerve at the mandibular foramen, and is an accessory TMJ ligament.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 60 ↑

Q67 FOUNDATION · Block A

Best answer: A. Choose a path or block out one side, reducing surgically only if both still prevent insertion

Justification: A suitable path of insertion or relief on one side usually allows insertion; surgery is reserved for undercuts that still prevent it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6, Unit 7 · LO 1.1

Return to Question 67 ↑

Q68 FOUNDATION · Block A

Best answer: D. A labial bar, or surgical removal of the tori if a sound design needs it

Justification: Tori block a lingual connector; a labial bar avoids them, or the tori are removed if a sound design needs a lingual connector.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6, Unit 6 (connectors) · LO 1.1, 2c.3

Return to Question 68 ↑

BLOCK B · Physiology and pathology

Q72 FOUNDATION · Block B

Best answer: B. Failure of the peripheral seal compromises pressure-related retention

Justification: A functional peripheral seal is an important contributor to complete denture retention; loss of the seal can permit air entry and loss of resistance to dislodgement.

Why not the closest alternatives: Internal adaptation and saliva also matter, but the movement-related posterior seal break is the decisive finding here.

Revise: Unit 2 §2.1 · LO 1.2

Return to Question 72 ↑

Q76 FOUNDATION · Block B

Best answer: A. Bicarbonate

Justification: Bicarbonate concentration rises with flow rate and is the main buffer of stimulated saliva.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.1 · LO 1.2

Return to Question 76 ↑

Q77 FOUNDATION · Block B

Best answer: C. Secretory immunoglobulin A

Justification: Secretory IgA contributes antigen-specific mucosal defence, while lysozyme and lactoferrin provide additional innate antimicrobial functions.

Why not the closest alternatives: Bicarbonate is a buffer and mucin mainly lubricates; neither is the specific antibody-mediated mechanism asked about.

Revise: Unit 2 §2.1 · LO 1.2

Return to Question 77 ↑

Q82 FOUNDATION · Block B

Best answer: B. 20

Justification: The WHO goal is retention of at least 20 functional teeth throughout life.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.2 · LO 1.2

Return to Question 82 ↑

Q85 FOUNDATION · Block B

Best answer: C. About four times as much

Justification: Tallgren showed the anterior mandible loses about four times the height of the anterior maxilla in the long term.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2, In one minute, §2.3 · LO 1.3

Return to Question 85 ↑

Q86 FOUNDATION · Block B

Best answer: B. In the first 6–12 months

Justification: Resorption is fastest early after extraction and continues more slowly for life. This is why immediate dentures need relining.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.3 · LO 1.3

Return to Question 86 ↑

Q87 FOUNDATION · Block B

Best answer: B. Retain and prepare suitable canine roots as overdenture abutments

Justification: When suitable, retained roots can provide overdenture support and preserve periodontal mechanoreceptor function while avoiding extraction.

Why not the closest alternatives: Implants may be considered but are not uniquely required where acceptable natural abutments and patient preferences favour conservation.

Revise: Unit 2 §2.3 · LO 1.3

Return to Question 87 ↑

Q89 FOUNDATION · Block B

Best answer: D. Reduced interocclusal space, so the over-erupted tooth must be managed first

Justification: Over-eruption after tooth loss reduces the space for a replacement; the opposing tooth may need adjustment, a restoration or orthodontic intrusion first.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.3 · LO 1.3, 2c.1

Return to Question 89 ↑

Q91 FOUNDATION · Block B

Best answer: D. Reduce the frequency of sugar intake

Justification: The Stephan curve shows each intake drops plaque pH for 30–60 minutes. Frequency, not total quantity, drives caries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.4 · LO 1.4

Return to Question 91 ↑

Q92 FOUNDATION · Block B

Best answer: A. Actinomyces

Justification: Actinomyces is particularly associated with root caries; Streptococcus mutans initiates coronal caries and Lactobacillus drives progression.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.4 · LO 1.4

Return to Question 92 ↑

Q93 FOUNDATION · Block B

Best answer: B. Body of the lesion

Justification: Deep to surface, the zones are translucent, dark, body and surface; the body has the greatest mineral loss.

Why not the closest alternatives: The surface zone stays relatively intact.

Revise: Unit 2 §2.4 · LO 1.4

Return to Question 93 ↑

Q96 FOUNDATION · Block B

Best answer: A. Fluoride varnish 22,600 ppm at least twice a year

Justification: This is the DBOH recommendation for adults at higher caries risk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.4 · LO 1.4

Return to Question 96 ↑

Q97 FOUNDATION · Block B

Best answer: C. Salivary flow and its medication-related causes

Justification: Hyposalivation markedly reduces buffering and clearance, magnifying root caries risk in a patient with recession and an RPD.

Why not the closest alternatives: Ridge shape and TMJ motion do not explain active root-surface demineralisation.

Revise: Unit 2 §2.4 · LO 1.4

Return to Question 97 ↑

Q99 FOUNDATION · Block B

Best answer: E. Erosion

Justification: Acid dissolves tooth around the restorations, leaving them standing proud, with smooth, silky enamel and cupped cusps.

Why not the closest alternatives: In attrition, restorations wear flush with the tooth.

Revise: Unit 2 §2.5 (exam traps) · LO 1.4

Return to Question 99 ↑

Q100 FOUNDATION · Block B

Best answer: C. Attrition

Justification: Matching facets on opposing teeth, fractured cusps and masseteric hypertrophy show tooth-to-tooth wear from bruxism.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.5 · LO 1.4

Return to Question 100 ↑

Q101 FOUNDATION · Block B

Best answer: B. Abrasion

Justification: Sharp, V-shaped cervical notches from a foreign object (the toothbrush) are worst on the side opposite the brushing hand — the left for a right-handed patient.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.5 · LO 1.4

Return to Question 101 ↑

Q104 FOUNDATION · Block B

Best answer: E. 0–3 for the worst surface in each sextant, with the sum guiding management

Justification: BEWE scores the most affected surface in each sextant 0–3; the cumulative score directs prevention, monitoring or treatment.

Why not the closest alternatives: A describes Smith and Knight.

Revise: Unit 2 §2.5, Unit 5 · LO 1.4, 2a.8

Return to Question 104 ↑

Q105 FOUNDATION · Block B

Best answer: C. Smith and Knight Tooth Wear Index

Justification: It scores every surface 0–4 and suits detailed research or monitoring rather than screening.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.5 · LO 1.4

Return to Question 105 ↑

Q110 FOUNDATION · Block B

Best answer: C. Full blood count, haematinics and blood glucose

Justification: Angular cheilitis is predisposed by haematinic deficiency, diabetes and immunosuppression, as well as low OVD and denture infection.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.6 · LO 1.8, 2a.7

Return to Question 110 ↑

Q117 FOUNDATION · Block B

Best answer: A. Combined alcohol and tobacco exposure can act synergistically and reducing both lowers preventable risk

Justification: Alcohol and tobacco are established oral cancer risk factors, with synergistic effects; reducing both is a reasonable preventive priority.

Why not the closest alternatives: Other exposures may also matter, but screening and single-factor assumptions do not negate risk reduction.

Revise: Unit 2 §2.7 · LO 1.11

Return to Question 117 ↑

Q118 FOUNDATION · Block B

Best answer: A. Urgent referral on the suspected cancer pathway

Justification: NICE NG12 lists a red or red-and-white patch, persistent ulceration over 3 weeks, or a persistent lump as triggers for urgent referral.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.7 · LO 1.11, 2a.6

Return to Question 118 ↑

Q121 FOUNDATION · Block B

Best answer: E. The relationship is bidirectional

Justification: Poor glycaemic control worsens periodontitis, and periodontal inflammation worsens glycaemic control.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.8 · LO 1.10

Return to Question 121 ↑

Q123 FOUNDATION · Block B

Best answer: B. Do not soak it in hypochlorite, because it damages Co-Cr

Justification: Sodium hypochlorite corrodes cobalt–chromium frameworks. Use non-abrasive cleaners suitable for metal.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 companion note 03 · LO 1.4, 2b.4

Return to Question 123 ↑

Q125 FOUNDATION · Block B

Best answer: C. Hygiene and risk-factor control, subgingival instrumentation, surgery where needed, then supportive care

Justification: This is the stepwise periodontal sequence; definitive prosthodontics follows stability.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 companion note 02 · LO 2b.3

Return to Question 125 ↑

Q127 FOUNDATION · Block B

Best answer: C. Review medication contributions with the prescriber and intensify fluoride, diet and salivary support

Justification: The prevention plan should address the drug-related salivary deficit collaboratively and intensify caries protection; high-fluoride products may be appropriate according to current prescribing guidance.

Why not the closest alternatives: Simply replacing prostheses or making unilateral medication changes does not address the interacting caries risks.

Revise: Unit 2 §2.1 · LO 1.2, 2a.7

Return to Question 127 ↑

BLOCK C · Dental materials science

Q133 FOUNDATION · Block C

Best answer: D. Match the undercut to clasp material, length, cross-section, approach and abutment support so flexure remains within the elastic limit

Justification: Retention depends on the effective flexibility of the clasp arm, which is influenced by length, shape, cross-section and alloy modulus as well as measured undercut. A simple one-to-one undercut/material table cannot replace full design assessment.

Why not the closest alternatives: Co–Cr cast arms tend to be stiffer than comparable wrought-wire or suitable precious-metal forms; abutment support and planned reciprocation remain critical.

Revise: Volume I, Unit 3 · Diploma LO 1.9; 2c.4; 2c.6 · Reference themes: 2017 MRD notes; clasp selection review

Return to Question 133 ↑

Q136 FOUNDATION · Block C

Best answer: B. Resilience

Justification: Resilience is the energy absorbed elastically (the area under the elastic part of the stress–strain curve).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.1 · LO 1.9

Return to Question 136 ↑

Q148 FOUNDATION · Block C

Best answer: E. Syneresis and imbibition distort it

Justification: Alginate loses water (syneresis) or absorbs it (imbibition) and distorts; pour within about 10–15 minutes unless it is an extended-pour product.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.2 · LO 1.9

Return to Question 148 ↑

Q159 FOUNDATION · Block C

Best answer: E. Distortion if the set putty is reseated without adequate relief

Justification: Reseating set putty over the wash compresses it; it rebounds after removal, giving undersized dies and tight crowns.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.2 · LO 1.9

Return to Question 159 ↑

Q160 FOUNDATION · Block C

Best answer: C. Rinse it, then use a material-specific disinfectant for the time on the label, avoiding prolonged immersion

Justification: Follow the label time; prolonged immersion distorts alginate and polyether.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.2, Unit 13 · LO 1.9, 3a.4

Return to Question 160 ↑

Q161 FOUNDATION · Block C

Best answer: B. Weaker, with less expansion and a slower set

Justification: Extra water leaves more porosity in the set gypsum.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.3 · LO 1.9

Return to Question 161 ↑

Q162 FOUNDATION · Block C

Best answer: E. A faster set and more expansion

Justification: Spatulation breaks up crystals and creates more nuclei for crystallisation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.3 · LO 1.9

Return to Question 162 ↑

Q172 FOUNDATION · Block C

Best answer: A. Invest promptly

Justification: Wax relaxes and distorts as stored stresses release; delays reduce casting accuracy.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.4 · LO 1.9

Return to Question 172 ↑

Q173 FOUNDATION · Block C

Best answer: D. Benzoyl peroxide

Justification: Benzoyl peroxide in the powder decomposes on heating to free radicals. In self-cure acrylic a tertiary amine activates it.

Why not the closest alternatives: Camphorquinone is the light-cure initiator; hydroquinone is the inhibitor.

Revise: Unit 3 §3.5 · LO 1.9

Return to Question 173 ↑

Q182 FOUNDATION · Block C

Best answer: B. Very low residual monomer and porosity, with no polymerisation shrinkage in the clinic

Justification: Industrially polymerised blocks are milled, so there is no processing shrinkage and the fit is better.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.5, Unit 11 · LO 1.9

Return to Question 182 ↑

Q184 FOUNDATION · Block C

Best answer: D. Abrasion of the opposing natural teeth

Justification: Porcelain teeth resist wear but abrade enamel; they also click, are brittle and need more space. Acrylic teeth are preferred opposite natural teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.6 · LO 1.9

Return to Question 184 ↑

Q185 FOUNDATION · Block C

Best answer: C. PEMA powder with an aromatic ester plasticiser in ethanol

Justification: This gel stays viscoelastic for days to weeks, conditioning traumatised mucosa and allowing functional impressions.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.6 · LO 1.9

Return to Question 185 ↑

Q190 FOUNDATION · Block C

Best answer: D. Forms excess carbides, making the alloy brittle

Justification: Carbon forms carbides that harden the alloy, but too much makes it brittle.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.7 · LO 1.9

Return to Question 190 ↑

Q192 FOUNDATION · Block C

Best answer: D. About 110 GPa

Justification: Titanium is about half as stiff as Co-Cr and forms a passive TiO2 layer.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.7 · LO 1.9

Return to Question 192 ↑

Q203 FOUNDATION · Block C

Best answer: B. Stress at a crack tip converts tetragonal to monoclinic zirconia, with about 3–5% volume expansion that closes the crack

Justification: The 3–5% expansion at a crack tip compresses and arrests the crack.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.9 · LO 1.9

Return to Question 203 ↑

Q204 FOUNDATION · Block C

Best answer: A. Low-temperature degradation (ageing): spontaneous tetragonal to monoclinic change in moisture

Justification: In moisture, zirconia can transform spontaneously, roughening and weakening its surface.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.9 · LO 1.9

Return to Question 204 ↑

Q210 FOUNDATION · Block C

Best answer: D. Fluoride release, CTE close to tooth, and sensitivity to moisture early on

Justification: GIC releases fluoride and expands like tooth, but must be protected from moisture while it sets.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.10 · LO 1.9

Return to Question 210 ↑

Q213 FOUNDATION · Block C

Best answer: C. 25 µm

Justification: ISO sets 25 µm as the maximum film thickness for luting cements.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.10 · LO 1.9

Return to Question 213 ↑

Q214 FOUNDATION · Block C

Best answer: D. 30–37% phosphoric acid for 15–30 s

Justification: This produces the type I–III etch patterns; enamel bonding is the most reliable bond in dentistry.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.11 · LO 1.9

Return to Question 214 ↑

Q217 FOUNDATION · Block C

Best answer: D. Air-abraded base metal bonded with an MDP-containing resin cement

Justification: Modern RBB wings use air-abraded base metal bonded with MDP-containing cement (for example Panavia); Rochette, Maryland and Virginia are historical.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.11 · LO 1.9

Return to Question 217 ↑

BLOCK D · Pharmacology, pain control, emergencies and the medically compromised patient

Q224 FOUNDATION · Block D

Best answer: D. Early local anaesthetic toxicity

Justification: Toxicity affects the CNS first (perioral tingling, tinnitus, drowsiness, convulsions), then depresses the cardiovascular system. Stop, call for help and support the airway.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.1 · LO 1.7

Return to Question 224 ↑

Q226 FOUNDATION · Block D

Best answer: A. Low tissue pH reduces the non-ionised fraction of the anaesthetic

Justification: Inflamed, acidic tissue reduces the non-ionised (lipid-soluble) fraction that crosses the nerve membrane.

Why not the closest alternatives: Accessory supply comes from the mylohyoid nerve or cervical plexus, not the facial nerve.

Revise: Unit 4 §4.1 · LO 1.7

Return to Question 226 ↑

Q228 FOUNDATION · Block D

Best answer: E. 30% (a maximum of 70% nitrous oxide)

Justification: Machines must deliver at least 30% oxygen (maximum 70% nitrous oxide), with fail-safe and scavenging.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.2 · LO 2c.8

Return to Question 228 ↑

Q234 FOUNDATION · Block D

Best answer: A. Clarithromycin 500 mg twice daily (or metronidazole 400 mg three times daily for anaerobes)

Justification: These are the SDCEP alternatives for penicillin allergy.

Why not the closest alternatives: Co-amoxiclav and flucloxacillin are penicillins.

Revise: Unit 4 §4.3 · LO 1.7

Return to Question 234 ↑

Q238 FOUNDATION · Block D

Best answer: C. Not routinely recommended

Justification: NICE CG64 does not recommend routine prophylaxis. SDCEP advises considering it for special-risk patients (previous endocarditis, prosthetic valves, some congenital heart disease) with the cardiologist.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.3 (exam traps) · LO 1.7, 2b.12

Return to Question 238 ↑

Q241 FOUNDATION · Block D

Best answer: C. The usual maximum is 4 g in 24 hours, reduced for low body weight and hepatic impairment

Justification: The dose is reduced in low body weight, hepatic impairment and alcohol dependence.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.4 · LO 1.7

Return to Question 241 ↑

Q243 FOUNDATION · Block D

Best answer: B. Proceed in primary care with local haemostatic measures

Justification: SDCEP: check the INR ideally within 24 hours; if it is below 4, treat in primary care with haemostatic measures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.5 · Schematic 23 · LO 2b.11

Return to Question 243 ↑

Q245 FOUNDATION · Block D

Best answer: A. Miss the morning dose

Justification: For twice-daily apixaban or dabigatran and a higher-risk procedure, SDCEP advises missing the morning dose; low-risk procedures need no interruption.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.5 · LO 2b.11

Return to Question 245 ↑

Q246 FOUNDATION · Block D

Best answer: B. Delay the morning dose until about 4 hours after haemostasis

Justification: That is the SDCEP advice for once-daily rivaroxaban or edoxaban and a higher-risk procedure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.5 · LO 2b.11

Return to Question 246 ↑

Q247 FOUNDATION · Block D

Best answer: E. Do not stop either

Justification: Stopping dual antiplatelet therapy after a stent risks thrombosis. Use local measures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4, In one minute, §4.5 · LO 2b.11

Return to Question 247 ↑

Q248 FOUNDATION · Block D

Best answer: A. Liaise with the haemophilia centre

Justification: An inferior alveolar block without factor cover risks a pterygomandibular haematoma and airway compromise.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.5 · LO 2b.11

Return to Question 248 ↑

Q252 FOUNDATION · Block D

Best answer: C. Cancer patient on high-dose IV zoledronic acid

Justification: High-dose IV bisphosphonate or denosumab for cancer carries the highest MRONJ risk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.6 · Schematic 24 · LO 2b.11

Return to Question 252 ↑

Q254 FOUNDATION · Block D

Best answer: B. Drug holidays are not recommended for routine dental care

Justification: SDCEP does not recommend interrupting antiresorptives for routine dentistry.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.6 · LO 2b.11

Return to Question 254 ↑

Q260 FOUNDATION · Block D

Best answer: C. 48 mmol/mol (6.5%)

Justification: An HbA1c of 48 mmol/mol or above diagnoses diabetes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.8 · LO 2a.13

Return to Question 260 ↑

Q261 FOUNDATION · Block D

Best answer: D. Prediabetes (42–47 mmol/mol)

Justification: 42–47 mmol/mol is prediabetes. Refer to the GMP for confirmation and management, as the Level 2 curriculum expects.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.8 · LO 2a.13

Return to Question 261 ↑

Q263 FOUNDATION · Block D

Best answer: C. Adrenaline 1:1000, 500 micrograms IM into the anterolateral thigh, repeated after 5 minutes if needed

Justification: Give it into the anterolateral thigh and repeat after 5 minutes if there is no improvement. IV adrenaline is for specialists only.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 263 ↑

Q264 FOUNDATION · Block D

Best answer: B. 300 micrograms

Justification: Children aged 6–12 receive 300 micrograms; under 6, 150 micrograms.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 264 ↑

Q265 FOUNDATION · Block D

Best answer: E. GTN spray 400 micrograms sublingually

Justification: GTN is the first drug for angina; it can be repeated.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 265 ↑

Q272 FOUNDATION · Block D

Best answer: D. Call 999 and give hydrocortisone if prescribed and you are trained

Justification: Adrenal crisis needs urgent steroid replacement and transfer.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 272 ↑

Q273 FOUNDATION · Block D

Best answer: A. 30 compressions to 2 breaths, at 100–120 a minute and 5–6 cm deep, with an AED as soon as available

Justification: These are the Resuscitation Council UK adult standards.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 273 ↑

Q275 FOUNDATION · Block D

Best answer: B. 15 L/min via a non-rebreathe mask

Justification: High-flow oxygen is given to the acutely ill patient.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 275 ↑

Q282 FOUNDATION · Block D

Best answer: A. Continue the rivaroxaban

Justification: Do not tell patients to stop anticoagulants for low-risk procedures; follow procedure-specific guidance.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 companion note (SBA traps) · LO 2b.11

Return to Question 282 ↑

BLOCK E · Assessment, diagnosis, imaging, classifications and planning

Q294 FOUNDATION · Block E

Best answer: B. A full six-point periodontal chart of the whole dentition

Justification: BSP guidance: code 4 or * in any sextant prompts full charting; code 3 prompts charting of that sextant after initial therapy.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 294 ↑

Q298 FOUNDATION · Block E

Best answer: A. Class III

Justification: A unilateral bounded space with teeth in front and behind.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.2 · LO 2a.10

Return to Question 298 ↑

Q299 FOUNDATION · Block E

Best answer: C. Class IV

Justification: A single bilateral anterior space crossing the midline.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.2 · LO 2a.10

Return to Question 299 ↑

Q305 FOUNDATION · Block E

Best answer: B. Class IV (knife-edge)

Justification: Cawood and Howell IV: adequate height, inadequate width.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.3 · LO 2a.11

Return to Question 305 ↑

Q306 FOUNDATION · Block E

Best answer: E. Class V

Justification: A flat ridge, inadequate in height and width.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.3 · LO 2a.11

Return to Question 306 ↑

Q307 FOUNDATION · Block E

Best answer: B. Class VI

Justification: A depressed ridge with basal bone loss.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.3 · LO 2a.11

Return to Question 307 ↑

Q310 FOUNDATION · Block E

Best answer: D. Stage III

Justification: Stage is set by the worst interproximal site: coronal third is Stage II, mid third Stage III, apical third Stage IV.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 310 ↑

Q311 FOUNDATION · Block E

Best answer: D. Recognise that heavy smoking is a Grade C modifier despite a ratio compatible with Grade B, and document uncertainty and full case findings

Justification: Periodontitis grading incorporates direct or indirect progression evidence and grade modifiers. Smoking at 10 or more cigarettes daily is a Grade C modifier and should be documented alongside the severity and risk profile.

Why not the closest alternatives: Staging and grading answer different questions: stage describes severity and complexity, while grade estimates progression and risk.

Revise: Volume I, Unit 5 · Diploma LO 2a.5; 2a.17; 2b.3 · Reference themes: 2019 Edinburgh MPerio; 2017 World Workshop classification

Return to Question 311 ↑

Q312 FOUNDATION · Block E

Best answer: E. Grade C (60 ÷ 30 = 2.0)

Justification: 60 ÷ 30 = 2.0, above 1.0, showing rapid progression.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 312 ↑

Q316 FOUNDATION · Block E

Best answer: C. Currently unstable

Justification: Any PPD of 5 mm or more, or a 4 mm site with bleeding, is unstable.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 316 ↑

Q321 FOUNDATION · Block E

Best answer: B. Grade 3

Justification: Over 1 mm horizontal and/or vertical (depressible) mobility.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 321 ↑

Q324 FOUNDATION · Block E

Best answer: C. High risk: oral hygiene and dietary assessment, find the cause, monitor, and consider restoration

Justification: A cumulative BEWE of 14 or more is high risk: identify the cause, prevent, monitor and consider restoration.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.5 · Number card 4 · LO 2a.8

Return to Question 324 ↑

Q327 FOUNDATION · Block E

Best answer: E. Class I (normal is about 2–4°)

Justification: A normal ANB angle is about 2–4°.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.6 · LO 2a.15

Return to Question 327 ↑

Q328 FOUNDATION · Block E

Best answer: D. Ideal 2:3, minimum 1:1

Justification: A crown longer than its root in bone gives an unfavourable lever for an abutment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.6 · LO 2a.16

Return to Question 328 ↑

Q332 FOUNDATION · Block E

Best answer: C. CBCT is justified only for a three-dimensional question that lower-dose imaging cannot answer

Justification: CBCT has the highest dental dose; it is never routine screening.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7, companion note 02 · LO 2a.12, 2a.19

Return to Question 332 ↑

Q334 FOUNDATION · Block E

Best answer: D. Justification, optimisation (ALARP) and a recorded clinical evaluation

Justification: Every exposure must be justified, kept as low as reasonably practicable and clinically evaluated with a written report.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 334 ↑

Q343 FOUNDATION · Block E

Best answer: C. Vitamin B12 or folate deficiency

Justification: A high MCV (macrocytosis) suggests B12 or folate deficiency.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.7

Return to Question 343 ↑

Q344 FOUNDATION · Block E

Best answer: C. A predictable ferrule combined with root/support prognosis

Justification: Ferrule geometry, sound tooth structure and periodontal/root condition drive restorability; a post is primarily for core retention rather than tooth strengthening.

Why not the closest alternatives: A longer post or stronger cement cannot by itself create a protective ferrule.

Revise: Unit 5 §5.9 · LO 2a.16

Return to Question 344 ↑

Q357 FOUNDATION · Block E

Best answer: D. About 2–4 mm

Justification: It is usually about 3 mm. Less suggests excessive OVD; more suggests overclosure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.1, Number card 2 · LO 2a.15

Return to Question 357 ↑

Q359 FOUNDATION · Block E

Best answer: B. Anterior tissue displaceability and undercut geometry

Justification: Mobile ridge tissue requires controlled recording without excessive displacement, and bony undercuts modify tray and path of insertion planning.

Why not the closest alternatives: Aesthetic characteristics of an old prosthesis do not determine whether the ridge should be recorded selectively.

Revise: Unit 5 §5.1 · LO 2a.5

Return to Question 359 ↑

BLOCK F · Removable partial dentures

Q371 FOUNDATION · Block F

Best answer: D. The outline of the saddles (which spaces will be restored)

Justification: The design sequence is saddles, support, retention, bracing and reciprocation, connection, review. First decide which spaces need restoring.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.1 · Schematic 11 · LO 2c.3

Return to Question 371 ↑

Q376 FOUNDATION · Block F

Best answer: C. The greatest convexity of the tooth relative to the chosen path of insertion

Justification: The undercut lies gingival to the survey line, which changes with the chosen tilt.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.2 · LO 2c.3

Return to Question 376 ↑

Q377 FOUNDATION · Block F

Best answer: B. Re-survey insertion path and guide-plane geometry before adjusting retention

Justification: Surveying defines a feasible insertion path, guide planes and interference control; incomplete seating should not be solved by tightening clasps.

Why not the closest alternatives: A tighter clasp can increase abutment loading while the rigid framework remains obstructed.

Revise: Unit 6 §6.2 · LO 2c.3

Return to Question 377 ↑

Q389 FOUNDATION · Block F

Best answer: A. More than 180°

Justification: Encirclement stops the tooth escaping from the clasp. Other requirements are retention, support, reciprocation, passivity and bracing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.4 · LO 2c.3

Return to Question 389 ↑

Q396 FOUNDATION · Block F

Best answer: D. In the gingival third undercut, passive when fully seated

Justification: An active clasp at rest moves the tooth orthodontically.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.4 · LO 2c.3

Return to Question 396 ↑

Q400 FOUNDATION · Block F

Best answer: B. Mesial rest, distal proximal plate and buccal I-bar

Justification: That is Kratochvil and Krol's RPI assembly.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 400 ↑

Q405 FOUNDATION · Block F

Best answer: A. They give support and stop the major connector settling

Justification: Indirect retainers also support anterior extensions and stop the major connector sinking.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.6 · LO 2c.3

Return to Question 405 ↑

Q407 FOUNDATION · Block F

Best answer: C. Reline of the saddles

Justification: Lifting rests on pressing the saddle show ridge resorption and rotation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 407 ↑

Q416 FOUNDATION · Block F

Best answer: A. It traps food

Justification: A continuous clasp gives indirect retention but lies on the cingula, where food collects.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 416 ↑

Q420 FOUNDATION · Block F

Best answer: E. Gingival stripping, with more periodontal disease and caries

Justification: Mucosa-borne acrylic RPDs covering the margins are known as "gum strippers".

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.3

Return to Question 420 ↑

Q425 FOUNDATION · Block F

Best answer: C. It is hard to adjust, reline or add to

Justification: Nylon is aesthetic and clasp-free but mainly mucosa-borne and difficult to modify.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 425 ↑

Q426 FOUNDATION · Block F

Best answer: C. They need crowned abutments and usually lie beyond Level 2

Justification: Attachments need crowns, laboratory precision and maintenance.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 426 ↑

Q427 FOUNDATION · Block F

Best answer: A. Correct the over-eruption and occlusal plane (pre-prosthetic optimisation, a Level 2 task)

Justification: Correct over-eruption, tilts and the occlusal plane before design; this is within Level 2.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.3

Return to Question 427 ↑

Q430 FOUNDATION · Block F

Best answer: B. About 1 week, then at regular recalls

Justification: Early review catches pressure points and high rests.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 430 ↑

Q431 FOUNDATION · Block F

Best answer: D. Monitoring caries and periodontal health at the abutments, and relining saddles as needed

Justification: Caries and periodontal disease at abutments, and ridge resorption under free-end saddles, are the main long-term risks.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 431 ↑

Q432 FOUNDATION · Block F

Best answer: C. A well-designed, tooth-supported Co-Cr RPD with good hygiene and recall does little harm

Justification: Bergman showed tooth-supported Co-Cr RPDs, with good hygiene and recall, do little harm to the remaining teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6, In one minute · LO 2c.3

Return to Question 432 ↑

Q437 FOUNDATION · Block F

Best answer: B. Posteriorly positioned indirect retention at an appropriate distance from the fulcrum

Justification: The position of indirect retention relative to the functional fulcrum line affects resistance to anterior displacement.

Why not the closest alternatives: Lip support and aesthetics matter but do not substitute for rotational control.

Revise: Unit 6 §6.6 · LO 2c.3

Return to Question 437 ↑

Q440 FOUNDATION · Block F

Best answer: D. Survey the study cast and plan, prepare the teeth, then take the master impression

Justification: Preparing teeth to a surveyed design ensures guide planes and rests match the path.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 companion note 02 · LO 2c.3

Return to Question 440 ↑

Q441 FOUNDATION · Block F

Best answer: A. Short, about 2–3 mm, so the plate can disengage under load

Justification: A short guide plane lets the proximal plate move gingivally and disengage.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 441 ↑

Q446 FOUNDATION · Block F

Best answer: E. Excellent oral hygiene, as it covers the gingival margins

Justification: A lingual plate covers the gingival margins, so plaque control is critical.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 446 ↑

Q448 FOUNDATION · Block F

Best answer: B. An alkaline peroxide cleanser

Justification: Hypochlorite corrodes Co-Cr.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 (exam traps) · LO 2c.20

Return to Question 448 ↑

Q449 FOUNDATION · Block F

Best answer: A. Remove it at night

Justification: Night removal rests the tissues and reduces candidosis and plaque stagnation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 449 ↑

Q459 FOUNDATION · Block F

Best answer: E. It reverses the design logic

Justification: Support and the path of insertion come first; clasps are chosen to suit them.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 companion note (SBA traps) · LO 2c.3

Return to Question 459 ↑

Q461 FOUNDATION · Block F

Best answer: A. Record a functional impression of the distal-extension saddle on the seated framework and produce an altered master cast

Justification: An altered-cast impression can record differential tissue displacement of a distal-extension base while the tooth-supported framework is fully seated. The technique aims to improve load sharing and limit rotation.

Why not the closest alternatives: Over-tightening clasps does not stop the saddle sinking and may overload the abutment; loss of tissue coverage worsens support.

Revise: Volume I, Unit 6 · Diploma LO 2c.4; 2c.6; 2c.13 · Reference themes: 2015 MRD; 2019 Edinburgh MPros

Return to Question 461 ↑

Q462 FOUNDATION · Block F

Best answer: D. The clasps will torque the abutments distally as the saddle moves

Justification: Rigid clasps in distal undercuts lever the abutments as the saddle sinks.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 462 ↑

BLOCK G · Complete dentures

Q471 FOUNDATION · Block G

Best answer: C. Stability

Justification: Stability resists horizontal and rotational forces; it depends on ridge form, flange extension, balanced occlusion and teeth in the neutral zone. Retention resists displacement away from the tissues.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.1 · LO 2c.9

Return to Question 471 ↑

Q472 FOUNDATION · Block G

Best answer: B. Loss of peripheral seal during functional movement

Justification: Complete denture retention depends on both adaptation and maintenance of the functional border seal; movement-specific seal loss is key here.

Why not the closest alternatives: Acrylic tooth material and clasp mechanics are not the dominant determinants of this complete denture symptom.

Revise: Unit 7 §7.1 · LO 2c.9

Return to Question 472 ↑

Q476 FOUNDATION · Block G

Best answer: B. Recording the full sulcus depth so that special trays can be made

Justification: The primary cast must show the whole denture-bearing area so special trays can be made.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.2 · LO 2c.9

Return to Question 476 ↑

Q480 FOUNDATION · Block G

Best answer: C. OVD, RCP, occlusal plane, centre and canine lines, lip line, tooth mould and shade

Justification: Everything the technician needs to set the teeth is recorded at this visit, with a facebow if a semi-adjustable articulator is used.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.2 · LO 2c.9

Return to Question 480 ↑

Q485 FOUNDATION · Block G

Best answer: D. About 1–2 mm (less with age)

Justification: Incisal display reduces with age.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.3 · LO 2c.9

Return to Question 485 ↑

Q487 FOUNDATION · Block G

Best answer: C. About two-thirds up the retromolar pad, level with the lateral border of the tongue

Justification: At this height, level with the lateral border of the tongue, the lower plane is stable.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.3 · LO 2c.9

Return to Question 487 ↑

Q490 FOUNDATION · Block G

Best answer: B. Base of the nose to the underside of the chin

Justification: Willis noted this roughly equals the pupil-to-stomion distance.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.9

Return to Question 490 ↑

Q491 FOUNDATION · Block G

Best answer: A. About 1–2 mm

Justification: If the teeth touch on "s", the OVD is too high.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.9

Return to Question 491 ↑

Q496 FOUNDATION · Block G

Best answer: E. It is reproducible

Justification: With no teeth to guide ICP, RCP is the repeatable reference.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.5 · LO 2c.19

Return to Question 496 ↑

Q499 FOUNDATION · Block G

Best answer: C. Take a new RCP registration and re-articulate

Justification: Grinding the teeth to fit a wrong record makes things worse.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.5 · LO 2c.19

Return to Question 499 ↑

Q500 FOUNDATION · Block G

Best answer: A. Condylar guidance about 30° and a Bonwill triangle with sides of about 10 cm

Justification: These fixed averages are adequate for most complete dentures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · Number card 2 · LO 2c.19

Return to Question 500 ↑

Q504 FOUNDATION · Block G

Best answer: A. The relationship of the maxillary arch to the hinge axis

Justification: It makes the arcs of closure on the articulator match the patient's.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 504 ↑

Q506 FOUNDATION · Block G

Best answer: C. Condylar guidance, incisal guidance, cusp angle, plane of orientation, compensating curve

Justification: Condylar guidance, incisal guidance, cusp angle, plane of orientation and compensating curve must be balanced together.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 506 ↑

Q507 FOUNDATION · Block G

Best answer: A. Simultaneous contacts on working and non-working sides keep the dentures stable in excursions

Justification: They stop the dentures tipping in excursions.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 507 ↑

Q513 FOUNDATION · Block G

Best answer: A. The lower denture is pushed forward in function

Justification: Occlusal force on an inclined surface drives the denture anteriorly; lower posteriors go over the crest, never up the ramus.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.7 · LO 2c.9

Return to Question 513 ↑

Q521 FOUNDATION · Block G

Best answer: E. That the upper incisal edges touch the wet–dry line of the lower lip (upper incisor length and labial position)

Justification: The f/v sounds test upper incisor length and labial position.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.8 · LO 2c.9

Return to Question 521 ↑

Q541 FOUNDATION · Block G

Best answer: C. Leave it in for the first 24 hours, review the next day, and reline at about 3–6 months

Justification: Rapid early resorption loosens the denture.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.4

Return to Question 541 ↑

Q543 FOUNDATION · Block G

Best answer: B. Fluoride gel placed in the fitting surface of the denture, with meticulous hygiene

Justification: Caries and periodontitis are the main risks to overdenture abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.4

Return to Question 543 ↑

Q548 FOUNDATION · Block G

Best answer: D. A rebase replaces the whole base

Justification: A reline adds a new fitting surface; a rebase replaces all the base material, keeping the teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.23

Return to Question 548 ↑

Q553 FOUNDATION · Block G

Best answer: E. Discuss implant-assisted dentures as an option and refer for implant surgery

Justification: Implant-assisted dentures should be offered; the surgery itself is beyond Level 2.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 companion note · LO 2c.5

Return to Question 553 ↑

Q554 FOUNDATION · Block G

Best answer: B. An RCP registration error

Justification: Uneven contacts with a slide mean the jaw record is wrong.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.5 · LO 2c.19

Return to Question 554 ↑

Q556 FOUNDATION · Block G

Best answer: A. The occlusal plane against the interpupillary and ala–tragal lines

Justification: Fox's guide checks the plane against the interpupillary and ala–tragal lines.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.3 · LO 2c.9

Return to Question 556 ↑

Q565 FOUNDATION · Block G

Best answer: A. Yes: regular review of the oral mucosa and denture fit

Justification: Edentulous patients still need oral cancer screening, and their dentures loosen over time.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7, Unit 12 · LO 2c.27

Return to Question 565 ↑

Q568 FOUNDATION · Block G

Best answer: A. An average-value articulator

Justification: It is adequate for most complete dentures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 568 ↑

Q569 FOUNDATION · Block G

Best answer: D. An average-value articulator

Justification: It uses a Bonwill triangle of about 10 cm and fixed condylar guidance of about 30°.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 569 ↑

BLOCK H · Fixed prosthodontics

Q574 FOUNDATION · Block H

Best answer: B. Reduce the taper where feasible and consider auxiliary grooves or boxes while preserving healthy tooth tissue

Justification: Resistance to rotation is compromised by short axial walls and excessive taper. Controlled convergence, adequate height, and selected auxiliary resistance forms can improve retention and resistance.

Why not the closest alternatives: Cement alone does not repair poor geometry. Subgingival placement without indication may damage periodontal tissues and does not necessarily create adequate resistance.

Revise: Volume I, Unit 8 · Diploma LO 2c.11; 2c.14 · Reference themes: 2020 Edinburgh MPros; 2019 Edinburgh MPros

Return to Question 574 ↑

Q587 FOUNDATION · Block H

Best answer: B. To protect the pulp, keep tooth position, protect the gingiva, restore function and appearance, and test the planned shape or OVD

Justification: Provisionals also let you trial the planned shape, OVD or occlusion.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.3 · LO 2c.13

Return to Question 587 ↑

Q599 FOUNDATION · Block H

Best answer: D. A good coronal seal placed promptly

Justification: Coronal leakage contributes to root canal failure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.5 · LO 2c.14

Return to Question 599 ↑

Q601 FOUNDATION · Block H

Best answer: E. The restoration on the abutment tooth that holds the bridge

Justification: The abutment is the tooth, the pontic the replacement, the connector the joint.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 601 ↑

Q603 FOUNDATION · Block H

Best answer: D. The total root surface area of the abutments should equal or exceed that of the teeth being replaced

Justification: That is Ante's law.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 603 ↑

Q604 FOUNDATION · Block H

Best answer: B. It is historical and not well supported by evidence, though still examined

Justification: Ante's law is still examined but lacks good evidence.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 604 ↑

Q609 FOUNDATION · Block H

Best answer: C. The most tooth destruction, the need for parallel preparations, and a loose retainer may go unnoticed

Justification: These are the drawbacks of a fixed-fixed design.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · Schematic 18 · LO 2c.11

Return to Question 609 ↑

Q615 FOUNDATION · Block H

Best answer: E. A single-wing resin-bonded cantilever bridge

Justification: It is first choice for a single missing incisor or premolar with sound abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.8 · LO 2c.11

Return to Question 615 ↑

Q616 FOUNDATION · Block H

Best answer: C. Differential movement has debonded one wing unnoticed, leading to caries

Justification: Differential movement between two wings leads to caries; single-wing designs perform better.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 616 ↑

Q618 FOUNDATION · Block H

Best answer: B. The pier acts as a fulcrum

Justification: Use a non-rigid connector at the pier, or separate bridges.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 (exam traps) · LO 2c.11

Return to Question 618 ↑

Q619 FOUNDATION · Block H

Best answer: C. It preserves sound tooth structure and permits conservative replacement in a suitable case

Justification: A suitable enamel-bonded adhesive design can preserve healthy abutments while providing a relatively conservative replacement option.

Why not the closest alternatives: Longevity and success depend on case selection, occlusion and bonding—not an absolute guarantee.

Revise: Unit 8 §8.8 · LO 2c.11

Return to Question 619 ↑

Q622 FOUNDATION · Block H

Best answer: E. Air-abrade the metal (50 µm alumina), etch the enamel, and use an MDP-containing resin cement under rubber dam

Justification: MDP bonds chemically to the base-metal oxide; isolation protects the bond.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.8, Unit 3 · LO 2c.11

Return to Question 622 ↑

Q623 FOUNDATION · Block H

Best answer: B. Bruxism and heavy occlusion make debonding more likely

Justification: They are the main risk factors for debonding.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 623 ↑

Q628 FOUNDATION · Block H

Best answer: A. A saddle (ridge lap) pontic; it is unhygienic

Justification: Its concave surface cannot be cleaned and causes inflammation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.9 (exam traps) · LO 2c.11

Return to Question 628 ↑

Q638 FOUNDATION · Block H

Best answer: D. HF-etched, silanated ceramic and light-cured resin cement on etched enamel

Justification: This is the standard protocol for silica-based veneers.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.11, Unit 3 · LO 2c.13

Return to Question 638 ↑

Q645 FOUNDATION · Block H

Best answer: D. It is not a mandatory rule for a natural smile

Justification: The golden proportion is not a natural-smile rule.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 01 · LO 2a.16

Return to Question 645 ↑

Q647 FOUNDATION · Block H

Best answer: D. Resin infiltration or microabrasion

Justification: These treat white spots without preparation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 02 · LO 2b.6

Return to Question 647 ↑

Q648 FOUNDATION · Block H

Best answer: C. Additive direct composite

Justification: It is the most conservative option for peg laterals and spacing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 02 · LO 2b.6

Return to Question 648 ↑

Q659 FOUNDATION · Block H

Best answer: B. Cuspal coverage with an onlay or crown

Justification: Root-filled posterior teeth with MOD cavities need cusp protection.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.5 · LO 2c.14

Return to Question 659 ↑

Q663 FOUNDATION · Block H

Best answer: C. Avoid it

Justification: Under heavy load it torques the single abutment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 663 ↑

Q664 FOUNDATION · Block H

Best answer: E. 15 acts as a pier fulcrum

Justification: A rigid bridge rocks over the pier and debonds the terminal retainers.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 664 ↑

Q665 FOUNDATION · Block H

Best answer: B. Explain that heavy occlusion contraindicates veneers, and discuss alternatives

Justification: Discuss alternatives rather than proceeding.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.11 · LO 2c.13

Return to Question 665 ↑

Q668 FOUNDATION · Block H

Best answer: A. The first cord controls fluid and the second displaces the tissue until just before the impression

Justification: The deep cord stays in; the upper cord is removed just before the impression.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.2 · LO 2c.13

Return to Question 668 ↑

Q669 FOUNDATION · Block H

Best answer: D. Recontour or remake the crown with a flat emergence profile

Justification: The cause is the crown contour; mouthwash and antibiotics only mask it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.2 · LO 2c.20

Return to Question 669 ↑

BLOCK I · Occlusion and temporomandibular disorders

Q673 FOUNDATION · Block I

Best answer: C. A jaw-to-jaw relation with the condyles in their most superior-anterior position against the eminence, discs correctly interposed, independent of the teeth

Justification: Centric relation is independent of the teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 673 ↑

Q674 FOUNDATION · Block I

Best answer: E. A slide of about 1 mm forwards occurs in about 90% of people

Justification: A slide is normal and needs no treatment by itself.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 674 ↑

Q675 FOUNDATION · Block I

Best answer: B. About 2–4 mm

Justification: The freeway space is the gap between rest position and occlusion.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 675 ↑

Q676 FOUNDATION · Block I

Best answer: E. Canine guidance

Justification: Only the canines contact on the working side and disclude all other teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · Schematic 20 · LO 2a.15

Return to Question 676 ↑

Q679 FOUNDATION · Block I

Best answer: A. A non-working (balancing) interference

Justification: It is most damaging in natural dentitions; desirable only in complete dentures (balanced occlusion).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 679 ↑

Q680 FOUNDATION · Block I

Best answer: C. A working-side interference

Justification: A posterior contact prevents canine or group guidance on the working side.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 680 ↑

Q681 FOUNDATION · Block I

Best answer: A. A deflective contact

Justification: It diverts the mandible from its normal path of closure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 681 ↑

Q683 FOUNDATION · Block I

Best answer: E. Curve of Wilson

Justification: Wilson is mediolateral; Spee is anteroposterior.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 683 ↑

Q685 FOUNDATION · Block I

Best answer: D. About 8–10 mm each

Justification: Those are typical lateral excursion and protrusion ranges.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 685 ↑

Q693 FOUNDATION · Block I

Best answer: B. Disc displacement without reduction, without limited opening

Justification: A history of lock, now opening 40 mm or more.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 693 ↑

Q697 FOUNDATION · Block I

Best answer: C. It is multifactorial and biopsychosocial

Justification: Parafunction, trauma, stress, sleep and widespread pain contribute; occlusion has only a weak association.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 697 ↑

Q702 FOUNDATION · Block I

Best answer: B. They are for short-term use and may increase clenching in some patients

Justification: Soft splints are not for long-term myalgia management.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 702 ↑

Q711 FOUNDATION · Block I

Best answer: E. Explain the uncertainty of a causal occlusal relationship and begin conservative, reversible self-care and pain management

Justification: Most common TMD presentations are initially managed conservatively with education, behaviour advice, analgesia when appropriate and reversible therapies. Extensive irreversible occlusal adjustment is not justified solely by a reported slide or joint click.

Why not the closest alternatives: Occlusal analysis can contribute to restorative planning, but it is not a substitute for a structured TMD diagnosis or evidence-based risk–benefit assessment.

Revise: Volume I, Unit 9 · Diploma LO 2a.17; 2a.20; 2c.19 · Reference themes: 2020 Edinburgh MPros; 2017 Dubai MPros

Return to Question 711 ↑

Q720 FOUNDATION · Block I

Best answer: E. Referral

Justification: A closed lock not responding to conservative care is a referral scenario.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 companion note · LO 2a.20

Return to Question 720 ↑

Q723 FOUNDATION · Block I

Best answer: C. Simple analgesia, such as short-term NSAIDs

Justification: Short-term NSAIDs suit acute arthralgia.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 723 ↑

Q726 FOUNDATION · Block I

Best answer: B. A full-coverage hard maxillary stabilisation splint

Justification: That is a Michigan splint.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 726 ↑

BLOCK J · Tooth wear, the failing dentition, interfaces and trauma

Q735 FOUNDATION · Block J

Best answer: C. Endodontics, periodontal surgery if needed, then fixed and removable prosthodontics

Justification: Endodontics, periodontal surgery, then prosthodontics.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.1 · LO 2c.1

Return to Question 735 ↑

Q741 FOUNDATION · Block J

Best answer: D. Direct resin composite: additive, adhesive and repairable

Justification: Direct composite. It is additive, adhesive, repairable and reversible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.2 · LO 2c.11

Return to Question 741 ↑

Q745 FOUNDATION · Block J

Best answer: E. About 4–6 months, occasionally up to 18

Justification: About 4–6 months, occasionally up to 18.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.3 · LO 2c.11

Return to Question 745 ↑

Q752 FOUNDATION · Block J

Best answer: C. It is Level 3, and risks root exposure and an unfavourable crown–root ratio

Justification: Level 3, with risks. Crown lengthening can expose roots and worsen the crown–root ratio.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.4 · LO 2c.11

Return to Question 752 ↑

Q760 FOUNDATION · Block J

Best answer: D. Teeth of doubtful prognosis should not be bridge abutments

Justification: Doubtful teeth are not bridge abutments. Design RPDs so teeth can be added instead.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5 · LO 2c.1

Return to Question 760 ↑

Q762 FOUNDATION · Block J

Best answer: A. Plaque under 20% and bleeding under 30%

Justification: Plaque under 20%, bleeding under 30%. Level 2–3 settings use stricter targets (bleeding under 10%).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 762 ↑

Q766 FOUNDATION · Block J

Best answer: E. Furcation-involved molars are poor abutments

Justification: Poor abutments; root resection is specialist.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 766 ↑

Q779 FOUNDATION · Block J

Best answer: E. Never replant an avulsed primary tooth

Justification: Never replant a primary tooth. It risks damage to the permanent successor.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 01 · LO 2a.18

Return to Question 779 ↑

Q782 FOUNDATION · Block J

Best answer: E. A fixed implant is generally deferred until growth is complete

Justification: Defer until growth is complete. Implants do not erupt with a growing jaw.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 03 · LO 2a.18

Return to Question 782 ↑

Q789 FOUNDATION · Block J

Best answer: D. 5000 ppm fluoride toothpaste

Justification: 5000 ppm fluoride toothpaste.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.2, Unit 2 · LO 2c.10

Return to Question 789 ↑

Q792 FOUNDATION · Block J

Best answer: A. The posterior teeth are out of contact

Justification: Posterior teeth out of contact. That is the intended starting point.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.3 · LO 2c.11

Return to Question 792 ↑

Q798 FOUNDATION · Block J

Best answer: D. Improve hygiene and disease control first

Justification: Disease control first. Her scores are far above the targets for definitive work.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 798 ↑

Q800 FOUNDATION · Block J

Best answer: B. Time, appliances and Level 3 planning

Justification: Time, appliances and Level 3 planning.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.8 · LO 2c.2

Return to Question 800 ↑

BLOCK K · Implants and digital prosthodontics

Q818 FOUNDATION · Block K

Best answer: B. Usually 4 or more implants

Justification: Usually 4 or more; palateless possible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 818 ↑

Q823 FOUNDATION · Block K

Best answer: E. Abutment screw loosening and ceramic chipping

Justification: Screw loosening and ceramic chipping.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.21

Return to Question 823 ↑

Q832 FOUNDATION · Block K

Best answer: C. It has no periodontal ligament or physiological movement

Justification: Osseoperception only. There is no periodontal ligament or physiological movement.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2, Unit 2 · LO 2c.5

Return to Question 832 ↑

Q835 FOUNDATION · Block K

Best answer: D. Refer; sinus grafting and ridge augmentation are not routine Level 2 care

Justification: Refer. Sinus grafting and augmentation are not Level 2.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 companion note 05 · LO 2b.10

Return to Question 835 ↑

Q837 FOUNDATION · Block K

Best answer: B. Address isolation, haemostasis and tissue management before deciding whether scanning or a conventional impression is feasible

Justification: Moisture, bleeding and inaccessible margins can prevent reliable digital capture; tissue management and suitable alternative workflows are crucial.

Why not the closest alternatives: A higher scanner resolution alone cannot expose an obscured finishing line.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 837 ↑

Q838 FOUNDATION · Block K

Best answer: C. Surface geometry as triangles, in an open format without colour

Justification: Surface triangles, open format, no colour.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 838 ↑

Q844 FOUNDATION · Block K

Best answer: A. Models, surgical guides, trays, splints and temporary crowns

Justification: Models, guides, trays, splints, temporaries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 844 ↑

Q845 FOUNDATION · Block K

Best answer: C. Fewer visits, stored records and low-monomer milled bases with good fit

Justification: Fewer visits, stored records, low-monomer bases; limited functional borders.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2c.4

Return to Question 845 ↑

Q850 FOUNDATION · Block K

Best answer: B. A validated technique, with a verification jig or conventional splinted impression where indicated

Justification: Validated technique with verification. Passive fit on multiple implants is a known digital limitation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 digital note 02 · LO 2a.19

Return to Question 850 ↑

Q853 FOUNDATION · Block K

Best answer: E. A protective night guard, as bruxism raises the risk of complications

Justification: A protective night guard. Bruxism raises complication risk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1 · LO 2c.5

Return to Question 853 ↑

Q854 FOUNDATION · Block K

Best answer: D. Higher failure rates; Level 3 care

Justification: Higher failure; Level 3.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1, Unit 4 · LO 2c.5

Return to Question 854 ↑

Q856 FOUNDATION · Block K

Best answer: E. Recognise the raised risk and optimise glycaemic control first

Justification: Optimise glycaemic control first.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1, Unit 4 · LO 2c.5

Return to Question 856 ↑

Q858 FOUNDATION · Block K

Best answer: B. Refer as peri-implantitis (Level 3)

Justification: Refer as peri-implantitis.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.3, Unit 10 · LO 2c.21

Return to Question 858 ↑

BLOCK L · Complications, failures and maintenance

Q867 FOUNDATION · Block L

Best answer: B. Extraction; prevention depends on ferrule and post design

Justification: Usually extraction. Prevention depends on ferrule and post design.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.1 · LO 2c.22

Return to Question 867 ↑

Q877 FOUNDATION · Block L

Best answer: B. Assess tissue support, saddle adaptation, occlusal load and the need for a functional reline or altered-cast approach

Justification: Distal-extension support is partly mucosal. Under loading, loss of saddle adaptation and rotational movement can produce recurrent trauma despite accurate framework seating. Evaluate these mechanics before further indiscriminate relief.

Why not the closest alternatives: Increasing direct retention can overload abutments rather than solve tissue displacement, while shortened base extension reduces support.

Revise: Volume I, Unit 12 · Diploma LO 2c.4; 2c.21; 2c.27 · Reference themes: 2015 MRD; 2019 Dubai MPros

Return to Question 877 ↑

Q879 FOUNDATION · Block L

Best answer: C. At least annually, for mucosal examination, fit and occlusal wear

Justification: At least annually. Check the mucosa, fit and occlusal wear.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.3 · LO 2c.27

Return to Question 879 ↑

Q882 FOUNDATION · Block L

Best answer: C. Brush with a soft brush and soap or non-abrasive cleanser over a basin of water, soak overnight, and leave them out at night

Justification: Soft brush and non-abrasive cleanser over water, overnight soak, night removal.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.3 · LO 2c.27

Return to Question 882 ↑

Q884 FOUNDATION · Block L

Best answer: A. Clear written prescriptions, inspection of impressions and casts at each stage, photographs and shade information, and agreed delivery times

Justification: Clear prescriptions, quality control, photographs, agreed times.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.4 · LO 3a.3

Return to Question 884 ↑

Q886 FOUNDATION · Block L

Best answer: C. About 89% at 10 years

Justification: About 89% at 10 years.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.5 · LO 2c.21

Return to Question 886 ↑

Q888 FOUNDATION · Block L

Best answer: B. About 80–90% at 5–10 years, with cantilevers performing well

Justification: About 80–90% at 5–10 years. Cantilever designs perform well.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.5 · LO 2c.21

Return to Question 888 ↑

BLOCK M · Governance, law and communication

Q899 FOUNDATION · Block M

Best answer: B. 16–17-year-olds are presumed to have capacity to consent (Family Law Reform Act 1969)

Justification: Presumed capacity at 16–17.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 (exam traps) · LO 3b.2

Return to Question 899 ↑

Q904 FOUNDATION · Block M

Best answer: A. At least 100 verifiable hours per 5-year cycle, at least 10 hours in every 2-year period, with a personal development plan

Justification: 100 hours in 5 years, at least 10 in every 2, with a PDP.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.2 · LO 3a.2

Return to Question 904 ↑

Q905 FOUNDATION · Block M

Best answer: B. Appropriate indemnity is mandatory

Justification: Indemnity is mandatory.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.2 · LO 3a.2

Return to Question 905 ↑

Q907 FOUNDATION · Block M

Best answer: E. An apology is not an admission of liability

Justification: An apology is not an admission of liability.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 (exam traps) · LO 3a.7

Return to Question 907 ↑

Q918 FOUNDATION · Block M

Best answer: A. A response within one month, normally free of charge

Justification: One month, normally free.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.4 · LO 3a.5

Return to Question 918 ↑

Q921 FOUNDATION · Block M

Best answer: D. Registering with the HSE, appointing a radiation protection adviser and supervisor, and setting local rules and a controlled area

Justification: HSE registration, RPA, RPS, local rules, controlled area.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.5 · LO 3a.6

Return to Question 921 ↑

Q922 FOUNDATION · Block M

Best answer: B. Within about 1.5 m of the tube and patient, and within the primary beam until it is attenuated

Justification: About 1.5 m, and the primary beam.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.5 · LO 3a.6

Return to Question 922 ↑

Q925 FOUNDATION · Block M

Best answer: A. 134–137°C for at least 3 minutes

Justification: 134–137°C for at least 3 minutes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · Schematic 26 · LO 3a.4

Return to Question 925 ↑

Q927 FOUNDATION · Block M

Best answer: C. A washer-disinfector

Justification: A washer-disinfector.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · LO 3a.4

Return to Question 927 ↑

Q928 FOUNDATION · Block M

Best answer: B. An automatic control test, plus a steam penetration test for vacuum sterilisers

Justification: Automatic control test, plus steam penetration for vacuum.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · LO 3a.4

Return to Question 928 ↑

Q932 FOUNDATION · Block M

Best answer: D. Hepatitis B vaccination with a documented response

Justification: Hepatitis B vaccination with documented response.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · LO 3a.4

Return to Question 932 ↑

Q935 FOUNDATION · Block M

Best answer: E. Reasonable adjustments for a person with a protected characteristic (disability)

Justification: Reasonable adjustments. Disability is one of nine protected characteristics.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.7 · LO 3a.11

Return to Question 935 ↑

Q937 FOUNDATION · Block M

Best answer: B. Audit, measuring practice against a standard

Justification: Audit. Audit measures against a standard; research creates new knowledge.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 (exam traps) · LO 3a.12

Return to Question 937 ↑

Q944 FOUNDATION · Block M

Best answer: D. 10

Justification: 10. The absolute risk reduction is 10%, so NNT = 1 ÷ 0.10.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 944 ↑

Q947 FOUNDATION · Block M

Best answer: B. A formal link with a consultant-led managed clinical network, a minimum caseload, outcome recording (PROMs and PREMs), audit and complication reporting

Justification: Network link, caseload, PROMs and PREMs, audit, complication reporting.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 947 ↑

BLOCK N · Integrated cases

Q968 FOUNDATION · Block N

Best answer: C. Acknowledge it within 3 working days and investigate locally

Justification: Acknowledge within 3 working days.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3a.8

Return to Question 968 ↑

Q980 FOUNDATION · Block N

Best answer: E. 10

Justification: 10. The absolute risk reduction is 10%, so NNT = 1 ÷ 0.10.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3a.12

Return to Question 980 ↑

Q981 FOUNDATION · Block N

Best answer: A. Each exposure must be justified individually

Justification: Individual justification.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 5, 11, 13 · LO 2a.12

Return to Question 981 ↑

Q982 FOUNDATION · Block N

Best answer: D. Explain that records must be retained (adults at least 11 years after last treatment), and offer copies

Justification: Retain the records; offer copies.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3a.5

Return to Question 982 ↑

Q983 FOUNDATION · Block N

Best answer: B. The 17-year-old is presumed to have capacity and can consent

Justification: Presumed capacity at 17.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3b.2

Return to Question 983 ↑

Q986 FOUNDATION · Block N

Best answer: E. Standard precautions: rinse, disinfect per protocol, label and send, as for every patient

Justification: Standard precautions for every patient.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 3, 13 · LO 3a.4

Return to Question 986 ↑

Q991 FOUNDATION · Block N

Best answer: B. An implant-retained palateless prosthesis (after referral), with desensitisation

Justification: Implant-retained palateless prosthesis.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 7, 11 · LO 2c.5

Return to Question 991 ↑

Q992 FOUNDATION · Block N

Best answer: E. It has survived, but success means in place and free of complications

Justification: Survived, not a success.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 · LO 2c.21, 3b.1

Return to Question 992 ↑

Q997 FOUNDATION · Block N

Best answer: B. Continue both antiplatelets

Justification: Continue antiplatelets.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 · LO 2b.11

Return to Question 997 ↑

Q999 FOUNDATION · Block N

Best answer: A. Re-audit to close the loop

Justification: Re-audit.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3a.12

Return to Question 999 ↑

Intermediate answers · 730 items

BLOCK A · Applied anatomy

Q1 INTERMEDIATE · Block A

Best answer: B. Masseteric nerve, from the anterior division of V3, passing through the mandibular notch

Justification: Masseter is supplied by the masseteric nerve, a motor branch of the anterior division of V3 that passes through the mandibular (sigmoid) notch. Masseteric hypertrophy is a recognised sign of bruxism.

Why not the closest alternatives: The buccal nerve is the only sensory branch of the anterior division; the facial nerve supplies muscles of facial expression, not mastication.

Revise: Unit 1 §1.1 · LO 1.1

Return to Question 1 ↑

Q4 INTERMEDIATE · Block A

Best answer: C. It deviates to the right

Justification: Each lateral pterygoid pulls its own condyle forward. If the right one fails, the left condyle translates alone and swings the chin towards the paralysed (right) side.

Why not the closest alternatives: Opening still occurs through the left muscle and the depressors, so D is wrong.

Revise: Unit 1 §1.1 · LO 1.1

Return to Question 4 ↑

Q6 INTERMEDIATE · Block A

Best answer: B. Rotation (hinge movement)

Justification: The lower compartment (condyle against disc) allows hinge rotation for the first 20–25 mm; translation then occurs in the upper compartment.

Why not the closest alternatives: Translation is an upper-compartment movement.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 6 ↑

Q7 INTERMEDIATE · Block A

Best answer: E. Auriculotemporal nerve

Justification: The auriculotemporal nerve is the main sensory supply of the TMJ (with the masseteric and deep temporal nerves, Hilton's law). It also supplies the ear, which explains referred earache.

Why not the closest alternatives: The chorda tympani carries taste; the great auricular supplies skin over the parotid.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 7 ↑

Q8 INTERMEDIATE · Block A

Best answer: A. Pain aggravated by ipsilateral joint loading with reproducible preauricular tenderness

Justification: Joint-localised familiar pain reproduced by movement and loading strengthens an arthrogenous diagnosis. Temporalis tenderness alone supports muscle pain but does not establish the joint as the primary source.

Why not the closest alternatives: Cold-provoked symptoms imply a pulpal source; tactile shocks suggest neuropathic pain, and a headache history alone is nonspecific.

Revise: Volume I, Unit 1 · Diploma LO 1.1; 2a.5; 2a.17 · Reference themes: 2019 Edinburgh MPros; 2017 Dubai MPros

Return to Question 8 ↑

Q9 INTERMEDIATE · Block A

Best answer: D. Lateral (temporomandibular) ligament

Justification: It is the true stabiliser, limiting posterior and inferior condylar movement.

Why not the closest alternatives: The sphenomandibular and stylomandibular ligaments are accessory and contribute little.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 9 ↑

Q10 INTERMEDIATE · Block A

Best answer: C. Fibrocartilage

Justification: The TMJ surfaces are fibrocartilage, reflecting their intramembranous origin. This tissue can remodel under load.

Why not the closest alternatives: Hyaline cartilage covers most other synovial joints.

Revise: Unit 1 §1.5 · LO 1.5

Return to Question 10 ↑

Q12 INTERMEDIATE · Block A

Best answer: B. Genioglossus; hypoglossal nerve

Justification: Genioglossus and the lingual frenum limit the anterior lingual flange; protrusion tightens them and lifts an over-extended flange.

Why not the closest alternatives: Mylohyoid limits the flange in the molar region, not anteriorly.

Revise: Unit 1 §1.4, §1.6 · LO 1.1, 2a.5

Return to Question 12 ↑

Q13 INTERMEDIATE · Block A

Best answer: E. Mylohyoid elevation affecting lingual flange movement

Justification: Mylohyoid elevation changes the floor of mouth and can dislodge a flange that impinges functionally.

Why not the closest alternatives: Coronoid interference is generally a maxillary distobuccal issue.

Revise: Volume I · Volume I Unit 1 · Diploma LO 1.1; 1.5; 2a.5

Return to Question 13 ↑

Q17 INTERMEDIATE · Block A

Best answer: A. The posterior limit of the tuberosity and the lateral extent of the post-dam

Justification: The post-dam runs from one hamular notch to the other, and the denture must reach into the notches for a complete seal.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 17 ↑

Q18 INTERMEDIATE · Block A

Best answer: B. Pressure on the mental nerve where the foramen now lies near the ridge crest

Justification: In severe resorption the mental foramen comes to lie on or near the ridge crest, so the denture compresses the nerve. Relieve the fitting surface over it.

Why not the closest alternatives: Allergy causes generalised mucosal changes, not lip paraesthesia.

Revise: Unit 1 §1.6 · LO 1.1, 2a.5

Return to Question 18 ↑

Q19 INTERMEDIATE · Block A

Best answer: E. Incisive papilla (nasopalatine nerve)

Justification: The papilla overlies the incisive foramen and the nasopalatine nerve, a standard relief area. Pressure causes burning anterior pain.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 19 ↑

Q21 INTERMEDIATE · Block A

Best answer: D. Submandibular space

Justification: By the mylohyoid line rule, lower second and third molar apices lie below the mylohyoid attachment, so infection spreads into the submandibular space.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7 · LO 1.1

Return to Question 21 ↑

Q22 INTERMEDIATE · Block A

Best answer: A. Sublingual space

Justification: Teeth from the incisors to the first molar have apices above mylohyoid; infection spreads into the sublingual space.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7 · LO 1.1

Return to Question 22 ↑

Q23 INTERMEDIATE · Block A

Best answer: E. Emergency referral for airway management

Justification: Bilateral submandibular, sublingual and submental involvement is Ludwig's angina, an airway emergency.

Why not the closest alternatives: Treating in practice, or delaying with antibiotics, risks airway obstruction.

Revise: Unit 1 §1.7, companion note 03 (risk and referral) · LO 1.1, 2a.6

Return to Question 23 ↑

Q25 INTERMEDIATE · Block A

Best answer: C. Through emissary veins

Justification: The pterygoid plexus connects to the cavernous sinus through emissary veins, a route for intracranial spread of infection.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.3 · LO 1.1

Return to Question 25 ↑

Q27 INTERMEDIATE · Block A

Best answer: A. Submasseteric space

Justification: Submasseteric infection, often from a lower third molar, presents with marked trismus and swelling over the angle.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7 · LO 1.1

Return to Question 27 ↑

Q33 INTERMEDIATE · Block A

Best answer: D. Descending palatine artery (third part of the maxillary artery)

Justification: The third (pterygopalatine) part of the maxillary artery gives the descending palatine artery, which divides into the greater and lesser palatine arteries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.3 · LO 1.1

Return to Question 33 ↑

Q36 INTERMEDIATE · Block A

Best answer: A. Masseter and medial pterygoid

Justification: Masseter on the lateral surface and medial pterygoid on the medial surface together form the pterygomasseteric sling around the angle.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.1 (exam traps) · LO 1.1

Return to Question 36 ↑

Q42 INTERMEDIATE · Block A

Best answer: C. Tensor veli palatini (nerve to medial pterygoid, V3)

Justification: It is supplied by the nerve to medial pterygoid (V3). All other soft palate muscles are supplied by the pharyngeal plexus.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.4 · LO 1.1

Return to Question 42 ↑

Q43 INTERMEDIATE · Block A

Best answer: A. Buccinator

Justification: Buccinator (facial nerve) keeps food on the occlusal table and grips the buccal polished surface. Weakness lets food pocket and reduces muscular control of the denture.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.4 · LO 1.1, 2a.5

Return to Question 43 ↑

Q44 INTERMEDIATE · Block A

Best answer: B. Pressure over a superficial mental foramen on a resorbed ridge

Justification: Resorption may place the mental foramen close to the denture-bearing surface, allowing focal pressure injury.

Why not the closest alternatives: Persistent paraesthesia needs review and investigation rather than assuming every case is prosthetic trauma.

Revise: Volume I · Volume I Unit 1 · Diploma LO 1.1; 1.5; 2a.5

Return to Question 44 ↑

Q46 INTERMEDIATE · Block A

Best answer: E. Posterior belly of digastric (and stylohyoid)

Justification: Both are second-arch muscles supplied by the facial nerve. The anterior belly is first-arch, supplied by the nerve to mylohyoid.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.4 (exam traps) · LO 1.1

Return to Question 46 ↑

Q48 INTERMEDIATE · Block A

Best answer: A. The buccal shelf

Justification: Between the external oblique ridge and the residual ridge, its cortical bone lies at right angles to occlusal load, making it the primary support area.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 48 ↑

Q50 INTERMEDIATE · Block A

Best answer: B. Pterygomandibular raphe

Justification: The raphe runs from the hamulus to the retromolar region and tightens on wide opening, lifting a base that extends too far distally.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.6 · LO 1.1

Return to Question 50 ↑

Q56 INTERMEDIATE · Block A

Best answer: C. Facial nerve, chorda tympani, lingual nerve, submandibular ganglion

Justification: The chorda tympani passes through the middle ear, so ear surgery can reduce submandibular and sublingual secretion.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.8 · LO 1.1

Return to Question 56 ↑

Q57 INTERMEDIATE · Block A

Best answer: E. It is mainly mucous and drains through multiple ducts of Rivinus on the sublingual fold

Justification: The sublingual gland drains through many small ducts on the sublingual fold and shares the submandibular parasympathetic supply.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.8 · LO 1.1

Return to Question 57 ↑

Q61 INTERMEDIATE · Block A

Best answer: B. Assess the border extension and polished-surface contours against muscle action

Justification: A denture retentive at rest but displaced in function suggests over-extension or polished-surface contours that conflict with muscles. Diagnose the functional border first.

Why not the closest alternatives: Adhesive, occlusal changes or a remake do not address the cause.

Revise: Unit 1 companion note 01, 04 · LO 1.1, 2a.5

Return to Question 61 ↑

Q62 INTERMEDIATE · Block A

Best answer: B. Loss of posterior palatal seal at the vibrating line

Justification: The vibrating line/posterior palatal seal anatomy determines posterior border extension and seal.

Why not the closest alternatives: An isolated mylohyoid adjustment cannot correct an upper denture post-dam.

Revise: Volume I · Volume I Unit 1 · Diploma LO 1.1; 1.5; 2a.5

Return to Question 62 ↑

Q63 INTERMEDIATE · Block A

Best answer: C. Arrange prompt assessment for unexplained neurosensory change and potential serious pathology

Justification: Persistent unexplained unilateral sensory change should not be attributed automatically to a denture, particularly after the presumed cause has been removed. Clinical examination and appropriate referral are indicated.

Why not the closest alternatives: Mechanical mental nerve compression may be relevant in severe resorption, but persistence after relief requires reassessment rather than repeated empirical denture changes.

Revise: Volume I, Unit 1 · Diploma LO 1.1; 2a.7; 2a.8; 2a.14 · Reference themes: 2020 Edinburgh MPros; Diploma LO

Return to Question 63 ↑

Q64 INTERMEDIATE · Block A

Best answer: D. Refer urgently to the maxillofacial team

Justification: Fever, trismus and dysphagia show a spreading infection that may threaten the airway.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7, companion note 03 · LO 1.1, 2a.6

Return to Question 64 ↑

Q65 INTERMEDIATE · Block A

Best answer: B. Coronoid interference with a maxillary distobuccal flange

Justification: The coronoid process moves forward during opening and can impinge on an overly bulky maxillary flange.

Why not the closest alternatives: The mental foramen is a mandibular sensory landmark.

Revise: Volume I · Volume I Unit 1 · Diploma LO 1.1; 1.5; 2a.5

Return to Question 65 ↑

Q66 INTERMEDIATE · Block A

Best answer: C. Soft-palate form and movement at the vibrating line

Justification: The posterior seal depends on the vibrating line, and its position varies with soft-palate form; examine it before deciding the seal.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 companion note 02 · LO 1.1, 2a.5

Return to Question 66 ↑

Q69 INTERMEDIATE · Block A

Best answer: E. Lateral pharyngeal space

Justification: The pterygomandibular space communicates with the lateral pharyngeal space, which can lead to airway compromise.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 1 §1.7 · LO 1.1

Return to Question 69 ↑

Q70 INTERMEDIATE · Block A

Best answer: C. Masseteric action against the mandibular distobuccal flange

Justification: Masseteric contraction can impinge on the mandibular distobuccal flange, requiring appropriate functional relief.

Why not the closest alternatives: Posterior tongue muscles are a more likely cause of lingual flange displacement.

Revise: Volume I · Volume I Unit 1 · Diploma LO 1.1; 1.5; 2a.5

Return to Question 70 ↑

BLOCK B · Physiology and pathology

Q71 INTERMEDIATE · Block B

Best answer: B. Exclude secondary causes including xerostomia, medications, haematinic deficiency and metabolic disease before diagnosing a primary neuropathic disorder

Justification: Burning tongue is a symptom with multiple causes. When dryness or a relevant drug is present, review and investigations for secondary burning are important before attributing symptoms to primary burning mouth disorder.

Why not the closest alternatives: Antifungals are appropriate when candidiasis is supported; they should not replace assessment of salivary function and systemic/neuropathic differentials.

Revise: Volume I · Unit 2 · Diploma LO 1.2; 2a.7; 2a.11–2a.17; source themes: 2016 Dubai general; user-specified themes

Return to Question 71 ↑

Q73 INTERMEDIATE · Block B

Best answer: C. Arrange timely specialist oral-medicine assessment with consideration of biopsy/immunopathology and urgent evaluation of ocular involvement

Justification: Mucous membrane pemphigoid can involve the eyes and cause scarring, while pemphigus vulgaris can produce serious mucosal disease. Accurate diagnosis and appropriate specialist follow-up are important; ocular symptoms increase urgency.

Why not the closest alternatives: The choice of recall frequency depends on the confirmed disorder and severity; one cannot safely rank all patients from an undifferentiated presentation alone.

Revise: Volume I · Unit 2 · Diploma LO 2a.7–2a.8; 2a.14; source themes: 2019 Dubai MRD; user-specified themes

Return to Question 73 ↑

Q74 INTERMEDIATE · Block B

Best answer: D. Correct steroid inhaler technique and rinsing, and evaluate/manage the confirmed candidiasis at the same time

Justification: Inhaled corticosteroids predispose to candidiasis due to local deposition; mouth rinsing after steroid dosing and improved delivery technique reduce risk. Existing symptomatic candidiasis may warrant appropriate topical treatment depending on severity and context.

Why not the closest alternatives: Salbutamol is a short-acting beta2 agonist, not an inhaled corticosteroid. Rinsing is preventive and is not necessarily sufficient as the sole treatment of established candidiasis.

Revise: Volume I · Unit 2 · Diploma LO 1.7; 2a.7; 2b.2; source themes: 2015 Dubai MRD; user-specified themes

Return to Question 74 ↑

Q75 INTERMEDIATE · Block B

Best answer: C. Erythematous denture-associated stomatitis: correct denture hygiene, night removal and fit, and add appropriate antifungal treatment if indicated

Justification: Denture-associated erythematous candidosis/stomatitis commonly presents as diffuse erythema of the denture-bearing palate. Hygiene, night removal and correcting the prosthesis are essential; antifungal treatment may also be indicated after clinical assessment.

Why not the closest alternatives: Pseudomembranous candidiasis usually has removable white plaques. A red lesion warrants review and reconsideration if it fails to resolve.

Revise: Volume I · Unit 2 · Diploma LO 1.2; 2a.7; 2c.21; source themes: 2015 Dubai MRD; 2019 Edinburgh MPros

Return to Question 75 ↑

Q78 INTERMEDIATE · Block B

Best answer: C. About one-fifth to one-sixth

Justification: Complete denture wearers achieve about one-fifth to one-sixth of dentate bite force and chewing efficiency. Explaining this sets realistic expectations.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.2 · LO 1.2

Return to Question 78 ↑

Q79 INTERMEDIATE · Block B

Best answer: B. Mucosal and TMJ receptors

Justification: Periodontal mechanoreceptors are lost with the teeth, so edentulous patients rely on mucosal and TMJ receptors for feedback.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.2 · LO 1.2

Return to Question 79 ↑

Q80 INTERMEDIATE · Block B

Best answer: D. Less sensitive

Justification: Implants lack a periodontal ligament; their osseoperception is less sensitive than a natural tooth's feedback.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.2 · LO 1.2

Return to Question 80 ↑

Q81 INTERMEDIATE · Block B

Best answer: E. No replacement

Justification: Käyser showed that premolar-to-premolar arches (about 3–5 occlusal units) usually give adequate function in older adults. A free-end RPD often adds little and risks plaque and caries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.2 · LO 1.2, 2c.1

Return to Question 81 ↑

Q83 INTERMEDIATE · Block B

Best answer: D. Copy (replica) dentures

Justification: Ability to adapt falls with age and neuromuscular disease. Copying keeps the familiar shapes she has learned to control while correcting wear and fit.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.2, Unit 7 · LO 1.2, 2c.9

Return to Question 83 ↑

Q84 INTERMEDIATE · Block B

Best answer: A. The maxilla resorbs upwards and inwards, and the mandible downwards and outwards

Justification: The upper arch becomes smaller and the lower wider, giving an apparent Class III relationship and posterior crossbite.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.3 · LO 1.3

Return to Question 84 ↑

Q88 INTERMEDIATE · Block B

Best answer: A. Continuous 24-hour wear of ill-fitting dentures

Justification: Prosthetic factors (poor fit, continuous wear, inadequate support) accelerate resorption.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.3 · LO 1.3

Return to Question 88 ↑

Q90 INTERMEDIATE · Block B

Best answer: C. 6.2–6.7

Justification: Root dentine and cementum demineralise at a higher pH than enamel (5.5), so root caries starts with milder acid challenges.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.4 (exam traps) · LO 1.4

Return to Question 90 ↑

Q94 INTERMEDIATE · Block B

Best answer: E. Remove infected dentine, leave affected dentine near the pulp, and seal

Justification: Remove soft, infected dentine; leave affected, remineralisable dentine over the pulp and seal well, avoiding pulp exposure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.4, companion note 05 · LO 1.4

Return to Question 94 ↑

Q95 INTERMEDIATE · Block B

Best answer: A. Xerostomia reduces buffering and clearance

Justification: Reduced salivary flow weakens buffering, clearance and lubrication as well as denture interfacial function.

Why not the closest alternatives: A plaque-retentive clasp could contribute to local caries but does not explain global dryness.

Revise: Volume I · Volume I Unit 2 · Diploma LO 1.2–1.4; 1.8; 1.11

Return to Question 95 ↑

Q98 INTERMEDIATE · Block B

Best answer: B. Bulimia nervosa

Justification: Palatal erosion (perimylolysis), parotid enlargement and Russell's sign (knuckle calluses) together point to bulimia.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.5 · LO 1.4, 2a.8

Return to Question 98 ↑

Q102 INTERMEDIATE · Block B

Best answer: D. Abfraction (flexural stress at the cervical margin)

Justification: Abfraction is the proposed (debated) flexural-stress mechanism for wedge-shaped cervical lesions with sharp angles, sometimes subgingival, on teeth under heavy lateral load.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.5 · LO 1.4

Return to Question 102 ↑

Q103 INTERMEDIATE · Block B

Best answer: A. Compensatory alveolar growth has kept the worn teeth in contact

Justification: In slow wear, dentoalveolar compensation keeps worn teeth in contact and maintains the OVD, leaving no space. This is the rationale for the Dahl approach.

Why not the closest alternatives: OVD is usually maintained, not lost.

Revise: Unit 2 §2.5, Unit 10 · LO 1.4, 2c.10

Return to Question 103 ↑

Q106 INTERMEDIATE · Block B

Best answer: B. Approach with empathy, liaise with her GMP with consent, and focus on prevention and monitoring before restoration

Justification: Patients may be secretive. A non-judgemental approach, consent-based liaison with the GMP, and protective, monitoring-led care come before any restoration.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.5 · LO 1.4, 3b.1

Return to Question 106 ↑

Q107 INTERMEDIATE · Block B

Best answer: A. Type III (papillary hyperplasia)

Justification: Type III is papillary hyperplasia of the central palate. Type I is pinpoint hyperaemia; type II is diffuse erythema.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.6 (exam traps) · LO 1.8

Return to Question 107 ↑

Q108 INTERMEDIATE · Block B

Best answer: E. Institute denture hygiene, overnight removal and fit correction; use a suitable antifungal if indicated after interaction review

Justification: Chronic denture-associated erythematous candidosis is promoted by continuous wear, plaque and poor fitting. Local risk-factor control is essential, and potential drug interactions, including with miconazole and warfarin, must be checked before prescribing.

Why not the closest alternatives: An antifungal alone often fails if the denture reservoir persists. Systemic treatment is not an automatic first-line option in a stable localised presentation.

Revise: Volume I, Unit 2 · Diploma LO 1.2; 1.7; 2c.21 · Reference themes: 2015 Dubai MRD; SDCEP oral candidosis

Return to Question 108 ↑

Q109 INTERMEDIATE · Block B

Best answer: A. Rinse and gargle with water, then spit, after the corticosteroid inhaler and assess technique or spacer use

Justification: Inhaled corticosteroids can deposit locally and predispose to oral candidosis. Rinsing and spitting after the steroid dose and considering a spacer reduce that local exposure. Salbutamol is a bronchodilator, not a corticosteroid.

Why not the closest alternatives: If clinically established candidosis persists or is extensive, additional treatment may be appropriate; prevention alone does not replace assessment of symptom severity or diagnosis.

Revise: Volume I, Unit 2 · Diploma LO 1.7; 2a.7; 2b.2 · Reference themes: SDCEP candidosis local measures (checked November 2026)

Return to Question 109 ↑

Q111 INTERMEDIATE · Block B

Best answer: A. Miconazole cream

Justification: Miconazole is active against both Candida and Staphylococcus aureus. Treat the denture as the source as well.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.6 · LO 1.8

Return to Question 111 ↑

Q112 INTERMEDIATE · Block B

Best answer: D. Trim the flange, use a tissue conditioner, and excise with histology if it does not resolve

Justification: Removing the irritant and using a tissue conditioner often lets the hyperplasia shrink. Persistent tissue is excised and examined histologically.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.6 · LO 1.8

Return to Question 112 ↑

Q113 INTERMEDIATE · Block B

Best answer: B. Exposed root surfaces demineralise at a higher pH than enamel

Justification: Cementum and root dentine are susceptible to demineralisation at higher pH than enamel.

Why not the closest alternatives: The observation is not evidence that root dentine is more acid resistant.

Revise: Volume I · Volume I Unit 2 · Diploma LO 1.2–1.4; 1.8; 1.11

Return to Question 113 ↑

Q114 INTERMEDIATE · Block B

Best answer: E. It is fibrous replacement of bone, best recorded with a mucostatic or selective-pressure impression

Justification: A flabby ridge, often opposed by lower natural anteriors (combination syndrome), is recorded undisplaced with a windowed tray so it does not rebound under the denture.

Why not the closest alternatives: Excision loses ridge height and is rarely needed.

Revise: Unit 2 §2.6, Unit 7 · LO 1.8, 2c.9

Return to Question 114 ↑

Q115 INTERMEDIATE · Block B

Best answer: C. Erythroplakia

Justification: Erythroplakia carries the highest malignant transformation risk of the potentially malignant disorders.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.7 · LO 1.11

Return to Question 115 ↑

Q116 INTERMEDIATE · Block B

Best answer: D. Lateral border of the tongue and floor of the mouth

Justification: These are the highest-risk sites for oral squamous cell carcinoma.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.7 · LO 1.11

Return to Question 116 ↑

Q119 INTERMEDIATE · Block B

Best answer: B. Secondary dentine deposition

Justification: Ageing teeth lay down secondary dentine, shrinking the pulp, lowering sensibility responses and making access harder.

Why not the closest alternatives: A weak response does not mean necrosis.

Revise: Unit 2 §2.8 · LO 1.10

Return to Question 119 ↑

Q120 INTERMEDIATE · Block B

Best answer: E. Occlusal wear may be compensated by dentoalveolar change

Justification: Compensatory dentoalveolar eruption can maintain occlusal contact and OVD despite gradual wear.

Why not the closest alternatives: Assuming lost OVD simply from short crowns can lead to overtreatment.

Revise: Volume I · Volume I Unit 2 · Diploma LO 1.2–1.4; 1.8; 1.11

Return to Question 120 ↑

Q122 INTERMEDIATE · Block B

Best answer: B. RPD components can increase local plaque retention

Justification: Clasp and saddle margins can favour plaque accumulation when hygiene is inadequate.

Why not the closest alternatives: Treating a single lesion without reviewing prosthesis design risks recurrence.

Revise: Volume I · Volume I Unit 2 · Diploma LO 1.2–1.4; 1.8; 1.11

Return to Question 122 ↑

Q124 INTERMEDIATE · Block B

Best answer: D. A simple, hygienic, maintainable design

Justification: When cleaning and attendance are unreliable, favour designs the patient and carers can maintain over elaborate fixed or implant work.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 companion note 04 · LO 2b.5, 2c.1

Return to Question 124 ↑

Q126 INTERMEDIATE · Block B

Best answer: A. No replacement is a valid option

Justification: Not every missing tooth needs replacing. A single missing second molar with a stable occlusion and no complaint can be left, after the options are discussed.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 companion note 05 · LO 2c.1

Return to Question 126 ↑

Q128 INTERMEDIATE · Block B

Best answer: D. Assess and address medication-related xerostomia, caries risk, hygiene and salivary support before revising the prosthesis

Justification: Antimuscarinic and antidepressant treatment can contribute to hyposalivation. Reduced salivary film impairs retention and increases root-caries risk. Disease control and liaison about medication are priorities before replacing a sound prosthesis.

Why not the closest alternatives: A new denture cannot reverse the underlying salivary problem; drug changes require the prescriber and acidic substitutes may worsen demineralisation.

Revise: Volume I, Unit 2 · Diploma LO 1.2; 1.8; 2b.1; 2c.1 · Reference themes: 2017 Dubai General; 2020 RCS England MPros

Return to Question 128 ↑

Q129 INTERMEDIATE · Block B

Best answer: B. Repeated sugar exposure increases time below the critical pH

Justification: Repeated acidogenic episodes prolong demineralising conditions even if total sugar is similar.

Why not the closest alternatives: Total sugar alone does not explain the frequency-associated challenge.

Revise: Volume I · Volume I Unit 2 · Diploma LO 1.2–1.4; 1.8; 1.11

Return to Question 129 ↑

Q130 INTERMEDIATE · Block B

Best answer: C. The ecological plaque hypothesis

Justification: Environmental change, such as frequent sugar and low saliva, selects for acidogenic, aciduric species, shifting the plaque towards caries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 2 §2.4 · LO 1.4

Return to Question 130 ↑

BLOCK C · Dental materials science

Q131 INTERMEDIATE · Block C

Best answer: C. Re-survey and make a conservative contour change to achieve a shallower usable undercut for the specified cast arm

Justification: A typical short cast Co–Cr arm is relatively stiff and is often designed around a 0.25 mm undercut. Surveyed undercut depth is only one determinant of deflection; length, diameter, taper and alloy properties modify flexibility. A 0.50 mm undercut demands a justified alternative design or changed tooth contour.

Why not the closest alternatives: Simply increasing engagement in a stiff cast arm risks exceeding its elastic limit. A flexible wrought-wire or suitable alternative may sometimes be chosen, but 0.50 mm is not universally appropriate without analysis of its dimensions and path.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.3; 2c.6; source themes: MRD Past questions; 2019 Edinburgh MPros

Return to Question 131 ↑

Q132 INTERMEDIATE · Block C

Best answer: C. Delayed investing permits wax pattern stress relaxation

Justification: Dental wax relaxes internal stresses and can distort with time and temperature; invest promptly.

Why not the closest alternatives: The observation is not primarily caused by denture acrylic curing or tray adhesive.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 132 ↑

Q134 INTERMEDIATE · Block C

Best answer: B. Granular porosity from loss of monomer during the dough stage

Justification: Loss of monomer from an uncovered or over-mature dough prevents proper wetting and fusion of polymer beads, causing granular porosity.

Why not the closest alternatives: Polymerisation heat and insufficient pressure have different characteristic defect patterns.

Revise: Volume I · PMMA review PMC7599472; Volume I Unit 3 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 134 ↑

Q135 INTERMEDIATE · Block C

Best answer: D. Plastic deformation and fatigue damage at a stress concentration have reduced reliable springback

Justification: Co–Cr has limited ductility; repeated plastic bending may create microcracks at high-stress sites, especially near the arm origin. Retention is lost as the elastic response changes, and fatigue fracture becomes a concern.

Why not the closest alternatives: Polishing cannot reliably reverse microcracks or permanent set. Redesign or remake may be indicated after assessing the full RPD, rather than simply bending again.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.3; 2c.6; source themes: 2019 Dubai MPros; MRD Past questions

Return to Question 135 ↑

Q137 INTERMEDIATE · Block C

Best answer: E. Moist dentine collapse after over-drying etched collagen

Justification: Over-drying demineralised dentine can collapse collagen, compromising resin infiltration.

Why not the closest alternatives: Different substrates require different coupling protocols; no ceramic is involved here.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.6; 2c.14

Return to Question 137 ↑

Q138 INTERMEDIATE · Block C

Best answer: D. Adjust repair surface preparation and appropriate acrylic technique

Justification: Improve mechanical and chemical surface preparation and use appropriate validated repair technique.

Why not the closest alternatives: Cementing the denture teeth differently is unrelated to the repair junction.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 138 ↑

Q139 INTERMEDIATE · Block C

Best answer: A. Review the wax design and invest without unnecessary delay

Justification: Invest wax patterns promptly to minimise distortion from stress relaxation.

Why not the closest alternatives: Recasting without altering the delay reproduces the problem.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 139 ↑

Q140 INTERMEDIATE · Block C

Best answer: A. Residual methyl methacrylate caused by incomplete conversion during the shortened polymerisation cycle

Justification: Insufficient heat/time can leave residual monomer, which can irritate tissues and weaken properties. The odour and diffuse mucosal complaint after an abbreviated cure support incomplete conversion.

Why not the closest alternatives: Excessive heating may create gaseous porosity, particularly in thick sections, but is a different mechanism from abbreviated low-temperature curing. Other causes of burning must still be assessed clinically.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.21; source themes: 2019 Edinburgh MPros; MRD Past questions

Return to Question 140 ↑

Q141 INTERMEDIATE · Block C

Best answer: E. Use a validated heating cycle that limits local monomer vaporisation in thick acrylic

Justification: Rapid excessive heating in thick acrylic can raise internal monomer temperature to its boiling range before adequate polymerisation, producing gaseous porosity. Controlled curing addresses the causal temperature gradient.

Why not the closest alternatives: Underpacking can create contraction porosity, but the rounded voids concentrated in a thick region after rapid boiling point to gaseous porosity. Polishing cannot repair internal bubbles.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.21; source themes: 2019 Edinburgh MPros; MRD Past questions

Return to Question 141 ↑

Q142 INTERMEDIATE · Block C

Best answer: B. Insufficient dough bulk and packing compensation during polymerisation contraction

Justification: An inadequate dough charge or insufficient packing pressure prevents material flow to compensate for polymerisation shrinkage, leaving contraction porosity. The process history and void distribution are decisive.

Why not the closest alternatives: Gaseous porosity is more classically associated with rapidly overheated thick acrylic. It is important not to attribute every void to the same manufacturing defect.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.21; source themes: MRD Past questions; 2019 Dubai MPros

Return to Question 142 ↑

Q143 INTERMEDIATE · Block C

Best answer: C. Reassess support and loading, remove the recurring flexural cause, and plan a suitable structural repair or remake

Justification: The recurring location and persistent uneven loading suggest flexural fatigue and a weak repair interface. Durable treatment requires analysis of occlusion, support, fit and structural design, not simply additional repair resin.

Why not the closest alternatives: A technically sound cold-cure repair can be serviceable, but repeated recurrence makes untreated bending stresses and interface preparation central to the decision.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.21–2c.23; source themes: MRD Past questions; 2020 Edinburgh MPros

Return to Question 143 ↑

Q144 INTERMEDIATE · Block C

Best answer: B. High-temperature breakdown of gypsum-bonded investment

Justification: Gypsum-bound investments lack sufficient high-temperature stability for many base-metal casting cycles; phosphate-bonded systems are preferred.

Why not the closest alternatives: The failure is chemistry and thermal stability, not simply wax pattern handling.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 144 ↑

Q145 INTERMEDIATE · Block C

Best answer: B. Cyclic flexural fatigue with crack initiation and propagation at a stress concentrator

Justification: Repeated flexure concentrates tensile stresses at notches and weak midline regions. A small crack can propagate under subcritical repeated loads until a visible fracture occurs.

Why not the closest alternatives: Impact fracture requires an acute loading event; the gradual failure, stress concentrator and antagonistic loading favour fatigue rather than impact.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.21; source themes: MRD Past questions; 2019 Dubai MPros

Return to Question 145 ↑

Q146 INTERMEDIATE · Block C

Best answer: C. Repeat the cure with the specified validated time-temperature profile

Justification: Use a validated material-specific curing programme; incomplete cure increases residual monomer and may weaken the base.

Why not the closest alternatives: Changing the dental stone does not solve PMMA polymer conversion.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 146 ↑

Q147 INTERMEDIATE · Block C

Best answer: B. Record posterior support selectively while recording the flabby anterior tissue without displacement, using a suitable relieved/window technique

Justification: Mobile tissue recorded under pressure rebounds after impression removal, producing an unstable fitting surface. Selective pressure for firm support and minimal displacement of flabby tissue more faithfully reproduce the functional situation.

Why not the closest alternatives: A fully mucocompressive approach may work in selected cases but is particularly problematic where the mobile anterior segment is the source of rocking.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.13; 2c.21; source themes: 2019 Edinburgh MPros; 2015 MRD

Return to Question 147 ↑

Q149 INTERMEDIATE · Block C

Best answer: A. Syneresis or imbibition during delayed alginate pouring

Justification: Alginate is susceptible to water loss and gain; prompt pouring or a validated extended-pour product is required.

Why not the closest alternatives: Ethanol loss describes condensation silicone, not alginate.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 149 ↑

Q150 INTERMEDIATE · Block C

Best answer: C. Excess water–powder ratio reduces cast strength

Justification: Excess mixing water increases final porosity after evaporation and reduces gypsum strength.

Why not the closest alternatives: Heat breakdown and wax distortion occur at different stages.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 150 ↑

Q151 INTERMEDIATE · Block C

Best answer: B. Switch to a suitable phosphate-bonded refractory investment and validate burnout/casting expansion for the alloy

Justification: Base-metal Co–Cr alloys require high casting temperatures; phosphate-bonded investments are appropriate where gypsum binders can degrade. Investment and casting expansion must compensate for alloy contraction without arbitrary overgrinding.

Why not the closest alternatives: Mechanical finishing can correct minor nodules but cannot reliably repair a systematic casting/investment misfit without compromising support or connector geometry.

Revise: Volume I · Unit 3 · Diploma LO 1.9; 2c.6; source themes: 2019 Dubai MPros; 2019 Edinburgh MPros

Return to Question 151 ↑

Q152 INTERMEDIATE · Block C

Best answer: C. Hydrogen release from the set material

Justification: Set addition silicone can release hydrogen, causing porous die surfaces if poured immediately. Wait about an hour unless the product contains a hydrogen absorber.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.2 · LO 1.9

Return to Question 152 ↑

Q153 INTERMEDIATE · Block C

Best answer: B. High stiffness

Justification: Set polyether is very rigid, so it is hard to remove from undercuts and can fracture stone dies.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.2 · LO 1.9

Return to Question 153 ↑

Q154 INTERMEDIATE · Block C

Best answer: A. Polyether

Justification: Polyether is inherently hydrophilic; addition silicone needs added surfactant.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 (exam traps) · LO 1.9

Return to Question 154 ↑

Q155 INTERMEDIATE · Block C

Best answer: A. Cyclic stress propagates a crack below the single-load fracture threshold

Justification: Fatigue can extend a crack under repeated stresses below the material's one-time fracture strength.

Why not the closest alternatives: A sudden impact is absent; an initiation site and progressive fracture line are present.

Revise: Volume I · PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 155 ↑

Q156 INTERMEDIATE · Block C

Best answer: D. Inadequate bonding at a self-cure repair interface

Justification: Poor surface preparation and inadequate chemical/mechanical integration weaken the repair interface.

Why not the closest alternatives: The fracture at the same interface argues for adhesion quality rather than processing voids elsewhere.

Revise: Volume I · Volume I Unit 3; PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 156 ↑

Q157 INTERMEDIATE · Block C

Best answer: D. Tray flexure and lack of adhesive

Justification: A flexible tray distorts during removal and springs back afterwards; elastomers also need the matching adhesive.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.2 · LO 1.9

Return to Question 157 ↑

Q158 INTERMEDIATE · Block C

Best answer: E. Residual monomer from incomplete polymerisation

Justification: A shortened cure can leave excess unreacted methyl methacrylate, causing mucosal irritation. Review clinical causes of stomatitis too; do not infer this from odour alone.

Why not the closest alternatives: Visible voids and fracture mechanisms do not specifically explain the shortened cycle plus chemical irritation.

Revise: Volume I · PMMA review PMC7599472; Volume I Unit 3 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 158 ↑

Q163 INTERMEDIATE · Block C

Best answer: C. Repeat processing with a validated controlled curing cycle

Justification: Use the manufacturer-validated curing cycle; inadequate polymerisation leaves more unreacted monomer.

Why not the closest alternatives: Polishing, changing denture teeth or adding adhesive does not correct an under-cured denture base.

Revise: Volume I · PMMA review PMC7599472; Volume I Units 3 and 12 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 163 ↑

Q164 INTERMEDIATE · Block C

Best answer: B. Tray flexure or inadequate tray support

Justification: Tray distortion or flexure during seating/removal can deform the impression, even if the impression material is dimensionally stable.

Why not the closest alternatives: A uniform direction of error in a flexible tray is not explained by local inhibition alone.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 164 ↑

Q165 INTERMEDIATE · Block C

Best answer: D. Hygroscopic expansion

Justification: Gypsum immersed in water while setting expands more than when it sets in air.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.3 · LO 1.9

Return to Question 165 ↑

Q166 INTERMEDIATE · Block C

Best answer: D. A weak repair junction concentrates load at its boundary

Justification: A weak interfacial bond and abrupt stiffness/geometry changes concentrate stress at the repair line.

Why not the closest alternatives: Replacing the entire occlusal scheme without inspecting the failed junction misses the likely local mechanism.

Revise: Volume I · PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 166 ↑

Q167 INTERMEDIATE · Block C

Best answer: E. Investigate possible contact irritation and material exposure before elective replacement

Justification: Investigate mechanical trauma, candidosis, mucosal disease and possible leachable residual monomer; cold-cure material alone is not diagnostic.

Why not the closest alternatives: Immediate remaking or assuming an allergy without examination misses other common causes.

Revise: Volume I · PMMA review PMC7599472; Volume I Units 3 and 12 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 167 ↑

Q168 INTERMEDIATE · Block C

Best answer: E. Thermal distortion of PMMA after hot-water cleaning

Justification: Excessive heat can release residual processing stresses and distort the denture base, impairing its fit.

Why not the closest alternatives: Acrylic porosity and interfacial repair failure are not the primary explanation for global distortion.

Revise: Volume I · Volume I Unit 3; PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 168 ↑

Q169 INTERMEDIATE · Block C

Best answer: E. Reject and recast the defective framework after reviewing sprueing, investment, burnout and casting practice

Justification: Porosity in a stressed metal framework junction is a structural defect, not merely a finishing blemish. A satisfactory re-cast requires investigation of the laboratory stage generating the defect.

Why not the closest alternatives: Polishing cannot restore lost continuity, and adding resin does not reliably compensate for compromised cast-metal strength.

Revise: Volume I, Unit 3 · Diploma LO 1.9; 2c.4; 2c.21 · Reference themes: 2019 Dubai MPros; Co–Cr laboratory workflow

Return to Question 169 ↑

Q170 INTERMEDIATE · Block C

Best answer: C. Latex/sulphur inhibition of addition-silicone polymerisation

Justification: Sulphur-containing glove contamination can inhibit platinum-catalysed addition silicone setting.

Why not the closest alternatives: Delaying the pour cannot repair material that failed to polymerise.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 170 ↑

Q171 INTERMEDIATE · Block C

Best answer: B. Impact loading from a one-off fall

Justification: A sudden high-energy event can create an impact fracture; this differs from progressive functional fatigue.

Why not the closest alternatives: Persistent chewing fatigue is less likely when the temporal association with a fall is clear.

Revise: Volume I · Volume I Unit 3; PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 171 ↑

Q174 INTERMEDIATE · Block C

Best answer: D. Isolation failure with salivary contamination at bonding

Justification: Salivary contamination can compromise adhesive bonding despite otherwise appropriate preparation and materials.

Why not the closest alternatives: Improving luting agent strength cannot fully compensate for contaminated bonding.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.6; 2c.14

Return to Question 174 ↑

Q175 INTERMEDIATE · Block C

Best answer: B. A cross-linking agent (ethylene glycol dimethacrylate)

Justification: Ethylene glycol dimethacrylate cross-links the polymer and improves resistance to crazing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.5 · LO 1.9

Return to Question 175 ↑

Q176 INTERMEDIATE · Block C

Best answer: B. Use the validated controlled polymerisation cycle to reduce internal monomer vaporisation

Justification: Thick acrylic can undergo internal temperature elevation during rapid curing, producing monomer-vapour bubbles. A controlled cure cycle addresses the heat-management mechanism, whereas contraction porosity is associated with packing deficiencies.

Why not the closest alternatives: Underpacking and incomplete dough adaptation can also create voids, but the location, adequate packing and documented thermal cycle discriminate gaseous porosity.

Revise: Volume I, Unit 3 · Diploma LO 1.9; 2c.13; 2c.21 · Reference themes: 2019 Edinburgh MPros; dental materials

Return to Question 176 ↑

Q177 INTERMEDIATE · Block C

Best answer: B. Reinvest/reprocess after correcting inadequate packing conditions

Justification: Appropriate dough volume and controlled packing/closure are required to compensate for polymerisation shrinkage.

Why not the closest alternatives: A more aggressive boiling cycle increases the risk of a different porosity mechanism.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 177 ↑

Q178 INTERMEDIATE · Block C

Best answer: A. Lack of silanisation after conditioning a silica-based ceramic

Justification: A silica-based glass ceramic requires an appropriate silane coupling strategy after its approved surface-conditioning protocol.

Why not the closest alternatives: Zirconia MDP protocols are not interchangeable with conventional glass-ceramic silanisation.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.6; 2c.14

Return to Question 178 ↑

Q179 INTERMEDIATE · Block C

Best answer: C. Investigate fit and functional loading, then redesign or rebase as appropriate with a durable laboratory repair or replacement

Justification: Recurrent failure at a repaired interface can reflect material discontinuity and the same uncorrected bending stress. A repair alone does not solve instability, occlusal overload or insufficient support.

Why not the closest alternatives: Self-cure repairs may be clinically useful, but repeat repair in an unstable distal-extension appliance is not the most durable treatment without eliminating the underlying load.

Revise: Volume I, Unit 3 · Diploma LO 1.9; 2c.4; 2c.21 · Reference themes: 2019 Edinburgh MPros; prior Membership restorative notes

Return to Question 179 ↑

Q180 INTERMEDIATE · Block C

Best answer: C. Ethanol loss during delayed condensation-silicone pouring

Justification: Volatile ethanol by-product loss causes dimensional shrinkage with delayed pouring of condensation silicone.

Why not the closest alternatives: Addition silicone has a different set chemistry; latex contamination causes tacky unset material.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 180 ↑

Q181 INTERMEDIATE · Block C

Best answer: D. Excess spatulation alters setting kinetics and expansion

Justification: Spatulation affects nucleation, setting rate and expansion; it is not a neutral manipulation.

Why not the closest alternatives: An investment defect is not needed to explain the gypsum setting change.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 181 ↑

Q183 INTERMEDIATE · Block C

Best answer: E. PMMA is radiolucent

Justification: Standard acrylic does not show on radiographs, which matters when fragments may be swallowed or inhaled.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.5 · LO 1.9

Return to Question 183 ↑

Q186 INTERMEDIATE · Block C

Best answer: E. Correct the base thickness and occlusal loading before repair

Justification: A repair will fail again if excessive bending stress and insufficient cross-section remain uncorrected.

Why not the closest alternatives: Changing acrylic chemistry alone cannot solve continuing stress concentration and flexural overload.

Revise: Volume I · PMMA review PMC7599472; Volume I Units 3 and 12 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 186 ↑

Q187 INTERMEDIATE · Block C

Best answer: A. They bond less well to the base and support Candida growth

Justification: Silicone liners stay resilient longer but bond poorly to PMMA, are hard to trim and polish, and support Candida.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.6 · LO 1.9

Return to Question 187 ↑

Q188 INTERMEDIATE · Block C

Best answer: D. A pre-existing flaw creates a stress-intensity concentration

Justification: A defect increases local stress intensity, encouraging crack initiation and subsequent propagation.

Why not the closest alternatives: The void location is a more persuasive explanation than general thermal aging alone.

Revise: Volume I · PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 188 ↑

Q189 INTERMEDIATE · Block C

Best answer: B. Inadequate micromechanical/MDP protocol for zirconia

Justification: Zirconia is not a silica-rich glass ceramic; appropriate airborne abrasion and phosphate-monomer (MDP) protocol are commonly indicated.

Why not the closest alternatives: The issue is substrate chemistry, not specifically dentine over-drying.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.6; 2c.14

Return to Question 189 ↑

Q191 INTERMEDIATE · Block C

Best answer: D. Make a new impression with a rigid tray and suitable adhesive

Justification: A sufficiently rigid tray and appropriate adhesive reduce distortion under functional loading and removal.

Why not the closest alternatives: Another pour of the same distorted impression does not remove the error.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 191 ↑

Q193 INTERMEDIATE · Block C

Best answer: C. An abrupt overload causes an impact-type catastrophic fracture

Justification: One sudden excessive load can exceed fracture strength and cause catastrophic failure.

Why not the closest alternatives: There is no evidence of progressive fatigue in service.

Revise: Volume I · PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 193 ↑

Q194 INTERMEDIATE · Block C

Best answer: C. Stress concentration at the frenum notch with cyclic flexure

Justification: A sharp notch concentrates tensile stress and repeated flexion propagates a fatigue crack.

Why not the closest alternatives: An impact fracture would normally have a more discrete event, and thermal warpage changes overall fit rather than initiating this characteristic line.

Revise: Volume I · Volume I Unit 3; PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 194 ↑

Q195 INTERMEDIATE · Block C

Best answer: B. Galvanic current between dissimilar metals in saliva

Justification: Dissimilar metals in saliva form a cell; contact creates a current felt as a shock.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.7 · LO 1.9

Return to Question 195 ↑

Q196 INTERMEDIATE · Block C

Best answer: C. Chemical bonding through the metal oxide layer

Justification: Tin and indium oxides on noble alloys, or chromium oxide on base metals, bond chemically to porcelain. Mechanical, compressive and van der Waals forces contribute less.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.8 · LO 1.9

Return to Question 196 ↑

Q197 INTERMEDIATE · Block C

Best answer: E. An oxide layer that is too thick weakens the bond

Justification: Base metals form a thick oxide that fails cohesively, weakening the bond.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.8 · LO 1.9

Return to Question 197 ↑

Q198 INTERMEDIATE · Block C

Best answer: A. Insufficient investment expansion produces an undersized casting

Justification: Casting shrinkage requires appropriate controlled setting/thermal investment expansion.

Why not the closest alternatives: Adding water to working stone does not reliably correct the investment's expansion balance.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 198 ↑

Q199 INTERMEDIATE · Block C

Best answer: C. Water uptake and thermal cycling change properties over time

Justification: PMMA absorbs water and responds to thermal history; dimensional and property changes can matter over prolonged use.

Why not the closest alternatives: This presentation does not specifically implicate an impact fracture.

Revise: Volume I · PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 199 ↑

Q200 INTERMEDIATE · Block C

Best answer: A. Poor resistance form of an over-tapered, short preparation

Justification: Poor preparation height/taper means inadequate resistance form; review the mechanical design before recementing.

Why not the closest alternatives: Repeated application of a different cement does not restore missing resistance form.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.6; 2c.14

Return to Question 200 ↑

Q201 INTERMEDIATE · Block C

Best answer: D. Low-pressure air abrasion (about 50 µm alumina) with an MDP-containing primer or cement

Justification: Zirconia has no glass phase to etch; MDP bonds chemically to its oxide. Conventional GIC or RMGIC is acceptable when the preparation is retentive.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.9 · LO 1.9

Return to Question 201 ↑

Q202 INTERMEDIATE · Block C

Best answer: C. Silane needs a silica-based surface

Justification: Zirconia is polycrystalline with no silica. Use MDP, or tribochemical silica coating before silane.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.9 (exam traps) · LO 1.9

Return to Question 202 ↑

Q205 INTERMEDIATE · Block C

Best answer: E. Autopolymerising repair with counselling about residual monomer and lower bulk strength

Justification: Autopolymerising resin is useful for appropriate repairs but generally has greater residual monomer and inferior mechanical properties; explain limits and arrange review.

Why not the closest alternatives: It is neither identical to laboratory heat-curing nor inevitably contraindicated for every repair.

Revise: Volume I · PMMA review PMC7599472; Volume I Units 3 and 12 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 205 ↑

Q206 INTERMEDIATE · Block C

Best answer: C. Fatigue crack growth under repeated functional flexure

Justification: Repeated subcritical flexural loading can initiate and propagate a fatigue crack from a stress concentration.

Why not the closest alternatives: A single high-impact event is not described, and newly processed porosity is not sufficient to explain progressive service-life crack growth.

Revise: Volume I · PMMA review PMC7599472; Volume I Unit 3 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 206 ↑

Q207 INTERMEDIATE · Block C

Best answer: B. Unglazed or roughened feldspathic porcelain

Justification: It is the most abrasive surface; polished monolithic zirconia is relatively kind. Always polish after adjustment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.9 · LO 1.9

Return to Question 207 ↑

Q208 INTERMEDIATE · Block C

Best answer: E. Low initial pH

Justification: The acid can irritate the pulp; incremental mixing on a cool slab controls the exothermic reaction and prolongs working time.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.10 · LO 1.9

Return to Question 208 ↑

Q209 INTERMEDIATE · Block C

Best answer: C. Hygroscopic expansion of the RMGIC fracturing a weak glass ceramic

Justification: RMGIC absorbs water and expands, which can crack weak glass ceramics; use resin cement for them.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.10 · LO 1.9

Return to Question 209 ↑

Q211 INTERMEDIATE · Block C

Best answer: B. Laboratory heat-polymerised repair with appropriate reinforcement/design review

Justification: Controlled laboratory processing with suitable reinforcement/design is more robust than repeatedly adding large volumes of cold-cure acrylic to a stressed midline.

Why not the closest alternatives: A large unplanned cold-cure build-up may introduce more residual monomer and a weak interface.

Revise: Volume I · PMMA review PMC7599472; Volume I Units 3 and 12 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 211 ↑

Q212 INTERMEDIATE · Block C

Best answer: B. Gaseous porosity from excessive temperature rise during cure

Justification: An excessively rapid or hot curing cycle can vaporise monomer inside thick sections, producing internal gaseous porosity.

Why not the closest alternatives: Underpacking creates contraction voids; monomer evaporation before packing gives a different, typically granular, defect.

Revise: Volume I · PMMA review PMC7599472; Volume I Unit 3 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 212 ↑

Q215 INTERMEDIATE · Block C

Best answer: B. Contraction porosity from inadequate packing or pressure

Justification: Insufficient material or pressure permits shrinkage voids during polymerisation and is consistent with contraction porosity.

Why not the closest alternatives: Gaseous pores favour overheating in thick sections; residual monomer is not the dominant explanation for these irregular voids.

Revise: Volume I · PMMA review PMC7599472; Volume I Unit 3 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 215 ↑

Q216 INTERMEDIATE · Block C

Best answer: B. Excessive water uptake by polyether during prolonged immersion

Justification: Polyether is hydrophilic and can absorb water when immersed too long; follow product-specific disinfection instructions.

Why not the closest alternatives: Alginate and condensation silicone have different dominant dimensional-change mechanisms.

Revise: Volume I · Volume I Unit 3 · Diploma LO 1.9; 2c.13

Return to Question 216 ↑

Q218 INTERMEDIATE · Block C

Best answer: D. Early processing dimensional change due to premature deflasking

Justification: Premature deflasking/cooling can release polymerisation and thermal stresses, allowing dimensional change.

Why not the closest alternatives: A global initial fit discrepancy is less characteristic of a later fatigue crack.

Revise: Volume I · Volume I Unit 3; PMMA review PMC7599472 · Diploma LO 1.9; 2c.13; 2c.21

Return to Question 218 ↑

Q219 INTERMEDIATE · Block C

Best answer: A. A ferrule of 1.5–2 mm of sound dentine all round

Justification: The ferrule, not the post, protects the root. A post only retains the core; it does not reinforce the tooth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.12 (exam traps) · LO 1.9, 2c.14

Return to Question 219 ↑

Q220 INTERMEDIATE · Block C

Best answer: B. It is greatest over short spans and lower over full arches and mobile, featureless edentulous mucosa

Justification: Errors accumulate as images are stitched across long spans, and mobile, featureless mucosa scans poorly.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 3 §3.14, Unit 11 · LO 1.9, 2a.19

Return to Question 220 ↑

BLOCK D · Pharmacology, pain control, emergencies and the medically compromised patient

Q221 INTERMEDIATE · Block D

Best answer: B. Consider a suitable alternative such as nystatin and coordinate prescribing according to current guidance rather than exposing the patient to a known azole–warfarin interaction

Justification: Miconazole and systemic azoles can markedly potentiate warfarin. A non-azole option such as nystatin may avoid this interaction, with prescribing tailored to current guidance and the clinical condition.

Why not the closest alternatives: Stopping anticoagulation in a mechanical-valve patient creates serious thrombotic risk; never adjust it solely to facilitate routine treatment without appropriate medical coordination.

Revise: Volume I · Unit 4 · Diploma LO 1.7; 2a.3; 2b.2; source themes: 2015 MRD; 2014 Dubai MRD

Return to Question 221 ↑

Q222 INTERMEDIATE · Block D

Best answer: E. Control active periodontal disease and modifiable risk factors, assess maintenance capacity and reassess before definitive treatment or referral

Justification: Poor glycaemic control, tobacco exposure and inadequate maintenance complicate periodontal treatment response and implant prognosis. Stabilisation and shared decision-making precede definitive complex prosthodontics.

Why not the closest alternatives: Neither a purely mechanical approach nor a blanket permanent prohibition accounts for severity, patient preference, treatment alternatives and the ability to control risks.

Revise: Volume I · Unit 4 · Diploma LO 1.8; 2a.3; 2b.4; 2c.5; source themes: 2019 Edinburgh MPerio; 2020 England MPros

Return to Question 222 ↑

Q223 INTERMEDIATE · Block D

Best answer: A. Prilocaine with felypressin

Justification: Felypressin has oxytocic action, so avoid it in late pregnancy. Large doses of prilocaine also cause methaemoglobinaemia.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.1 (exam traps) · LO 1.7

Return to Question 223 ↑

Q225 INTERMEDIATE · Block D

Best answer: C. Hypertension with reflex bradycardia

Justification: Non-selective beta-blockers leave adrenaline's alpha effects unopposed. Limit the dose.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.1 (exam traps) · LO 1.7

Return to Question 225 ↑

Q227 INTERMEDIATE · Block D

Best answer: C. Akinosi (closed-mouth) block

Justification: It is given with the teeth together, suiting trismus. Gow-Gates is another alternative to the standard block.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.1 · LO 1.7

Return to Question 227 ↑

Q229 INTERMEDIATE · Block D

Best answer: B. First-trimester pregnancy

Justification: It is a relative contraindication, with nasal obstruction, severe COPD, recent middle-ear or eye surgery with gas, and B12 deficiency.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.2 · LO 2c.8

Return to Question 229 ↑

Q230 INTERMEDIATE · Block D

Best answer: D. Flumazenil

Justification: It is the benzodiazepine antagonist. Naloxone reverses opioids.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.2 · LO 2c.8

Return to Question 230 ↑

Q231 INTERMEDIATE · Block D

Best answer: E. A responsible adult escort, and no driving or operating machinery until the next day

Justification: IACSD standards require an escort after IV sedation, because of residual sedation and amnesia.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.2 · LO 2c.8

Return to Question 231 ↑

Q232 INTERMEDIATE · Block D

Best answer: B. ASA I–II

Justification: ASA III and above are generally treated in secondary care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.2 · LO 2c.8

Return to Question 232 ↑

Q233 INTERMEDIATE · Block D

Best answer: D. Phenoxymethylpenicillin 500 mg four times daily for 5 days (or amoxicillin 500 mg three times daily)

Justification: For spreading infection with systemic signs, SDCEP recommends this, or amoxicillin 500 mg three times daily, after drainage.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.3 · LO 1.7, 2c.15

Return to Question 233 ↑

Q235 INTERMEDIATE · Block D

Best answer: C. Metronidazole 400 mg three times daily for 3 days

Justification: It is the SDCEP regimen for necrotising periodontal disease, alongside debridement and oral hygiene.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.3 · LO 1.7

Return to Question 235 ↑

Q236 INTERMEDIATE · Block D

Best answer: B. Confirm the diagnosis and provide definitive pulpal intervention or arrange appropriate timely referral with analgesia advice

Justification: Spontaneous and prolonged cold-provoked pain supports symptomatic irreversible pulpitis despite a normal apical radiograph. Local pulpal treatment, not routine systemic antibiotics, addresses the source.

Why not the closest alternatives: Radiographic apical changes can be absent early. Analgesia is an adjunct, and treatment choice depends on restorability and patient consent.

Revise: Volume I, Unit 4 · Diploma LO 1.7; 2a.17; 2b.1 · Reference themes: 2020 Edinburgh MEndo; 2020 RCS England MPros

Return to Question 236 ↑

Q237 INTERMEDIATE · Block D

Best answer: A. Check current interaction guidance and choose an appropriate treatment without exposing the patient to a major warfarin interaction

Justification: Miconazole can potentiate warfarin even when used as an oral gel. Antifungal choice must account for medicine interactions, severity and local measures; anticoagulation should not be stopped casually.

Why not the closest alternatives: Local application does not remove the interaction risk. Systemic azoles may also interact and require an appropriate prescribing assessment.

Revise: Volume I, Unit 4 · Diploma LO 1.7; 2a.3; 2b.2 · Reference themes: 2015 MRD; SDCEP prescribing guidance

Return to Question 237 ↑

Q239 INTERMEDIATE · Block D

Best answer: C. Urgent medical evaluation of significant unexplained or progressive infection

Justification: Systemic infection with dysphagia and potential airway compromise requires urgent escalation.

Why not the closest alternatives: Routine outpatient antibiotic-only management can be dangerously inadequate.

Revise: Volume I · Volume I Unit 4 · Diploma LO 1.7; 2a.13; 2b.11–2b.12

Return to Question 239 ↑

Q240 INTERMEDIATE · Block D

Best answer: B. A patient with NSAID-sensitive asthma

Justification: NSAIDs can trigger bronchospasm. Also avoid or take care with peptic ulcer, renal impairment, anticoagulants and third-trimester pregnancy.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.4 · LO 1.7

Return to Question 240 ↑

Q242 INTERMEDIATE · Block D

Best answer: E. Identify interaction between oral miconazole and warfarin

Justification: Miconazole can inhibit warfarin metabolism and increase anticoagulant effect; check local prescribing guidance and alternatives.

Why not the closest alternatives: Topical/oral gel formulations are not automatically interaction-free.

Revise: Volume I · Volume I Unit 4 · Diploma LO 1.7; 2a.13; 2b.11–2b.12

Return to Question 242 ↑

Q244 INTERMEDIATE · Block D

Best answer: E. Defer treatment and seek advice from the anticoagulation service

Justification: An INR of 4 or above is outside the threshold for primary care treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.5 · LO 2b.11

Return to Question 244 ↑

Q249 INTERMEDIATE · Block D

Best answer: C. No interruption

Justification: Crown preparation with cord, impressions and denture work are low bleeding-risk procedures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.5 · LO 2b.11

Return to Question 249 ↑

Q250 INTERMEDIATE · Block D

Best answer: D. When the bone has been exposed for more than 8 weeks

Justification: MRONJ is exposed (or probeable) bone for over 8 weeks, with antiresorptive or antiangiogenic history and no jaw radiotherapy or metastasis.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.6 · LO 2b.11

Return to Question 250 ↑

Q251 INTERMEDIATE · Block D

Best answer: A. Denosumab's effect wears off within months

Justification: Denosumab is a RANKL antibody; bisphosphonates bind to bone and persist.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.6 · LO 1.7

Return to Question 251 ↑

Q253 INTERMEDIATE · Block D

Best answer: E. Retain the root (root treatment and decoronation as an overdenture abutment)

Justification: In high-risk patients, root treatment and decoronation avoid an extraction and support the denture as an overdenture abutment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.6 · LO 2b.11, 2c.9

Return to Question 253 ↑

Q255 INTERMEDIATE · Block D

Best answer: D. A dental assessment, getting the mouth healthy before treatment starts if possible

Justification: Getting the mouth healthy before antiresorptive therapy, including well-fitting dentures, reduces MRONJ risk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.6 · LO 2b.11, 2b.12

Return to Question 255 ↑

Q256 INTERMEDIATE · Block D

Best answer: E. At least 10–14 days before radiotherapy, to allow healing

Justification: Teeth of poor prognosis in the field should be removed early enough to heal before irradiation, reducing later osteoradionecrosis risk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.7 · LO 2b.11, 2b.12

Return to Question 256 ↑

Q257 INTERMEDIATE · Block D

Best answer: D. 5000 ppm toothpaste, fluoride gel in custom trays, saliva substitutes and trismus exercises

Justification: Radiation caries, xerostomia and trismus are managed preventively; avoid extractions in the field where possible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.7 · LO 2b.11

Return to Question 257 ↑

Q258 INTERMEDIATE · Block D

Best answer: B. The risk is lifelong, dose-dependent and much higher in the mandible than the maxilla

Justification: Osteoradionecrosis risk persists for life and rises with doses above about 50–60 Gy.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.7 · LO 2b.11

Return to Question 258 ↑

Q259 INTERMEDIATE · Block D

Best answer: A. Level 3

Justification: Obturators and implants in irradiated bone are Level 3 (specialist) care under the NHS restorative standard.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.7, Unit 5 · LO 2b.12

Return to Question 259 ↑

Q262 INTERMEDIATE · Block D

Best answer: E. Refer high-risk sedation cases to an appropriate service

Justification: Sedation risk and setting suitability must be formally assessed; complex patients may require specialist/secondary care services.

Why not the closest alternatives: More anxiolytic medication in an unprepared practice is not a safe substitute for risk assessment.

Revise: Volume I · Volume I Unit 4 · Diploma LO 1.7; 2a.13; 2b.11–2b.12

Return to Question 262 ↑

Q266 INTERMEDIATE · Block D

Best answer: C. Call 999, give aspirin 300 mg chewed, and give oxygen if he is hypoxic

Justification: Persistent pain unrelieved by GTN suggests myocardial infarction.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 266 ↑

Q267 INTERMEDIATE · Block D

Best answer: B. Salbutamol 100 micrograms per puff, 4–6 puffs via a spacer, repeated up to 10 puffs

Justification: Give 4–6 puffs, repeating up to 10. Call 999 if it is life-threatening or not improving.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 267 ↑

Q268 INTERMEDIATE · Block D

Best answer: C. Stop dental treatment and provide fast-acting oral glucose in accordance with emergency guidance, then recheck and monitor

Justification: The symptoms, missed meal and low glucose support hypoglycaemia. In a conscious cooperative adult able to swallow, fast-acting carbohydrate and repeat assessment are the immediate priorities.

Why not the closest alternatives: Glucagon is considered when oral glucose cannot safely be administered; insulin worsens hypoglycaemia. Escalation depends on recovery and local emergency protocols.

Revise: Volume I, Unit 4 · Diploma LO 1.7; 2a.13 · Reference themes: 2014 Dubai MRD; SDCEP emergencies

Return to Question 268 ↑

Q269 INTERMEDIATE · Block D

Best answer: E. Glucagon 1 mg IM

Justification: An unconscious patient cannot swallow safely; give glucagon IM and call for help.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 (exam traps) · LO 2b.1

Return to Question 269 ↑

Q270 INTERMEDIATE · Block D

Best answer: A. Buccal midazolam 10 mg

Justification: For a seizure lasting 5 minutes or more, protect the patient from injury and give buccal midazolam.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 270 ↑

Q271 INTERMEDIATE · Block D

Best answer: E. Lie the patient flat, raise the legs and give oxygen

Justification: Vasovagal syncope restores cerebral perfusion quickly when the patient is laid flat with the legs raised.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 271 ↑

Q274 INTERMEDIATE · Block D

Best answer: C. Up to 5 back blows, then up to 5 abdominal thrusts

Justification: For ineffective coughing in a conscious adult: up to 5 back blows, then up to 5 abdominal thrusts, repeated.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.9 · LO 2b.1

Return to Question 274 ↑

Q276 INTERMEDIATE · Block D

Best answer: A. Prefer a fixed option, avoiding small removable prostheses that could be inhaled or fractured

Justification: In uncontrolled epilepsy, small removable prostheses can be inhaled or fracture during a seizure. Metal-based or reinforced dentures are used if removable. Check for phenytoin-induced gingival overgrowth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10 · LO 2c.15

Return to Question 276 ↑

Q277 INTERMEDIATE · Block D

Best answer: E. Second trimester

Justification: Necessary care is safest in the second trimester; take radiographs only if justified.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10 · LO 2c.15

Return to Question 277 ↑

Q278 INTERMEDIATE · Block D

Best answer: A. Review dental bleeding risk and local haemostatic strategy without routine drug cessation

Justification: Use current procedure-specific anticoagulant guidance, evaluate bleeding risk and local haemostasis; avoid automatic prolonged cessation.

Why not the closest alternatives: Drug interruption may expose the patient to avoidable thrombotic risk.

Revise: Volume I · Volume I Unit 4 · Diploma LO 1.7; 2a.13; 2b.11–2b.12

Return to Question 278 ↑

Q279 INTERMEDIATE · Block D

Best answer: B. Adapt treatment and preventive planning to xerostomia-inducing polypharmacy

Justification: Review contributing medications with prescriber, manage dry mouth and caries risk, and adapt prosthetic planning.

Why not the closest alternatives: A new denture alone may fail if salivary problems persist.

Revise: Volume I · Volume I Unit 4 · Diploma LO 1.7; 2a.13; 2b.11–2b.12

Return to Question 279 ↑

Q280 INTERMEDIATE · Block D

Best answer: C. Fully cured heat-cured PMMA, polyamide or polycarbonate

Justification: True allergy is to residual monomer; fully cured or methacrylate-free bases avoid it.

Why not the closest alternatives: Self-cure and chairside resins contain the most residual monomer.

Revise: Unit 4 §4.10 · LO 2c.15

Return to Question 280 ↑

Q281 INTERMEDIATE · Block D

Best answer: B. Electrosurgery (and some ultrasonic devices)

Justification: Electrosurgery, and some ultrasonic devices, may interfere with pacemakers and ICDs.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10 · LO 2c.15

Return to Question 281 ↑

Q283 INTERMEDIATE · Block D

Best answer: D. Stabilise the periodontal disease first, then reassess before any definitive bridge

Justification: Uncontrolled periodontal disease is not a suitable foundation for definitive bridgework; reassess after therapy.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 companion note 02 · LO 2b.3, 2c.1

Return to Question 283 ↑

Q284 INTERMEDIATE · Block D

Best answer: B. Explain benefit, maintenance, reasonable alternatives and relevant medical risks including MRONJ where applicable

Justification: Valid informed consent requires a balanced account of material risks and maintenance burdens, alongside reasonable alternatives and patient goals.

Why not the closest alternatives: Expected benefit is essential but not enough when systemic risk and life-long maintenance affect decisions.

Revise: Unit 4 companion note 03, Unit 13 · LO 3b.1

Return to Question 284 ↑

Q285 INTERMEDIATE · Block D

Best answer: C. Copy dentures with minimal changes, and labelling for identification

Justification: Neuromuscular disease limits adaptation; copying keeps learned control. Labelling prevents loss in care homes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10 · LO 2c.15

Return to Question 285 ↑

Q286 INTERMEDIATE · Block D

Best answer: C. Increased risk of candidosis and delayed healing

Justification: Adrenal suppression is rarely relevant for routine prosthodontics; infection and healing are.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10 · LO 2c.15

Return to Question 286 ↑

Q287 INTERMEDIATE · Block D

Best answer: B. An implant-retained overdenture, after assessment

Justification: Without saliva, physical retention fails; implants provide mechanical retention.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10, Unit 11 · LO 2c.15

Return to Question 287 ↑

Q288 INTERMEDIATE · Block D

Best answer: E. Manage the cause with her GMP first

Justification: Ongoing acid would undermine new restorations; stabilise with the GMP, then restore.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.10, Unit 10 · LO 2c.15

Return to Question 288 ↑

Q289 INTERMEDIATE · Block D

Best answer: A. Stabilise oral disease, liaise with her physician, and defer elective definitive prosthodontics until her condition is stable

Justification: With unstable disease, stabilise oral disease and defer elective definitive care until it is medically safe.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 companion note 02 · LO 2b.12

Return to Question 289 ↑

Q290 INTERMEDIATE · Block D

Best answer: D. Refer to a secondary care setting

Justification: ASA III patients, and severe COPD, are outside primary-care sedation standards.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 §4.2 · LO 2c.8, 2b.12

Return to Question 290 ↑

BLOCK E · Assessment, diagnosis, imaging, classifications and planning

Q291 INTERMEDIATE · Block E

Best answer: C. Apply Grade C because smoking at least 10 cigarettes daily is a major grade modifier

Justification: When reliable direct progression data are unavailable, bone loss/age provides initial grading evidence, but smoking at 10 or more cigarettes per day is a Grade C modifier in the 2017 classification framework.

Why not the closest alternatives: An initially calculated Grade B cannot override a sufficiently strong documented risk modifier. Stage and grade describe different aspects of severity/complexity and progression risk.

Revise: Volume I · Unit 5 · Diploma LO 1.8; 2a.5; 2a.17; source themes: 2019 Edinburgh MPerio; 2019 Dubai MPerio

Return to Question 291 ↑

Q292 INTERMEDIATE · Block E

Best answer: B. Use Grade C as a risk-modified classification and prioritise glycaemic/periodontal control before complex reconstruction

Justification: Within the periodontitis grading framework, diabetes with HbA1c at or above 7% is a Grade C modifier. Glycaemic control also matters for prognosis and sequencing of definitive restorative care.

Why not the closest alternatives: Grade does not automatically determine stage. Risk control and periodontal stabilisation matter more than indiscriminate antibiotic prescribing.

Revise: Volume I · Unit 5 · Diploma LO 1.8; 2a.5; 2b.4; source themes: 2019 Edinburgh MPerio; 2019 Dubai MPerio

Return to Question 292 ↑

Q293 INTERMEDIATE · Block E

Best answer: D. Offer further periodontal treatment discussion and referral for surgical/regenerative assessment when appropriate

Justification: Class II furcation does not automatically justify extraction. A persistent deep defect warrants risk evaluation and consideration of further periodontal therapy, including referral for regenerative assessment in suitable cases.

Why not the closest alternatives: Furcation involvement increases complexity and maintenance difficulty, but treatment and prognosis depend on remaining support, access, restoration and patient factors.

Revise: Volume I · Unit 5 · Diploma LO 2a.17–2a.18; 2c.20; source themes: 2019 Edinburgh MPerio; 2019 Dubai MPerio

Return to Question 293 ↑

Q295 INTERMEDIATE · Block E

Best answer: D. Use them as a reference for accepted contours while re-assessing fit, jaw relation, aesthetics and wear

Justification: An old denture can preserve accepted contour and tooth-position information, but deficiencies in vertical relation, occlusion and fit require targeted corrections.

Why not the closest alternatives: Blind copying would reproduce known functional problems.

Revise: Unit 5 §5.1 · LO 2a.3

Return to Question 295 ↑

Q296 INTERMEDIATE · Block E

Best answer: E. Neuropathic pain requiring further assessment

Justification: Neuropathic causes may need assessment when symptoms are discordant with a mechanical lesion.

Why not the closest alternatives: Assuming an acrylic allergy without examining the symptom pattern is premature.

Revise: Volume I · Volume I Units 2 and 5 · Diploma LO 2a.5–2a.19; 2b.3

Return to Question 296 ↑

Q297 INTERMEDIATE · Block E

Best answer: E. Clarify the patient's main concerns and expectations

Justification: Establish goals, expectations and understanding before selecting an irreversible prosthetic design.

Why not the closest alternatives: Proceeding directly to preparations without shared goals increases dissatisfaction risk.

Revise: Volume I · Volume I Unit 5 · Diploma LO 2a.1–2a.21; 2b.1–2b.12

Return to Question 297 ↑

Q300 INTERMEDIATE · Block E

Best answer: E. Establish periodontal, endodontic and restorative prognosis of proposed abutments

Justification: The prognosis of individual abutments and supporting tissues drives the viability of the fixed plan.

Why not the closest alternatives: Aesthetic mock-up cannot substitute for biological assessment.

Revise: Volume I · Volume I Unit 5 · Diploma LO 2a.1–2a.21; 2b.1–2b.12

Return to Question 300 ↑

Q301 INTERMEDIATE · Block E

Best answer: A. Papillon–Lefèvre syndrome

Justification: Papillon–Lefèvre syndrome classically combines palmoplantar keratoderma and severe early-onset periodontitis, often with primary and permanent dentition involvement. Specialist periodontal and medical assessment is indicated.

Why not the closest alternatives: Other syndromes can predispose to periodontal breakdown, but the skin pattern plus precocious severe tooth loss is a key discriminator.

Revise: Volume I · Unit 5 · Diploma LO 1.2; 2a.7; 2a.17; source themes: 2019 Edinburgh MPerio; user-specified themes

Return to Question 301 ↑

Q302 INTERMEDIATE · Block E

Best answer: E. Chediak–Higashi syndrome involving defective lysosomal trafficking and immune dysfunction

Justification: Chediak–Higashi syndrome features immunodeficiency, recurrent pyogenic infections, partial albinism and bleeding abnormalities; periodontal disease can be severe. It is a medical referral concern, not a routine restorative diagnosis.

Why not the closest alternatives: Papillon–Lefèvre typically has palmoplantar keratoderma, whereas immunodeficiency with hypopigmentation is the stronger clue here.

Revise: Volume I · Unit 5 · Diploma LO 1.2; 2a.7; 2a.17; source themes: 2019 Edinburgh MPerio; user-specified themes

Return to Question 302 ↑

Q303 INTERMEDIATE · Block E

Best answer: A. Reconcile medical risks and drug interactions before invasive care

Justification: Review medication indication, procedure bleeding risk, MRONJ risk and current guidance collaboratively where appropriate.

Why not the closest alternatives: Stopping prescribed drugs arbitrarily is unsafe.

Revise: Volume I · Volume I Unit 5 · Diploma LO 2a.1–2a.21; 2b.1–2b.12

Return to Question 303 ↑

Q304 INTERMEDIATE · Block E

Best answer: A. Investigate persistent unexplained soft-tissue lesions urgently

Justification: A persistent suspicious oral lesion warrants urgent investigation/referral rather than prosthesis redesign alone.

Why not the closest alternatives: Relying on mechanical trauma without follow-up could delay cancer diagnosis.

Revise: Volume I · Volume I Unit 5 · Diploma LO 2a.1–2a.21; 2b.1–2b.12

Return to Question 304 ↑

Q308 INTERMEDIATE · Block E

Best answer: D. Reassess overall risk and prognosis, pursue further site-specific periodontal therapy and discuss referral for advanced furcation treatment before committing to the RPD design

Justification: Persistent deep bleeding pockets indicate incomplete periodontal control. A potential RPD abutment should have a considered prognosis before definitive design; advanced furcation treatment may be appropriate in skilled hands.

Why not the closest alternatives: Prosthetic splinting does not replace disease management, and furcation presence alone does not mandate extraction.

Revise: Volume I · Unit 5 · Diploma LO 2a.17–2a.18; 2c.20; source themes: 2019 Edinburgh MPerio; 2019 Dubai MPerio

Return to Question 308 ↑

Q309 INTERMEDIATE · Block E

Best answer: C. Denture-associated candidosis requiring hygiene/fit review

Justification: Denture stomatitis is associated with plaque/biofilm, continuous wear and impaired denture hygiene.

Why not the closest alternatives: A diffuse palatal pattern is less characteristic of a single over-extended edge.

Revise: Volume I · Volume I Units 2 and 5 · Diploma LO 2a.5–2a.19; 2b.3

Return to Question 309 ↑

Q313 INTERMEDIATE · Block E

Best answer: B. Localised, because fewer than 30% of teeth are affected

Justification: 6 of 26 teeth is 23%, under the 30% threshold, so the extent is localised; 30% or more of teeth is generalised.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 313 ↑

Q314 INTERMEDIATE · Block E

Best answer: A. Currently stable

Justification: BOP under 10%, PPD 4 mm or less and no BOP at 4 mm sites.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 314 ↑

Q315 INTERMEDIATE · Block E

Best answer: C. Currently in remission

Justification: BOP 10% or more, but PPD 4 mm or less with no BOP at 4 mm sites.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 315 ↑

Q317 INTERMEDIATE · Block E

Best answer: A. Stage IV

Justification: Stage IV involves tooth loss from periodontitis and a need for complex rehabilitation (fewer than 20 teeth, masticatory dysfunction, drifting).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 317 ↑

Q318 INTERMEDIATE · Block E

Best answer: E. Level 2

Justification: NHS standard: after steps 1–2 in primary care, Stage III Grade A is Level 2.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2b.10

Return to Question 318 ↑

Q319 INTERMEDIATE · Block E

Best answer: D. Level 3

Justification: Stage III Grade B or C, and all Stage IV, are Level 3.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2b.10

Return to Question 319 ↑

Q320 INTERMEDIATE · Block E

Best answer: B. Code 3 (pocket 3.5–5.5 mm)

Justification: The band partly visible means a 3.5–5.5 mm pocket; code 4 is the band disappearing (over 5.5 mm).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 320 ↑

Q322 INTERMEDIATE · Block E

Best answer: D. Grade III

Justification: Hamp III is through-and-through; I is under 3 mm horizontal loss; II is over 3 mm but not through.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 322 ↑

Q323 INTERMEDIATE · Block E

Best answer: A. Potential oral malignancy requiring urgent referral

Justification: Persistent induration and non-healing mucosal ulceration demand urgent investigation for malignancy.

Why not the closest alternatives: Repeated relines or antiseptic mouthwash should not delay referral.

Revise: Volume I · Volume I Units 2 and 5 · Diploma LO 2a.5–2a.19; 2b.3

Return to Question 323 ↑

Q325 INTERMEDIATE · Block E

Best answer: E. Class II division 2

Justification: Retroclined upper centrals with lower edges behind the cingulum plateau.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.6 · LO 2a.15

Return to Question 325 ↑

Q326 INTERMEDIATE · Block E

Best answer: E. Use diagnostic wax-up/mock-up to test functional and aesthetic changes

Justification: A reversible diagnostic wax-up/mock-up or provisional trial allows functional and aesthetic evaluation before definitive preparation.

Why not the closest alternatives: Choosing crowns without testing a major change may be premature.

Revise: Volume I · Volume I Unit 5 · Diploma LO 2a.1–2a.21; 2b.1–2b.12

Return to Question 326 ↑

Q329 INTERMEDIATE · Block E

Best answer: B. Periapical radiograph using the paralleling technique

Justification: It shows bone level, crown–root ratio, root form, periapical status and root-filling quality with least distortion.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 329 ↑

Q330 INTERMEDIATE · Block E

Best answer: C. Vertical bitewings

Justification: Vertical bitewings show crestal bone in periodontitis; horizontal bitewings are for caries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 330 ↑

Q331 INTERMEDIATE · Block E

Best answer: E. It is a ghost image, projected on the opposite side, higher and larger

Justification: Objects between the source and the rotation centre project as blurred ghosts on the opposite side, higher and magnified.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 (exam traps) · LO 2a.12

Return to Question 331 ↑

Q333 INTERMEDIATE · Block E

Best answer: A. The practitioner (here, the dentist)

Justification: Under IR(ME)R the practitioner justifies, the operator exposes, the referrer requests and the employer sets procedures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12, 3a.6

Return to Question 333 ↑

Q335 INTERMEDIATE · Block E

Best answer: B. Intraoral, panoramic, CBCT

Justification: Roughly 0.001–0.008, 0.003–0.024 and 0.02–1 mSv respectively.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 335 ↑

Q336 INTERMEDIATE · Block E

Best answer: D. Cold, using tetrafluoroethane or ethyl chloride on a pledget

Justification: Tetrafluoroethane or ethyl chloride cold tests are more reliable than heat.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.14

Return to Question 336 ↑

Q337 INTERMEDIATE · Block E

Best answer: B. The EPT measures nerve response, not blood flow

Justification: After trauma, nerve conduction may be lost temporarily while the blood supply survives; a single negative result does not justify root canal treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 (exam traps) · LO 2a.14

Return to Question 337 ↑

Q338 INTERMEDIATE · Block E

Best answer: E. Laser Doppler flowmetry or pulse oximetry

Justification: These assess pulpal blood flow, true vitality; thermal and electric tests assess sensibility only.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.14

Return to Question 338 ↑

Q339 INTERMEDIATE · Block E

Best answer: C. A bite test on individual cusps (for example a Tooth Slooth)

Justification: Pain on release when loading a single cusp localises a crack; transillumination also helps.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.14

Return to Question 339 ↑

Q340 INTERMEDIATE · Block E

Best answer: A. To assess interocclusal space, plan a wax-up and survey the RPD design

Justification: Mounted casts in RCP show space, guidance and interferences, and let you survey the RPD path and design.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.15, 2c.2

Return to Question 340 ↑

Q341 INTERMEDIATE · Block E

Best answer: A. A diagnostic wax-up and intraoral mock-up

Justification: They test aesthetics and OVD reversibly and help communication with patient and laboratory.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.17

Return to Question 341 ↑

Q342 INTERMEDIATE · Block E

Best answer: B. Iron

Justification: A low MCV (microcytosis) suggests iron deficiency; confirm with ferritin.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.7

Return to Question 342 ↑

Q345 INTERMEDIATE · Block E

Best answer: D. Sound coronal tissue above the crest (about 2 mm high and 1 mm wide, or over 30% of the original crown)

Justification: The NHS entry criteria require about 2 mm high and 1 mm wide sound tissue, or over 30% of the original crown.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.9 · LO 2a.16, 2b.10

Return to Question 345 ↑

Q346 INTERMEDIATE · Block E

Best answer: E. Assess for vertical root fracture and endodontic pathology using careful history, probing pattern, tests and appropriate imaging

Justification: A narrow isolated deep pocket on a restored or root-treated tooth raises concern for a vertical root fracture or endodontic communication. A diagnosis based only on probing depth risks inappropriate periodontal intervention.

Why not the closest alternatives: Imaging can support but not always confirm a root fracture; interpret the complete clinical pattern and refer when diagnosis or treatment exceeds competence.

Revise: Volume I, Unit 5 · Diploma LO 2a.9; 2a.17; 2c.17 · Reference themes: 2019 Edinburgh MPerio; 2015 MRD

Return to Question 346 ↑

Q347 INTERMEDIATE · Block E

Best answer: C. Investigations should be chosen only when their results may change the diagnosis, prognosis or plan

Justification: Choose investigations whose results could alter diagnosis, prognosis or treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 companion note · LO 2a.12

Return to Question 347 ↑

Q348 INTERMEDIATE · Block E

Best answer: B. Refer urgently

Justification: A normal radiograph does not overrule a suspicious clinical lesion; an indurated persistent ulcer needs the suspected cancer pathway.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 companion note (traps), Unit 2 · LO 2a.6

Return to Question 348 ↑

Q349 INTERMEDIATE · Block E

Best answer: D. Urgent care, stabilisation, re-evaluation, definitive treatment, supportive maintenance

Justification: This five-phase sequence ensures definitive work rests on controlled disease.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 treatment strategies note 01 · LO 2b.3

Return to Question 349 ↑

Q350 INTERMEDIATE · Block E

Best answer: A. Leaving the space, a resin-bonded bridge, a conventional bridge, an RPD or an implant, with their pros and cons

Justification: Compare preparation, reversibility, aesthetics, cost, longevity, cleaning and failure for each, including no treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 treatment strategies note 02 · LO 2b.4, 2c.2

Return to Question 350 ↑

Q351 INTERMEDIATE · Block E

Best answer: D. A conformative approach, matching the existing ICP

Justification: A stable, comfortable ICP is kept when space and function allow.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 treatment strategies note 03, Unit 9 · LO 2c.19

Return to Question 351 ↑

Q352 INTERMEDIATE · Block E

Best answer: B. Periodontal disease of a poorly cleansable abutment

Justification: Periodontal inflammation and hygiene access of the planned abutment must be addressed before redesigning prostheses.

Why not the closest alternatives: More elaborate clasping does not substitute for periodontal treatment.

Revise: Volume I · Volume I Units 2 and 5 · Diploma LO 2a.5–2a.19; 2b.3

Return to Question 352 ↑

Q353 INTERMEDIATE · Block E

Best answer: B. Material risks and reasonable alternatives, including no treatment, must be discussed

Justification: Montgomery (2015) requires disclosure of what this patient would consider material, plus alternatives including no treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 treatment strategies note 04, Unit 13 · LO 3b.1

Return to Question 353 ↑

Q354 INTERMEDIATE · Block E

Best answer: C. The diagnosis, key-tooth prognosis, proposed phases, responsibilities and relevant records

Justification: A good referral lets the receiving clinician act without repeating the assessment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 treatment strategies note 05 · LO 2b.10

Return to Question 354 ↑

Q355 INTERMEDIATE · Block E

Best answer: E. Definitive restorations before disease control are unsound

Justification: Active periodontitis threatens margins, abutments and long-term outcome; stabilise first.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 treatment strategies note 01 · LO 2b.3

Return to Question 355 ↑

Q356 INTERMEDIATE · Block E

Best answer: C. Reduced OVD

Justification: Overclosure produces deep folds, a prominent chin and angular cheilitis. An excessive freeway space confirms it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.1, Unit 7 · LO 2a.15

Return to Question 356 ↑

Q358 INTERMEDIATE · Block E

Best answer: A. It may affect whether a conformative or reorganised approach is needed

Justification: A significant slide with symptoms or interferences may favour reorganisation; a stable one may be conformed to.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.1, Unit 9 · LO 2a.15

Return to Question 358 ↑

Q360 INTERMEDIATE · Block E

Best answer: B. Timed collection by spitting or drooling for 5–15 minutes

Justification: Unstimulated flow is collected by spitting or drooling over a set time; under 0.1 mL/min is hyposalivation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.1, Unit 2 · LO 2a.7

Return to Question 360 ↑

Q361 INTERMEDIATE · Block E

Best answer: C. Traumatic ulcer from denture flange movement

Justification: A focal lesion corresponding to a new mechanical interference suggests trauma, provided healing is confirmed after correction.

Why not the closest alternatives: Never assume persistent ulcers remain traumatic after the causative contact is removed.

Revise: Volume I · Volume I Units 2 and 5 · Diploma LO 2a.5–2a.19; 2b.3

Return to Question 361 ↑

Q362 INTERMEDIATE · Block E

Best answer: E. Molar–incisor pattern

Justification: The 2017 classification describes extent as localised, generalised or molar–incisor pattern.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2a.9

Return to Question 362 ↑

Q363 INTERMEDIATE · Block E

Best answer: C. Level 3

Justification: Stage III Grade B is Level 3, whatever the extent.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.4 · LO 2b.10

Return to Question 363 ↑

Q364 INTERMEDIATE · Block E

Best answer: A. Horizontal bitewings

Justification: They are the standard view for proximal caries and restoration margins.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 364 ↑

Q365 INTERMEDIATE · Block E

Best answer: D. Focal trough distortion and overlap limit fine detail

Justification: Panoramic images give an overview, not the fine detail needed for early caries.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 365 ↑

Q366 INTERMEDIATE · Block E

Best answer: A. Intraoral about 0.001–0.008 mSv

Justification: The CBCT dose depends heavily on the field of view.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.7 · LO 2a.12

Return to Question 366 ↑

Q367 INTERMEDIATE · Block E

Best answer: C. Anxiety, or liquefactive necrosis conducting the current

Justification: Both can give a false-positive EPT response.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.14

Return to Question 367 ↑

Q368 INTERMEDIATE · Block E

Best answer: D. Calcification and reduced pulp size with age

Justification: Older and calcified teeth often fail to respond despite vital pulps.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.8 · LO 2a.14

Return to Question 368 ↑

Q369 INTERMEDIATE · Block E

Best answer: B. They are counted as modification spaces by number, whatever their size

Justification: Applegate: the extent of a modification is not considered, only how many extra spaces there are.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 5 §5.2 · LO 2a.10

Return to Question 369 ↑

Q370 INTERMEDIATE · Block E

Best answer: D. Integrated periodontal, endodontic and restorability prognosis

Justification: Abutment selection must integrate periodontal support, remaining tooth structure, pulpal/endodontic status and strategic value.

Why not the closest alternatives: A good undercut is insufficient if the proposed abutment has an unacceptable prognosis.

Revise: Unit 5 §5.9 · LO 2a.16

Return to Question 370 ↑

BLOCK F · Removable partial dentures

Q372 INTERMEDIATE · Block F

Best answer: E. Failure of reciprocation transmits lateral force to the abutment

Justification: Lack of reciprocal/bracing support allows lateral clasp forces to act on the tooth.

Why not the closest alternatives: Increasing the retentive tip depth would tend to worsen the force.

Revise: Volume I · Clasp fatigue studies PMC9760146 and PMC9275311 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 372 ↑

Q373 INTERMEDIATE · Block F

Best answer: B. Correct an interfering framework component rather than increasing retention

Justification: Use pressure-indicating material and survey information to identify and correct the obstruction; do not tighten an unseated framework.

Why not the closest alternatives: Retention is secondary to achieving passive, complete framework seating.

Revise: Volume I · Volume I Unit 6; clasp review PMC9275311 · Diploma LO 2c.2–2c.6

Return to Question 373 ↑

Q374 INTERMEDIATE · Block F

Best answer: D. Reconsider mesial rest support, guide planes, appropriate stress release and tissue support as an integrated redesign

Justification: Distal-extension rotation around the rest/fulcrum creates torque on terminal abutments. Mesial-rest stress-releasing concepts can reduce harmful distal leverage while still providing appropriate support and controlled movement.

Why not the closest alternatives: Increasing a rigid retainer’s resistance may increase abutment torque rather than solve the differential tooth–mucosa displacement problem.

Revise: Volume I · Unit 6 · Diploma LO 2c.2–2c.6; 2c.13; source themes: 2015 Dubai MRD; 2020 Edinburgh MPros

Return to Question 374 ↑

Q375 INTERMEDIATE · Block F

Best answer: A. Excessive undercut increases strain at insertion/removal

Justification: Greater displacement can exceed the clasp’s elastic limit and accelerate fatigue.

Why not the closest alternatives: Clasp retention must be balanced against flexibility and undercut depth.

Revise: Volume I · Clasp fatigue studies PMC9760146 and PMC9275311 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 375 ↑

Q378 INTERMEDIATE · Block F

Best answer: A. Record the displaceable distal saddle tissues with the metal framework fully seated on stable tooth rests

Justification: The altered-cast impression records the distal-extension tissues with the RPD framework positioned on their tooth-supported rests, improving the relationship between displaceable ridge support and stable tooth support.

Why not the closest alternatives: It does not replace correct framework design or ensure zero rotation; its utility depends on a stable seated framework and clinical ridge conditions.

Revise: Volume I · Unit 6 · Diploma LO 2c.3; 2c.13; 2c.21; source themes: 2015 MRD; MRD Past questions

Return to Question 378 ↑

Q379 INTERMEDIATE · Block F

Best answer: E. Re-survey available paths and evaluate tooth modification, sectional insertion and alternative prostheses against support and maintainability

Justification: The problem is conflicting insertion paths and undercuts, not merely lack of flexibility. A sectional prosthesis can sometimes accommodate a difficult insertion path, whereas flexible polyamide has limitations in rigid support, adjustment and repair.

Why not the closest alternatives: A flexible base may have selected indications, but its flexibility does not automatically substitute for framework support, rest seats or manageable undercuts.

Revise: Volume I · Unit 6 · Diploma LO 2c.3; 2c.6; 2c.13; source themes: 2019 Edinburgh MPros; user-specified clinical themes

Return to Question 379 ↑

Q380 INTERMEDIATE · Block F

Best answer: C. Decide a surveyed path of insertion and evaluate available lingual clearance, tooth modifications and alternative major connectors

Justification: Path of insertion, available functional sulcus depth, undercuts and connector rigidity govern whether a proposed major connector is anatomically and biomechanically feasible. Lingual tori and tipped posterior teeth can substantially alter the design.

Why not the closest alternatives: A bar relieved until flexible loses essential rigidity; ignoring the posterior teeth risks non-seating or inappropriate clasp trajectories.

Revise: Volume I · Unit 6 · Diploma LO 2c.3; 2c.6; source themes: 2015 Dubai; user-specified clinical themes

Return to Question 380 ↑

Q381 INTERMEDIATE · Block F

Best answer: D. Identify framework distortion, casting nodules, guide-plane interferences and rest seating; correct or remake the framework before jaw registration and acrylic processing

Justification: Framework fit and rest seating are prerequisites for a reliable distal-extension impression, occlusal records and processed bases. Rocking metal cannot be reliably corrected by more saddle bulk; a passively seated framework should be obtained first.

Why not the closest alternatives: The altered-cast technique depends on a seated framework and does not repair fundamental casting distortion or surveying errors.

Revise: Volume I · Unit 6 · Diploma LO 1.9; 2c.3; 2c.6; source themes: 2015 MRD; MRD Past questions

Return to Question 381 ↑

Q382 INTERMEDIATE · Block F

Best answer: A. Add composite to create a contour with a suitable undercut

Justification: Additive composite can create retentive contour or a rest seat without crowning.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.2 · LO 2c.3

Return to Question 382 ↑

Q383 INTERMEDIATE · Block F

Best answer: B. Change the insertion path and survey design

Justification: Path of insertion and guide-plane compatibility determine whether rigid components seat without interference.

Why not the closest alternatives: A stronger clasp cannot overcome a rigid framework interference.

Revise: Volume I · Volume I Unit 6; clasp review PMC9275311 · Diploma LO 2c.2–2c.6

Return to Question 383 ↑

Q384 INTERMEDIATE · Block F

Best answer: B. Alter clasp approach to avoid a soft-tissue undercut

Justification: Select a feasible clasp approach or redesign the undercut/path to avoid tissue impingement.

Why not the closest alternatives: A deeper retentive tooth undercut does not remove the tissue obstruction.

Revise: Volume I · Volume I Units 5–6 · Diploma LO 2c.2–2c.6; 2a.18

Return to Question 384 ↑

Q385 INTERMEDIATE · Block F

Best answer: C. Select a longer or more flexible retentive arm in a suitable undercut

Justification: Appropriate arm length, cross-section, material and undercut can reduce cyclic strain and fatigue fracture risk.

Why not the closest alternatives: A more rigid or shorter cast arm would increase strain at the same displacement.

Revise: Volume I · Volume I Unit 6; clasp review PMC9275311 · Diploma LO 2c.2–2c.6

Return to Question 385 ↑

Q386 INTERMEDIATE · Block F

Best answer: B. A cingulum rest (on a natural or composite-built cingulum)

Justification: It is less visible, gives a shorter lever and directs load better than an incisal rest.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.3 · LO 2c.3

Return to Question 386 ↑

Q387 INTERMEDIATE · Block F

Best answer: C. Modify a guide plane/reciprocal element to improve stability

Justification: Reciprocation and bracing stabilise the prosthesis and counter lateral forces exerted by the retentive arm.

Why not the closest alternatives: Simply increasing the undercut magnifies forces rather than corrects reciprocation.

Revise: Volume I · Volume I Unit 6; clasp review PMC9275311 · Diploma LO 2c.2–2c.6

Return to Question 387 ↑

Q388 INTERMEDIATE · Block F

Best answer: D. Alter the rest position/design to accommodate tissueward rotation

Justification: Mesial rest and suitable direct retainer geometry can reduce adverse abutment torquing during tissueward saddle rotation.

Why not the closest alternatives: It does not remove the need for saddle support and sound surveying.

Revise: Volume I · Volume I Unit 6 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 388 ↑

Q390 INTERMEDIATE · Block F

Best answer: C. Re-survey with a corrected path of insertion

Justification: The cast must be surveyed and a feasible insertion path established; rigid components cannot bend around undercuts.

Why not the closest alternatives: Retention modifications cannot solve a fundamentally incompatible insertion path.

Revise: Volume I · Volume I Units 5–6 · Diploma LO 2c.2–2c.6; 2a.18

Return to Question 390 ↑

Q391 INTERMEDIATE · Block F

Best answer: A. An embrasure clasp, with the embrasure prepared for space

Justification: It gives cross-arch retention and bracing on the dentate side of a Kennedy II; prepare enough embrasure space.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.4 · LO 2c.3

Return to Question 391 ↑

Q392 INTERMEDIATE · Block F

Best answer: C. Reassess the distal-extension design and abutment prognosis before altering clasp tightness

Justification: Re-examine periodontal health, support, saddle fit and entire biomechanical design before modifying clasp force.

Why not the closest alternatives: Tightening an unstable appliance may worsen the underlying abutment trauma.

Revise: Volume I · Volume I Unit 6 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 392 ↑

Q393 INTERMEDIATE · Block F

Best answer: B. Wrought wire

Justification: Wrought wire is flexible, adjustable and suits 0.5–0.75 mm undercuts on free-end abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.4 · LO 2c.3

Return to Question 393 ↑

Q394 INTERMEDIATE · Block F

Best answer: D. Review occlusal rest-seat form to obtain tooth-supported load

Justification: An appropriately prepared occlusal rest seat provides vertical tooth-supported load and resists tissueward displacement.

Why not the closest alternatives: Clasp engagement is not a substitute for support.

Revise: Volume I · Volume I Unit 6; clasp review PMC9275311 · Diploma LO 2c.2–2c.6

Return to Question 394 ↑

Q395 INTERMEDIATE · Block F

Best answer: E. Provide appropriately placed indirect retention

Justification: A suitably positioned indirect retainer resists rotation away from the mucosa around the fulcrum line.

Why not the closest alternatives: Tightening a clasp does not substitute for an indirect retainer where indicated.

Revise: Volume I · Volume I Unit 6 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 395 ↑

Q397 INTERMEDIATE · Block F

Best answer: A. Teeth move about 0.02–0.1 mm under load, while mucosa compresses about 0.4–2 mm

Justification: Teeth move 0.02–0.1 mm; mucosa compresses 0.4–2 mm. The saddle rotates about the fulcrum line and torques the abutment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 397 ↑

Q398 INTERMEDIATE · Block F

Best answer: C. The retromolar pad and buccal shelf

Justification: Maximum extension spreads load over the largest supporting area.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 398 ↑

Q399 INTERMEDIATE · Block F

Best answer: E. The altered cast technique (Applegate)

Justification: It records the saddle area under functional load on the framework, matching tooth and mucosal support.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 399 ↑

Q401 INTERMEDIATE · Block F

Best answer: B. They move gingivally and mesially, disengaging from the tooth and reducing torque

Justification: Releasing from the tooth under load minimises torque on the abutment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 401 ↑

Q402 INTERMEDIATE · Block F

Best answer: E. RPA, replacing the I-bar with an Akers-type arm

Justification: Eliason's RPA replaces the I-bar with an Akers-type arm when a gingivally approaching clasp is contraindicated.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 402 ↑

Q403 INTERMEDIATE · Block F

Best answer: D. Through the rests nearest to the free-end saddles

Justification: The saddle rotates about this line.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.6 · LO 2c.3

Return to Question 403 ↑

Q404 INTERMEDIATE · Block F

Best answer: C. As far anterior to the fulcrum line as possible

Justification: The longer the lever arm, the better the indirect retainer resists lifting.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.6 · LO 2c.3

Return to Question 404 ↑

Q406 INTERMEDIATE · Block F

Best answer: E. Narrow, reduced occlusal tables in harmonious occlusion

Justification: Narrower tables reduce the load on the resorbing ridge and the torque on abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5 · LO 2c.3

Return to Question 406 ↑

Q408 INTERMEDIATE · Block F

Best answer: A. Full palatal plate

Justification: Few teeth, long saddles and a flat palate need maximum support and retention.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · Schematic 13 · LO 2c.3

Return to Question 408 ↑

Q409 INTERMEDIATE · Block F

Best answer: B. Mid-palatal strap, at least 8 mm wide

Justification: Short bounded saddles on both sides need a thin, rigid strap.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 409 ↑

Q410 INTERMEDIATE · Block F

Best answer: D. Anteroposterior strap avoiding the torus

Justification: It is rigid with less coverage and avoids the torus.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 410 ↑

Q411 INTERMEDIATE · Block F

Best answer: A. A short thick arm has excessive bending stiffness

Justification: Shorter and thicker arms are less flexible, so similar deflection produces greater stress.

Why not the closest alternatives: The alloy alone does not determine flexibility independent of shape.

Revise: Volume I · Clasp fatigue studies PMC9760146 and PMC9275311 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 411 ↑

Q412 INTERMEDIATE · Block F

Best answer: D. Lingual bar: half-pear, about 4 mm high, at least 3 mm clear of the gingival margin

Justification: At least 7–8 mm of sulcus allows a half-pear bar about 4 mm high and 3 mm clear of the gingiva.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · Number card 2 · LO 2c.3

Return to Question 412 ↑

Q413 INTERMEDIATE · Block F

Best answer: A. Lingual plate

Justification: A shallow sulcus, mobile incisors needing splinting and likely additions favour a plate.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 413 ↑

Q414 INTERMEDIATE · Block F

Best answer: B. Sublingual bar

Justification: It sits deeper in the sulcus and is recorded in a functional impression.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 414 ↑

Q415 INTERMEDIATE · Block F

Best answer: E. Labial bar

Justification: It is used for lingually inclined teeth or large tori, despite its bulk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 415 ↑

Q417 INTERMEDIATE · Block F

Best answer: E. Loss of an occlusal rest shifts load to soft tissue

Justification: Without tooth-supported rest contact, load may shift onto the mucosa and alter rotation.

Why not the closest alternatives: Clasp retention cannot substitute for vertical occlusal support.

Revise: Volume I · Clasp fatigue studies PMC9760146 and PMC9275311 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 417 ↑

Q418 INTERMEDIATE · Block F

Best answer: E. Reassess indirect retention about the functional fulcrum line

Justification: Indirect retention should be placed to oppose rotation away from tissue about the functional fulcrum line.

Why not the closest alternatives: A more rigid major connector alone may not counter the lever arm.

Revise: Volume I · Volume I Units 5–6 · Diploma LO 2c.2–2c.6; 2a.18

Return to Question 418 ↑

Q419 INTERMEDIATE · Block F

Best answer: C. Major connectors must be rigid so that forces spread across the arch

Justification: Rigidity distributes forces across the arch; a flexible bar concentrates load on single abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 419 ↑

Q421 INTERMEDIATE · Block F

Best answer: E. An acrylic interim (transitional) denture

Justification: Acrylic is easy to add to as teeth are lost; a Co-Cr framework would be wasted on teeth of uncertain prognosis.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 421 ↑

Q422 INTERMEDIATE · Block F

Best answer: B. An Every denture

Justification: Every (1949): point contacts, open embrasures, free gingival margins and distal stabilising wires maximise gingival health in an acrylic design.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 422 ↑

Q423 INTERMEDIATE · Block F

Best answer: A. Reassess periodontal prognosis before using an abutment

Justification: Assess and stabilise periodontal disease, then reconsider the tooth’s prognosis and suitability for support.

Why not the closest alternatives: An elaborate clasp design cannot rescue a hopeless abutment.

Revise: Volume I · Volume I Units 5–6 · Diploma LO 2c.2–2c.6; 2a.18

Return to Question 423 ↑

Q424 INTERMEDIATE · Block F

Best answer: D. A swing-lock denture

Justification: A hinged labial bar engages labial undercuts of many teeth, suiting compromised teeth where conventional clasps fail.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 424 ↑

Q428 INTERMEDIATE · Block F

Best answer: C. Use a disclosing medium to locate interference and verify full rest seating before evaluating retention

Justification: The sequence is to verify passive complete seating and locate interference; retention cannot be evaluated reliably when the framework is not seated.

Why not the closest alternatives: Arbitrary clasp adjustment may alter force without addressing the actual obstruction.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 428 ↑

Q429 INTERMEDIATE · Block F

Best answer: C. Recontour the survey crown for a suitable guide plane

Justification: Restore/modify guide-plane anatomy consistent with the surveyed plan before framework completion.

Why not the closest alternatives: Simply forcing the framework risks abutment trauma.

Revise: Volume I · Volume I Units 5–6 · Diploma LO 2c.2–2c.6; 2a.18

Return to Question 429 ↑

Q433 INTERMEDIATE · Block F

Best answer: E. The dentist, whose prescription must specify every component

Justification: The dentist is responsible for the design; the prescription must specify every component.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6, In one minute, Unit 13 · LO 2c.3, 3a.3

Return to Question 433 ↑

Q434 INTERMEDIATE · Block F

Best answer: B. Without reciprocation, the retentive arm pushes the tooth as it flexes

Justification: A retentive clasp without reciprocation is not a complete design.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 companion note (SBA traps) · LO 2c.3

Return to Question 434 ↑

Q435 INTERMEDIATE · Block F

Best answer: A. A high rest

Justification: Adjust the rest or deepen the seat; pain after fitting is often from a high rest, over-extension or tissue pressure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 companion note · LO 2c.20

Return to Question 435 ↑

Q436 INTERMEDIATE · Block F

Best answer: B. Tooth support from rests at both ends

Justification: A Kennedy III saddle is tooth-supported, so it does not rotate about a fulcrum; indirect retention is generally unnecessary.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.1, §6.6 · LO 2c.3

Return to Question 436 ↑

Q438 INTERMEDIATE · Block F

Best answer: A. Mesial rests and RPI clasps on 34 and 44, cingulum rests on the canines, a lingual bar and maximum saddle extension

Justification: This classic free-end design controls torque and rotation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.5–6.7 · Schematic 14 · LO 2c.3

Return to Question 438 ↑

Q439 INTERMEDIATE · Block F

Best answer: C. Full palatal plate

Justification: With only four teeth and bilateral free ends, palatal coverage gives most support and retention.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 439 ↑

Q442 INTERMEDIATE · Block F

Best answer: A. Improve broad, functional saddle support while maintaining hygiene

Justification: Broader well-adapted saddle support reduces unit loading on the residual ridge; impression and extent should be reviewed.

Why not the closest alternatives: More clasp retention may worsen abutment torque rather than solve inadequate tissue support.

Revise: Volume I · Volume I Unit 6 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 442 ↑

Q443 INTERMEDIATE · Block F

Best answer: B. Distal undercuts increase

Justification: Lowering the heels tips the crowns distally relative to the rod.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.2 · LO 2c.3

Return to Question 443 ↑

Q444 INTERMEDIATE · Block F

Best answer: B. An ill-fitting distal saddle increases torque on the terminal abutment

Justification: Unsupported tissueward rotation increases torque on the terminal abutment as the RPD functions.

Why not the closest alternatives: Tightening the clasp does not repair loss of saddle support.

Revise: Volume I · Clasp fatigue studies PMC9760146 and PMC9275311 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 444 ↑

Q445 INTERMEDIATE · Block F

Best answer: D. A gingivally approaching I-bar into a mesiobuccal undercut, or moving retention to a premolar

Justification: A gingivally approaching bar is less visible; retention can also be placed on less visible teeth or gained from guide planes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.4 · LO 2c.3

Return to Question 445 ↑

Q447 INTERMEDIATE · Block F

Best answer: D. A minimal, open design with little tooth and gingival coverage

Justification: Reducing tooth and gingival coverage limits plaque stagnation in a high-risk mouth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8, Unit 2 · LO 2c.3

Return to Question 447 ↑

Q450 INTERMEDIATE · Block F

Best answer: C. Reline the free-end saddle

Justification: Rocking and lifting indirect retainers mean ridge resorption; tightening clasps would add torque.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 450 ↑

Q451 INTERMEDIATE · Block F

Best answer: B. Preventing over-eruption and maintaining tooth position

Justification: Occlusal rests maintain the abutment's vertical position as well as supporting the denture.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.3 · LO 2c.3

Return to Question 451 ↑

Q452 INTERMEDIATE · Block F

Best answer: B. Re-survey the cast and evaluate an alternative path, relief, connector design or multidisciplinary management before selecting a definitive prosthesis

Justification: Path of insertion, connector location and anatomical obstacles require case-specific surveying. A flexible material alone does not solve the need for support, bracing and healthy tissue clearance.

Why not the closest alternatives: A sectional design can be considered in selected cases, but only after analysing path, hygiene, connectors and tissue tolerance; no single material substitutes for sound design.

Revise: Volume I, Unit 6 · Diploma LO 2a.5; 2c.4; 2c.6 · Reference themes: 2015 MRD; user-specified clinical themes

Return to Question 452 ↑

Q453 INTERMEDIATE · Block F

Best answer: D. A proximal plate on a guide plane

Justification: Close contact between plate and guide plane reduces food trapping.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.2 · LO 2c.3

Return to Question 453 ↑

Q454 INTERMEDIATE · Block F

Best answer: E. In harmony with the existing ICP

Justification: RPD occlusion conforms to the natural occlusion; balanced occlusion is for complete dentures.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.19

Return to Question 454 ↑

Q455 INTERMEDIATE · Block F

Best answer: C. Pre-prosthetic optimisation, within Level 2

Justification: Adjusting over-eruption and tilts before design is part of Level 2 care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.3

Return to Question 455 ↑

Q456 INTERMEDIATE · Block F

Best answer: D. Lingual plate, splinting the incisors

Justification: A lingual plate splints mobile incisors and provides indirect retention.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.3

Return to Question 456 ↑

Q457 INTERMEDIATE · Block F

Best answer: C. Inadequate rest support, so the denture settles

Justification: Without seated rests the denture settles and the bar impinges on the gingiva.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.3, §6.7 · LO 2c.3

Return to Question 457 ↑

Q458 INTERMEDIATE · Block F

Best answer: C. A properly located indirect retainer anterior to the fulcrum line, supported by suitable prepared rests

Justification: Indirect retention uses an appropriately placed rest on a tooth away from the fulcrum line to resist rotation of distal extension bases away from tissues. It complements, rather than substitutes for, adequate support and direct retention.

Why not the closest alternatives: Excessive clasp retention can damage abutments and does not correct the rotational mechanics of distal extension bases.

Revise: Volume I, Unit 6 · Diploma LO 2c.4; 2c.6 · Reference themes: 2017 MRD; RPD design notes

Return to Question 458 ↑

Q460 INTERMEDIATE · Block F

Best answer: B. As far forward of the fulcrum line as possible, such as a cingulum rest on a canine

Justification: An indirect retainer far anterior to the fulcrum resists the saddle lifting.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.6 · LO 2c.3

Return to Question 460 ↑

Q463 INTERMEDIATE · Block F

Best answer: B. A swing-lock denture

Justification: It gains retention from labial undercuts across many teeth through a hinged labial bar.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 463 ↑

Q464 INTERMEDIATE · Block F

Best answer: A. An acrylic interim RPD

Justification: It suits uncertain prognosis and can be modified as treatment progresses.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2c.4

Return to Question 464 ↑

Q465 INTERMEDIATE · Block F

Best answer: E. Beyond Level 2 (specialist)

Justification: Precision attachments on crowned abutments are usually specialist care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.8 · LO 2b.10

Return to Question 465 ↑

Q466 INTERMEDIATE · Block F

Best answer: A. Adjust only the marked area and re-check

Justification: Disclosing medium identifies the interference; relieve only where it marks. Remake only if no interference explains the misfit.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.9 · LO 2c.20

Return to Question 466 ↑

Q467 INTERMEDIATE · Block F

Best answer: B. Adjust the high rest

Justification: A high rest overloads the abutment and causes tenderness.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 companion note · LO 2c.20

Return to Question 467 ↑

Q468 INTERMEDIATE · Block F

Best answer: D. Review clasp flexibility and location of the retentive tip

Justification: A too-rigid clasp with an excessive undercut can transmit destructive torque as the saddle rotates.

Why not the closest alternatives: Increasing retention without reviewing arm length, undercut and rest design is unsafe.

Revise: Volume I · Volume I Unit 6 · Diploma LO 2c.2–2c.6; 2c.20

Return to Question 468 ↑

Q469 INTERMEDIATE · Block F

Best answer: D. The thickness and position of the anterior strap

Justification: Bulk over the anterior palate disturbs tongue contact for "s"; thin or reposition it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 §6.7 · LO 2c.20

Return to Question 469 ↑

Q470 INTERMEDIATE · Block F

Best answer: D. Maintain necessary tooth support but minimise unnecessary soft-tissue coverage and plan hygiene review

Justification: Support through appropriate rests should be preserved, while plaque-retentive unnecessary coverage and poor cleansability should be addressed.

Why not the closest alternatives: Eliminating rests shifts load onto soft tissues and is not a biologically sound shortcut.

Revise: Unit 6 §6.8 · LO 2c.3

Return to Question 470 ↑

BLOCK G · Complete dentures

Q473 INTERMEDIATE · Block G

Best answer: D. Use a functional neutral-zone impression to reorganise the arch position and polished contours within the muscle envelope

Justification: With advanced ridge resorption, muscle balance and polished-surface contours become central to stability. A neutral-zone technique identifies the space where tongue and cheek/lip forces are balanced during function.

Why not the closest alternatives: Increasing OVD may create other problems and will not correct an obviously misplaced tooth arrangement; simple flange shortening does not address the polished surface.

Revise: Volume I · Unit 7 · Diploma LO 2c.13; 2c.19–2c.23; source themes: 2017 Dubai MPros; MRD Past questions

Return to Question 473 ↑

Q474 INTERMEDIATE · Block G

Best answer: B. Triangulate reproducible jaw records, facial appearance, phonetics, interocclusal space and functional tolerance at trial

Justification: Physiological rest position and interocclusal space are variable, particularly with neuromuscular or adaptation difficulties. A defensible OVD is triangulated from repeatable clinical records, appearance, phonetics and functional testing.

Why not the closest alternatives: Freeway space is supportive information rather than an invariant value; no single anthropometric line reliably determines OVD.

Revise: Volume I · Unit 7 · Diploma LO 2a.20; 2c.13; 2c.19; source themes: 2017 Dubai MPros; MRD Past questions

Return to Question 474 ↑

Q475 INTERMEDIATE · Block G

Best answer: A. Assess anterior tooth position and palatal contour during sibilants before making irreversible OVD changes

Justification: Sibilant sounds depend on a narrow speaking space and appropriate anterior/palatal contours. Palatal thickness and incisor placement may disrupt airflow even if the rest position and OVD are reasonable.

Why not the closest alternatives: OVD can contribute to phonetic problems, but changing it without checking tooth position and palatal form risks an unnecessary global change.

Revise: Volume I · Unit 7 · Diploma LO 2a.20; 2c.13; 2c.19; source themes: 2019 Dubai MPros; user-specified clinical themes

Return to Question 475 ↑

Q477 INTERMEDIATE · Block G

Best answer: E. It supplies an anatomical cranial reference for orientation; occlusal-plane position still requires intraoral, facial and functional assessment

Justification: Frankfort horizontal is an anatomical reference approximating head orientation. Facebow transfer helps relate the maxillary arch to an articulator, but tooth display, lip support and the occlusal plane must be clinically assessed.

Why not the closest alternatives: Equating a cranial reference plane to the desired occlusal plane confuses orientation with restorative design.

Revise: Volume I · Unit 7 · Diploma LO 2c.13; 2a.20; source themes: 2020 Edinburgh MPros; user-specified clinical themes

Return to Question 477 ↑

Q478 INTERMEDIATE · Block G

Best answer: B. Assess the hamular notches, soft-palate mobility and posterior palatal seal before modifying the border

Justification: The hamular notch and vibrating-line region help define the posterior limit and seal. An anteriorly positioned posterior border may allow air ingress and loss of maxillary retention.

Why not the closest alternatives: Changes to occlusal guidance do not correct a deficient posterior peripheral seal and may worsen stability.

Revise: Volume I · Unit 7 · Diploma LO 1.1; 2c.13; 2c.21; source themes: 2015 MRD; 2020 Edinburgh MPros

Return to Question 478 ↑

Q479 INTERMEDIATE · Block G

Best answer: C. Optimise broad stress distribution over suitable support areas such as the buccal shelf, with selective relief of painful prominences and functional flange shaping

Justification: The buccal shelf often offers favourable mandibular denture support. In atrophic cases, tissue relief and maximising stable support within functional borders are preferable to concentrating pressure on sharp or fragile ridge crests.

Why not the closest alternatives: Flange shortening can reduce support and stability; occlusal and polished-surface factors remain essential.

Revise: Volume I · Unit 7 · Diploma LO 1.1; 2c.13; 2c.21; source themes: 2014 Dubai MRD; MRD Past questions

Return to Question 479 ↑

Q481 INTERMEDIATE · Block G

Best answer: A. Replicate useful established tooth position and polished-surface features while selectively correcting occlusal wear, fitting surface and border defects

Justification: Copy/replica techniques can conserve features to which a patient has adapted while allowing planned correction of deficits. A patient with impaired neuromuscular adaptation may benefit from preserving familiar geometry rather than changing all variables.

Why not the closest alternatives: An exact duplicate of every old defect would preserve the causes of instability; copy techniques involve selective change, not blind replication.

Revise: Volume I · Unit 7 · Diploma LO 2c.13; 2c.21–2c.27; source themes: 2017 Dubai MPros; 2015 MRD

Return to Question 481 ↑

Q482 INTERMEDIATE · Block G

Best answer: E. Evaluate unstable record bases prior to jaw registration

Justification: Stable bases are prerequisite to reproducible jaw relations.

Why not the closest alternatives: A sophisticated articulator cannot fix unstable clinical record bases.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.21–2c.27

Return to Question 482 ↑

Q483 INTERMEDIATE · Block G

Best answer: B. Review occlusal load and tissue support before adding soft liner

Justification: Support distribution and occlusal loading determine tissue stress and should be corrected at source.

Why not the closest alternatives: Soft liner placement alone does not resolve excessive or poorly distributed loading.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 483 ↑

Q484 INTERMEDIATE · Block G

Best answer: A. Anterior tooth arrangement, closest-speaking space and palatal contour during repeated phonetic testing

Justification: Sibilant production depends on anterior tooth position, the closest-speaking space and palatal shape. Testing those relationships at trial can isolate the error without unnecessary global OVD changes.

Why not the closest alternatives: Freeway space is a supportive guide, not a sole determinant of phonetic function; over-thick palatal contours can themselves distort speech.

Revise: Volume I, Unit 7 · Diploma LO 2c.13; 2a.20 · Reference themes: 2017 Dubai MPros; 2020 Edinburgh MPros

Return to Question 484 ↑

Q486 INTERMEDIATE · Block G

Best answer: D. Parallel to the interpupillary line anteriorly and to the ala–tragal line (Camper's plane) posteriorly

Justification: These references orient the occlusal plane.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.3 (exam traps) · LO 2c.9

Return to Question 486 ↑

Q488 INTERMEDIATE · Block G

Best answer: A. Use a copy technique to preserve accepted contours for a poor adaptor

Justification: A copy/replica technique preserves accepted features while allowing targeted changes.

Why not the closest alternatives: A radically redesigned prosthesis is often difficult to adapt to in this situation.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 488 ↑

Q489 INTERMEDIATE · Block G

Best answer: E. Salivary deficit that reduces the interfacial film

Justification: An inadequate or viscous salivary film impairs adhesion/cohesion and surface tension.

Why not the closest alternatives: Improving processing strength does not restore the missing fluid film.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 489 ↑

Q492 INTERMEDIATE · Block G

Best answer: D. Rest position varies with posture, fatigue and emotion

Justification: Head posture, fatigue and emotion all change it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.9

Return to Question 492 ↑

Q493 INTERMEDIATE · Block G

Best answer: C. OVD too high (freeway space too small)

Justification: Generalised soreness, clicking and strained lips are classic signs of too little freeway space.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 (exam traps) · LO 2c.9, 2c.21

Return to Question 493 ↑

Q494 INTERMEDIATE · Block G

Best answer: E. OVD too low (freeway space too large)

Justification: Overclosure causes commissural folds (angular cheilitis), a collapsed look and a prominent chin.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.9

Return to Question 494 ↑

Q495 INTERMEDIATE · Block G

Best answer: B. Muscle fatigue, soreness and faster ridge resorption

Justification: Constant loading without rest accelerates resorption and fatigues the muscles.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.9

Return to Question 495 ↑

Q497 INTERMEDIATE · Block G

Best answer: E. Adjust an overextended border using pressure-indicating and functional movements

Justification: Identify the interfering border during movement and adjust locally, followed by review for healing.

Why not the closest alternatives: A soft liner over an uncorrected impinging flange often perpetuates the cause.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 497 ↑

Q498 INTERMEDIATE · Block G

Best answer: D. At the apex of the arrowhead

Justification: It marks the most retruded, reproducible position.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.5 · LO 2c.19

Return to Question 498 ↑

Q501 INTERMEDIATE · Block G

Best answer: B. Deficient border seal despite adequate adaptation

Justification: A deficient functional border seal can compromise retention even when the fitting surface adapts well.

Why not the closest alternatives: A reline without reassessing border extension may not solve the problem.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 501 ↑

Q502 INTERMEDIATE · Block G

Best answer: C. L = H/8 + 12 (Hanau)

Justification: Hanau's formula estimates the Bennett angle (L) from the horizontal condylar guidance (H).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 502 ↑

Q503 INTERMEDIATE · Block G

Best answer: E. Local traumatic denture instability from a deflective occlusal contact

Justification: A deflective contact can destabilise an otherwise well-fitting base; examine static and dynamic contacts.

Why not the closest alternatives: This is not necessarily a problem of too little retention.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.19; 2c.21

Return to Question 503 ↑

Q505 INTERMEDIATE · Block G

Best answer: C. Reassess posterior palatal seal and soft-palate movement

Justification: Posterior palatal seal position and soft-palate mobility are central to posterior retention under function.

Why not the closest alternatives: Relining the entire fitting surface cannot specifically correct the posterior border seal without assessment.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.9; 2c.21

Return to Question 505 ↑

Q508 INTERMEDIATE · Block G

Best answer: A. A reduced OVD with loss of facial support and commissural folding

Justification: Worn occlusion and support may contribute to a reduced OVD; confirm with multiple clinical assessments.

Why not the closest alternatives: An increase in OVD may be considered only after examination and assessment of adaptability.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.19; 2c.21

Return to Question 508 ↑

Q509 INTERMEDIATE · Block G

Best answer: D. Monoplane (zero-degree) teeth

Justification: Zero-degree teeth generate the least horizontal force, which suits severe resorption, Class III and poor control.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.6 · LO 2c.19

Return to Question 509 ↑

Q510 INTERMEDIATE · Block G

Best answer: E. About one-sixteenth of the bizygomatic width

Justification: Berry's proportion is one guide, alongside the canine lines and inter-alar width.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.7 · LO 2c.9

Return to Question 510 ↑

Q511 INTERMEDIATE · Block G

Best answer: A. Review polished-surface contours and the neutral zone

Justification: The neutral zone/polished-surface contour must accommodate tongue and cheek forces rather than compete with them.

Why not the closest alternatives: Clasp tightening is inapplicable, and occlusal changes are not the first response to contour-driven instability.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.9; 2c.21

Return to Question 511 ↑

Q512 INTERMEDIATE · Block G

Best answer: B. Involve her in the choice, usually favouring a darker, characterised shade that suits her age

Justification: Very light teeth create a "denture look"; shared decision-making avoids later disputes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.7 · LO 2c.9, 3b.1

Return to Question 512 ↑

Q514 INTERMEDIATE · Block G

Best answer: E. In crossbite, with the upper teeth possibly buccal to the resorbed ridge

Justification: Centripetal maxillary resorption makes the upper arch narrower; setting in crossbite keeps teeth over the ridges.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.7, Unit 2 · LO 2c.9

Return to Question 514 ↑

Q515 INTERMEDIATE · Block G

Best answer: D. Record the neutral zone and reshape tooth position and polished surfaces to the patient’s muscular balance

Justification: For a very resorbed ridge, the balance of tongue, cheek and lip forces becomes especially important. A functional neutral-zone record guides both tooth position and polished-surface contours.

Why not the closest alternatives: Placement strictly over the ridge crest is not universally stable in compromised anatomy; overcontoured surfaces may worsen muscle displacement.

Revise: Volume I, Unit 7 · Diploma LO 2a.20; 2c.13; 2c.21 · Reference themes: 2019 Dubai MPros; 2015 MRD

Return to Question 515 ↑

Q516 INTERMEDIATE · Block G

Best answer: C. Poor tissue adaptation requiring reline or remake assessment

Justification: Loss of intaglio adaptation after ridge resorption reduces available support and retention.

Why not the closest alternatives: An overextended flange would more characteristically dislodge during function.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 516 ↑

Q517 INTERMEDIATE · Block G

Best answer: D. Re-record the jaw relationship before extensive grinding

Justification: Repeat the horizontal/vertical records before indiscriminate grinding of all occlusal surfaces.

Why not the closest alternatives: Grinding cannot reliably correct an erroneous original relation.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.21–2c.27

Return to Question 517 ↑

Q518 INTERMEDIATE · Block G

Best answer: B. Selective-pressure/window impression for a displaceable anterior ridge

Justification: A window or selective-pressure approach can record the mobile tissue with less displacement.

Why not the closest alternatives: A uniformly heavy mucocompressive impression may record the tissue in a displaced position.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 518 ↑

Q519 INTERMEDIATE · Block G

Best answer: B. The need for a cautious reversible trial due to limited adaptive capacity

Justification: Where adaptation is limited, test major occlusal changes conservatively and reversibly, such as by provisional/replica approaches.

Why not the closest alternatives: An immediate irreversible substantial change risks poor tolerance.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.19; 2c.21

Return to Question 519 ↑

Q520 INTERMEDIATE · Block G

Best answer: D. Posterior: by asking the patient to say "ah"

Justification: Saying "ah" shows the posterior vibrating line; the Valsalva manoeuvre shows the anterior one.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.8 · LO 2c.9

Return to Question 520 ↑

Q522 INTERMEDIATE · Block G

Best answer: D. An erroneous horizontal jaw relation despite acceptable vertical dimension

Justification: An incorrect horizontal record may produce instability despite an acceptable vertical dimension.

Why not the closest alternatives: Alteration of OVD without first checking horizontal relation can compound the problem.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.19; 2c.21

Return to Question 522 ↑

Q523 INTERMEDIATE · Block G

Best answer: E. Excessive flange length displaced by a muscle

Justification: Functional muscle movement dislodges the overextended flange.

Why not the closest alternatives: The movement-specific displacement is more diagnostic than global loss of saliva.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 523 ↑

Q524 INTERMEDIATE · Block G

Best answer: B. An over-extended distobuccal flange (coronoid process)

Justification: The coronoid process moves forward on opening and pushes the flange down.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9 (exam traps) · Schematic 16 · LO 2c.21

Return to Question 524 ↑

Q525 INTERMEDIATE · Block G

Best answer: C. An inadequate post-dam or peripheral seal

Justification: Looseness at rest means air enters at the border; under-extension or poor fit are other causes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9 · LO 2c.21

Return to Question 525 ↑

Q526 INTERMEDIATE · Block G

Best answer: E. Occlusal interference that destabilises the base during contact

Justification: A deflective contact can lever a well-fitting base off its supporting tissues.

Why not the closest alternatives: The timing with tooth contact makes this more likely than a passive border-seal problem.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 526 ↑

Q527 INTERMEDIATE · Block G

Best answer: C. Excessive OVD, an RCP error, parafunction, inadequate support, or systemic factors such as anaemia or diabetes

Justification: Generalised soreness points to overload or systemic susceptibility rather than a local fault.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9 · LO 2c.21

Return to Question 527 ↑

Q528 INTERMEDIATE · Block G

Best answer: B. Trial a reversible OVD change where adaptation is uncertain

Justification: A reversible trial can test adaptation before irreversible definitive change.

Why not the closest alternatives: A large untested change risks adaptation difficulties.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.21–2c.27

Return to Question 528 ↑

Q529 INTERMEDIATE · Block G

Best answer: A. An occlusal interference, a pressure spot or a sharp underlying bony crest

Justification: Crestal ulcers usually come from occlusal or fitting-surface pressure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9 · LO 2c.21

Return to Question 529 ↑

Q530 INTERMEDIATE · Block G

Best answer: D. The denture pressing on the mental nerve

Justification: In a resorbed mandible the foramen lies near the crest; relieve the fitting surface over it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9, Unit 1 · LO 2c.21

Return to Question 530 ↑

Q531 INTERMEDIATE · Block G

Best answer: A. Lack of posterior buccal overjet

Justification: Edge-to-edge posteriors and low OVD trap the cheek.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9 · LO 2c.21

Return to Question 531 ↑

Q532 INTERMEDIATE · Block G

Best answer: B. Functional border moulding to capture muscular limits

Justification: Functional border moulding helps define extensions compatible with muscle movement.

Why not the closest alternatives: Occlusal adjustment before diagnosing extension may be misdirected.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.21–2c.27

Return to Question 532 ↑

Q533 INTERMEDIATE · Block G

Best answer: C. Correct the posterior border, and use a training base, desensitisation or palateless (implant) designs if needed

Justification: Treat the technical cause first; training bases, desensitisation and palateless implant designs help persistent gaggers.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9 · LO 2c.21

Return to Question 533 ↑

Q534 INTERMEDIATE · Block G

Best answer: A. Correct an early contact after remount verification

Justification: A verified occlusal interference can be adjusted conservatively after remount assessment.

Why not the closest alternatives: Unrelated border shortening risks compromising suction.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.21–2c.27

Return to Question 534 ↑

Q535 INTERMEDIATE · Block G

Best answer: B. Denture stomatitis, xerostomia, haematinic deficiency or diabetes, and very rarely monomer allergy

Justification: Investigate local and systemic causes before assuming allergy or a psychological cause.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9, Unit 2 · LO 2c.21

Return to Question 535 ↑

Q536 INTERMEDIATE · Block G

Best answer: D. Correct a peripheral extension interference first

Justification: Correct the demonstrated border interference before occlusal equilibration.

Why not the closest alternatives: Occlusal adjustment cannot address dislodgement that occurs before contact.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.21–2c.27

Return to Question 536 ↑

Q537 INTERMEDIATE · Block G

Best answer: B. Fatigue from flexure over a midline fulcrum, worsened by opposing natural teeth

Justification: Flexure over a torus or a poor fit, plus natural opposing teeth, causes midline fatigue fracture.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.9, Unit 3 · LO 2c.22

Return to Question 537 ↑

Q538 INTERMEDIATE · Block G

Best answer: B. Adjust the masseteric notch/distobuccal flange

Justification: Functional masseteric action can displace an excessively extended distobuccal flange; use functional assessment to adjust the notch.

Why not the closest alternatives: Arbitrary occlusal grinding before border evaluation misses the reproducible muscle trigger.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.9; 2c.21

Return to Question 538 ↑

Q539 INTERMEDIATE · Block G

Best answer: E. Copy (replica) dentures, changing only what is necessary

Justification: They keep the polished and occlusal surfaces the patient has learned, changing only what is needed.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.9

Return to Question 539 ↑

Q540 INTERMEDIATE · Block G

Best answer: C. Poor retention, and it cannot be relined labially

Justification: Socketed immediates look natural at first but lose the labial seal as the ridge resorbs.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.4

Return to Question 540 ↑

Q542 INTERMEDIATE · Block G

Best answer: D. Root-filled and reduced to 2–3 mm above the gingiva with domed surfaces

Justification: This reduces leverage while preserving bone and proprioception; canines are ideal abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.4

Return to Question 542 ↑

Q544 INTERMEDIATE · Block G

Best answer: A. A two-implant mandibular overdenture as the minimum standard of care

Justification: This is the minimum standard of care for the edentulous mandible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 (exam traps), Unit 11 · LO 2c.5

Return to Question 544 ↑

Q545 INTERMEDIATE · Block G

Best answer: C. Occlusal load imbalance and characteristic combination-syndrome changes

Justification: The described opposing-arch arrangement can produce a characteristic combination-syndrome pattern of load-related anterior maxillary resorption and other changes.

Why not the closest alternatives: A post-dam fault alone does not explain the group of anterior ridge and opposing-arch changes.

Revise: Unit 7 §7.10 · LO 2c.9

Return to Question 545 ↑

Q546 INTERMEDIATE · Block G

Best answer: E. Restoring posterior support (a well-fitting lower RPD) and reducing anterior loading

Justification: Unsupported posterior lower segments throw load onto the premaxilla.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.9

Return to Question 546 ↑

Q547 INTERMEDIATE · Block G

Best answer: E. Record the firm supporting tissues appropriately while using a window or selective relief to record the flabby tissue minimally displaced

Justification: Different support and mobility across the ridge favour a selective-pressure or window impression. Displacing flabby tissue during recording creates a surface that does not match the resting mucosal contour.

Why not the closest alternatives: Mucocompressive recording is not automatically appropriate for a displaceable anterior ridge, and omitting the area prevents controlled fitting.

Revise: Volume I, Unit 7 · Diploma LO 2c.13; 2c.21 · Reference themes: 2019 Edinburgh MPros; 2015 MRD

Return to Question 547 ↑

Q549 INTERMEDIATE · Block G

Best answer: C. The patient bit too firmly during the reline impression, increasing the OVD

Justification: Closed-mouth relines raise the OVD if the patient bites into the material.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.23

Return to Question 549 ↑

Q550 INTERMEDIATE · Block G

Best answer: B. Level 3

Justification: Tuberosity reduction, vestibuloplasty and torus removal are usually specialist care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2b.10

Return to Question 550 ↑

Q551 INTERMEDIATE · Block G

Best answer: A. Correct overextension along the mylohyoid region

Justification: Excessive extension into the functional mylohyoid region dislodges the denture during floor-of-mouth activity.

Why not the closest alternatives: Increasing adhesive or OVD does not remove the mechanical border interference.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.9; 2c.21

Return to Question 551 ↑

Q552 INTERMEDIATE · Block G

Best answer: C. Adhesive and OVD changes rarely cure an over-extended flange; correct the border

Justification: Adhesive or OVD changes rarely cure an over-extended flange.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 companion note (SBA traps) · LO 2c.21

Return to Question 552 ↑

Q555 INTERMEDIATE · Block G

Best answer: A. Cutting the post-dam on the master cast along the marked vibrating line

Justification: Without it, polymerisation contraction lifts the posterior border and breaks the seal.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.8 · LO 2c.9

Return to Question 555 ↑

Q557 INTERMEDIATE · Block G

Best answer: E. Reassess occlusal contacts and jaw relation after verifying border fit

Justification: When borders and support are satisfactory, occlusal disharmony or an erroneous jaw relation can drive tipping.

Why not the closest alternatives: Aggressive border relief would sacrifice retention and may not address the demonstrated premature contact.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.4; 2c.9; 2c.21

Return to Question 557 ↑

Q558 INTERMEDIATE · Block G

Best answer: C. Copy dentures with minimal changes

Justification: Dementia limits adaptation; preserve the familiar shapes.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10, Unit 4 · LO 2c.9

Return to Question 558 ↑

Q559 INTERMEDIATE · Block G

Best answer: B. A metal (Co-Cr) or reinforced palate, with balanced contacts

Justification: Natural opposing teeth overload a single denture and cause fatigue fracture.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.9

Return to Question 559 ↑

Q560 INTERMEDIATE · Block G

Best answer: E. Undercuts, implants and adhesives

Justification: These are mechanical, as opposed to physical or muscular, retention.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.1 · LO 2c.9

Return to Question 560 ↑

Q561 INTERMEDIATE · Block G

Best answer: B. OVD too high

Justification: Lips that cannot meet at rest look strained.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.21

Return to Question 561 ↑

Q562 INTERMEDIATE · Block G

Best answer: E. Re-establish the OVD with a new jaw record and reset or remake the dentures

Justification: Excessive OVD can only be cured by re-establishing the correct vertical relation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.4 · LO 2c.21

Return to Question 562 ↑

Q563 INTERMEDIATE · Block G

Best answer: D. It acts as a splint and limits swelling

Justification: Once swelling develops, reinsertion may be impossible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.4

Return to Question 563 ↑

Q564 INTERMEDIATE · Block G

Best answer: C. Assessment and referral for a two-implant overdenture

Justification: It is the minimum standard for the failing edentulous mandible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.10 · LO 2c.5

Return to Question 564 ↑

Q566 INTERMEDIATE · Block G

Best answer: A. An excessively increased OVD producing muscular symptoms

Justification: The cluster of reduced freeway space, speech clicking and muscular strain suggests excessive OVD.

Why not the closest alternatives: A reduced OVD typically has different facial and commissural signs.

Revise: Volume I · Volume I Unit 7 · Diploma LO 2c.9; 2c.19; 2c.21

Return to Question 566 ↑

Q567 INTERMEDIATE · Block G

Best answer: B. The canine lines and inter-alar width

Justification: They estimate the width of the six upper anterior teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 §7.7 · LO 2c.9

Return to Question 567 ↑

Q570 INTERMEDIATE · Block G

Best answer: A. Copy tooth display but reassess posterior extension, seal, fit and jaw relation

Justification: Accepted aesthetics may be retained while functional failures—particularly borders and seal—must be reassessed before a replacement.

Why not the closest alternatives: An exact duplication of the ineffective posterior seal would repeat the problem.

Revise: Unit 7, Unit 13 · LO 2c.9, 3a.3

Return to Question 570 ↑

BLOCK H · Fixed prosthodontics

Q571 INTERMEDIATE · Block H

Best answer: D. Use a shoulder/deep chamfer where porcelain needs bulk and a more conservative metal-compatible palatal chamfer

Justification: Porcelain needs sufficient supported thickness and a clear finish line, commonly a rounded shoulder or deep chamfer. A metal-only margin may permit a more conservative chamfer. Margin depth and periodontal health matter as well as shape.

Why not the closest alternatives: A feather or knife-edge facial finish line may leave inadequate porcelain bulk or create overcontour; indiscriminate deep preparation sacrifices tooth structure.

Revise: Volume I · Unit 8 · Diploma LO 1.9; 2c.11; 2c.14; source themes: 2020 Edinburgh MPros; 2019 Dubai MPros

Return to Question 571 ↑

Q572 INTERMEDIATE · Block H

Best answer: A. It can be considered selectively for a metal restoration when conservation is critical, but risks difficult margin identification and overcontour if poorly controlled

Justification: Knife-edge or feather margins may conserve cervical tooth tissue for selected metal restorations but offer less distinct laboratory finish lines and can encourage overcontour. They are not generally a universal preparation for brittle ceramic margins.

Why not the closest alternatives: For all-ceramic designs, preparation must be guided by the manufacturer’s requirements and appropriate material thickness rather than universal use of feather edges.

Revise: Volume I · Unit 8 · Diploma LO 1.9; 2c.11; 2c.14; source themes: 2020 Edinburgh MPros; user-specified clinical themes

Return to Question 572 ↑

Q573 INTERMEDIATE · Block H

Best answer: E. Preserving adequate axial wall height and controlled taper, with auxiliary resistance features when appropriate

Justification: Resistance form concerns prevention of rotation/dislodgement under non-axial forces. Short walls and excessive taper reduce resistance; adequate geometry and auxiliary grooves can improve stability.

Why not the closest alternatives: Cement selection cannot compensate reliably for a fundamentally unfavourable preparation design.

Revise: Volume I · Unit 8 · Diploma LO 2c.11; 2c.14; source themes: 2019 Edinburgh MPros; 2020 Edinburgh MPros

Return to Question 573 ↑

Q575 INTERMEDIATE · Block H

Best answer: B. Assess suitability for a single-retainer cantilever RBB, including adhesive substrate and dynamic occlusion

Justification: A fixed–fixed RBB may debond on one wing because abutments move independently, potentially leaving the bridge partially retained and risking caries. A suitable single-retainer cantilever reduces differential-movement loading.

Why not the closest alternatives: More wings do not necessarily solve differential physiological tooth movement. The abutment, bonding substrate and occlusion still need assessment.

Revise: Volume I · Unit 8 · Diploma LO 2c.2; 2c.11; 2c.14; source themes: 2019 Edinburgh MPros; MRD Past questions

Return to Question 575 ↑

Q576 INTERMEDIATE · Block H

Best answer: E. Consider an adhesive resin-bonded single-retainer bridge in favourable enamel/occlusion

Justification: An appropriately designed adhesive bridge may preserve sound abutments if isolation, enamel and occlusion are favourable.

Why not the closest alternatives: A conventional bridge would require more preparation of healthy teeth.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2b.1–2b.12; 2c.11

Return to Question 576 ↑

Q577 INTERMEDIATE · Block H

Best answer: A. Inadequate material-specific substrate conditioning

Justification: Zirconia requires material-specific bonding such as appropriate abrasion/MDP rather than HF glass-ceramic protocol alone.

Why not the closest alternatives: The weak point is the material-specific interface, not necessarily tooth preparation.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 577 ↑

Q578 INTERMEDIATE · Block H

Best answer: E. Enamel-supported conservative single-retainer design

Justification: A single-retainer adhesive bridge may replace the tooth without circumferential reduction of abutments.

Why not the closest alternatives: A conventional bridge would sacrifice more sound tissue.

Revise: Volume I · Volume I Units 5 and 8 · Diploma LO 2a.9; 2a.18; 2c.11

Return to Question 578 ↑

Q579 INTERMEDIATE · Block H

Best answer: E. Correct unsupported veneering porcelain/design before remaking a metal–ceramic crown

Justification: Porcelain requires adequate framework support and compatible design to limit tensile fracture.

Why not the closest alternatives: Simply changing cement or polishing chips does not correct unsupported veneer geometry.

Revise: Volume I · Volume I Unit 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 579 ↑

Q580 INTERMEDIATE · Block H

Best answer: B. Choose adhesive glass ceramic where enamel bonding and aesthetics dominate

Justification: Bonded enamel-supported glass ceramic can be appropriate when aesthetics and conservative preparations align.

Why not the closest alternatives: A full-coverage option sacrifices additional healthy enamel without clear benefit.

Revise: Volume I · Volume I Unit 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 580 ↑

Q581 INTERMEDIATE · Block H

Best answer: B. Reassess occlusal reduction and restorative space before material change

Justification: Assess restorative space, reduction and occlusion against material-specific thickness requirements before fabrication.

Why not the closest alternatives: Changing shade or cement cannot overcome insufficient structural thickness.

Revise: Volume I · Volume I Unit 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 581 ↑

Q582 INTERMEDIATE · Block H

Best answer: C. High functional tensile stress acting on poorly supported veneer promotes crack initiation and propagation

Justification: Brittle porcelain is susceptible to tensile stress and unsupported span geometry. Heavy guidance loads and inadequate framework support create a plausible mechanical pathway for repeated veneer fracture.

Why not the closest alternatives: Mere recementation or polishing does not correct load distribution or supporting framework design.

Revise: Volume I · Unit 8 · Diploma LO 1.9; 2c.11; 2c.14; source themes: 2020 Edinburgh MPros; 2019 Dubai MPros

Return to Question 582 ↑

Q583 INTERMEDIATE · Block H

Best answer: C. Assess whether the restoration impinges on the supracrestal tissue attachment and whether a sound cleansable margin can be established with appropriate periodontal/restorative planning

Justification: Encroachment on the supracrestal tissue attachment can contribute to persistent inflammation and tissue breakdown. The restoration must be biologically compatible, cleansable and maintainable, potentially requiring multidisciplinary planning.

Why not the closest alternatives: A more apical margin may worsen rather than relieve tissue impingement; a change in cement alone does not solve an inaccessible or biologically inappropriate margin.

Revise: Volume I · Unit 8 · Diploma LO 2a.5; 2c.11; 2c.14; source themes: 2020 Edinburgh MPros; 2019 Edinburgh MPerio

Return to Question 583 ↑

Q584 INTERMEDIATE · Block H

Best answer: C. Invasion of the supracrestal tissue attachment, causing chronic inflammation, recession or bone loss

Justification: It causes chronic inflammation, recession or bone loss.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.2 · LO 2c.13

Return to Question 584 ↑

Q585 INTERMEDIATE · Block H

Best answer: C. An adequately supported facial shoulder or deep chamfer and a more conservative palatal chamfer for metal

Justification: Ceramic coverage requires sufficient supported bulk and a clear finish line, often a shoulder or deep chamfer. Metal-only surfaces permit more conservative margin forms. Supragingival placement is desirable where clinically appropriate.

Why not the closest alternatives: Feather edges can promote thin unsupported ceramic and overcontour, while deep shoulders all around remove more tooth than necessary for metal-only surfaces.

Revise: Volume I, Unit 8 · Diploma LO 1.9; 2c.11; 2c.14 · Reference themes: 2020 Edinburgh MPros; 2019 Dubai MPros

Return to Question 585 ↑

Q586 INTERMEDIATE · Block H

Best answer: E. Adrenaline-impregnated cord

Justification: Systemic absorption can raise blood pressure; use aluminium chloride, ferric sulphate or plain cord.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.2 · LO 2c.13

Return to Question 586 ↑

Q588 INTERMEDIATE · Block H

Best answer: C. Reassess connector design where porcelain/metal fatigue occurs

Justification: Connector dimensions, support, alloy/ceramic compatibility and occlusal load are relevant to repeated framework-adjacent fracture.

Why not the closest alternatives: Simply re-cementing a stable bridge does not address the structural failure.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2b.1–2b.12; 2c.11

Return to Question 588 ↑

Q589 INTERMEDIATE · Block H

Best answer: A. A eugenol-free temporary cement

Justification: Eugenol inhibits polymerisation of the later resin cement.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.3 · LO 2c.13

Return to Question 589 ↑

Q590 INTERMEDIATE · Block H

Best answer: C. Conformative: restore to the existing ICP

Justification: Single units and short bridges are restored to the existing ICP.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.4 · LO 2c.19

Return to Question 590 ↑

Q591 INTERMEDIATE · Block H

Best answer: C. Reorganised (RCP), usually Level 3 when major

Justification: An unstable ICP, extensive work or an OVD change needs a reorganised approach in RCP.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.4 · LO 2c.19, 2b.10

Return to Question 591 ↑

Q592 INTERMEDIATE · Block H

Best answer: C. Discuss extraction/replacement when a deep fracture makes predictable restoration untenable

Justification: Predictability is poor with a deep vertical fracture; discuss prognosis, extraction and replacement choices rather than promising definitive crown treatment.

Why not the closest alternatives: A longer post or more aggressive preparation cannot predictably restore a vertically split root.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2c.11; 2c.14

Return to Question 592 ↑

Q593 INTERMEDIATE · Block H

Best answer: C. Avoid the design when occlusal clearance cannot be provided safely

Justification: Adhesive bridge success depends on suitable clearance and loading; do not force an unsuitable design.

Why not the closest alternatives: A thinner-than-recommended metal wing may flex and fail.

Revise: Volume I · Volume I Units 5 and 8 · Diploma LO 2a.9; 2a.18; 2c.11

Return to Question 593 ↑

Q594 INTERMEDIATE · Block H

Best answer: A. Dual-arch trays suit single units with stable intercuspation only

Justification: Dual-arch trays flex and cannot record longer spans accurately.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.4 · LO 2c.13

Return to Question 594 ↑

Q595 INTERMEDIATE · Block H

Best answer: D. About 20–40 µm, stopping short of the margin

Justification: It gives room for cement without opening the margin.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.4 · LO 2c.13

Return to Question 595 ↑

Q596 INTERMEDIATE · Block H

Best answer: B. Cuspal coverage (an onlay or crown)

Justification: Sorensen and Martinoff showed much better survival for root-filled posterior teeth with cusp coverage.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.5 · LO 2c.14

Return to Question 596 ↑

Q597 INTERMEDIATE · Block H

Best answer: A. A composite restoration of the access cavity

Justification: Anterior root-filled teeth with little tissue loss need no crown or post.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.5 · LO 2c.14

Return to Question 597 ↑

Q598 INTERMEDIATE · Block H

Best answer: C. A Nayyar core (amalgam or composite retained in the pulp chamber)

Justification: The pulp chamber retains an amalgam or composite core without a post.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.5 · LO 2c.14

Return to Question 598 ↑

Q600 INTERMEDIATE · Block H

Best answer: D. Refer for coordinated implant/orthodontic planning where space and roots are unsuitable

Justification: Coordinated orthodontic and implant assessment may be needed to optimise space and root divergence.

Why not the closest alternatives: Implant selection purely from crown width risks iatrogenic injury.

Revise: Volume I · Volume I Units 5 and 8 · Diploma LO 2a.9; 2a.18; 2c.11

Return to Question 600 ↑

Q602 INTERMEDIATE · Block H

Best answer: C. Evaluate abutment periodontal prognosis and overload before choosing a long-span fixed bridge

Justification: Abutment prognosis and load distribution must be established before committing to a long fixed span.

Why not the closest alternatives: Span length and cement choice cannot compensate for inadequate periodontal prognosis.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2b.1–2b.12; 2c.11

Return to Question 602 ↑

Q605 INTERMEDIATE · Block H

Best answer: A. Canine

Justification: The upper canine has the largest root of the single-rooted teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 605 ↑

Q606 INTERMEDIATE · Block H

Best answer: E. Defer definitive bridge while active caries risk is controlled

Justification: Disease risk and abutment stability must be controlled before elective bridgework.

Why not the closest alternatives: Immediate bonding does not eliminate future caries risk around wings.

Revise: Volume I · Volume I Units 5 and 8 · Diploma LO 2a.9; 2a.18; 2c.11

Return to Question 606 ↑

Q607 INTERMEDIATE · Block H

Best answer: E. It falls to about one-eighth

Justification: Deflection decreases with the cube of connector thickness.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 607 ↑

Q608 INTERMEDIATE · Block H

Best answer: A. The pontics lie outside the line joining the abutments and act as a lever arm

Justification: Pontics outside the line between abutments tip the bridge labially.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 608 ↑

Q610 INTERMEDIATE · Block H

Best answer: B. A cantilever from 13 replacing 12

Justification: A long-rooted canine supporting a lateral incisor pontic is a classic cantilever.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 610 ↑

Q611 INTERMEDIATE · Block H

Best answer: B. Consider orthodontic extrusion when biologically and strategically suitable

Justification: Orthodontic extrusion can move the tooth and fracture margin coronally where root length/support allow a useful ferrule.

Why not the closest alternatives: It is not automatic; prognosis and patient preferences remain decisive.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2c.11; 2c.14

Return to Question 611 ↑

Q612 INTERMEDIATE · Block H

Best answer: B. A fixed-movable bridge

Justification: A movable joint avoids preparing non-parallel abutments to a single path.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 612 ↑

Q613 INTERMEDIATE · Block H

Best answer: A. Avoid a post when adequate coronal structure and a core are already present

Justification: Posts are not routinely indicated in root-filled teeth when adequate core retention already exists.

Why not the closest alternatives: Posts can remove sound root dentine and create additional risks without improving tooth strength.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2c.11; 2c.14

Return to Question 613 ↑

Q614 INTERMEDIATE · Block H

Best answer: D. A spring cantilever

Justification: It is historic and rarely used now.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 614 ↑

Q617 INTERMEDIATE · Block H

Best answer: E. Reassess ferrule and remaining coronal dentine before post placement

Justification: A ferrule and sound remaining tooth structure influence long-term fracture resistance more than simply increasing post length.

Why not the closest alternatives: A post provides core retention; it does not inherently strengthen an otherwise unrestorable root.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2c.11; 2c.14

Return to Question 617 ↑

Q620 INTERMEDIATE · Block H

Best answer: A. About 0.7 mm

Justification: Placing the wing at a slightly raised occlusion lets the teeth re-establish contact, avoiding preparation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.8 · LO 2c.11

Return to Question 620 ↑

Q621 INTERMEDIATE · Block H

Best answer: D. Maximum enamel coverage within aesthetic limits, about 180° wraparound, a cingulum rest and minimal preparation for a path

Justification: These maximise the bond area and give resistance and a defined path.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.8 · LO 2c.11

Return to Question 621 ↑

Q624 INTERMEDIATE · Block H

Best answer: D. Consider an appropriately designed single-retainer cantilever RBB after assessing occlusion, support and available enamel

Justification: Differential abutment movement can flex a two-wing resin-bonded restoration and cause unilateral debonding. A single-retainer cantilever design may avoid this mechanism when the case is suitable.

Why not the closest alternatives: Any RBB requires appropriate occlusion, retainer surface and periodontal support; the conclusion is not that all two-wing bridges necessarily fail.

Revise: Volume I, Unit 8 · Diploma LO 2c.2; 2c.14 · Reference themes: 2019 Edinburgh MPros; 2016 MRD

Return to Question 624 ↑

Q625 INTERMEDIATE · Block H

Best answer: C. Sanitary (hygienic), with no ridge contact and at least a 3 mm gap

Justification: A gap of at least 3 mm suits lower posterior, non-aesthetic sites.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.9 · Schematic 19 · LO 2c.11

Return to Question 625 ↑

Q626 INTERMEDIATE · Block H

Best answer: E. Modified ridge lap, contacting the labial side of the ridge only

Justification: Labial contact only balances appearance and hygiene in upper anterior and premolar sites.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.9 · LO 2c.11

Return to Question 626 ↑

Q627 INTERMEDIATE · Block H

Best answer: C. Ovate, sitting in a concavity in the ridge

Justification: It gives the most natural emergence but needs a thick ridge or socket preservation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.9 · LO 2c.11

Return to Question 627 ↑

Q629 INTERMEDIATE · Block H

Best answer: E. Discuss a removable temporary replacement while prognosis is uncertain

Justification: A temporary removable solution can restore appearance while biological prognosis becomes clearer.

Why not the closest alternatives: Definitive cementation before prognosis is established can commit a poor plan.

Revise: Volume I · Volume I Units 5 and 8 · Diploma LO 2a.9; 2a.18; 2c.11

Return to Question 629 ↑

Q630 INTERMEDIATE · Block H

Best answer: D. Narrow the buccolingual width of the pontic

Justification: A narrower table reduces load on the abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.9 · LO 2c.11

Return to Question 630 ↑

Q631 INTERMEDIATE · Block H

Best answer: B. Prefer repairable direct composite when a reversible additive solution is appropriate

Justification: Direct composite provides a repairable, modifiable and conservative option when additive reshaping suffices.

Why not the closest alternatives: Definitive circumferential ceramic coverage would usually be unnecessarily invasive.

Revise: Volume I · Volume I Unit 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 631 ↑

Q632 INTERMEDIATE · Block H

Best answer: A. About 16 mm² in the premolar region

Justification: Lithium disilicate bridges are limited in length and position; anterior connectors need about 12 mm².

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.10 · Number card 3 · LO 2c.6

Return to Question 632 ↑

Q633 INTERMEDIATE · Block H

Best answer: D. About 9 mm²

Justification: That is for a three-unit posterior zirconia bridge; longer spans need more.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.10 · LO 2c.6

Return to Question 633 ↑

Q634 INTERMEDIATE · Block H

Best answer: E. Height, because strength rises with the square of height and deflection falls with its cube

Justification: Strength rises with height squared and deflection falls with height cubed.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.10 · LO 2c.6

Return to Question 634 ↑

Q635 INTERMEDIATE · Block H

Best answer: C. Discolouration unresponsive to bleaching

Justification: That is a classic indication, with diastema closure, shape change and minor malalignment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.11 · LO 2c.13

Return to Question 635 ↑

Q636 INTERMEDIATE · Block H

Best answer: C. Inadequate isolation of adhesive surfaces

Justification: Adhesive interfaces depend on adequate moisture control and isolation.

Why not the closest alternatives: Perfect ceramic conditioning does not remove the effect of contaminated substrate.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 636 ↑

Q637 INTERMEDIATE · Block H

Best answer: E. Incisal overlap with a palatal finish line

Justification: It distributes stress and gives a positive seat.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.11 · LO 2c.13

Return to Question 637 ↑

Q639 INTERMEDIATE · Block H

Best answer: C. Cusp coverage with an onlay

Justification: A cusp narrower than half its height is weak and needs protection.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.11 · LO 2c.13

Return to Question 639 ↑

Q640 INTERMEDIATE · Block H

Best answer: B. Use a high-strength monolithic material when load and connector geometry dominate

Justification: Monolithic high-strength ceramics may offer favourable fracture resistance for selected posterior loaded restorations, subject to design and polishing.

Why not the closest alternatives: Aesthetics alone should not override structural requirements.

Revise: Volume I · Volume I Unit 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 640 ↑

Q641 INTERMEDIATE · Block H

Best answer: D. Undersized ceramic thickness in a high-load area

Justification: Material thickness and functional load influence ceramic fracture; a retained restoration can fail structurally.

Why not the closest alternatives: Debonding and fracture are different modes and need different remedies.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 641 ↑

Q642 INTERMEDIATE · Block H

Best answer: B. Compromised abutment prognosis from untreated disease

Justification: Reassess biological prognosis and treat disease before repeating elective prosthetic replacement.

Why not the closest alternatives: Retentive design cannot by itself solve active endodontic disease.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 642 ↑

Q643 INTERMEDIATE · Block H

Best answer: A. The GDC

Justification: Technicians and clinical dental technicians are GDC registrants with their own scope of practice.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.12, Unit 13 · LO 3a.3

Return to Question 643 ↑

Q644 INTERMEDIATE · Block H

Best answer: D. Discuss no replacement as a valid choice when function and patient preferences permit

Justification: No replacement can be an evidence-based option when chewing, comfort and preferences are satisfactory.

Why not the closest alternatives: Replacing every missing tooth is not a universal clinical obligation.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2b.1–2b.12; 2c.11

Return to Question 644 ↑

Q646 INTERMEDIATE · Block H

Best answer: B. Investigating the cause, then non-vital (internal) bleaching

Justification: Least invasive first: whitening before any preparation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 02 · LO 2b.6

Return to Question 646 ↑

Q649 INTERMEDIATE · Block H

Best answer: E. Orthodontics, before any elective tooth preparation

Justification: Malalignment is corrected before any elective tooth preparation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 02 · LO 2b.6

Return to Question 649 ↑

Q650 INTERMEDIATE · Block H

Best answer: A. A diagnostic wax-up and mock-up, assessment of enamel, parafunction and overbite, and a discussion of irreversibility and replacement

Justification: Veneers are irreversible and will need replacing; test the result and discuss risks first.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 03 · LO 2b.6, 3b.1

Return to Question 650 ↑

Q651 INTERMEDIATE · Block H

Best answer: C. They are less conservative than whitening, orthodontics or composite

Justification: Crowning sound teeth for a minor concern is a classic trap.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 05 · LO 2b.6

Return to Question 651 ↑

Q652 INTERMEDIATE · Block H

Best answer: B. It has survived but is not a success

Justification: Success includes biological health, function and satisfaction, not just presence.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 companion note 05 · LO 2c.20

Return to Question 652 ↑

Q653 INTERMEDIATE · Block H

Best answer: B. Avoid an irreversible fixed solution until disease and tooth prognosis are stable

Justification: Stabilise active disease and reassess the abutments before an irreversible fixed prosthesis.

Why not the closest alternatives: Immediate preparation risks building on an unstable biological foundation.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2b.1–2b.12; 2c.11

Return to Question 653 ↑

Q654 INTERMEDIATE · Block H

Best answer: D. Level 3 (referral)

Justification: Full-mouth reconstruction with OVD change is specialist care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 companion note · LO 2b.10

Return to Question 654 ↑

Q655 INTERMEDIATE · Block H

Best answer: A. Adrenaline-impregnated cord

Justification: Systemic absorption can raise blood pressure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.2 · LO 2c.13

Return to Question 655 ↑

Q656 INTERMEDIATE · Block H

Best answer: C. Inadequate resistance form in a short tapered preparation

Justification: Axial height and taper determine resistance form; cement cannot reliably overcome a deficient preparation.

Why not the closest alternatives: Changing adhesive brand without redesigning the preparation does not create resistance form.

Revise: Volume I · Volume I Units 3 and 8 · Diploma LO 1.9; 2c.11; 2c.14

Return to Question 656 ↑

Q657 INTERMEDIATE · Block H

Best answer: A. They suit short-span work best

Justification: Stitching errors accumulate across full arches.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.4, Unit 11 · LO 2a.19

Return to Question 657 ↑

Q658 INTERMEDIATE · Block H

Best answer: E. Consider crown lengthening only after evaluating crown-to-root and biological consequences

Justification: Crown lengthening changes clinical crown length, crown–root ratio, furcation proximity and neighbouring tissues.

Why not the closest alternatives: Making the margin visible is not sufficient if the procedure sacrifices strategic support.

Revise: Volume I · Volume I Unit 8 · Diploma LO 2a.18; 2c.11; 2c.14

Return to Question 658 ↑

Q660 INTERMEDIATE · Block H

Best answer: C. An adhesive onlay preserves more tooth than a crown while covering the cusps

Justification: It covers cusps without full-coverage reduction.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.11 · LO 2c.13

Return to Question 660 ↑

Q661 INTERMEDIATE · Block H

Best answer: A. Central incisor

Justification: In the mandible, root surface area runs 6 > 7 > 3 > 5 ≈ 4 > 2 > 1.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.6 · LO 2c.11

Return to Question 661 ↑

Q662 INTERMEDIATE · Block H

Best answer: D. A fixed-movable bridge, or uprighting 47 orthodontically

Justification: A tilted abutment cannot share a single path with an upright one.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 §8.7 · LO 2c.11

Return to Question 662 ↑

Q666 INTERMEDIATE · Block H

Best answer: C. Direct composite build-ups

Justification: It is reversible and suits a young patient who is still growing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 02 · LO 2b.6

Return to Question 666 ↑

Q667 INTERMEDIATE · Block H

Best answer: B. The shade may still change and residual oxygen reduces the resin bond for a time

Justification: Residual oxygen after bleaching inhibits resin polymerisation for a period, and the shade settles.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 aesthetic note 03 · LO 2b.6

Return to Question 667 ↑

Q670 INTERMEDIATE · Block H

Best answer: D. Caries, tissue health, margins, contacts and occlusion before cosmetic changes

Justification: Bridge maintenance evaluates biological and technical health as well as function and patient satisfaction; caries and inflammation are clinically significant.

Why not the closest alternatives: A good aesthetic result does not mean the bridge remains biologically sound.

Revise: Unit 8 companion note 05 · LO 2c.20

Return to Question 670 ↑

BLOCK I · Occlusion and temporomandibular disorders

Q671 INTERMEDIATE · Block I

Best answer: D. Explain the uncertain causal relationship and favour reversible care, monitoring and further assessment if symptoms progress

Justification: Conservative reversible care is first-line for uncomplicated TMD. An isolated click is not an indication for irreversible occlusal alteration, and clinical improvement with a splint does not prove the occlusion caused the disorder.

Why not the closest alternatives: Permanent equilibration cannot be inferred as necessary from the presence of a click or an improvement while wearing a reversible appliance.

Revise: Volume I · Unit 9 · Diploma LO 2a.15; 2a.20; 2c.19; source themes: 2017 Dubai MPros; 2020 Edinburgh MPros

Return to Question 671 ↑

Q672 INTERMEDIATE · Block I

Best answer: E. Control periodontal inflammation and reassess traumatic loading/mobility; occlusal trauma may modify support and function but does not replace the microbial diagnosis

Justification: Periodontal inflammation and traumatic loading can coexist. Traumatic forces may contribute to mobility and tissue adaptation, but treatment must address plaque-associated disease; occlusal management is adjunctive and case-specific.

Why not the closest alternatives: Occlusal adjustment alone does not cure periodontitis, and mobility is not automatically evidence of hopeless prognosis.

Revise: Volume I · Unit 9 · Diploma LO 1.8; 2a.20; 2c.19; source themes: 2015 MRD; 2019 Edinburgh MPerio

Return to Question 672 ↑

Q677 INTERMEDIATE · Block I

Best answer: E. Reassess the occlusal scheme and eliminate destabilising excursive interferences while considering denture support and opposing natural dentition

Justification: Mucosa-supported prostheses respond differently to lateral tipping forces than periodontal-ligament-supported teeth. Denture stability and the existing opposing natural tooth guidance must be evaluated together when arranging occlusal contacts.

Why not the closest alternatives: Imposing an identical scheme on every removable, fixed or implant restoration is not justified; the clinical mechanics and available stability differ.

Revise: Volume I · Unit 9 · Diploma LO 2a.20; 2c.19; 2c.21; source themes: 2019 Edinburgh MPros; MRD Past questions

Return to Question 677 ↑

Q678 INTERMEDIATE · Block I

Best answer: D. Mutually protected occlusion

Justification: Anteriors protect posteriors in excursions; posteriors protect anteriors in ICP.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 678 ↑

Q682 INTERMEDIATE · Block I

Best answer: C. Avoid irreversible occlusal reconstruction solely to treat nonspecific TMD pain

Justification: Irreversible bite reconstruction is not justified solely to treat nonspecific TMD symptoms; consider reversible care and appropriate assessment.

Why not the closest alternatives: Restoration decisions should be based on dental needs, not an unsupported promise to abolish TMD.

Revise: Volume I · Volume I Unit 9 · Diploma LO 2a.15; 2a.20; 2c.19

Return to Question 682 ↑

Q684 INTERMEDIATE · Block I

Best answer: B. Restricted (normal is about 40–55 mm)

Justification: Normal maximum opening is about 40–55 mm; under 40 mm is restricted.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.20

Return to Question 684 ↑

Q686 INTERMEDIATE · Block I

Best answer: B. 20–25 mm

Justification: Further opening needs condylar translation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.1 · LO 2a.15

Return to Question 686 ↑

Q687 INTERMEDIATE · Block I

Best answer: A. Urgent specialist investigation for unexplained progressive neurosensory change

Justification: Unexplained progressive neurosensory change warrants investigation, not routine splint-only treatment.

Why not the closest alternatives: Masticatory muscle tenderness should not distract from a serious neurological red flag.

Revise: Volume I · Volume I Unit 9 · Diploma LO 2a.15; 2a.20; 2c.19

Return to Question 687 ↑

Q688 INTERMEDIATE · Block I

Best answer: A. Myalgia

Justification: Familiar pain on palpating the masticatory muscles; it is the commonest TMD.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · Schematic 21 · LO 2a.20

Return to Question 688 ↑

Q689 INTERMEDIATE · Block I

Best answer: E. Differentiate disc displacement with reduction from persistent closed lock

Justification: Disc displacement with reduction commonly produces reciprocal clicking with preserved opening.

Why not the closest alternatives: Persistent closed lock typically limits opening rather than merely clicking.

Revise: Volume I · Volume I Unit 9 · Diploma LO 2a.15; 2a.20; 2c.19

Return to Question 689 ↑

Q690 INTERMEDIATE · Block I

Best answer: D. Disc displacement with reduction

Justification: A reciprocal click shows the disc is recaptured on opening.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 690 ↑

Q691 INTERMEDIATE · Block I

Best answer: C. Disc displacement with reduction, with intermittent locking

Justification: Clicking plus episodes of limited opening.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 691 ↑

Q692 INTERMEDIATE · Block I

Best answer: D. Disc displacement without reduction, with limited opening (right closed lock)

Justification: Opening under 40 mm, deflection to the affected side and restricted contralateral excursion.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 692 ↑

Q694 INTERMEDIATE · Block I

Best answer: A. Degenerative joint disease

Justification: Crepitus with radiographic change.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 694 ↑

Q695 INTERMEDIATE · Block I

Best answer: E. Dislocation (open lock)

Justification: The condyle is trapped anterior to the eminence and the patient cannot close.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 695 ↑

Q696 INTERMEDIATE · Block I

Best answer: A. Headache attributed to TMD

Justification: Temporal headache modified by jaw function.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 696 ↑

Q698 INTERMEDIATE · Block I

Best answer: E. Women aged about 20–40

Justification: That is the commonest demographic.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.2 · LO 2a.20

Return to Question 698 ↑

Q699 INTERMEDIATE · Block I

Best answer: D. Reassurance only

Justification: A click without pain or locking needs no treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 699 ↑

Q700 INTERMEDIATE · Block I

Best answer: B. A soft diet, avoiding wide opening and gum, warm compresses, jaw rest, and keeping "lips together, teeth apart"

Justification: Self-care is first-line and most TMD is self-limiting.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 700 ↑

Q701 INTERMEDIATE · Block I

Best answer: E. Full-coverage hard acrylic (often maxillary), with even contacts in RCP and canine guidance

Justification: That is a stabilisation (Michigan-type) splint.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 701 ↑

Q703 INTERMEDIATE · Block I

Best answer: D. Posterior open bite

Justification: Long-term anterior repositioning changes the occlusion irreversibly.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 (exam traps) · LO 2a.20

Return to Question 703 ↑

Q704 INTERMEDIATE · Block I

Best answer: C. Over-eruption of the uncovered posterior teeth

Justification: Partial-coverage appliances worn continuously let uncovered teeth erupt.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 704 ↑

Q705 INTERMEDIATE · Block I

Best answer: A. Manipulation to try to recapture the disc, with reassurance, a splint and exercises

Justification: A recent closed lock may be reduced; otherwise reassure, splint and exercise.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 705 ↑

Q706 INTERMEDIATE · Block I

Best answer: D. Press the mandible downwards and backwards, with the thumbs on the lower molars or external oblique ridges

Justification: This lowers the condyle below the eminence and guides it back into the fossa; then advise limited opening.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 (exam traps) · LO 2a.20

Return to Question 706 ↑

Q707 INTERMEDIATE · Block I

Best answer: A. Same-day medical referral for suspected giant cell arteritis (urgent ESR or CRP)

Justification: Jaw claudication over age 50 risks sight; ESR and CRP are urgent.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.4 · LO 2a.20

Return to Question 707 ↑

Q708 INTERMEDIATE · Block I

Best answer: B. Malignancy (for example nasopharyngeal, parotid or infratemporal)

Justification: Progressive trismus, numbness, weight loss and referred otalgia are red flags.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.4 · LO 2a.20

Return to Question 708 ↑

Q709 INTERMEDIATE · Block I

Best answer: E. Cardiac pain referred to the jaw

Justification: Exertional jaw pain can be angina.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.4 · LO 2a.20

Return to Question 709 ↑

Q710 INTERMEDIATE · Block I

Best answer: C. Trigeminal neuralgia

Justification: Paroxysmal electric-shock pain triggered by light touch is characteristic.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.4 · LO 2a.20

Return to Question 710 ↑

Q712 INTERMEDIATE · Block I

Best answer: D. Education, self-care, conservative analgesia or reversible therapy with review

Justification: Most uncomplicated TMD myalgia is initially managed with education and reversible care; invasive and irreversible interventions need more specific indications.

Why not the closest alternatives: Arthrocentesis and surgical approaches target selected joint disorders, not routine initial care for nonspecific myalgia.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 712 ↑

Q713 INTERMEDIATE · Block I

Best answer: A. The GMP or specialist

Justification: Low-dose tricyclics are prescribed medically, not by the dentist routinely.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.3 · LO 2a.20

Return to Question 713 ↑

Q714 INTERMEDIATE · Block I

Best answer: D. Attrition facets, fractured restorations and cusps, masseteric hypertrophy, a scalloped tongue and linea alba

Justification: These are the classic signs of bruxism.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 714 ↑

Q715 INTERMEDIATE · Block I

Best answer: C. A hard night-time splint, with metal or monolithic zirconia occlusal surfaces

Justification: Layered porcelain chips under parafunction.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 715 ↑

Q716 INTERMEDIATE · Block I

Best answer: C. Examination on mounted casts to find interferences, the slide and the guidance

Justification: It identifies interferences, the slide and guidance before multiple restorations.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 716 ↑

Q717 INTERMEDIATE · Block I

Best answer: D. Reorganise to RCP

Justification: An unstable ICP or OVD change needs a reproducible reference.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 717 ↑

Q718 INTERMEDIATE · Block I

Best answer: A. Only to remove specific interferences for restorative reasons

Justification: Equilibration is irreversible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 718 ↑

Q719 INTERMEDIATE · Block I

Best answer: B. No; imaging is targeted to specific questions such as bone pathology or an alternative diagnosis

Justification: Imaging is targeted to a specific clinical question.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 companion note · LO 2a.20

Return to Question 719 ↑

Q721 INTERMEDIATE · Block I

Best answer: A. Canine guidance discludes the posterior teeth, reducing lateral load on the new crowns

Justification: Canine guidance takes lateral load off the posterior restorations.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9, In one minute · LO 2c.19

Return to Question 721 ↑

Q722 INTERMEDIATE · Block I

Best answer: E. Recognition of acute mandibular dislocation requiring timely reduction/referral

Justification: An acute dislocation requires prompt assessment and appropriate reduction/referral, including consideration of trauma and patient factors.

Why not the closest alternatives: An opening click is distinct from inability to close after wide opening.

Revise: Volume I · Volume I Unit 9 · Diploma LO 2a.15; 2a.20; 2c.19

Return to Question 722 ↑

Q724 INTERMEDIATE · Block I

Best answer: B. Left (towards the affected side)

Justification: The locked condyle cannot translate, so the jaw swings towards it.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9, In one minute · LO 2a.20

Return to Question 724 ↑

Q725 INTERMEDIATE · Block I

Best answer: E. Conservative reversible measures for myalgia without red flags

Justification: Education, self-care and appropriate reversible management suit uncomplicated masticatory myalgia.

Why not the closest alternatives: Irreversible occlusal alteration is not an evidence-based first-line cure for nonspecific muscle pain.

Revise: Volume I · Volume I Unit 9 · Diploma LO 2a.15; 2a.20; 2c.19

Return to Question 725 ↑

Q727 INTERMEDIATE · Block I

Best answer: D. Pain location, movement limitation, joint noises, locking and functional impact

Justification: These support a DC/TMD diagnosis.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 companion note 01 · LO 2a.20

Return to Question 727 ↑

Q728 INTERMEDIATE · Block I

Best answer: C. Group function, sharing the working-side load

Justification: It shares lateral load when a canine cannot take it alone.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9, In one minute · LO 2c.19

Return to Question 728 ↑

Q729 INTERMEDIATE · Block I

Best answer: E. Conform to the existing ICP

Justification: A stable, comfortable ICP is kept for single restorations.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 729 ↑

Q730 INTERMEDIATE · Block I

Best answer: A. Clenching or bruxism (linea alba and a scalloped tongue)

Justification: Linea alba and a scalloped tongue are soft-tissue signs of clenching.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 §9.5 · LO 2c.19

Return to Question 730 ↑

BLOCK J · Tooth wear, the failing dentition, interfaces and trauma

Q731 INTERMEDIATE · Block J

Best answer: C. A cracked tooth with a vital pulp and probable reversible pulpal inflammation

Justification: Pain on release from a specific cusp is characteristic of crack-related flexure. A short non-lingering cold response supports a vital pulp without strong evidence of irreversible inflammation; further crack testing is needed.

Why not the closest alternatives: Root fracture is not established from the presented findings, while isolated muscle pain does not explain reproducible cusp-release pain.

Revise: Volume I · Unit 10 · Diploma LO 2a.6; 2a.17; 2c.20; source themes: 2015 MRD; 2019 Edinburgh MPros

Return to Question 731 ↑

Q732 INTERMEDIATE · Block J

Best answer: D. Consider additive anterior Dahl restoration where space is limited but posterior support stable

Justification: Additive composite at an increased anterior vertical dimension (Dahl concept) may provide conservative space generation.

Why not the closest alternatives: Generalised full-mouth rehabilitation is disproportionate when posterior support and wear distribution are favourable.

Revise: Volume I · Volume I Unit 10 · Diploma LO 1.4; 2c.1; 2c.10–2c.12

Return to Question 732 ↑

Q733 INTERMEDIATE · Block J

Best answer: A. Prioritise investigations for possible vertical root fracture, integrating isolated probing, sinus tracing, imaging and history

Justification: A narrow deep isolated defect beside a root-treated tooth, especially with a J/halo-type lesion and sinus, raises concern for vertical root fracture. Confirmation may require careful history, probing, transillumination and appropriate imaging or referral.

Why not the closest alternatives: Primary endodontic or periodontal lesions remain in the differential; a single imaging shape is not pathognomonic and should not automatically trigger extraction.

Revise: Volume I · Unit 10 · Diploma LO 2a.11; 2a.17–2a.18; 2c.20; source themes: 2019 Edinburgh MPerio; 2020 Edinburgh MPros

Return to Question 733 ↑

Q734 INTERMEDIATE · Block J

Best answer: A. Refer for multidisciplinary assessment of severe generalised wear with complex occlusal changes

Justification: Complex rehabilitation warrants integrated specialist assessment and staged planning.

Why not the closest alternatives: An isolated anterior composite intervention cannot resolve the complex global occlusion.

Revise: Volume I · Volume I Unit 10 · Diploma LO 1.4; 2c.1; 2c.10–2c.12

Return to Question 734 ↑

Q736 INTERMEDIATE · Block J

Best answer: B. Assess and control a probable pulpal infection through an endodontic pathway, then reassess periodontal healing

Justification: Pulp necrosis with an isolated sinus-like sulcular tract and little generalised periodontal breakdown favours primary endodontic infection draining through the periodontal apparatus. Endodontic infection control precedes reassessment of healing.

Why not the closest alternatives: A combined lesion or fracture must still be excluded; early periodontal surgery before endodontic assessment risks treating the secondary manifestation rather than its source.

Revise: Volume I · Unit 10 · Diploma LO 2a.11; 2a.17–2a.18; 2c.20; source themes: 2014 Dubai MRD; 2019 Edinburgh MPerio

Return to Question 736 ↑

Q737 INTERMEDIATE · Block J

Best answer: D. Long-standing periodontal disease may have progressed to secondary pulpal involvement, requiring integrated endodontic and periodontal assessment

Justification: The documented periodontal disease predates signs of pulpal involvement. Although present tests are imperfect, the chronology supports periodontal destruction with possible secondary endodontic involvement and a combined management plan.

Why not the closest alternatives: Current non-response alone cannot establish whether the initial origin was pulpal or periodontal; chronology, probing, pulp tests and imaging are essential.

Revise: Volume I · Unit 10 · Diploma LO 2a.11; 2a.17–2a.18; 2c.20; source themes: 2015 MRD; 2019 Edinburgh MPerio

Return to Question 737 ↑

Q738 INTERMEDIATE · Block J

Best answer: E. Promptly reposition the coronal segment, assess stabilisation options and arrange trauma-guideline follow-up/referral

Justification: For a root fracture with displaced coronal segment, prompt repositioning and appropriate flexible stabilisation with follow-up are important. More cervical fractures have a guarded prognosis and need careful specialist assessment, but initial conservative management is not uniformly inappropriate.

Why not the closest alternatives: Immediate pulp testing can be falsely negative; prophylactic endodontic treatment of both fragments is generally not the initial default.

Revise: Volume I · Unit 10 · Diploma LO 2a.17; 2c.20; source themes: 2015 Dubai MRD; 2020 Edinburgh MPros

Return to Question 738 ↑

Q739 INTERMEDIATE · Block J

Best answer: D. Control erosive drivers and defer irreversible restorations

Justification: Address the ongoing intrinsic acid exposure and prevention/medical support before definitive adhesive work.

Why not the closest alternatives: Materials do not remove the cause of active erosion.

Revise: Volume I · Volume I Unit 10 · Diploma LO 2c.1; 2c.10–2c.12

Return to Question 739 ↑

Q740 INTERMEDIATE · Block J

Best answer: C. Hypoplastic amelogenesis imperfecta with insufficient enamel matrix formation

Justification: Hypoplastic AI typically produces quantitatively deficient enamel: thin enamel, pits or grooves, often with a hereditary and widespread presentation. The radiographic reduction in thickness is a key discriminator.

Why not the closest alternatives: Hypomaturation/hypocalcified forms more often have near-normal enamel thickness but altered quality; fluorosis and MIH differ in exposure history and tooth distribution.

Revise: Volume I · Unit 10 · Diploma LO 1.2; 2a.6; 2c.20; source themes: 2020 Edinburgh MPros; 2019 Dubai MPros

Return to Question 740 ↑

Q742 INTERMEDIATE · Block J

Best answer: A. Dentinogenesis imperfecta

Justification: Opalescent discoloration combined with bulbous crowns, cervical constriction, pulp obliteration and family history is characteristic of dentinogenesis imperfecta; assess for associated systemic findings as appropriate.

Why not the closest alternatives: Enamel-focused conditions do not typically cause the described dentine-related crown and pulp morphology.

Revise: Volume I · Unit 10 · Diploma LO 1.2; 2a.6; 2c.20; source themes: 2019 Dubai MPros; user-specified themes

Return to Question 742 ↑

Q743 INTERMEDIATE · Block J

Best answer: E. Modify parafunctional loading and restore selectively after risk assessment

Justification: Evaluate occlusal loading, restoration bulk and appropriate risk mitigation before choosing another definitive material.

Why not the closest alternatives: Repeating the same restorations without managing mechanical risk invites recurrence.

Revise: Volume I · Volume I Unit 10 · Diploma LO 2c.1; 2c.10–2c.12

Return to Question 743 ↑

Q744 INTERMEDIATE · Block J

Best answer: B. Symptomatic irreversible pulpitis in a vital tooth, requiring a discussion of appropriate definitive pulpal treatment or extraction

Justification: Prolonged lingering thermal pain with spontaneous episodes strongly supports symptomatic irreversible pulpitis. Apical radiographs and percussion may be unremarkable early in the course; clinical diagnosis combines history and testing.

Why not the closest alternatives: Dentine hypersensitivity is usually short and stimulus-linked, whereas pulp necrosis often yields absent response to sensibility testing rather than a marked lingering cold response.

Revise: Volume I · Unit 10 · Diploma LO 2a.6; 2a.17; 2c.20; source themes: 2019 Dubai MEndo; 2014 MRD

Return to Question 744 ↑

Q746 INTERMEDIATE · Block J

Best answer: D. Prolonged dry extraoral time markedly worsens periodontal ligament prognosis, requiring guideline-based replantation decisions and monitoring for replacement resorption/ankylosis

Justification: Extended dry time compromises periodontal-ligament cells and increases the risk of ankylosis and replacement resorption after replantation. Mature permanent teeth may still be replanted under guideline-directed care, but counselling and long-term review are essential.

Why not the closest alternatives: A good immediate position does not imply normal long-term healing; time, storage and root maturity affect risk and follow-up.

Revise: Volume I · Unit 10 · Diploma LO 2a.17; 2c.20; source themes: 2019 Dubai MEndo; 2020 RCS England MPros

Return to Question 746 ↑

Q747 INTERMEDIATE · Block J

Best answer: A. Protect exposed dentine and restore the fractured crown, after assessing for associated luxation or root injury and arranging follow-up

Justification: This is consistent with an uncomplicated enamel–dentine crown fracture. Timely sealing and conservative restoration, with examination for associated injuries and follow-up, protects the pulp and restores form.

Why not the closest alternatives: Visible pulp exposure would change the classification and management toward a vital pulp assessment; it is not described here.

Revise: Volume I · Unit 10 · Diploma LO 2a.17; 2c.20; source themes: 2020 Edinburgh MPros; 2015 Dubai MRD

Return to Question 747 ↑

Q748 INTERMEDIATE · Block J

Best answer: C. Long-term bisphosphonate therapy

Justification: Bisphosphonates. Drugs affecting bone turnover are a relative contraindication, with active periodontal disease, TMD, an anterior open bite tendency and poor compliance.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.3 · LO 2c.11

Return to Question 748 ↑

Q749 INTERMEDIATE · Block J

Best answer: E. Temporary speech change, tenderness and eating difficulty, with posterior contact taking months

Justification: Temporary speech change, tenderness, eating difficulty and months to contact.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.3 · LO 2c.11, 3b.1

Return to Question 749 ↑

Q750 INTERMEDIATE · Block J

Best answer: B. A diagnostic wax-up and a provisional trial at the new OVD

Justification: Wax-up and a provisional trial at the new OVD. Extensive reorganisation is generally Level 3.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.3 · LO 2c.11

Return to Question 750 ↑

Q751 INTERMEDIATE · Block J

Best answer: E. Elective root canal treatment to gain space

Justification: Elective root canal treatment. It sacrifices pulp vitality just to gain space.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.4 · LO 2c.11

Return to Question 751 ↑

Q753 INTERMEDIATE · Block J

Best answer: B. Dentoalveolar compensation often maintains OVD, so it should not be inferred from wear alone

Justification: Compensation often maintains OVD. Do not infer reduced OVD from worn teeth alone.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 companion note (traps) · LO 2a.15

Return to Question 753 ↑

Q754 INTERMEDIATE · Block J

Best answer: D. The cause must be stabilised and the remaining tissue reviewed first

Justification: Stabilise the cause first. Then review the remaining tissue.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 companion note (traps) · LO 2c.10

Return to Question 754 ↑

Q755 INTERMEDIATE · Block J

Best answer: A. Recognise that mixed mechanisms are common and address all relevant risk factors

Justification: Mixed mechanisms are common. Address every relevant risk factor.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 companion note 01 · LO 1.4

Return to Question 755 ↑

Q756 INTERMEDIATE · Block J

Best answer: B. Its abutment potential, occlusal stops and role in canine guidance

Justification: Abutment potential, occlusal stops and canine guidance.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5 · LO 2c.1

Return to Question 756 ↑

Q757 INTERMEDIATE · Block J

Best answer: E. A shortened dental arch, avoiding heroic treatment of the molars when function is adequate

Justification: Shortened dental arch. Avoid heroic molar treatment when function is adequate.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5 · LO 2c.1

Return to Question 757 ↑

Q758 INTERMEDIATE · Block J

Best answer: A. Transitional acrylic RPDs to which teeth can be added, with overdenture abutments where possible

Justification: Transitional acrylic RPDs and overdenture abutments. Avoid sudden total clearance.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5 · LO 2c.1

Return to Question 758 ↑

Q759 INTERMEDIATE · Block J

Best answer: C. It must be root-filled or asymptomatic and free of caries

Justification: Root-filled or asymptomatic, and caries-free. Retained roots preserve bone and proprioception.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5 · LO 2c.1

Return to Question 759 ↑

Q761 INTERMEDIATE · Block J

Best answer: C. PPD 4 mm or less with no bleeding at those sites

Justification: PPD 4 mm or less with no bleeding.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 761 ↑

Q763 INTERMEDIATE · Block J

Best answer: E. Supragingival margins, correct contour, and cleansable embrasures and pontics

Justification: Supragingival margins, correct contour, cleansable design.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 763 ↑

Q764 INTERMEDIATE · Block J

Best answer: B. Tooth-supported Co-Cr with rests, an open design away from the gingivae, guide planes and minimal clasp torque

Justification: Tooth-supported Co-Cr, open design, minimal torque.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 764 ↑

Q765 INTERMEDIATE · Block J

Best answer: D. When increasing mobility interferes with function or comfort; adhesive splints

Justification: When mobility affects function or comfort; adhesive splints.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 765 ↑

Q767 INTERMEDIATE · Block J

Best answer: C. Peri-implant mucositis (reversible, no bone loss) is Level 2

Justification: Mucositis Level 2; peri-implantitis Level 3.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 (exam traps) · LO 2c.16

Return to Question 767 ↑

Q768 INTERMEDIATE · Block J

Best answer: A. Complete crack assessment, test the segments and pulpal status, stabilise the cusps and plan appropriate coverage if the tooth is restorable

Justification: Pain on release of biting and a short normal response to cold can indicate a symptomatic cracked tooth with a sensate pulp. Extent and restorability guide stabilisation and cuspal protection rather than automatic endodontics.

Why not the closest alternatives: Persistent lingering thermal pain, spontaneous pain or an isolated deep pocket would alter the diagnosis and management. A normal radiograph does not exclude an enamel/dentine crack.

Revise: Volume I, Unit 10 · Diploma LO 2a.17; 2c.11; 2c.16 · Reference themes: 2015 MRD; 2020 Edinburgh MEndo

Return to Question 768 ↑

Q769 INTERMEDIATE · Block J

Best answer: C. Preventive non-restorative approach with monitoring

Justification: Prevention and monitoring avoid unnecessary operative intervention in stable asymptomatic wear.

Why not the closest alternatives: The presence of wear alone is not an indication for crowns.

Revise: Volume I · Volume I Unit 10 · Diploma LO 2c.1; 2c.10–2c.12

Return to Question 769 ↑

Q770 INTERMEDIATE · Block J

Best answer: B. Level 3

Justification: Level 3. Combined endodontic–periodontal–prosthodontic problems on strategic teeth are specialist care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.7 · LO 2c.18

Return to Question 770 ↑

Q771 INTERMEDIATE · Block J

Best answer: A. Localised additive composite using the Dahl principle

Justification: An additive anterior Dahl approach can provide localised restoration and space creation without extensive crown preparation.

Why not the closest alternatives: Complex full-arch rehabilitation may not be needed where wear is localised.

Revise: Volume I · Volume I Unit 10 · Diploma LO 2c.1; 2c.10–2c.12

Return to Question 771 ↑

Q772 INTERMEDIATE · Block J

Best answer: D. Surgery, cost, time, bone requirements, peri-implant disease and limited NHS access

Justification: Surgery, cost, time, bone, peri-implant disease, NHS access.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.8 · LO 2c.5

Return to Question 772 ↑

Q773 INTERMEDIATE · Block J

Best answer: B. Level 1

Justification: Level 1.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.9 · LO 2b.10

Return to Question 773 ↑

Q774 INTERMEDIATE · Block J

Best answer: A. Staged provisional increase in OVD for complex generalised rehabilitation

Justification: Staged reversible or provisional OVD testing and integrated planning help assess tolerance and functional outcomes.

Why not the closest alternatives: Immediate definitive full-mouth preparations omit an important risk-reduction phase.

Revise: Volume I · Volume I Unit 10 · Diploma LO 2c.1; 2c.10–2c.12

Return to Question 774 ↑

Q775 INTERMEDIATE · Block J

Best answer: C. Level 3, along with cleft, amelogenesis imperfecta and head and neck cancer rehabilitation

Justification: Level 3. It covers developmental conditions and head and neck cancer rehabilitation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.9 · LO 2b.10

Return to Question 775 ↑

Q776 INTERMEDIATE · Block J

Best answer: D. Level 3

Justification: Level 3. Severe gagging is Level 3; minor gagging is Level 2.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.9 · LO 2b.10

Return to Question 776 ↑

Q777 INTERMEDIATE · Block J

Best answer: B. Level 3

Justification: Level 3. Undiagnosed pain is complex care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.9 · LO 2b.10

Return to Question 777 ↑

Q778 INTERMEDIATE · Block J

Best answer: C. Review bruxism and restorative material/load before replacing repeated fractured restorations

Justification: Review function, parafunction, contact distribution, available material bulk and whether protective strategies are indicated.

Why not the closest alternatives: A different shade or surface polish alone cannot manage excessive mechanical stress.

Revise: Volume I · Volume I Unit 10 · Diploma LO 1.4; 2c.1; 2c.10–2c.12

Return to Question 778 ↑

Q780 INTERMEDIATE · Block J

Best answer: C. Store it in an appropriate medium and arrange urgent expert care

Justification: Appropriate storage medium and urgent care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 01 · LO 2a.18

Return to Question 780 ↑

Q781 INTERMEDIATE · Block J

Best answer: B. Pulpal necrosis, inflammatory or replacement resorption, ankylosis, colour change and root fracture healing

Justification: Necrosis, resorption, ankylosis, colour change, fracture healing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 02 · LO 2a.18

Return to Question 781 ↑

Q783 INTERMEDIATE · Block J

Best answer: D. A resin-bonded bridge, a removable partial denture or orthodontic space management

Justification: RBB, RPD or orthodontic space management. These keep future options open.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 03 · LO 2a.18

Return to Question 783 ↑

Q784 INTERMEDIATE · Block J

Best answer: C. Ankylosis (replacement resorption)

Justification: Ankylosis. An ankylosed tooth cannot erupt with growth and becomes infra-occluded.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 02 · LO 2a.18

Return to Question 784 ↑

Q785 INTERMEDIATE · Block J

Best answer: B. Aim to preserve pulp vitality with appropriate vital-pulp therapy after assessment and provide a sealed restoration

Justification: Timely vital-pulp treatment can preserve a healthy pulp after a complicated crown fracture when the biological and clinical conditions are favourable. The defect also requires protection and appropriate follow-up.

Why not the closest alternatives: Exposure alone does not mandate extraction or routine root treatment. Depth, contamination, elapsed time, pulpal condition and restoration feasibility are relevant.

Revise: Volume I, Unit 10 · Diploma LO 2a.17; 2c.16 · Reference themes: 2020 RCS England MPros; IADT 2020 trauma

Return to Question 785 ↑

Q786 INTERMEDIATE · Block J

Best answer: A. With multidisciplinary advice

Justification: Multidisciplinary advice.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 trauma note 04 · LO 2a.18

Return to Question 786 ↑

Q787 INTERMEDIATE · Block J

Best answer: C. Liaise with the GMP, prevent and monitor, and delay definitive restoration until the condition is stable

Justification: Liaise, prevent, delay definitive work.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.2, Unit 2 · LO 2c.10

Return to Question 787 ↑

Q788 INTERMEDIATE · Block J

Best answer: B. A night-time hard splint

Justification: A night-time hard splint.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.2 · LO 2c.10

Return to Question 788 ↑

Q790 INTERMEDIATE · Block J

Best answer: B. Use monitoring/prevention when wear is localised, asymptomatic and stable

Justification: Preventive advice and longitudinal monitoring can avoid unnecessary operative treatment.

Why not the closest alternatives: An index score alone does not mandate full-coverage restorations.

Revise: Volume I · Volume I Unit 10 · Diploma LO 1.4; 2c.1; 2c.10–2c.12

Return to Question 790 ↑

Q791 INTERMEDIATE · Block J

Best answer: C. Hypoplastic amelogenesis imperfecta with a familial presentation

Justification: Familial, generalised developmental loss of enamel quantity across dentitions favours hypoplastic amelogenesis imperfecta. Radiographic enamel is thin rather than normally thick but poorly mineralised.

Why not the closest alternatives: Fluorosis is exposure-related and does not typically show Mendelian clustering; DI predominantly affects dentine and can show bulbous crowns and pulp changes.

Revise: Volume I, Unit 10 · Diploma LO 1.2; 2a.17; 2c.10 · Reference themes: 2020 Edinburgh MPros; 2019 Dubai MPros

Return to Question 791 ↑

Q793 INTERMEDIATE · Block J

Best answer: C. Assess pulp status pre-operatively and record it

Justification: Assess and record pulp status. Prepared abutments carry a long-term necrosis risk.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.7 · LO 2c.17

Return to Question 793 ↑

Q794 INTERMEDIATE · Block J

Best answer: B. Canines, with their long roots and strategic position

Justification: Canines. Long roots and a strategic position make them ideal overdenture abutments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5, Unit 7 · LO 2c.1

Return to Question 794 ↑

Q795 INTERMEDIATE · Block J

Best answer: D. A planned, staged transition, avoiding sudden total clearance where possible

Justification: A planned, staged transition.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.5 · LO 2c.1

Return to Question 795 ↑

Q796 INTERMEDIATE · Block J

Best answer: C. A conservative composite restoration

Justification: Conservative composite. Anterior root-filled teeth with little tissue loss need no crown.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.7 · LO 2c.17

Return to Question 796 ↑

Q797 INTERMEDIATE · Block J

Best answer: E. Only if the patient's oral hygiene allows

Justification: Only if hygiene allows. A plate covers the gingival margins.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 10 §10.6 · LO 2c.16

Return to Question 797 ↑

Q799 INTERMEDIATE · Block J

Best answer: D. Control intrinsic or dietary acid challenge before definitive restoration

Justification: Manage the ongoing erosive challenge and appropriate medical referral/support before elective definitive restorations.

Why not the closest alternatives: No restorative material withstands uncontrolled recurrent acid exposure predictably.

Revise: Volume I · Volume I Unit 10 · Diploma LO 1.4; 2c.1; 2c.10–2c.12

Return to Question 799 ↑

BLOCK K · Implants and digital prosthodontics

Q801 INTERMEDIATE · Block K

Best answer: D. Escalate progressive radiographic bone loss with bleeding/suppuration for specialist evaluation

Justification: Progressive bone loss with inflammatory signs suggests peri-implantitis requiring thorough evaluation and appropriate referral.

Why not the closest alternatives: Isolated polishing or a more aesthetic crown does not treat progressive peri-implant disease.

Revise: Volume I · Volume I Unit 11 · Diploma LO 2a.19; 2c.5

Return to Question 801 ↑

Q802 INTERMEDIATE · Block K

Best answer: D. Manage peri-implant mucositis with plaque control, accessible debridement and tissue reassessment

Justification: Bleeding and inflammation without progressive supporting bone loss fit peri-implant mucositis. Plaque-control improvement and mechanical professional care are central, with repeated evaluation of tissue response and cleaning access.

Why not the closest alternatives: Peri-implantitis implies a bone-loss component; neither implant removal nor replacement of attachments is indicated purely by bleeding.

Revise: Volume I · Unit 11 · Diploma LO 2a.19; 2c.5; 2c.21; source themes: 2019 Edinburgh MPerio; 2019 Edinburgh MPros

Return to Question 802 ↑

Q803 INTERMEDIATE · Block K

Best answer: A. Assess prosthesis design and keratinised-tissue comfort without assuming implant failure

Justification: Assess soft tissue phenotype, comfort, plaque control and whether specialist mucogingival input is appropriate.

Why not the closest alternatives: The symptoms alone do not prove implant failure or mandate removal.

Revise: Volume I · Volume I Unit 11 · Diploma LO 2a.19; 2c.5

Return to Question 803 ↑

Q804 INTERMEDIATE · Block K

Best answer: B. Recognise probable peri-implantitis, control accessible biofilm/risk factors and refer progressive bone loss appropriately

Justification: Inflammatory findings plus progressive bone loss are concerning for peri-implantitis. Tobacco and poor glycaemic control increase risk; initial management should control biofilm and risk while escalating progressive disease appropriately.

Why not the closest alternatives: Attachments or occlusion may need assessment but do not replace diagnosis and management of progressive inflammatory bone loss.

Revise: Volume I · Unit 11 · Diploma LO 2a.19; 2c.5; 2c.21; source themes: 2019 Edinburgh MPerio; 2019 Edinburgh MPros

Return to Question 804 ↑

Q805 INTERMEDIATE · Block K

Best answer: E. Measure the restorative envelope for components, hygiene access, acrylic thickness and attachment maintenance before selecting the design

Justification: Attachment choice is constrained by available vertical/restorative space and maintenance needs. Bars require space for the bar, hygiene clearance and denture housing; solitary attachments may be lower-profile but need adequate acrylic and appropriate parallelism.

Why not the closest alternatives: Universal numerical clearance rules are unsafe without identifying the particular system and manufacturer instructions.

Revise: Volume I · Unit 11 · Diploma LO 2c.5; 2c.9; 2c.13; source themes: 2019 Dubai MPros; 2019 Edinburgh MPros

Return to Question 805 ↑

Q806 INTERMEDIATE · Block K

Best answer: A. Review the entire occlusion, attachment retention, tooth positions, ridge support and denture stability under function, then modify the prosthetic scheme as indicated

Justification: A stable implant-retained mandible may transfer higher chewing forces to the opposed mucosa-supported maxillary denture. Occlusal balance, tooth positions, maxillary support and attachment characteristics need integrated evaluation.

Why not the closest alternatives: Maximising lower retention or simply removing posterior contacts may worsen differential loading and fail to address upper denture support.

Revise: Volume I · Unit 11 · Diploma LO 2c.5; 2c.9; 2c.19; source themes: 2019 Dubai MPros; user-specified themes

Return to Question 806 ↑

Q807 INTERMEDIATE · Block K

Best answer: C. Clinical inflammation and progressive radiographic bone loss are central to diagnosis; the microbial biofilm is polymicrobial and cultures alone are not sufficient

Justification: Peri-implant biofilms are complex and often share taxa with periodontal disease. The diagnostic combination of inflammatory signs, probing changes and progressive bone loss matters more than presence or absence of any one species.

Why not the closest alternatives: Culture alone does not distinguish colonisation from active disease; risk factors and prosthesis hygiene access must be considered.

Revise: Volume I · Unit 11 · Diploma LO 1.6; 2a.19; 2c.5; source themes: 2019 Edinburgh MPerio; user-specified themes

Return to Question 807 ↑

Q808 INTERMEDIATE · Block K

Best answer: E. Balance retentive needs with available vertical space, patient dexterity, cleaning access and realistic maintenance

Justification: Attachment selection is a patient- and prosthesis-specific compromise. Hygiene access, manual skill, available restorative space and maintenance burden can outweigh pursuit of maximum initial retention.

Why not the closest alternatives: Bars can assist in selected designs, but splinting does not eliminate biologic risk or cleaning requirements, and low-profile attachments still need sufficient acrylic support.

Revise: Volume I · Unit 11 · Diploma LO 2c.5; 2c.9; 2c.13; source themes: 2019 Dubai MPros; user-specified themes

Return to Question 808 ↑

Q809 INTERMEDIATE · Block K

Best answer: C. Immediate (within 1 week)

Justification: Immediate (within 1 week).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1 · LO 2c.5

Return to Question 809 ↑

Q810 INTERMEDIATE · Block K

Best answer: B. Early (1 week to 2 months)

Justification: Early (1 week to 2 months). Conventional loading is after 2 months.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1 · LO 2c.5

Return to Question 810 ↑

Q811 INTERMEDIATE · Block K

Best answer: B. External hex

Justification: External hex. This historic design has more screw loosening.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1 · LO 2c.5

Return to Question 811 ↑

Q812 INTERMEDIATE · Block K

Best answer: D. Conical (Morse taper)

Justification: Conical (Morse taper). It gives a better seal and stability.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1 · LO 2c.5

Return to Question 812 ↑

Q813 INTERMEDIATE · Block K

Best answer: E. Investigate peri-implant inflammation and hygiene access before simply polishing the crown

Justification: Examine peri-implant disease status and whether the restoration allows plaque control.

Why not the closest alternatives: A mechanically stable implant can still develop mucosal inflammation.

Revise: Volume I · Volume I Unit 11 · Diploma LO 2a.19; 2c.5

Return to Question 813 ↑

Q814 INTERMEDIATE · Block K

Best answer: A. A history of periodontitis, especially with smoking

Justification: History of periodontitis with smoking. Other risks: poorly controlled diabetes, poor plaque control, no maintenance, radiotherapy, IV antiresorptives and bruxism.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.1 · LO 2c.5

Return to Question 814 ↑

Q815 INTERMEDIATE · Block K

Best answer: E. Evaluate available restorative height, implant position/divergence, retention requirement, opposing arch and hygiene to select the most maintainable system

Justification: Attachment systems differ in space requirement, hygiene access, tolerance of implant alignment and maintenance. Selection is patient-specific; no single attachment is universally superior.

Why not the closest alternatives: The two-implant mandibular overdenture is an established treatment option but does not mean every patient requires the same prosthetic attachment or fixed alternative.

Revise: Volume I, Unit 11 · Diploma LO 2a.19; 2c.5; 2c.13 · Reference themes: 2019 Dubai MPros; overdenture consensus

Return to Question 815 ↑

Q816 INTERMEDIATE · Block K

Best answer: A. Easier hygiene and maintenance, less restorative space needed

Justification: Easier hygiene, less space; inserts wear.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 816 ↑

Q817 INTERMEDIATE · Block K

Best answer: A. A bar

Justification: A bar. It splints divergent implants but needs more vertical space and hygiene effort.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 817 ↑

Q819 INTERMEDIATE · Block K

Best answer: D. Screw-retained: retrievable, with no subgingival cement

Justification: Screw-retained. It is retrievable with no subgingival cement.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 819 ↑

Q820 INTERMEDIATE · Block K

Best answer: C. Residual subgingival cement

Justification: Residual cement. Excess subgingival cement is a recognised cause of peri-implant disease.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 820 ↑

Q821 INTERMEDIATE · Block K

Best answer: D. It is Level 3

Justification: Level 3; passive fit; short cantilevers.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 821 ↑

Q822 INTERMEDIATE · Block K

Best answer: A. Wear of the attachment inserts, needing replacement

Justification: Insert wear. Loss of retention is the commonest overdenture complication.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.21

Return to Question 822 ↑

Q824 INTERMEDIATE · Block K

Best answer: C. With a calibrated torque driver to the manufacturer's value

Justification: A calibrated torque driver. Tighten to the manufacturer's value.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.5

Return to Question 824 ↑

Q825 INTERMEDIATE · Block K

Best answer: B. Implants have no periodontal ligament, so differential mobility causes problems

Justification: No periodontal ligament. Differential mobility makes rigid tooth–implant connection a problem.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 (exam traps) · LO 2c.5

Return to Question 825 ↑

Q826 INTERMEDIATE · Block K

Best answer: C. No erythema, bleeding or suppuration, and no bone loss beyond initial remodelling

Justification: No erythema, bleeding or suppuration; no bone loss.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.3 · LO 2c.21

Return to Question 826 ↑

Q827 INTERMEDIATE · Block K

Best answer: D. Diagnose peri-implant mucositis, address plaque control and prosthetic cleansability, and reassess

Justification: Bleeding with local inflammation but without progressive bone loss beyond initial remodelling is consistent with peri-implant mucositis. Local biofilm reduction, hygiene optimisation and reassessment are appropriate first steps.

Why not the closest alternatives: Peri-implantitis requires evidence of progressive supporting bone loss in addition to inflammation; baseline measurements are vital for interpretation.

Revise: Volume I, Unit 11 · Diploma LO 2a.19; 2c.5; 2c.27 · Reference themes: 2019 Edinburgh MPerio; 2017 World Workshop classification

Return to Question 827 ↑

Q828 INTERMEDIATE · Block K

Best answer: D. Bone 3 mm or more apical to the implant platform, with PPD 6 mm or more and bleeding

Justification: Bone 3 mm or more apical to the platform, PPD 6 mm or more with bleeding. That is the 2017 World Workshop definition without baseline records.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.3 · LO 2c.21

Return to Question 828 ↑

Q829 INTERMEDIATE · Block K

Best answer: A. A baseline radiograph and probing depths

Justification: Baseline radiograph and probing depths.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.3 · LO 2c.21

Return to Question 829 ↑

Q830 INTERMEDIATE · Block K

Best answer: E. Interdental brushes and floss under the bar, professional debridement with instruments that will not scratch titanium, and annual review

Justification: Interdental brushes, floss under the bar, non-scratch debridement, annual review.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.3 · LO 2c.21

Return to Question 830 ↑

Q831 INTERMEDIATE · Block K

Best answer: A. Instruments that will not scratch titanium (such as plastic, titanium or carbon-fibre tips)

Justification: Instruments that will not scratch titanium.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 (exam traps) · LO 2c.21

Return to Question 831 ↑

Q833 INTERMEDIATE · Block K

Best answer: B. It has survived, but is not a success

Justification: Survived, not a success.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 companion note 04 · LO 2c.21

Return to Question 833 ↑

Q834 INTERMEDIATE · Block K

Best answer: E. The prosthesis: the missing-tooth problem, desired restoration, smile line, restorative space, occlusion, cleansability and the patient's priorities

Justification: Prosthesis-first assessment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 companion note 01 · LO 2a.19

Return to Question 834 ↑

Q836 INTERMEDIATE · Block K

Best answer: B. They are excellent for short spans

Justification: Short spans best; stitching errors over full arches.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 836 ↑

Q839 INTERMEDIATE · Block K

Best answer: D. PLY or OBJ

Justification: PLY or OBJ. These carry colour and texture.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 839 ↑

Q840 INTERMEDIATE · Block K

Best answer: A. CBCT (DICOM) with surface scans (STL)

Justification: DICOM merged with STL.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 840 ↑

Q841 INTERMEDIATE · Block K

Best answer: E. Trueness is closeness to the reference; precision is repeatability

Justification: Trueness is closeness to the reference; precision is repeatability.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 digital note 03 · LO 2a.19

Return to Question 841 ↑

Q842 INTERMEDIATE · Block K

Best answer: A. Provide conservative shared planning where implant maintenance capacity is poor

Justification: Choose maintainable shared-care options based on patient dexterity, attendance, biological risk and informed preferences.

Why not the closest alternatives: Technical feasibility alone does not establish a sustainable prosthetic choice.

Revise: Volume I · Volume I Unit 11 · Diploma LO 2a.19; 2c.5

Return to Question 842 ↑

Q843 INTERMEDIATE · Block K

Best answer: D. Stereolithography (SLA), digital light processing (DLP) and selective laser melting (for metals)

Justification: SLA, DLP and selective laser melting.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 843 ↑

Q846 INTERMEDIATE · Block K

Best answer: D. A physical mock-up from a wax-up for the patient's approval

Justification: A physical mock-up. It remains useful for approval and consent.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19, 3b.1

Return to Question 846 ↑

Q847 INTERMEDIATE · Block K

Best answer: C. Jaw motion tracking

Justification: Jaw motion tracking.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 847 ↑

Q848 INTERMEDIATE · Block K

Best answer: A. Save source files securely and document revisions

Justification: Secure storage and documented revisions.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 digital note 05 · LO 3a.5

Return to Question 848 ↑

Q849 INTERMEDIATE · Block K

Best answer: E. Post-curing and using a material indicated for the purpose

Justification: Post-curing and material indication.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 digital note 04 · LO 2a.19

Return to Question 849 ↑

Q851 INTERMEDIATE · Block K

Best answer: C. Screw torque, occlusal loading (including bruxism) and the connection

Justification: Torque, occlusal load and connection.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.21

Return to Question 851 ↑

Q852 INTERMEDIATE · Block K

Best answer: A. Fracture of the upper denture

Justification: Fracture of the upper denture. It is a recognised complication opposite implants.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.21

Return to Question 852 ↑

Q855 INTERMEDIATE · Block K

Best answer: B. Evaluate abutment screw loosening, fit and occlusal forces before retightening

Justification: Check component compatibility, screw integrity, passive fit and occlusal loads before repeating an identical intervention.

Why not the closest alternatives: Repeated loosening can reflect a mechanical cause rather than a need for tighter blind torque.

Revise: Volume I · Volume I Unit 11 · Diploma LO 2a.19; 2c.5

Return to Question 855 ↑

Q857 INTERMEDIATE · Block K

Best answer: D. Mechanical debridement, OHI and a check of the prosthesis design for cleansability

Justification: Debridement, OHI and a cleansability check.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.3 · LO 2c.21

Return to Question 857 ↑

Q859 INTERMEDIATE · Block K

Best answer: A. Replace the worn matrix (insert)

Justification: Replace the matrix.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.2 · LO 2c.21

Return to Question 859 ↑

Q860 INTERMEDIATE · Block K

Best answer: C. They integrate facial references into the digital plan

Justification: Facial references in the plan. They support, not replace, examination and consent.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 §11.4 · LO 2a.19

Return to Question 860 ↑

BLOCK L · Complications, failures and maintenance

Q861 INTERMEDIATE · Block L

Best answer: C. Assess peri-implant tissue health and prosthesis cleansability at review

Justification: Evaluate peri-implant plaque control, prosthesis contours, probing findings and radiographic history, with referral if disease is suspected.

Why not the closest alternatives: A mechanically intact crown can still have a biologic maintenance problem.

Revise: Volume I · Volume I Unit 12 · Diploma LO 2c.21–2c.27

Return to Question 861 ↑

Q862 INTERMEDIATE · Block L

Best answer: B. Identify the true fitting-surface pressure and the destabilising occlusal contact, correct each conservatively and review healing

Justification: A sharp ridge can create a local pressure spot, while excursive occlusal interferences can increase instability and local loading. Both fitting surface and functional contacts need assessment before adjustments are selected.

Why not the closest alternatives: Indiscriminate flange relief can reduce useful support; targeted corrections preserve function while addressing the causes.

Revise: Volume I · Unit 12 · Diploma LO 2c.21–2c.27; source themes: 2019 Edinburgh MPros; 2017 Dubai MPros

Return to Question 862 ↑

Q863 INTERMEDIATE · Block L

Best answer: E. Investigate cementation contamination and surface treatment after early adhesive debond

Justification: The bonding workflow and surface preparation are central to early adhesive debonding.

Why not the closest alternatives: Repeated recementation without fixing isolation is unlikely to be reliable.

Revise: Volume I · Volume I Unit 12 · Diploma LO 2c.21–2c.27

Return to Question 863 ↑

Q864 INTERMEDIATE · Block L

Best answer: C. Check occlusal contacts and opposing dentition for overload

Justification: High or concentrated occlusal load may contribute to repeated flexural fracture and should be assessed.

Why not the closest alternatives: Acrylic composition alone does not explain contact-related recurrence.

Revise: Volume I · Volume I Units 3 and 12 · Diploma LO 1.9; 2c.21–2c.27

Return to Question 864 ↑

Q865 INTERMEDIATE · Block L

Best answer: C. Reassess the dentition, surveyed path and support, then decide whether repair, redesign or replacement can achieve a reliable long-term fit

Justification: Fracture, repeated adjustment and tooth movement suggest that repairing the same geometry may recur. The entire RPD design and current abutment contours must be considered before choosing a repair or a new prosthesis.

Why not the closest alternatives: The clasp cannot be safely selected independently from its path, reciprocal components and support; a new tip alone is not a complete solution.

Revise: Volume I · Unit 12 · Diploma LO 1.9; 2c.3; 2c.21–2c.27; source themes: MRD Past questions; 2019 Edinburgh MPros

Return to Question 865 ↑

Q866 INTERMEDIATE · Block L

Best answer: C. Evaluate palatal vault/section thickness and flexural fatigue

Justification: High palatal geometry and inadequate acrylic cross-section can permit repeated bending and midline fatigue.

Why not the closest alternatives: Reinforcing a new base without diagnosing the geometry may miss other drivers.

Revise: Volume I · Volume I Units 3 and 12 · Diploma LO 1.9; 2c.21–2c.27

Return to Question 866 ↑

Q868 INTERMEDIATE · Block L

Best answer: D. Residual excess cement, until proven otherwise

Justification: Residual cement. Suspect it until proven otherwise.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 (exam traps) · LO 2c.21

Return to Question 868 ↑

Q869 INTERMEDIATE · Block L

Best answer: A. An open or weak contact

Justification: An open or weak contact. Adjust or replace the restoration.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.1 · LO 2c.21

Return to Question 869 ↑

Q870 INTERMEDIATE · Block L

Best answer: D. Check restoration thickness and load for ceramic chipping

Justification: Restorative thickness and occlusal loading must be evaluated before changing the material or repeating the onlay.

Why not the closest alternatives: Bond strength alone cannot make unsupported thin ceramic immune to fracture.

Revise: Volume I · Volume I Unit 12 · Diploma LO 2c.21–2c.27

Return to Question 870 ↑

Q871 INTERMEDIATE · Block L

Best answer: A. Caries and loss of retention

Justification: Caries and loss of retention.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.1 · LO 2c.22

Return to Question 871 ↑

Q872 INTERMEDIATE · Block L

Best answer: D. The later need for endodontic treatment

Justification: Later need for endodontics. It is a common biological complication of crowned vital teeth.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.1 · LO 2c.21, 3b.1

Return to Question 872 ↑

Q873 INTERMEDIATE · Block L

Best answer: C. A distorted impression or cast; re-impression and remake

Justification: Distorted impression or cast. Widespread misfit means re-impression and remake.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.2 · LO 2c.22

Return to Question 873 ↑

Q874 INTERMEDIATE · Block L

Best answer: C. Review fit and tissue support over the resorbed ridge

Justification: Loss of support allows additional flexure and can contribute to fatigue crack growth.

Why not the closest alternatives: Repair material selection without support assessment is insufficient.

Revise: Volume I · Volume I Units 3 and 12 · Diploma LO 1.9; 2c.21–2c.27

Return to Question 874 ↑

Q875 INTERMEDIATE · Block L

Best answer: B. Fatigue, aggravated by repeated adjustment of low-ductility Co-Cr or casting porosity

Justification: Fatigue. Repeated adjustment of low-ductility Co-Cr, or casting porosity, makes it worse.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.2 · LO 2c.22

Return to Question 875 ↑

Q876 INTERMEDIATE · Block L

Best answer: D. Occlusal overload, an unseated rest, free-end saddle rotation or a clasp active at rest

Justification: Overload, unseated rest, saddle rotation or active clasp.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.2 · LO 2c.21

Return to Question 876 ↑

Q878 INTERMEDIATE · Block L

Best answer: B. Discuss design reinforcement or alternative base material if risk persists

Justification: Consider appropriate material/design reinforcement after the underlying cause has been assessed.

Why not the closest alternatives: Repeated application of the same repair resin is less likely to be durable than a planned structural solution.

Revise: Volume I · Volume I Units 3 and 12 · Diploma LO 1.9; 2c.21–2c.27

Return to Question 878 ↑

Q880 INTERMEDIATE · Block L

Best answer: B. Reline or rebase

Justification: Reline or rebase. Remake only when OVD, appearance or occlusion are also wrong.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.3 · LO 2c.23

Return to Question 880 ↑

Q881 INTERMEDIATE · Block L

Best answer: B. Inspect porosity and repair-interface quality

Justification: Porosity and poor interfacial integrity can create sites of stress concentration and recurrent failure.

Why not the closest alternatives: A new palatal seal will not by itself improve a weak repaired junction.

Revise: Volume I · Volume I Units 3 and 12 · Diploma LO 1.9; 2c.21–2c.27

Return to Question 881 ↑

Q883 INTERMEDIATE · Block L

Best answer: D. Raised OVD from flash during processing

Justification: Raised OVD from flash. It is a processing error.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.4 · LO 2c.22

Return to Question 883 ↑

Q885 INTERMEDIATE · Block L

Best answer: A. Investigate occlusion and jaw relation when prosthesis tips during contact

Justification: Occlusal interference and incorrect jaw relations can destabilise otherwise acceptable bases.

Why not the closest alternatives: Another broad reline is unlikely to resolve a reproducible premature-contact problem.

Revise: Volume I · Volume I Unit 12 · Diploma LO 2c.21–2c.27

Return to Question 885 ↑

Q887 INTERMEDIATE · Block L

Best answer: E. Identify repetitive flexure and support deficiency before denture-base repair

Justification: Repeated failures demand evaluation of design, fit, support and flexural stresses rather than another identical repair.

Why not the closest alternatives: Choosing a stronger resin without addressing root causes often produces another failure.

Revise: Volume I · Volume I Unit 12 · Diploma LO 2c.21–2c.27

Return to Question 887 ↑

Q889 INTERMEDIATE · Block L

Best answer: E. Implant overdenture attachments

Justification: Implant overdenture attachments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 12 §12.5 · LO 2c.27

Return to Question 889 ↑

Q890 INTERMEDIATE · Block L

Best answer: A. Assess stress concentration, occlusion, extension and base thickness, then plan an appropriate laboratory repair or replacement with reinforcement where indicated

Justification: The frenal notch can concentrate stress under repeated flexure. A visible crack is evidence of possible fatigue propagation; repair decisions should account for fit, occlusion and material design rather than treating it as cosmetic.

Why not the closest alternatives: Reinforcement does not correct uncontrolled occlusal loading; both the source of flexion and material integrity matter.

Revise: Volume I, Unit 12 · Diploma LO 1.9; 2c.21; 2c.27 · Reference themes: 2017 MRD; 2019 Edinburgh MPros

Return to Question 890 ↑

BLOCK M · Governance, law and communication

Q891 INTERMEDIATE · Block M

Best answer: B. It is informed, voluntary, given by a person with capacity, specific to the treatment, documented, and can be withdrawn

Justification: Informed, voluntary, capacitous, specific, documented, withdrawable. Consent is a process, not a signature.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3b.1

Return to Question 891 ↑

Q892 INTERMEDIATE · Block M

Best answer: E. Montgomery v Lanarkshire (2015): material risks to a reasonable patient, or to this patient, and reasonable alternatives

Justification: Montgomery (2015). It requires disclosure of material risks and reasonable alternatives, and replaced Bolam for disclosure.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · Schematic 25 · LO 3b.1

Return to Question 892 ↑

Q893 INTERMEDIATE · Block M

Best answer: A. Bolam, qualified by Bolitho (opinion must withstand logical analysis)

Justification: Bolam qualified by Bolitho. They still apply to diagnosis and treatment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3a.1

Return to Question 893 ↑

Q894 INTERMEDIATE · Block M

Best answer: C. Duty of care, breach of that duty, causation, and resulting harm

Justification: Duty, breach, causation, harm.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3a.1

Return to Question 894 ↑

Q895 INTERMEDIATE · Block M

Best answer: D. Vicarious liability: an employer is liable for employees' acts in the course of employment

Justification: Vicarious liability.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3a.1

Return to Question 895 ↑

Q896 INTERMEDIATE · Block M

Best answer: D. An unwise decision does not mean the person lacks capacity

Justification: An unwise decision is not incapacity.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3b.2

Return to Question 896 ↑

Q897 INTERMEDIATE · Block M

Best answer: A. Use a qualified interpreter for important complex consent discussions

Justification: Arrange a suitably qualified interpreter or reliable communication support to establish the patient's own informed decision.

Why not the closest alternatives: A relative's summary is not a guaranteed substitute for valid consent.

Revise: Volume I · Volume I Unit 13 · Diploma LO 3a.1–3a.12; 3b.1–3b.5

Return to Question 897 ↑

Q898 INTERMEDIATE · Block M

Best answer: E. Capacity is decision-specific and time-specific

Justification: Decision-specific and time-specific. Reassess on the day.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3b.2

Return to Question 898 ↑

Q900 INTERMEDIATE · Block M

Best answer: C. If she is Gillick competent

Justification: Gillick competence.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3b.2

Return to Question 900 ↑

Q901 INTERMEDIATE · Block M

Best answer: E. Adults with Incapacity (Scotland) Act 2000

Justification: Adults with Incapacity (Scotland) Act 2000.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3b.2

Return to Question 901 ↑

Q902 INTERMEDIATE · Block M

Best answer: C. Benefits, risks, alternatives and doing nothing (BRAN)

Justification: BRAN. Discuss benefits, risks, alternatives and doing nothing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.1 · LO 3b.1

Return to Question 902 ↑

Q903 INTERMEDIATE · Block M

Best answer: D. Maximise the practice's income

Justification: Maximising income. It is not a GDC principle.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.2 · LO 3a.2

Return to Question 903 ↑

Q906 INTERMEDIATE · Block M

Best answer: A. All registrants

Justification: All registrants.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.7

Return to Question 906 ↑

Q908 INTERMEDIATE · Block M

Best answer: C. The statutory duty of candour

Justification: The statutory duty of candour. It applies to CQC-registered providers for notifiable incidents.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.7

Return to Question 908 ↑

Q909 INTERMEDIATE · Block M

Best answer: D. Through the MHRA Yellow Card scheme

Justification: MHRA Yellow Card.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.7

Return to Question 909 ↑

Q910 INTERMEDIATE · Block M

Best answer: B. The MHRA

Justification: MHRA.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.7

Return to Question 910 ↑

Q911 INTERMEDIATE · Block M

Best answer: C. RIDDOR, to the HSE

Justification: RIDDOR to the HSE.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.7

Return to Question 911 ↑

Q912 INTERMEDIATE · Block M

Best answer: A. A never event (wrong-site surgery)

Justification: A never event.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 (exam traps) · LO 3a.7

Return to Question 912 ↑

Q913 INTERMEDIATE · Block M

Best answer: B. Acknowledge within 3 working days

Justification: Acknowledge within 3 working days; 12 months.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.8

Return to Question 913 ↑

Q914 INTERMEDIATE · Block M

Best answer: E. NHS: the Parliamentary and Health Service Ombudsman

Justification: PHSO (NHS) or Dental Complaints Service (private).

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.8

Return to Question 914 ↑

Q915 INTERMEDIATE · Block M

Best answer: D. The Public Interest Disclosure Act 1998

Justification: Public Interest Disclosure Act 1998.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.3 · LO 3a.9

Return to Question 915 ↑

Q916 INTERMEDIATE · Block M

Best answer: B. Add a dated, signed note of the correction, leaving the original visible

Justification: A dated, signed correction. Never alter the original.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.4 · LO 3a.5

Return to Question 916 ↑

Q917 INTERMEDIATE · Block M

Best answer: D. Adults at least 11 years after last treatment

Justification: Adults 11 years; children to 25 or 11 years, whichever is longer.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.4 · LO 3a.5

Return to Question 917 ↑

Q919 INTERMEDIATE · Block M

Best answer: C. With consent, when required by law (such as a court order), or in the public interest to prevent serious harm, sharing the minimum necessary

Justification: Consent, law, or public interest. Share the minimum necessary.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.4 · LO 3a.5

Return to Question 919 ↑

Q920 INTERMEDIATE · Block M

Best answer: E. Caldicott principles

Justification: Caldicott principles.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.4 · LO 3a.5

Return to Question 920 ↑

Q923 INTERMEDIATE · Block M

Best answer: E. Employer, referrer, practitioner and operator

Justification: Employer, referrer, practitioner, operator.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.5, Unit 5 · LO 3a.6

Return to Question 923 ↑

Q924 INTERMEDIATE · Block M

Best answer: C. Rectangular collimation, film holders, the fastest suitable receptor, appropriate exposure settings, and image-quality audit

Justification: Rectangular collimation, holders, fast receptor, QA audit.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.5 · LO 3a.6

Return to Question 924 ↑

Q926 INTERMEDIATE · Block M

Best answer: E. A type B (vacuum) steriliser

Justification: A type B vacuum steriliser. It is needed for wrapped and hollow instruments.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · LO 3a.4

Return to Question 926 ↑

Q929 INTERMEDIATE · Block M

Best answer: E. Discuss reasonable alternatives, benefits, risks and maintenance in language the patient understands

Justification: Explain reasonable alternatives and material risks and burdens, including long-term cleaning and maintenance.

Why not the closest alternatives: Consent is not meaningful if reasonable options are withheld.

Revise: Volume I · Volume I Unit 13 · Diploma LO 3a.1–3a.12; 3b.1–3b.5

Return to Question 929 ↑

Q930 INTERMEDIATE · Block M

Best answer: D. About 2 minutes at the start of the day and 20–30 seconds between patients, with a biocide

Justification: 2 minutes at the start, 20–30 seconds between patients.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · LO 3a.4

Return to Question 930 ↑

Q931 INTERMEDIATE · Block M

Best answer: A. Encourage bleeding, wash with soap and water without scrubbing, cover, report, and contact occupational health urgently (HIV PEP ideally within 1 hour)

Justification: Bleed, wash, cover, report, occupational health urgently. HIV PEP ideally within 1 hour, up to 72 hours.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.6 · LO 3a.4

Return to Question 931 ↑

Q933 INTERMEDIATE · Block M

Best answer: C. Recognise, record and refer

Justification: Recognise, record, refer. Do not investigate yourself.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.7 · LO 3a.10

Return to Question 933 ↑

Q934 INTERMEDIATE · Block M

Best answer: B. Report the known case to the police

Justification: Report to the police. It is mandatory for known FGM in under-18s in England and Wales.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.7 · LO 3a.10

Return to Question 934 ↑

Q936 INTERMEDIATE · Block M

Best answer: C. Clinical effectiveness and audit, risk management, education and training, patient involvement, information management, staff management and research

Justification: The seven pillars.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 936 ↑

Q938 INTERMEDIATE · Block M

Best answer: D. Re-audit

Justification: Re-audit.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 938 ↑

Q939 INTERMEDIATE · Block M

Best answer: E. Plan–Do–Study–Act (PDSA) cycles

Justification: PDSA cycles.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 939 ↑

Q940 INTERMEDIATE · Block M

Best answer: A. A systematic review and meta-analysis of RCTs

Justification: A systematic review and meta-analysis of RCTs.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 940 ↑

Q941 INTERMEDIATE · Block M

Best answer: D. Stabilise active disease, explain the increased risks, obtain appropriate investigations and refer for multidisciplinary specialist planning

Justification: Level 2 competence includes recognising complexity beyond one's scope. Disease control, shared planning and appropriately timed referral provide safer care than proceeding with complex irreversible interventions.

Why not the closest alternatives: Consent does not eliminate an operator's duty to practise within competence. Interim disease control and supportive care may remain appropriate.

Revise: Volume I, Unit 13 · Diploma LO 2a.14; 2b.1; 3a.2 · Reference themes: NHS England clinical standards Level 2 and Level 3

Return to Question 941 ↑

Q942 INTERMEDIATE · Block M

Best answer: E. Obtain renewed informed consent when the agreed plan materially changes

Justification: A major change in treatment requires renewed discussion and valid informed consent.

Why not the closest alternatives: Consent for a crown does not automatically authorise extraction.

Revise: Volume I · Volume I Unit 13 · Diploma LO 3a.1–3a.12; 3b.1–3b.5

Return to Question 942 ↑

Q943 INTERMEDIATE · Block M

Best answer: A. It falls

Justification: PPV falls with low prevalence.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.12

Return to Question 943 ↑

Q945 INTERMEDIATE · Block M

Best answer: E. Audit an identified pattern of clinical failure and implement an improvement cycle

Justification: Review and audit the observed failure trend, investigate processing changes, agree corrective measures and re-audit outcomes.

Why not the closest alternatives: Anecdotal reassurance alone does not establish a safe process.

Revise: Volume I · Volume I Unit 13 · Diploma LO 3a.1–3a.12; 3b.1–3b.5

Return to Question 945 ↑

Q946 INTERMEDIATE · Block M

Best answer: A. CQC (England), Healthcare Improvement Scotland, Healthcare Inspectorate Wales, RQIA (Northern Ireland)

Justification: CQC, HIS, HIW, RQIA. The GDC is the professional regulator.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.8 · LO 3a.2

Return to Question 946 ↑

Q948 INTERMEDIATE · Block M

Best answer: C. SPIKES: setting, perception, invitation, knowledge, emotions, strategy and summary

Justification: SPIKES.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 §13.9 · LO 3b.3

Return to Question 948 ↑

Q949 INTERMEDIATE · Block M

Best answer: C. Disclose and manage significant errors transparently with appropriate documentation

Justification: Act to safeguard the patient, disclose the event candidly, document it and follow local governance requirements.

Why not the closest alternatives: Concealment and waiting for symptoms are unacceptable.

Revise: Volume I · Volume I Unit 13 · Diploma LO 3a.1–3a.12; 3b.1–3b.5

Return to Question 949 ↑

Q950 INTERMEDIATE · Block M

Best answer: C. Document the patient's informed choice, alternatives including no treatment, expected consequences, and a risk-based review plan

Justification: Valid consent and shared decision-making include accepting an informed decision not to replace a missing tooth. Document capacity, discussion of material risks and alternatives, the patient's goals and a review plan.

Why not the closest alternatives: A signed form without dialogue is not a substitute for informed choice; the least invasive reasonable option can be no replacement in a stable shortened dental arch.

Revise: Volume I, Unit 13 · Diploma LO 3a.2; 3b.1; 2b.3 · Reference themes: 2020 RCS England MPros; GDC consent standards

Return to Question 950 ↑

BLOCK N · Integrated cases

Q951 INTERMEDIATE · Block N

Best answer: C. Root-treat and retain the root as an overdenture abutment, if feasible, rather than extracting

Justification: Retain the root as an overdenture abutment. Over 5 years of oral bisphosphonate raises MRONJ risk, so root retention beats extraction; apixaban is not stopped.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 6, 7 · LO 2b.11, 2c.9

Return to Question 951 ↑

Q952 INTERMEDIATE · Block N

Best answer: A. Liaise with her GMP about the reflux, give preventive advice, and monitor before any restoration

Justification: Manage the cause and monitor. Stabilise reflux erosion before any irreversible veneers.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 8, 10 · LO 2c.10

Return to Question 952 ↑

Q953 INTERMEDIATE · Block N

Best answer: E. Assess capacity, act in her best interests with carer involvement, and provide simple conventional dentures with labelling

Justification: Capacity, best interests, simple dentures, labelling. Copying is impossible because the dentures are lost.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 7, 13 · LO 3b.2, 2c.9

Return to Question 953 ↑

Q954 INTERMEDIATE · Block N

Best answer: B. Remove the allergen, call for help, and give adrenaline 1:1000, 500 micrograms IM

Justification: Remove the allergen; adrenaline 500 micrograms IM. Anaphylaxis needs IM adrenaline first.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 13 · LO 2b.1

Return to Question 954 ↑

Q955 INTERMEDIATE · Block N

Best answer: D. Peri-implant mucositis from residual cement: remove the cement, debride, give OHI and review (Level 2)

Justification: Mucositis from residual cement; Level 2. Remove the cement and debride; there is no bone loss.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 11 · LO 2c.21

Return to Question 955 ↑

Q956 INTERMEDIATE · Block N

Best answer: B. Refer for specialist assessment because of the lifelong osteoradionecrosis risk

Justification: Specialist referral. Osteoradionecrosis risk is lifelong; avoid extraction in the field where possible.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 6 · LO 2b.11

Return to Question 956 ↑

Q957 INTERMEDIATE · Block N

Best answer: E. Approach with empathy, seek her consent to liaise with her GMP, and focus on prevention and monitoring before restoration

Justification: Empathy, consented liaison, prevention first. She is an adult, so her confidentiality is respected.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 10, 13 · LO 2c.10, 3a.5

Return to Question 957 ↑

Q958 INTERMEDIATE · Block N

Best answer: A. Denture hygiene, night removal, soaking, correcting the fit, and nystatin if it persists

Justification: Hygiene, night removal, fit; nystatin if needed. Azoles interact with warfarin and simvastatin.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 4 · LO 1.8, 1.7

Return to Question 958 ↑

Q959 INTERMEDIATE · Block N

Best answer: D. Review the occlusion and design, and discuss alternatives such as a conventional bridge, an implant or an RPD

Justification: Review the occlusion and design; discuss alternatives. Repeated debonding in a bruxist with heavy contact means the design must change.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 8, 12 · LO 2c.22

Return to Question 959 ↑

Q960 INTERMEDIATE · Block N

Best answer: C. A diagnostic wax-up and provisional trial at the new OVD, with referral for Level 3 planning

Justification: Wax-up, provisional trial, Level 3. Generalised wear with an OVD increase is reorganised rehabilitation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 9, 10 · LO 2c.11

Return to Question 960 ↑

Q961 INTERMEDIATE · Block N

Best answer: D. Co-Cr, tooth-supported, with a mesial rest and RPI on the terminal abutment, a lingual bar, an open design and maximum saddle extension

Justification: Tooth-supported Co-Cr with RPI, lingual bar and open design. A 9 mm sulcus allows a bar; high caries risk favours minimal coverage.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 6, 10 · LO 2c.3, 2c.16

Return to Question 961 ↑

Q962 INTERMEDIATE · Block N

Best answer: C. Trim the distobuccal flange and thin the posterior border, preserving the post-dam seal

Justification: Trim the distobuccal flange and thin the posterior border. Keep the post-dam seal.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 7 · LO 2c.21

Return to Question 962 ↑

Q963 INTERMEDIATE · Block N

Best answer: A. Monitor and seek multidisciplinary specialist advice

Justification: Monitor with multidisciplinary advice; defer implants. Ankylosed teeth cannot be extruded orthodontically.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 10, 11 · LO 2a.18

Return to Question 963 ↑

Q964 INTERMEDIATE · Block N

Best answer: E. A well-fitting conventional denture with relief over bony prominences

Justification: A well-fitting conventional denture. High-dose IV antiresorptives carry high MRONJ risk, so implants are generally avoided.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 7, 11 · LO 2b.11

Return to Question 964 ↑

Q965 INTERMEDIATE · Block N

Best answer: B. Periodontal therapy and referral to the GMP for diabetic control first

Justification: Periodontal therapy and diabetic control first. Definitive work waits for stability.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 4, 5, 10 · LO 2b.3, 2a.13

Return to Question 965 ↑

Q966 INTERMEDIATE · Block N

Best answer: A. Prompt cuspal coverage (onlay or crown) in metal or monolithic zirconia, plus a night guard

Justification: Prompt cuspal coverage, monolithic material, night guard.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 3, 8, 9 · LO 2c.14

Return to Question 966 ↑

Q967 INTERMEDIATE · Block N

Best answer: D. Contact her, explain, apologise, offer a remedy, report it as a never event and document everything

Justification: Candour, apology, remedy, never-event report, documentation.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3a.7

Return to Question 967 ↑

Q969 INTERMEDIATE · Block N

Best answer: E. Plain or aluminium chloride cord, with the margin just subgingival and about 3 mm from the bone crest

Justification: Cord retraction, margin about 3 mm from the crest. Avoid electrosurgery with an ICD; respect the supracrestal attachment.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 8 · LO 2c.13

Return to Question 969 ↑

Q970 INTERMEDIATE · Block N

Best answer: B. Anteroposterior strap avoiding the torus

Justification: Anteroposterior strap.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 6 · LO 2c.3

Return to Question 970 ↑

Q971 INTERMEDIATE · Block N

Best answer: E. Record your findings and refer through the practice safeguarding lead to adult social care

Justification: Record and refer via the safeguarding lead. Do not investigate yourself.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 13 · LO 3a.10

Return to Question 971 ↑

Q972 INTERMEDIATE · Block N

Best answer: B. Interim RBBs or an RPD, with referral for multidisciplinary Level 3 care

Justification: Interim prostheses and Level 3 referral. Hypodontia is complex multidisciplinary care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 10, 11 · LO 2b.10

Return to Question 972 ↑

Q973 INTERMEDIATE · Block N

Best answer: D. Encourage coughing

Justification: Encourage coughing; image if not retrieved. Back blows are only for ineffective coughing.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 12 · LO 2b.1

Return to Question 973 ↑

Q974 INTERMEDIATE · Block N

Best answer: C. Refer as peri-implantitis (Level 3)

Justification: Peri-implantitis: refer.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 10, 11 · LO 2c.21

Return to Question 974 ↑

Q975 INTERMEDIATE · Block N

Best answer: A. Explain that reversible care comes first (self-care and a stabilisation splint), and do not equilibrate to treat TMD

Justification: Reversible care first; no equilibration for TMD.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 · LO 2a.20

Return to Question 975 ↑

Q976 INTERMEDIATE · Block N

Best answer: C. Pressure on the mental nerve

Justification: Mental nerve pressure. Symptoms that resolve when the denture is removed point to pressure; relieve the fitting surface.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 1, 7 · LO 2c.21

Return to Question 976 ↑

Q977 INTERMEDIATE · Block N

Best answer: A. Fluoride gel in the fitting surface of the denture, with meticulous hygiene and a diet review

Justification: Fluoride gel in the denture, hygiene, diet.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 7 · LO 2c.9

Return to Question 977 ↑

Q978 INTERMEDIATE · Block N

Best answer: B. A selective-pressure (windowed) impression, with balanced occlusion avoiding heavy anterior contact

Justification: Selective pressure, balanced occlusion, light anterior contact. This limits combination-syndrome-type loading.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 7, 11 · LO 2c.9

Return to Question 978 ↑

Q979 INTERMEDIATE · Block N

Best answer: D. High-fluoride toothpaste, a neutral saliva substitute, an open RPD design and a short recall

Justification: High fluoride, neutral substitute, open design, short recall.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 6 · LO 2c.3

Return to Question 979 ↑

Q984 INTERMEDIATE · Block N

Best answer: C. Act in her best interests, consulting those close to her and choosing the least restrictive option, unless she regains capacity

Justification: Best interests, least restrictive option.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 13 · LO 3b.2

Return to Question 984 ↑

Q985 INTERMEDIATE · Block N

Best answer: E. Request the statement and confirm the laboratory is MHRA-registered before fitting

Justification: Statement of manufacture and MHRA registration.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 8, 13 · LO 3a.3

Return to Question 985 ↑

Q987 INTERMEDIATE · Block N

Best answer: C. Remove it, treat the caries, and redesign with a non-rigid connector at the pier or as separate bridges

Justification: Remove, treat, redesign at the pier.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 8, 12 · LO 2c.22

Return to Question 987 ↑

Q988 INTERMEDIATE · Block N

Best answer: D. Dehydration and poor lighting

Justification: Dehydration and poor lighting.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 · LO 2c.13

Return to Question 988 ↑

Q989 INTERMEDIATE · Block N

Best answer: B. Liaise with the haemophilia centre, avoiding regional blocks without factor cover

Justification: Haemophilia centre liaison.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 4, 7 · LO 2b.11

Return to Question 989 ↑

Q990 INTERMEDIATE · Block N

Best answer: A. Proceed with standard care and no drug holiday; review healing

Justification: Standard care, no drug holiday. Low-risk MRONJ profile.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 · LO 2b.11

Return to Question 990 ↑

Q993 INTERMEDIATE · Block N

Best answer: C. A single-wing resin-bonded cantilever from 13

Justification: Single-wing resin-bonded cantilever.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 8 · LO 2c.11

Return to Question 993 ↑

Q994 INTERMEDIATE · Block N

Best answer: A. Provide posterior support with a lower RPD, and use a selective-pressure impression for the flabby ridge

Justification: Posterior support and selective pressure. That manages combination syndrome.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 6, 7 · LO 2c.9

Return to Question 994 ↑

Q995 INTERMEDIATE · Block N

Best answer: D. Same-day medical referral for suspected giant cell arteritis

Justification: Same-day referral for suspected GCA.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 9 · LO 2a.20

Return to Question 995 ↑

Q996 INTERMEDIATE · Block N

Best answer: D. Liaise with the physician and defer elective treatment until he is medically stable

Justification: Liaise and defer elective care.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Unit 4 · LO 2b.12

Return to Question 996 ↑

Q998 INTERMEDIATE · Block N

Best answer: E. Urgent referral on the suspected cancer pathway

Justification: Urgent suspected cancer referral.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 2, 7 · LO 1.11

Return to Question 998 ↑

Q1000 INTERMEDIATE · Block N

Best answer: C. Risk-based recall with supportive periodontal and peri-implant care, prosthesis maintenance, and recording of patient-reported outcomes

Justification: Risk-based recall with maintenance and PROMs.

Distractor-by-distractor analysis was not provided in the source bank.

Revise: Units 10, 11, 12 · LO 2c.27

Return to Question 1000 ↑

Advanced answers · 500 items

BLOCK A · Applied anatomy

Q1088 ADVANCED · Block A

Best answer: E. Refer immediately for urgent airway and maxillofacial management

Justification: A spreading floor-of-mouth infection threatening the airway is an emergency.

Why not the closest alternatives: A local drainage or antibiotic-only outpatient plan may delay airway protection.

Revise: Diploma LO 1.1, 1.5, 2a.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anatomy changes the urgency of care

Return to Question 1088 ↑

Q1089 ADVANCED · Block A

Best answer: D. Manage localised swelling after appropriate assessment and source control

Justification: Local source control with appropriate operative assessment is the appropriate route for an uncomplicated localised dental infection.

Why not the closest alternatives: Hospital-level airway response is not indicated in an otherwise stable localised presentation.

Revise: Diploma LO 1.1, 1.5, 2a.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anatomy changes the urgency of care

Return to Question 1089 ↑

Q1090 ADVANCED · Block A

Best answer: D. Seek urgent assessment for a progressive cranial neuropathy

Justification: Unexplained progressive sensory disturbance requires prompt investigation for neurological or other significant disease.

Why not the closest alternatives: Routine denture relief should not substitute for evaluating a red flag.

Revise: Diploma LO 1.1, 1.5, 2a.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anatomy changes the urgency of care

Return to Question 1090 ↑

Q1091 ADVANCED · Block A

Best answer: B. Assess lingual flange extension against mylohyoid and tongue movements

Justification: Mylohyoid and tongue activity control the posterior lingual border; excessive extension produces functional displacement.

Why not the closest alternatives: Occlusion cannot explain a reproducible displacement with swallowing alone.

Revise: Diploma LO 1.1, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Functional denture border anatomy

Return to Question 1091 ↑

Q1092 ADVANCED · Block A

Best answer: D. Assess the distobuccal flange against masseter action

Justification: The masseteric notch and masseter activity shape the posterior buccal flange.

Why not the closest alternatives: Tongue-related lingual extension is not the principal observation here.

Revise: Diploma LO 1.1, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Functional denture border anatomy

Return to Question 1092 ↑

Q1093 ADVANCED · Block A

Best answer: C. Assess posterior maxillary flange interference from the coronoid process

Justification: The mandibular coronoid process can interfere with an overbulked maxillary distobuccal flange during opening.

Why not the closest alternatives: Changes to mandibular occlusion are not the first explanation.

Revise: Diploma LO 1.1, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Functional denture border anatomy

Return to Question 1093 ↑

Q1157 ADVANCED · Block A

Best answer: C. Identify the mental foramen before relieving a severely resorbed lower denture

Justification: Severe atrophy may bring the mental foramen near the denture-bearing crest; pressure can cause altered sensation.

Why not the closest alternatives: An occlusal adjustment does not address the likely nerve pressure directly.

Revise: Diploma LO 1.1, 2a.5, 2c.6 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anatomical limits in a prosthetic plan

Return to Question 1157 ↑

Q1158 ADVANCED · Block A

Best answer: A. Use functional border moulding to register a masseteric notch

Justification: A masseteric notch in the distobuccal extension permits muscle function without dislodgement.

Why not the closest alternatives: Arbitrary posterior palatal modifications are unrelated to a mandibular flange.

Revise: Diploma LO 1.1, 2a.5, 2c.6 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anatomical limits in a prosthetic plan

Return to Question 1158 ↑

Q1159 ADVANCED · Block A

Best answer: A. Assess the hamular notches and vibrating lines for a posterior palatal seal

Justification: Hamular notch anatomy and functional vibrating lines are essential to a correctly positioned posterior palatal seal.

Why not the closest alternatives: Overextending blindly can cause gagging or instability.

Revise: Diploma LO 1.1, 2a.5, 2c.6 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anatomical limits in a prosthetic plan

Return to Question 1159 ↑

Q1181 ADVANCED · Block A

Best answer: C. Relieve the denture over the mental foramen and review symptoms promptly

Justification: The anatomical change brings the mental nerve close to the denture-bearing crest, and a reproducible local pressure symptom merits prompt relief and re-evaluation.

Why not the closest alternatives: Persistent numbness in the absence of denture pressure would require investigation rather than routine adjustment.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.5

Return to Question 1181 ↑

Q1182 ADVANCED · Block A

Best answer: D. Investigate the lingual flange against dynamic tongue movement and adjust it

Justification: Dynamic tongue function identifies a functional-border problem, so evaluate and correct the offending lingual extension rather than arbitrarily altering the bite.

Why not the closest alternatives: A posterior palatal-seal assessment is irrelevant to a lower denture.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.5

Return to Question 1182 ↑

Q1183 ADVANCED · Block A

Best answer: A. Arrange an urgent assessment of unexplained persistent sensory change

Justification: Unexplained persistent sensory loss with local asymmetry is a red flag for pathology; further clinical investigation and urgent referral take priority.

Why not the closest alternatives: Relieving a non-tender denture risks delaying a significant diagnosis.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.5

Return to Question 1183 ↑

Q1184 ADVANCED · Block A

Best answer: A. Assess a distobuccal flange against coronoid interference

Justification: Coronoid movement can impinge upon the maxillary distobuccal flange; assess that movement before remaking a well-fitting prosthesis.

Why not the closest alternatives: Posterior palatal-seal changes do not directly correct a movement-induced distal buccal interference.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.5

Return to Question 1184 ↑

Q1185 ADVANCED · Block A

Best answer: E. Arrange emergency admission for airway assessment and infection control

Justification: The combination suggests spreading floor-of-mouth infection and threatened airway; immediate hospital escalation is essential.

Why not the closest alternatives: Office drainage or delayed radiography must not postpone airway management.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.13

Return to Question 1185 ↑

Q1186 ADVANCED · Block A

Best answer: A. Drain a localised fluctuant abscess and address the cause if appropriate within competence

Justification: Source control and appropriately delivered local drainage are central when infection remains local and the practitioner is competent.

Why not the closest alternatives: Emergency transfer is reserved for spreading or systemic risk, not every minor local collection.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.13

Return to Question 1186 ↑

Q1187 ADVANCED · Block A

Best answer: D. Obtain targeted imaging and refer for suspected deep-space infection

Justification: Possible submasseteric spread and significant trismus need urgent maxillofacial investigation; a superficial denture adjustment cannot resolve it.

Why not the closest alternatives: Do not treat a possible deep-space abscess as a straightforward localised mucosal swelling.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.13

Return to Question 1187 ↑

Q1188 ADVANCED · Block A

Best answer: D. Arrange review after adjustment of a traumatic denture flange

Justification: A clearly mechanical mucosal injury can initially be treated by removing the cause and arranging timely reassessment.

Why not the closest alternatives: If the lesion persists or new red flags develop, the management pathway changes to investigation/referral.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 1.5, 2a.13

Return to Question 1188 ↑

Q1189 ADVANCED · Block A

Best answer: D. Record the retromylohyoid space during functional moulding

Justification: The functional posterolingual sulcus contributes to mandibular border seal; appropriate recording and extension can improve stability.

Why not the closest alternatives: Adding a post-dam is an upper-denture intervention, not the answer here.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 2a.6, 2c.21

Return to Question 1189 ↑

Q1190 ADVANCED · Block A

Best answer: D. Relieve a flange interfering with the masseteric notch during clenching

Justification: Masseter action narrows the distobuccal sulcus; the masseteric notch must accommodate movement without impingement.

Why not the closest alternatives: Altering the lingual extension would miss the location and trigger of symptoms.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 2a.6, 2c.21

Return to Question 1190 ↑

Q1191 ADVANCED · Block A

Best answer: A. Assess and correct over-extension against the coronoid process

Justification: The coronoid process can displace an over-contoured maxillary distobuccal flange on opening.

Why not the closest alternatives: Posterior palatal seal adjustment would not address the movement-specific buccal collision.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 2a.6, 2c.21

Return to Question 1191 ↑

Q1192 ADVANCED · Block A

Best answer: E. Reassess neutral-zone and polished-surface contour

Justification: The polished surfaces must balance tongue and perioral muscle actions; correcting neutral-zone contours is more appropriate than extending already adequate borders.

Why not the closest alternatives: A reline alone does not correct a denture whose external contour is dynamically unfavourable.

Revise: Volume I — Unit 1 · Diploma LO 1.1, 2a.6, 2c.21

Return to Question 1192 ↑

BLOCK B · Physiology and pathology

Q1013 ADVANCED · Block B

Best answer: A. Investigate the medicines with the prescriber and institute saliva and caries prevention

Justification: Medication-associated hyposalivation requires coordinated medicines review plus intensive prevention, not only prosthesis repair.

Why not the closest alternatives: Crowns are not a primary response to an uncontrolled high-caries environment.

Revise: Diploma LO 1.2, 1.8, 2c.1 · Volume I and Diploma learning outcomes (August 2026) · Theme: Saliva and risk adaptation

Return to Question 1013 ↑

Q1014 ADVANCED · Block B

Best answer: A. Investigate for systemic causes and manage oral disease while awaiting results

Justification: The combination requires assessment for systemic causes while oral prevention and symptom care start promptly.

Why not the closest alternatives: A denture reline or acidic stimulants would neither explain nor safely address the presentation.

Revise: Diploma LO 1.2, 1.8, 2c.1 · Volume I and Diploma learning outcomes (August 2026) · Theme: Saliva and risk adaptation

Return to Question 1014 ↑

Q1015 ADVANCED · Block B

Best answer: B. Investigate the medicines with the prescriber and institute saliva and caries prevention

Justification: The major limiting factor is lack of a lubricating saliva film; manage xerostomia and review denture function rather than automatically relining.

Why not the closest alternatives: A hard reline cannot recreate missing saliva or cure mucosal dryness.

Revise: Diploma LO 1.2, 1.8, 2c.1 · Volume I and Diploma learning outcomes (August 2026) · Theme: Saliva and risk adaptation

Return to Question 1015 ↑

Q1016 ADVANCED · Block B

Best answer: D. Suspect intrinsic acid exposure and address the medical cause sensitively

Justification: The pattern and systemic clues raise intrinsic gastric acid exposure; a sensitive medical history and prevention are central.

Why not the closest alternatives: A mechanical diagnosis does not explain the palatal distribution and associated signs.

Revise: Diploma LO 1.4, 1.8, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Diagnosing tooth surface loss

Return to Question 1016 ↑

Q1017 ADVANCED · Block B

Best answer: C. Identify attrition linked to tooth contact and assess function

Justification: Matching wear facets and parafunctional fracture support tooth-to-tooth attrition.

Why not the closest alternatives: Acid erosion is not the best explanation for matching contact facets.

Revise: Diploma LO 1.4, 1.8, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Diagnosing tooth surface loss

Return to Question 1017 ↑

Q1018 ADVANCED · Block B

Best answer: C. Identify extrinsic acid erosion and counsel on beverage frequency

Justification: Frequent extrinsic acidic exposure is a plausible primary driver; frequency reduction is essential before restoration.

Why not the closest alternatives: Simply placing restorative coverage will not address ongoing erosion.

Revise: Diploma LO 1.4, 1.8, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Diagnosing tooth surface loss

Return to Question 1018 ↑

Q1094 ADVANCED · Block B

Best answer: B. Coordinate assessment of glycaemic control before complex elective care

Justification: Systemic and periodontal stabilisation materially influence surgical and maintenance prognosis.

Why not the closest alternatives: An elective complex plan should not bypass glycaemic assessment and disease control.

Revise: Diploma LO 1.3, 1.11, 2a.3 · Volume I and Diploma learning outcomes (August 2026) · Theme: Systemic modifiers of prognosis

Return to Question 1094 ↑

Q1095 ADVANCED · Block B

Best answer: E. Investigate haematinic deficiency alongside oral findings

Justification: Iron, B12 or folate deficiency can cause oral symptoms and merits appropriate medical investigation.

Why not the closest alternatives: Assuming all symptoms originate from the denture overlooks systemic disease.

Revise: Diploma LO 1.3, 1.11, 2a.3 · Volume I and Diploma learning outcomes (August 2026) · Theme: Systemic modifiers of prognosis

Return to Question 1095 ↑

Q1096 ADVANCED · Block B

Best answer: A. Prioritise an adaptable low-maintenance prosthetic approach

Justification: A simple, repairable, hygienic prosthesis with achievable maintenance is often preferable to elaborate alternatives.

Why not the closest alternatives: Technical sophistication alone does not improve real-world outcomes.

Revise: Diploma LO 1.3, 1.11, 2a.3 · Volume I and Diploma learning outcomes (August 2026) · Theme: Systemic modifiers of prognosis

Return to Question 1096 ↑

Q1193 ADVANCED · Block B

Best answer: D. Coordinate medication review and intensive caries/xerostomia prevention

Justification: Medication-associated salivary reduction drives root caries; coordinate medication review and protect the remaining dentition before prosthetic replacement.

Why not the closest alternatives: Restoring isolated defects without controlling the disease environment invites early recurrence.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 1.8, 2b.3

Return to Question 1193 ↑

Q1194 ADVANCED · Block B

Best answer: E. Manage suspected intrinsic acid exposure through sensitive medical liaison and prevention

Justification: Suspected gastric acid exposure requires sensitive assessment, appropriate medical care and preventive measures before definitive restorations.

Why not the closest alternatives: Abrasive wear or isolated bruxism would not explain the distribution as well.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 1.8, 2b.3

Return to Question 1194 ↑

Q1195 ADVANCED · Block B

Best answer: B. Apply evidence-based prevention and monitor stable tooth wear before intervening

Justification: Stable, asymptomatic tooth wear is primarily monitored with risk control and baseline records rather than aggressive reconstruction.

Why not the closest alternatives: The presence of wear alone does not justify irreversible crown preparation.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 1.8, 2b.3

Return to Question 1195 ↑

Q1196 ADVANCED · Block B

Best answer: C. Urgently investigate a persistent suspicious mucosal lesion

Justification: A non-resolving suspicious mucosal lesion needs urgent assessment; prosthodontic treatment should not delay a cancer pathway.

Why not the closest alternatives: Presuming simple trauma despite persistence and induration is unsafe.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 1.8, 2b.3

Return to Question 1196 ↑

Q1197 ADVANCED · Block B

Best answer: A. Reduce between-meal sugar exposure and strengthen fluoride-based caries control

Justification: Repeated sugar challenges in a vulnerable root environment are a principal preventable driver; frequency reduction plus fluoride protection is central.

Why not the closest alternatives: Repeated fillings alone cannot overcome unchanged risk behaviour.

Revise: Volume I — Unit 2 · Diploma LO 1.4, 1.8, 2c.1

Return to Question 1197 ↑

Q1198 ADVANCED · Block B

Best answer: C. Modify frequent dietary acid exposure and postpone elective bonding until stabilised

Justification: Frequent extrinsic acid exposure explains the lesion pattern; reduce exposure and protect tooth surfaces before definitive adhesive work.

Why not the closest alternatives: Caries-focused sugar advice alone does not adequately address erosive acidity.

Revise: Volume I — Unit 2 · Diploma LO 1.4, 1.8, 2c.1

Return to Question 1198 ↑

Q1199 ADVANCED · Block B

Best answer: C. Introduce a personalised denture hygiene and night-removal programme

Justification: Hygiene, appropriate cleaning and overnight removal address a likely denture-associated inflammatory cause; check fit and review.

Why not the closest alternatives: Antifungals without correcting denture practices often produce relapse.

Revise: Volume I — Unit 2 · Diploma LO 1.4, 1.8, 2c.1

Return to Question 1199 ↑

Q1200 ADVANCED · Block B

Best answer: B. Assess a possible eating disorder sensitively and coordinate appropriate support

Justification: Offer a confidential, empathetic discussion and coordinated care for potential intrinsic acid exposure; avoid confrontation.

Why not the closest alternatives: An immediate irreversible restorative plan would miss the possible underlying health problem.

Revise: Volume I — Unit 2 · Diploma LO 1.4, 1.8, 2c.1

Return to Question 1200 ↑

Q1201 ADVANCED · Block B

Best answer: B. Diagnose probable denture stomatitis and address hygiene and fit

Justification: Distribution and continuous wear support denture-associated stomatitis. Address prosthesis hygiene, fit, and relevant medical factors.

Why not the closest alternatives: Diffuse denture-contact erythema is not by itself a reason for urgent cancer referral.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 2a.7, 2c.21

Return to Question 1201 ↑

Q1202 ADVANCED · Block B

Best answer: C. Assess haematinic and systemic factors contributing to angular cheilitis

Justification: Angular cheilitis may reflect local moisture/vertical-dimension factors as well as iron, B12 or folate deficiency; assess both.

Why not the closest alternatives: Treating a single microbe without examining predisposing factors is incomplete.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 2a.7, 2c.21

Return to Question 1202 ↑

Q1203 ADVANCED · Block B

Best answer: C. Refer an unexplained persistent ulcer for urgent assessment

Justification: A non-healing, indurated lesion warrants urgent assessment rather than repeated adjustment or prolonged observation.

Why not the closest alternatives: Do not attribute an evolving suspicious lesion to prosthesis trauma indefinitely.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 2a.7, 2c.21

Return to Question 1203 ↑

Q1204 ADVANCED · Block B

Best answer: E. Provide a non-compressive impression approach for a mobile ridge

Justification: Mucostatic or selective-pressure records reduce distortion of displaceable mucosa and improve tissue adaptation.

Why not the closest alternatives: An aggressively compressive final impression records tissue in a non-resting position.

Revise: Volume I — Unit 2 · Diploma LO 1.2, 2a.7, 2c.21

Return to Question 1204 ↑

Q1205 ADVANCED · Block B

Best answer: C. Accept a shortened dental arch and monitor function without replacing every molar

Justification: A functional shortened dental arch is often acceptable; replacement should be guided by need rather than an arbitrary full-tooth-count target.

Why not the closest alternatives: An extensive posterior bridge would introduce risk without a patient-centred benefit.

Revise: Volume I — Unit 2 · Diploma LO 1.11, 2b.4, 2c.4

Return to Question 1205 ↑

Q1206 ADVANCED · Block B

Best answer: C. Use a carefully adapted copy-denture approach to preserve familiar contours

Justification: Copy techniques can preserve familiar tooth positions and polished surfaces while improving fit, reducing adaptation demands.

Why not the closest alternatives: A fully redesigned prosthesis may be more difficult for a patient with neuromuscular limitations.

Revise: Volume I — Unit 2 · Diploma LO 1.11, 2b.4, 2c.4

Return to Question 1206 ↑

Q1207 ADVANCED · Block B

Best answer: B. Assess implant-retained overdenture suitability when conventional mandibular retention is inadequate

Justification: Implant-retained overdenture assessment may improve lower prosthesis retention where suitable; assess anatomy, risk, cost, maintenance and referral needs.

Why not the closest alternatives: Yet another conventional reline may not solve severe anatomical retention problems.

Revise: Volume I — Unit 2 · Diploma LO 1.11, 2b.4, 2c.4

Return to Question 1207 ↑

Q1208 ADVANCED · Block B

Best answer: A. Evaluate a removable partial denture for an actual chewing or appearance deficit

Justification: A carefully designed RPD can be appropriate when actual function is impaired and patient preferences exclude implants.

Why not the closest alternatives: Shortened dental arch is not an absolute reason never to replace posterior teeth.

Revise: Volume I — Unit 2 · Diploma LO 1.11, 2b.4, 2c.4

Return to Question 1208 ↑

BLOCK C · Dental materials science

Q1019 ADVANCED · Block C

Best answer: D. Etch a silica-based ceramic with an appropriate HF protocol and silanise before resin bonding

Justification: Glass ceramics are silica-based and use a validated HF etch and silane protocol under correct safety controls.

Why not the closest alternatives: Zirconia primers target a different substrate; generic GIC is not ideal for adhesive veneers.

Revise: Diploma LO 1.9, 2c.6, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Adhesive material choice

Return to Question 1019 ↑

Q1020 ADVANCED · Block C

Best answer: D. Air-abrade zirconia and use an appropriate phosphate-monomer primer or cement

Justification: Zirconia is polycrystalline and benefits from controlled air abrasion and MDP-based chemical adhesion.

Why not the closest alternatives: Routine glass-ceramic HF treatment is ineffective for zirconia.

Revise: Diploma LO 1.9, 2c.6, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Adhesive material choice

Return to Question 1020 ↑

Q1021 ADVANCED · Block C

Best answer: C. Reassess isolation and ferrule before selecting any bonding protocol

Justification: Fundamental restorability, ferrule and a manageable margin are prerequisites to choosing a bonding system.

Why not the closest alternatives: No adhesive chemistry compensates reliably for unmanageable isolation and lack of residual structure.

Revise: Diploma LO 1.9, 2c.6, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Adhesive material choice

Return to Question 1021 ↑

Q1022 ADVANCED · Block C

Best answer: C. Repeat the impression using a rigid tray and correct material support

Justification: Tray flexure and inadequate wash support can distort dies and margins; remake the impression accurately.

Why not the closest alternatives: Occlusal adjustment cannot correct a gross marginal inaccuracy.

Revise: Diploma LO 1.9, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Impression selection and error

Return to Question 1022 ↑

Q1023 ADVANCED · Block C

Best answer: C. Use a highly hydrophilic impression material with appropriate moisture control

Justification: Accurate moisture-tolerant material selection and field control improve wetting; poor haemostasis must still be corrected.

Why not the closest alternatives: Prolonged indiscriminate immersion can distort materials and is not an impression technique.

Revise: Diploma LO 1.9, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Impression selection and error

Return to Question 1023 ↑

Q1024 ADVANCED · Block C

Best answer: D. Use a selective-pressure technique to avoid displacing the flabby tissue

Justification: A selective-pressure or window technique records mobile tissue without compressing it into a misleading position.

Why not the closest alternatives: A rigid tray alone does not solve the problem of tissue displacement.

Revise: Diploma LO 1.9, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Impression selection and error

Return to Question 1024 ↑

Q1025 ADVANCED · Block C

Best answer: E. Identify fatigue from repeated flexure and redesign support or reinforcement

Justification: Repeated subcritical cyclic loading is characteristic of fatigue; assess fit, geometry, occlusion and reinforcement.

Why not the closest alternatives: Impact is unlikely without an acute event.

Revise: Diploma LO 1.9, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Failure mechanism in dental materials

Return to Question 1025 ↑

Q1026 ADVANCED · Block C

Best answer: B. Identify impact fracture and consider a high-impact base

Justification: Impact toughness is important in accidental drops; a high-impact acrylic may reduce this risk.

Why not the closest alternatives: This is not a typical fatigue history.

Revise: Diploma LO 1.9, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Failure mechanism in dental materials

Return to Question 1026 ↑

Q1027 ADVANCED · Block C

Best answer: D. Identify investment wax distortion and make a fresh pattern promptly

Justification: Wax can relax internal stresses and change shape over time; prompt investment is essential.

Why not the closest alternatives: Higher stiffness alone does not prevent wax relaxation.

Revise: Diploma LO 1.9, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Failure mechanism in dental materials

Return to Question 1027 ↑

Q1097 ADVANCED · Block C

Best answer: C. Confirm whether the problem is inadequate retention form rather than cement strength

Justification: Poor resistance and retention geometry may defeat otherwise appropriate luting cement.

Why not the closest alternatives: Selecting a new adhesive without changing the mechanical problem is unlikely to solve it.

Revise: Diploma LO 1.9, 2c.14, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Cementation and bonding failure

Return to Question 1097 ↑

Q1098 ADVANCED · Block C

Best answer: D. Use MDP-compatible bonding for suitably conditioned zirconia

Justification: Zirconia benefits from suitable air abrasion and a phosphate monomer such as MDP.

Why not the closest alternatives: Silane alone is designed mainly for silica-based glass ceramics.

Revise: Diploma LO 1.9, 2c.14, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Cementation and bonding failure

Return to Question 1098 ↑

Q1099 ADVANCED · Block C

Best answer: D. Treat the margin caries before recementing the existing crown

Justification: Biological disease must be managed and restorability re-evaluated before simply rebonding.

Why not the closest alternatives: Even strong cement cannot restore diseased tissue.

Revise: Diploma LO 1.9, 2c.14, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Cementation and bonding failure

Return to Question 1099 ↑

Q1100 ADVANCED · Block C

Best answer: B. Review excessive water-to-powder ratio as a cause of low strength

Justification: Excess mixing water increases porosity and decreases cast strength.

Why not the closest alternatives: Immediate blame of a separate laboratory step would miss the known material error.

Revise: Diploma LO 1.9, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Gypsum and laboratory fit

Return to Question 1100 ↑

Q1101 ADVANCED · Block C

Best answer: D. Review insufficient strength and expansion control in die material selection

Justification: Investment and die expansion must be selected for the alloy casting shrinkage and the technique.

Why not the closest alternatives: An arbitrary stronger cement does not correct a casting dimensional mismatch.

Revise: Diploma LO 1.9, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Gypsum and laboratory fit

Return to Question 1101 ↑

Q1102 ADVANCED · Block C

Best answer: D. Review delayed pouring of dimensionally unstable alginate

Justification: Alginate is prone to syneresis and imbibition; prompt pouring according to manufacturer instructions is critical.

Why not the closest alternatives: Surface repair of the cast cannot reconstruct the distorted original dimensions.

Revise: Diploma LO 1.9, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Gypsum and laboratory fit

Return to Question 1102 ↑

Q1160 ADVANCED · Block C

Best answer: A. Re-evaluate rubber-dam isolation and re-treat contaminated enamel before bonding

Justification: Predictable adhesion demands a clean, properly prepared substrate; stop, control bleeding and repeat the required conditioning.

Why not the closest alternatives: Changing restoration material does not cure contaminated enamel or lack of isolation.

Revise: Diploma LO 1.9, 2c.14, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Surface-treatment contamination

Return to Question 1160 ↑

Q1161 ADVANCED · Block C

Best answer: B. Use a compatible zirconia primer after controlled surface preparation

Justification: Following an appropriate surface decontamination and conditioning protocol, MDP-mediated bonding may improve zirconia adhesion.

Why not the closest alternatives: Silane alone is not a universal primer for polycrystalline zirconia.

Revise: Diploma LO 1.9, 2c.14, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Surface-treatment contamination

Return to Question 1161 ↑

Q1162 ADVANCED · Block C

Best answer: E. Use silane on a correctly etched silica-based surface

Justification: A silane coupling agent links the silica-rich glass surface to compatible resin cement.

Why not the closest alternatives: Zirconia-only conditioning is not a substitute for glass-ceramic silanisation.

Revise: Diploma LO 1.9, 2c.14, 2c.24 · Volume I and Diploma learning outcomes (August 2026) · Theme: Surface-treatment contamination

Return to Question 1162 ↑

Q1209 ADVANCED · Block C

Best answer: E. Select a glass ceramic with enamel bonding after conservative preparation

Justification: Adhesively bonded glass ceramic can conserve enamel and provide predictable aesthetics when case selection and isolation are favourable.

Why not the closest alternatives: Monolithic zirconia would sacrifice the optical and adhesive advantages of glass ceramic here.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1209 ↑

Q1210 ADVANCED · Block C

Best answer: C. Select monolithic zirconia where high strength and limited clearance dominate

Justification: A high-strength, appropriately designed monolithic zirconia restoration may be advantageous for load and thickness constraints, with retention and preparation assessed.

Why not the closest alternatives: Highly aesthetic thin veneers are not the principal consideration in this posterior load case.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1210 ↑

Q1211 ADVANCED · Block C

Best answer: C. Use a bonded direct composite for repairable, minimally invasive restoration

Justification: Direct composite is conservative, repairable and proportionate to a limited defect.

Why not the closest alternatives: Full coverage would remove sound tooth structure without a commensurate benefit.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1211 ↑

Q1212 ADVANCED · Block C

Best answer: A. Use a metal-ceramic crown where adequate clearance and predictable retention exist

Justification: A well-designed metal-ceramic restoration can be a reasonable option with robust retention and acceptable aesthetics.

Why not the closest alternatives: No material is intrinsically superior without considering clearance, design, and patient preference.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1212 ↑

Q1213 ADVANCED · Block C

Best answer: E. Repeat the impression promptly after correcting tray flexure and material support

Justification: Tray flexure can distort the impression independently of material choice; use a properly supported rigid tray.

Why not the closest alternatives: Changing only the stone type cannot correct an elastically distorted impression.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1213 ↑

Q1214 ADVANCED · Block C

Best answer: E. Correct inadequate wash relief before repeating a two-stage PVS impression

Justification: Inadequate relief prevents a controlled wash space and can distort reseating; provide appropriate relief and repeat the technique.

Why not the closest alternatives: A precise die will still reproduce a flawed impression.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1214 ↑

Q1215 ADVANCED · Block C

Best answer: D. Avoid prolonged immersion of a polyether impression and follow the manufacturer’s disinfection protocol

Justification: Polyether can absorb water and deform with prolonged exposure; follow validated material-specific disinfection and pouring instructions.

Why not the closest alternatives: Assuming all elastomers tolerate extended immersion is incorrect.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1215 ↑

Q1216 ADVANCED · Block C

Best answer: A. Investigate latex-associated inhibition of addition-silicone setting

Justification: Sulphur-containing contaminants from some latex products can inhibit PVS polymerisation; avoid such contact and remake the impression.

Why not the closest alternatives: This localised inhibition is less consistent with general tray flexibility.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1216 ↑

Q1217 ADVANCED · Block C

Best answer: D. Hydrofluoric acid conditioning and silane for suitable silica-based glass ceramic

Justification: Appropriate glass-ceramic conditioning and silanisation facilitate resin bonding; use product-specific protocols and safe HF handling.

Why not the closest alternatives: Zirconia phosphate-monomer protocols are not a substitute for glass-ceramic conditioning.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1217 ↑

Q1218 ADVANCED · Block C

Best answer: C. Air abrasion and phosphate-monomer chemistry for zirconia

Justification: Zirconia bonding relies on controlled surface treatment and phosphate-functional monomers rather than conventional HF/silane alone.

Why not the closest alternatives: Treating zirconia as etchable glass ceramic is a material-category error.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1218 ↑

Q1219 ADVANCED · Block C

Best answer: A. Selective enamel etching with a suitable adhesive for direct resin composite

Justification: Selective enamel etching and appropriate adhesive placement support the direct composite bond without unnecessary preparation.

Why not the closest alternatives: Ceramic HF conditioning is not used directly on natural tooth enamel in this context.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1219 ↑

Q1220 ADVANCED · Block C

Best answer: D. A suitable metal primer and surface conditioning for a base-metal restoration

Justification: Metal-specific surface conditioning and primer chemistry can improve bonding where the design remains appropriate.

Why not the closest alternatives: Simply increasing cement thickness will not correct a contaminated bonding surface.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1220 ↑

Q1221 ADVANCED · Block C

Best answer: D. Correct the water-to-powder ratio and controlled spatulation for gypsum

Justification: Excess mixing water weakens gypsum and can change its expansion; control ratios and handling rather than blame the impression.

Why not the closest alternatives: A more expensive scanner would not correct gypsum mixing error.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1221 ↑

Q1222 ADVANCED · Block C

Best answer: A. Use a higher-strength, appropriate die stone for precision fixed restorations

Justification: Precision crown dies need suitable high-strength die stone and controlled handling, not low-strength model plaster.

Why not the closest alternatives: Investment expansion does not address a fragile working die.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1222 ↑

Q1223 ADVANCED · Block C

Best answer: C. Select investment expansion matched to the casting alloy and technique

Justification: Investment expansion must compensate for the specific alloy and technique; reassess the expansion protocol and casting parameters.

Why not the closest alternatives: Simply widening preparations risks masking a systematic laboratory error.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1223 ↑

Q1224 ADVANCED · Block C

Best answer: E. Invest wax patterns promptly to minimise time-dependent distortion

Justification: Wax experiences stress relaxation and thermal distortion over time, so prompt investment is a critical control.

Why not the closest alternatives: A different luting cement cannot compensate for a distorted pattern.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1224 ↑

Q1225 ADVANCED · Block C

Best answer: D. Diagnose fatigue from recurrent flexure and assess occlusal support

Justification: Recurrent flexure with occlusal loading can cause fatigue fracture; evaluate design, fit, and contacts before repair.

Why not the closest alternatives: Impact cannot explain every midline fracture without an impact history.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.21

Return to Question 1225 ↑

Q1226 ADVANCED · Block C

Best answer: E. Diagnose impact fracture and consider a tougher denture base

Justification: Impact toughness is the dominant requirement; consider high-impact acrylic and practical handling adaptations.

Why not the closest alternatives: Reinforcement for midline fatigue alone would not address repeated accidental drops.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.21

Return to Question 1226 ↑

Q1227 ADVANCED · Block C

Best answer: A. Correct porosity associated with inadequate processing control

Justification: Rapid heating can cause monomer boiling and gaseous porosity; a controlled polymerisation cycle is needed.

Why not the closest alternatives: Occlusal adjustment does not eliminate internal processing voids.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.21

Return to Question 1227 ↑

Q1228 ADVANCED · Block C

Best answer: B. Eliminate stress concentration from an excessively deep frenum notch

Justification: A deep sharp notch creates a local stress concentration; redesign the contour and reinforce appropriately.

Why not the closest alternatives: Treating a geometric design defect solely as a material failure may lead to recurrence.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.21

Return to Question 1228 ↑

Q1229 ADVANCED · Block C

Best answer: C. Use an elastomer with adequate tear resistance and controlled bulk across deep undercuts

Justification: Material tear resistance, bulk and safe withdrawal path are key when thin sections traverse undercuts.

Why not the closest alternatives: Simply adding more retraction cord does not correct an impression repeatedly tearing at the same site.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1229 ↑

Q1230 ADVANCED · Block C

Best answer: A. Select a more hydrophilic impression approach when moist surfaces cannot be completely dried

Justification: Wetting characteristics become critical in persistent moisture; choose a suitable approach while still managing contamination.

Why not the closest alternatives: A hydrophobic material without compatible wetting additives may reproduce the same voids.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1230 ↑

Q1231 ADVANCED · Block C

Best answer: A. Choose a suitable rigid mucostatic material only when undercuts do not preclude removal

Justification: Rigid mucostatic material may be suitable in an uncomplicated undercut-free zone where tissue displacement must be minimised.

Why not the closest alternatives: Rigid materials should not be trapped across significant tooth or ridge undercuts.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1231 ↑

Q1232 ADVANCED · Block C

Best answer: B. Avoid a condensation material if dimensional delay before pouring cannot be controlled

Justification: Material dimensional stability during delayed pouring matters; condensation materials lose volatile by-products and require prompt casting.

Why not the closest alternatives: Tray rigidity remains important, but cannot prevent ongoing material shrinkage.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.13

Return to Question 1232 ↑

Q1233 ADVANCED · Block C

Best answer: E. Prioritise elastic recovery and predictable removal from undercuts

Justification: Viscoelastic recovery depends on rate and magnitude of deformation; appropriate bulk and decisive removal help limit distortion.

Why not the closest alternatives: Cement choice cannot compensate for an inaccurate impression.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1233 ↑

Q1234 ADVANCED · Block C

Best answer: E. Provide sufficient ceramic thickness or choose a suitable high-strength design

Justification: Clinical material selection must integrate load, minimum thickness, geometry and restoration type; high strength alone is not a licence for a thin crown.

Why not the closest alternatives: Retreatment using the same under-thickness glass ceramic is unlikely to change the mechanism.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1234 ↑

Q1235 ADVANCED · Block C

Best answer: A. Check modulus, length, diameter and support before changing an RPD clasp alloy

Justification: Clasp strain depends on effective arm length, cross-section, undercut and material stiffness; redesign is required, not repeated chairside bending.

Why not the closest alternatives: A tougher adhesive cement does not solve a clasp mechanics problem.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1235 ↑

Q1236 ADVANCED · Block C

Best answer: D. Use a compatible coefficient of thermal expansion for metal-ceramic systems

Justification: The thermal expansion match and cooling protocol affect residual porcelain stress; material compatibility is essential.

Why not the closest alternatives: Increased interproximal contact strength cannot solve thermal mismatch.

Revise: Volume I — Unit 3 · Diploma LO 1.9, 2c.14

Return to Question 1236 ↑

BLOCK D · Pharmacology, pain control, emergencies and the medically compromised patient

Q1028 ADVANCED · Block D

Best answer: B. Stop elective treatment and seek emergency medical assessment

Justification: Suspected acute coronary syndrome requires emergency response, not continued dentistry.

Why not the closest alternatives: Reassurance, empiric antibiotics and a routine review dangerously delay care.

Revise: Diploma LO 1.7, 2a.13, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Medical warning signs during elective prosthodontics

Return to Question 1028 ↑

Q1029 ADVANCED · Block D

Best answer: C. Consult the medical team and plan care once systemic disease is stabilised

Justification: Non-urgent rehabilitation should be coordinated with the responsible medical team after stabilisation.

Why not the closest alternatives: Neither automatic antibiotics nor proceeding without risk assessment is justified.

Revise: Diploma LO 1.7, 2a.13, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Medical warning signs during elective prosthodontics

Return to Question 1029 ↑

Q1030 ADVANCED · Block D

Best answer: A. Continue treatment after reassuring the patient

Justification: There is no clinical reason to stop a minor, non-invasive adjustment solely because hypertension is controlled.

Why not the closest alternatives: Emergency assessment or antibiotics are not indicated in an asymptomatic stable patient.

Revise: Diploma LO 1.7, 2a.13, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Medical warning signs during elective prosthodontics

Return to Question 1030 ↑

Q1031 ADVANCED · Block D

Best answer: B. Assess procedure-specific bleeding risk and follow current anticoagulant guidance

Justification: A non-invasive impression does not generally justify changing anticoagulation; assess the actual procedure.

Why not the closest alternatives: Stopping anticoagulation indiscriminately increases thromboembolic risk.

Revise: Diploma LO 2a.3, 2b.10, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anticoagulation and planned treatment

Return to Question 1031 ↑

Q1032 ADVANCED · Block D

Best answer: B. Choose a non-surgical prosthetic option after shared decision-making

Justification: An elective surgical intervention carries competing haemorrhage and thrombotic risks; a non-surgical alternative should be discussed.

Why not the closest alternatives: Stopping antiplatelets without specialist advice may be dangerous.

Revise: Diploma LO 2a.3, 2b.10, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anticoagulation and planned treatment

Return to Question 1032 ↑

Q1033 ADVANCED · Block D

Best answer: D. Seek medical advice for a high-risk procedure when the indication and drug plan are uncertain

Justification: Seek relevant current guidance and individual medical advice before a higher-risk procedure with renal impairment.

Why not the closest alternatives: An automatic week-long interruption is not a safe general rule.

Revise: Diploma LO 2a.3, 2b.10, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anticoagulation and planned treatment

Return to Question 1033 ↑

Q1034 ADVANCED · Block D

Best answer: B. Confirm medication, indication, dose and exposure before assigning MRONJ risk

Justification: The indication and regimen profoundly affect MRONJ risk; clarify the drug and liaise with the prescribing team.

Why not the closest alternatives: It is unsafe to class all regimens as equal or proceed with irreversible intervention.

Revise: Diploma LO 1.7, 2c.7, 2c.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Medication-related bone risk

Return to Question 1034 ↑

Q1035 ADVANCED · Block D

Best answer: D. Arrange specialist input for a complex, high-risk oncology-related case

Justification: High-dose cancer-related antiresorptives materially elevate complexity and merit specialist multidisciplinary risk assessment.

Why not the closest alternatives: Routine surgery without specialist assessment risks avoidable harm.

Revise: Diploma LO 1.7, 2c.7, 2c.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Medication-related bone risk

Return to Question 1035 ↑

Q1036 ADVANCED · Block D

Best answer: B. Prefer non-surgical prosthetic treatment where this meets the patient’s aims

Justification: A satisfactory non-surgical solution avoids unnecessary bony trauma and addresses the complaint.

Why not the closest alternatives: Antiresorptive history does not preclude conservative prosthodontic care.

Revise: Diploma LO 1.7, 2c.7, 2c.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Medication-related bone risk

Return to Question 1036 ↑

Q1103 ADVANCED · Block D

Best answer: B. Stop the procedure and initiate the anaphylaxis emergency algorithm

Justification: Systemic skin, respiratory and circulatory compromise indicates possible anaphylaxis requiring the emergency response pathway.

Why not the closest alternatives: Oral antibiotics and reassurance do not treat anaphylaxis.

Revise: Diploma LO 1.7, 2a.13, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Adverse event recognition

Return to Question 1103 ↑

Q1104 ADVANCED · Block D

Best answer: D. Assess positional syncope and manage the airway, positioning and vital signs

Justification: The postural context and immediate recovery favour vasovagal syncope; manage and reassess, considering alternate diagnoses.

Why not the closest alternatives: Anaphylaxis is less likely without multisystem features.

Revise: Diploma LO 1.7, 2a.13, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Adverse event recognition

Return to Question 1104 ↑

Q1105 ADVANCED · Block D

Best answer: A. Assess a local anaesthetic systemic toxicity scenario and initiate emergency support

Justification: Neurological prodromes shortly after injection raise concern for local anaesthetic systemic toxicity and require emergency support.

Why not the closest alternatives: Treating the episode as routine anxiety may delay resuscitation.

Revise: Diploma LO 1.7, 2a.13, 2b.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Adverse event recognition

Return to Question 1105 ↑

Q1106 ADVANCED · Block D

Best answer: C. Coordinate pre-radiotherapy dental assessment with the oncology team

Justification: Early coordinated dental assessment and appropriate management before radiotherapy may reduce future complications without delaying oncology unnecessarily.

Why not the closest alternatives: Elective cosmetic reconstruction is not the immediate priority.

Revise: Diploma LO 2c.7, 2c.8, 2b.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Multidisciplinary risk before restorative intervention

Return to Question 1106 ↑

Q1107 ADVANCED · Block D

Best answer: C. Seek specialist guidance where post-radiotherapy extractions could threaten bone health

Justification: Post-radiotherapy extraction carries osteoradionecrosis risk; obtain dose and multidisciplinary advice for complex surgical decisions.

Why not the closest alternatives: Proceeding as a routine extraction without assessing the radiation field is unsafe.

Revise: Diploma LO 2c.7, 2c.8, 2b.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Multidisciplinary risk before restorative intervention

Return to Question 1107 ↑

Q1108 ADVANCED · Block D

Best answer: B. Plan routine removable repair without concern when no invasive treatment is needed

Justification: A simple non-invasive adjustment or repair can be provided with appropriate hygiene and medical awareness.

Why not the closest alternatives: Cancer treatment is not an automatic contraindication to all non-invasive prosthetic care.

Revise: Diploma LO 2c.7, 2c.8, 2b.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Multidisciplinary risk before restorative intervention

Return to Question 1108 ↑

Q1163 ADVANCED · Block D

Best answer: C. Stop treatment and assess airway, breathing and circulation

Justification: An unresponsive patient with abnormal breathing requires an immediate emergency assessment and response according to current resuscitation algorithms.

Why not the closest alternatives: Continuing prosthetic care delays lifesaving action.

Revise: Diploma LO 2a.13, 2b.7, 2c.27 · Volume I and Diploma learning outcomes (August 2026) · Theme: Decision-making when a clinical emergency interrupts treatment

Return to Question 1163 ↑

Q1164 ADVANCED · Block D

Best answer: A. Activate emergency escalation for rapidly spreading infection and airway symptoms

Justification: Rapidly spreading odontogenic infection with dysphagia may endanger the airway and warrants emergency escalation.

Why not the closest alternatives: Outpatient reassurance or a later appointment are unsafe.

Revise: Diploma LO 2a.13, 2b.7, 2c.27 · Volume I and Diploma learning outcomes (August 2026) · Theme: Decision-making when a clinical emergency interrupts treatment

Return to Question 1164 ↑

Q1165 ADVANCED · Block D

Best answer: C. Provide symptom-led conservative dental care after excluding red flags

Justification: Limited conservative care and short-interval review can be appropriate after excluding red flags.

Why not the closest alternatives: Emergency referral is disproportionate for a resolved uncomplicated traumatic sore spot.

Revise: Diploma LO 2a.13, 2b.7, 2c.27 · Volume I and Diploma learning outcomes (August 2026) · Theme: Decision-making when a clinical emergency interrupts treatment

Return to Question 1165 ↑

Q1237 ADVANCED · Block D

Best answer: D. Stop treatment, place the patient appropriately, assess and manage a probable vasovagal episode

Justification: This pattern is typical of syncope; remove the stimulus, assess airway and vital signs, and manage positioning appropriately.

Why not the closest alternatives: Calling it hypoglycaemia solely because of sweating overlooks the trigger and bradycardia.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 2b.11

Return to Question 1237 ↑

Q1238 ADVANCED · Block D

Best answer: D. Treat a suspected hypoglycaemic event promptly according to consciousness and local emergency protocol

Justification: Prompt glucose treatment with repeated monitoring is indicated for conscious hypoglycaemia, following emergency guidance.

Why not the closest alternatives: Syncope positioning alone will not correct neuroglycopenia.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 2b.11

Return to Question 1238 ↑

Q1239 ADVANCED · Block D

Best answer: E. Activate the emergency response for suspected anaphylaxis and give first-line emergency treatment

Justification: A rapidly evolving airway/circulatory reaction is anaphylaxis until proven otherwise; activate emergency care and give guideline-based adrenaline.

Why not the closest alternatives: Antihistamines alone do not replace the first-line emergency response.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 2b.11

Return to Question 1239 ↑

Q1240 ADVANCED · Block D

Best answer: D. Arrange immediate hospital assessment for suspected acute coronary syndrome

Justification: Suspected acute coronary syndrome requires emergency services, appropriate initial support and urgent transfer.

Why not the closest alternatives: Do not continue elective dentistry or delay transfer for a routine medication review.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 2b.11

Return to Question 1240 ↑

Q1241 ADVANCED · Block D

Best answer: C. Assess bleeding risk and use local haemostasis rather than routinely stopping antiplatelet treatment

Justification: Routine interruption of antiplatelets can carry substantial thrombotic risk; use local measures and follow current guidance.

Why not the closest alternatives: A unilateral decision to stop both agents is not a safe default.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.3

Return to Question 1241 ↑

Q1242 ADVANCED · Block D

Best answer: B. Consult the current DOAC dental guidance and tailor timing to procedure bleeding risk

Justification: DOAC plans are procedure- and patient-specific; consult current guidance regarding timing, haemostasis and liaison where needed.

Why not the closest alternatives: One blanket protocol cannot safely cover every DOAC and procedure.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.3

Return to Question 1242 ↑

Q1243 ADVANCED · Block D

Best answer: A. Identify an interacting antifungal and avoid unsafe concomitant use

Justification: Miconazole can substantially interact with warfarin even as an oral preparation; choose an appropriate alternative and check guidance.

Why not the closest alternatives: A locally applied product is not necessarily free from systemic interactions.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.3

Return to Question 1243 ↑

Q1244 ADVANCED · Block D

Best answer: D. Obtain relevant INR information before invasive care in a warfarin-treated patient

Justification: Up-to-date anticoagulation assessment is needed before invasive treatment, following local guidance and procedural risk.

Why not the closest alternatives: Proceeding without current information is not justified by a past stable result.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.3

Return to Question 1244 ↑

Q1245 ADVANCED · Block D

Best answer: E. Prioritise retention and non-surgical management where feasible in high-risk oncology therapy

Justification: High-risk oncology antiresorptive therapy shifts the risk-benefit balance toward preserving viable teeth when clinically reasonable.

Why not the closest alternatives: Simple extraction solely for expediency may expose the patient to avoidable MRONJ risk.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.14

Return to Question 1245 ↑

Q1246 ADVANCED · Block D

Best answer: C. Coordinate dental stabilisation and medical liaison before initiating high-risk therapy

Justification: Coordinated assessment, infection control and relevant dental care before high-risk therapy can reduce subsequent complications.

Why not the closest alternatives: Delaying all dental assessment until after treatment starts misses a preventive opportunity.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.14

Return to Question 1246 ↑

Q1247 ADVANCED · Block D

Best answer: D. Explain a comparatively low but non-zero risk in a stable osteoporosis-dose patient

Justification: MRONJ risk is generally much lower at osteoporosis doses than oncology doses, but still needs informed consent, case assessment and guidance.

Why not the closest alternatives: An absolute ban on all extractions is not appropriate.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.14

Return to Question 1247 ↑

Q1248 ADVANCED · Block D

Best answer: A. Investigate exposed necrotic bone and coordinate appropriate specialist assessment

Justification: Persistent exposed bone with medication history warrants assessment for MRONJ and specialist liaison where indicated.

Why not the closest alternatives: Repeatedly adjusting a denture without considering pathology can delay diagnosis.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2b.11, 2a.14

Return to Question 1248 ↑

Q1249 ADVANCED · Block D

Best answer: B. Defer elective extensive care until unstable cardiovascular or systemic disease has been assessed

Justification: Unstable cardiovascular symptoms are a priority over elective complex rehabilitation; arrange appropriate medical assessment.

Why not the closest alternatives: Proceeding because the proposed treatment is elective does not eliminate systemic risk.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.11, 2c.2

Return to Question 1249 ↑

Q1250 ADVANCED · Block D

Best answer: D. Adapt appointment structure and maintenance for cognitive and physical limitations

Justification: Simplify care, involve the appropriate supporter with consent, and select an easily maintainable design and realistic recall.

Why not the closest alternatives: Technical sophistication is less important than safe long-term function.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.11, 2c.2

Return to Question 1250 ↑

Q1251 ADVANCED · Block D

Best answer: C. Provide enhanced xerostomia prevention and review medicines with the prescriber

Justification: Medication review where possible plus high-risk caries prevention should precede definitive bridgework.

Why not the closest alternatives: Replacing prostheses without addressing salivary risk is prone to repeat failure.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.11, 2c.2

Return to Question 1251 ↑

Q1252 ADVANCED · Block D

Best answer: E. Coordinate periodontal control and glycaemic-risk assessment before invasive rehabilitation

Justification: Infection control, periodontal assessment and systemic risk optimisation should inform the sequencing and provider setting.

Why not the closest alternatives: Immediate extensive rehabilitation despite active disease risks poor outcomes.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.11, 2c.2

Return to Question 1252 ↑

Q1253 ADVANCED · Block D

Best answer: D. Manage a seizure safely, protect from injury and activate help according to duration or recovery

Justification: Protect from injury, avoid restraint and follow the emergency seizure pathway, including escalation thresholds.

Why not the closest alternatives: Attempting to insert an object into the mouth is hazardous.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 3a.2

Return to Question 1253 ↑

Q1254 ADVANCED · Block D

Best answer: A. Treat a severe asthma exacerbation with emergency inhaled therapy and escalation

Justification: A severe asthma attack requires urgent assessment, guideline-based bronchodilator treatment and escalation if not resolving.

Why not the closest alternatives: Continuing treatment or treating this as routine dental anxiety is unsafe.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 3a.2

Return to Question 1254 ↑

Q1255 ADVANCED · Block D

Best answer: D. Stop treatment and manage suspected myocardial ischaemia with emergency services

Justification: Treat symptoms as potential acute coronary syndrome; stop dental care and activate urgent medical response.

Why not the closest alternatives: A simple vasovagal faint would not account well for persistent radiating chest pain.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 3a.2

Return to Question 1255 ↑

Q1256 ADVANCED · Block D

Best answer: E. Respond to suspected stroke with immediate emergency activation and time documentation

Justification: Time-critical stroke recognition and emergency referral are essential; document symptom onset and do not delay transfer.

Why not the closest alternatives: Routine dental follow-up is inappropriate for acute focal neurological deficits.

Revise: Volume I — Unit 4 · Diploma LO 1.7, 2a.13, 3a.2

Return to Question 1256 ↑

Q1257 ADVANCED · Block D

Best answer: E. Establish decision-specific capacity and support understanding before proceeding

Justification: Capacity is decision-specific and may be supported by accessible explanation; a diagnosis alone does not establish incapacity.

Why not the closest alternatives: A proxy should not automatically override an adult who can decide with support.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.12, 3a.7

Return to Question 1257 ↑

Q1258 ADVANCED · Block D

Best answer: B. Offer accessible communication and the option of a suitable interpreter

Justification: Use appropriate interpretation and teach-back rather than assuming nodding indicates informed consent.

Why not the closest alternatives: A family member translating alone may not provide adequate independent informed communication.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.12, 3a.7

Return to Question 1258 ↑

Q1259 ADVANCED · Block D

Best answer: A. Respect a capacitous patient’s informed refusal and document alternatives

Justification: Respect informed refusal; document discussion and offer reasonable ongoing care.

Why not the closest alternatives: A clinician cannot impose irreversible treatment merely because it is recommended.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.12, 3a.7

Return to Question 1259 ↑

Q1260 ADVANCED · Block D

Best answer: D. Arrange best-interests decision making when the patient lacks decision-specific capacity

Justification: Follow the applicable legal and ethical best-interests framework, including appropriate consultees and least-restrictive options.

Why not the closest alternatives: A relative’s preference alone is not a substitute for the required decision process.

Revise: Volume I — Unit 4 · Diploma LO 2a.3, 2b.12, 3a.7

Return to Question 1260 ↑

BLOCK E · Assessment, diagnosis, imaging, classifications and planning

Q1001 ADVANCED · Block E

Best answer: B. Obtain a limited-field CBCT after confirming its diagnostic benefit and justification

Justification: A small-field CBCT may resolve a specific three-dimensional question after 2-D methods fail, if the result will alter management.

Why not the closest alternatives: A further periapical is less likely to answer the already unresolved question; CBCT is not routine screening.

Revise: Diploma LO 2a.9, 2a.17, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Choosing additional imaging

Return to Question 1001 ↑

Q1002 ADVANCED · Block E

Best answer: B. Repeat a high-quality periapical radiograph using a second horizontal angulation

Justification: Correct the technical limitation and obtain a diagnostically adequate intraoral image before escalating dose or intervening.

Why not the closest alternatives: CBCT adds dose and is not justified merely because the original radiograph is inadequate.

Revise: Diploma LO 2a.9, 2a.17, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Choosing additional imaging

Return to Question 1002 ↑

Q1003 ADVANCED · Block E

Best answer: B. Refer urgently on a suspected malignancy pathway

Justification: The examination identifies a possible oral malignancy; urgent suspected-cancer referral takes priority over prosthodontic imaging.

Why not the closest alternatives: Neither a panoramic nor CBCT substitutes for urgent specialist examination and appropriate investigation.

Revise: Diploma LO 2a.9, 2a.17, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Choosing additional imaging

Return to Question 1003 ↑

Q1004 ADVANCED · Block E

Best answer: A. Investigate for a vertical root fracture and discuss extraction if confirmed

Justification: The combination suggests a vertical root fracture; determine restorability rather than placing another crown.

Why not the closest alternatives: Posts do not cure root fractures; the isolated defect is not explained by routine crown failure alone.

Revise: Diploma LO 2a.6, 2a.17, 2a.18, 2c.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Testing restorability

Return to Question 1004 ↑

Q1005 ADVANCED · Block E

Best answer: A. Provide an indirect restoration after managing the limited coronal defect

Justification: A bonded partial-coverage restoration can preserve sound structure in an otherwise favourable tooth.

Why not the closest alternatives: Full-coverage post and core is unnecessary in a vital tooth and introduces avoidable damage.

Revise: Diploma LO 2a.6, 2a.17, 2a.18, 2c.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Testing restorability

Return to Question 1005 ↑

Q1006 ADVANCED · Block E

Best answer: D. Investigate periodontal and endodontic causes before assigning prognosis

Justification: Periodontal and endodontic diagnosis must precede prognosis and definitive prosthodontics.

Why not the closest alternatives: Restoring first risks cementing disease into an expensive and unstable plan.

Revise: Diploma LO 2a.6, 2a.17, 2a.18, 2c.11 · Volume I and Diploma learning outcomes (August 2026) · Theme: Testing restorability

Return to Question 1006 ↑

Q1007 ADVANCED · Block E

Best answer: A. Accept a shortened dental arch and monitor function

Justification: No replacement can be entirely appropriate where a functional shortened dental arch meets the patient’s needs.

Why not the closest alternatives: An elective prosthesis without a functional indication adds maintenance and risk.

Revise: Diploma LO 2a.16, 2b.6, 2b.7, 2c.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Patient-centred replacement decision

Return to Question 1007 ↑

Q1008 ADVANCED · Block E

Best answer: C. Use a simple hygienic removable partial denture

Justification: A simple, easily cleaned and repairable RPD can meet a specific functional aim where the patient prefers removable care.

Why not the closest alternatives: Multiple implants or extracting sound teeth would be disproportionate.

Revise: Diploma LO 2a.16, 2b.6, 2b.7, 2c.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Patient-centred replacement decision

Return to Question 1008 ↑

Q1009 ADVANCED · Block E

Best answer: A. Offer a resin-bonded adhesive bridge after occlusal assessment

Justification: An appropriately designed resin-bonded bridge offers a conservative fixed replacement without full-coverage preparation.

Why not the closest alternatives: Implants are not the only fixed option, and the patient explicitly wishes to avoid surgery.

Revise: Diploma LO 2a.16, 2b.6, 2b.7, 2c.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Patient-centred replacement decision

Return to Question 1009 ↑

Q1010 ADVANCED · Block E

Best answer: A. Stabilise periodontal disease and reassess prognosis before final prosthodontics

Justification: Definitive bridge design follows successful periodontal stabilisation and re-evaluation of abutments.

Why not the closest alternatives: Extensive fixed work during active periodontal disease risks predictable failure.

Revise: Diploma LO 2a.18, 2b.6, 2b.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Staging disease control

Return to Question 1010 ↑

Q1011 ADVANCED · Block E

Best answer: D. Arrange specialist multidisciplinary input before attempting full-mouth reconstruction

Justification: Complex multidisciplinary rehabilitation and vertical-dimension changes exceed routine Level 2 planning; specialist input is appropriate.

Why not the closest alternatives: Immediate full-mouth irreversible work before coherent diagnosis and a multidisciplinary plan is hazardous.

Revise: Diploma LO 2a.18, 2b.6, 2b.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Staging disease control

Return to Question 1011 ↑

Q1012 ADVANCED · Block E

Best answer: A. Provide provisional, repairable restorations while medical and behavioural risks are addressed

Justification: Stabilise caries, saliva-related risks and patient comfort with provisional repairs before committing to definitive aesthetics.

Why not the closest alternatives: Early definitive expensive work is unlikely to survive uncontrolled risk factors.

Revise: Diploma LO 2a.18, 2b.6, 2b.8 · Volume I and Diploma learning outcomes (August 2026) · Theme: Staging disease control

Return to Question 1012 ↑

Q1109 ADVANCED · Block E

Best answer: A. Use a second sensibility test and diagnostic imaging before assigning pulpal diagnosis

Justification: Sensibility tests can be temporarily false negative after trauma; repeat in context and monitor signs before irrevocable treatment.

Why not the closest alternatives: One negative thermal response does not establish necrosis.

Revise: Diploma LO 2a.11, 2a.17, 2b.6 · Volume I and Diploma learning outcomes (August 2026) · Theme: Combining clinical tests

Return to Question 1109 ↑

Q1110 ADVANCED · Block E

Best answer: A. Investigate cracks and occlusal loading with appropriate tests

Justification: A cracked-tooth differential warrants careful bite testing, transillumination and occlusal evaluation as appropriate.

Why not the closest alternatives: A routine panoramic image is a weak test for a fine crown-related crack.

Revise: Diploma LO 2a.11, 2a.17, 2b.6 · Volume I and Diploma learning outcomes (August 2026) · Theme: Combining clinical tests

Return to Question 1110 ↑

Q1111 ADVANCED · Block E

Best answer: E. Seek urgent investigation of a persistent unusual soft-tissue lesion

Justification: A persistent unexplained oral lesion needs urgent assessment via the appropriate pathway.

Why not the closest alternatives: Concentrating on pulpal or periodontal tests would delay the relevant diagnostic process.

Revise: Diploma LO 2a.11, 2a.17, 2b.6 · Volume I and Diploma learning outcomes (August 2026) · Theme: Combining clinical tests

Return to Question 1111 ↑

Q1166 ADVANCED · Block E

Best answer: D. Use a justified periapical radiograph to evaluate a specific abutment concern

Justification: A high-quality periapical radiograph addresses a specific tooth-related concern with relatively limited exposure.

Why not the closest alternatives: A panoramic image is less suited to fine periapical details.

Revise: Diploma LO 2a.9, 2a.10, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Radiographic decision thresholds

Return to Question 1166 ↑

Q1167 ADVANCED · Block E

Best answer: A. Request CBCT only when a defined 3-D clinical question will alter management

Justification: CBCT may be justified when the three-dimensional anatomy matters to surgical safety and treatment planning.

Why not the closest alternatives: CBCT should not be automatic for all restorative cases.

Revise: Diploma LO 2a.9, 2a.10, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Radiographic decision thresholds

Return to Question 1167 ↑

Q1168 ADVANCED · Block E

Best answer: B. Manage without imaging if there is no new clinical indication

Justification: Ionising imaging requires individual justification and should not be repeated without an appropriate benefit.

Why not the closest alternatives: Serial CBCT surveillance of a clinically stable restoration adds unnecessary dose.

Revise: Diploma LO 2a.9, 2a.10, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Radiographic decision thresholds

Return to Question 1168 ↑

Q1261 ADVANCED · Block E

Best answer: A. Use focused periapical imaging after assessing the clinical question

Justification: A targeted periapical image is proportionate to a tooth-specific periradicular and restorative question.

Why not the closest alternatives: A CBCT would usually be premature without adequate conventional imaging.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.10, 2b.4

Return to Question 1261 ↑

Q1262 ADVANCED · Block E

Best answer: C. Use a panoramic survey to assess the distribution of a partially dentate arch where justified

Justification: A panoramic image can give a justified overview of widespread dentition and bony structures, supplemented by intraoral views.

Why not the closest alternatives: It is less suitable than a good periapical for fine detail at a single root apex.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.10, 2b.4

Return to Question 1262 ↑

Q1263 ADVANCED · Block E

Best answer: C. Consider limited-field CBCT only after 2-D imaging fails to answer a decision-critical 3-D question

Justification: A limited-field CBCT may answer the unresolved three-dimensional question if justified after conventional imaging.

Why not the closest alternatives: Routine CBCT screening is not justified in a case that can be resolved with 2-D views.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.10, 2b.4

Return to Question 1263 ↑

Q1264 ADVANCED · Block E

Best answer: A. Arrange appropriate specialist evaluation without delaying for routine prosthodontic images

Justification: Suspicious soft-tissue and neurological findings require urgent clinical escalation; routine dental imaging must not delay appropriate assessment.

Why not the closest alternatives: An OPG alone cannot exclude serious soft-tissue pathology.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.10, 2b.4

Return to Question 1264 ↑

Q1265 ADVANCED · Block E

Best answer: E. Investigate a possible cracked tooth with targeted tests before choosing a restoration

Justification: Pain on release raises concern for a cracked tooth; targeted clinical testing informs whether cuspal coverage is suitable.

Why not the closest alternatives: Immediate extraction or crown preparation before localisation would be premature.

Revise: Volume I — Unit 5 · Diploma LO 2a.11, 2a.17, 2b.3

Return to Question 1265 ↑

Q1266 ADVANCED · Block E

Best answer: C. Establish pulpal and periapical status before assigning a restorative prognosis

Justification: Sensibility, percussion, clinical and radiographic correlation are needed before endodontic versus restorative treatment is selected.

Why not the closest alternatives: Assuming every painful crown is a mechanical failure misses pulpal disease.

Revise: Volume I — Unit 5 · Diploma LO 2a.11, 2a.17, 2b.3

Return to Question 1266 ↑

Q1267 ADVANCED · Block E

Best answer: A. Assess periodontal activity and overall support before bridge planning

Justification: Active periodontitis and uncertain support affect prognosis and must be addressed before committing to fixed abutment loading.

Why not the closest alternatives: Fabricating the bridge first can lock active disease into the final design.

Revise: Volume I — Unit 5 · Diploma LO 2a.11, 2a.17, 2b.3

Return to Question 1267 ↑

Q1268 ADVANCED · Block E

Best answer: D. Assess possible non-odontogenic pain and refer when findings do not fit a tooth cause

Justification: The pain pattern does not localise to a restorative source; avoid further irreversible dentistry and assess non-odontogenic causes.

Why not the closest alternatives: More crowns or endodontic procedures risk worsening persistent pain without a diagnosis.

Revise: Volume I — Unit 5 · Diploma LO 2a.11, 2a.17, 2b.3

Return to Question 1268 ↑

Q1269 ADVANCED · Block E

Best answer: B. Demonstrable sound circumferential ferrule with adequate retention after caries removal

Justification: A real ferrule and retention potential substantially improve restorability, subject to occlusion and endodontic status.

Why not the closest alternatives: The presence of a root filling alone does not imply a hopeless prognosis.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.3, 2c.2

Return to Question 1269 ↑

Q1270 ADVANCED · Block E

Best answer: B. A likely vertical root fracture despite otherwise acceptable crown margins

Justification: These findings raise suspicion of vertical root fracture and can make long-term retention untenable if confirmed.

Why not the closest alternatives: A new crown cannot repair a vertical root fracture.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.3, 2c.2

Return to Question 1270 ↑

Q1271 ADVANCED · Block E

Best answer: D. Uncontrolled periodontal support loss and mobility affecting strategic abutment value

Justification: Active progressive attachment loss and unreliable maintenance compromise abutment prognosis and prosthesis design.

Why not the closest alternatives: Crown height and enamel alone are insufficient measures of an abutment's fitness.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.3, 2c.2

Return to Question 1271 ↑

Q1272 ADVANCED · Block E

Best answer: E. Inability to maintain plaque control around a proposed complex restoration

Justification: The proposed restoration cannot be judged separately from maintainability; complex contours in an uncontrolled environment may be inappropriate.

Why not the closest alternatives: Selecting the strongest ceramic does not compensate for absent disease control.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.3, 2c.2

Return to Question 1272 ↑

Q1273 ADVANCED · Block E

Best answer: B. Stabilise caries and periodontal disease, then reassess definitive restorative options

Justification: Definitive prosthodontics should follow disease control and a realistic maintenance plan.

Why not the closest alternatives: The best designed bridge may fail prematurely if active risks persist.

Revise: Volume I — Unit 5 · Diploma LO 2b.3, 2b.6, 2c.1

Return to Question 1273 ↑

Q1274 ADVANCED · Block E

Best answer: A. Provide an interim removable prosthesis while establishing prognosis and patient priorities

Justification: An interim removable solution can maintain aesthetics while investigations and treatment planning proceed.

Why not the closest alternatives: Premature irreversible reconstruction would commit to uncertain abutments.

Revise: Volume I — Unit 5 · Diploma LO 2b.3, 2b.6, 2c.1

Return to Question 1274 ↑

Q1275 ADVANCED · Block E

Best answer: A. Perform a diagnostic wax-up/trial to assess the desired occlusal changes before irreversible work

Justification: A reversible diagnostic stage helps test occlusal and aesthetic goals before extensive irreversible rehabilitation.

Why not the closest alternatives: Proceeding immediately to multiple definitive crowns is unnecessarily risky.

Revise: Volume I — Unit 5 · Diploma LO 2b.3, 2b.6, 2c.1

Return to Question 1275 ↑

Q1276 ADVANCED · Block E

Best answer: E. Refer for multidisciplinary planning of a complex full-mouth rehabilitation

Justification: Complex developmental rehabilitation warrants coordinated specialist assessment beyond routine enhanced-skills planning.

Why not the closest alternatives: A single isolated implant decision may prejudice the definitive interdisciplinary plan.

Revise: Volume I — Unit 5 · Diploma LO 2b.3, 2b.6, 2c.1

Return to Question 1276 ↑

Q1277 ADVANCED · Block E

Best answer: A. Provide balanced options, including doing nothing, and elicit the patient's priorities

Justification: Non-replacement is a legitimate option; informed shared decision-making should address function, risk and preference.

Why not the closest alternatives: A clinician should not treat tooth count as an absolute prosthetic target.

Revise: Volume I — Unit 5 · Diploma LO 2a.16, 2b.7, 3b.3

Return to Question 1277 ↑

Q1278 ADVANCED · Block E

Best answer: B. Use a provisional aesthetic mock-up to resolve a realistic but uncertain visual objective

Justification: A reversible mock-up helps test aesthetics, lip support and speech before irreversible preparation.

Why not the closest alternatives: Immediate crown preparation would bypass an important assessment step.

Revise: Volume I — Unit 5 · Diploma LO 2a.16, 2b.7, 3b.3

Return to Question 1278 ↑

Q1279 ADVANCED · Block E

Best answer: A. Defer elective irreversible treatment if expectations remain demonstrably unrealistic

Justification: Unrealistic expectations and disproportionate harm justify deferral and further discussion rather than irreversible elective dentistry.

Why not the closest alternatives: Patient autonomy does not require provision of clinically inappropriate treatment.

Revise: Volume I — Unit 5 · Diploma LO 2a.16, 2b.7, 3b.3

Return to Question 1279 ↑

Q1280 ADVANCED · Block E

Best answer: C. Adapt the plan to affordability while retaining safety and minimum maintenance standards

Justification: An appropriately designed RPD can meet function and affordability goals within safe, maintainable care.

Why not the closest alternatives: Implants are not mandatory when a simpler solution meets the patient's needs.

Revise: Volume I — Unit 5 · Diploma LO 2a.16, 2b.7, 3b.3

Return to Question 1280 ↑

Q1281 ADVANCED · Block E

Best answer: C. Record extent and progression of erosive wear and establish risk control before restoration

Justification: Serial wear records and acid risk control determine when and how intervention is needed; occlusal change is not automatically required.

Why not the closest alternatives: Restoration without control of the cause can fail repeatedly.

Revise: Volume I — Unit 5 · Diploma LO 2a.15, 2b.3, 2c.10

Return to Question 1281 ↑

Q1282 ADVANCED · Block E

Best answer: B. Assess reduced restorative space versus true loss of vertical dimension

Justification: Compensatory dentoalveolar change may preserve OVD despite local tooth wear; distinguish a space problem from global vertical collapse.

Why not the closest alternatives: Simply increasing the whole arch's OVD may over-treat a localised defect.

Revise: Volume I — Unit 5 · Diploma LO 2a.15, 2b.3, 2c.10

Return to Question 1282 ↑

Q1283 ADVANCED · Block E

Best answer: C. Investigate a discrepancy between retruded contact and habitual intercuspation

Justification: A reproducible slide may alter prosthetic contacts and should be understood before occlusal reconstruction.

Why not the closest alternatives: An asymptomatic slide does not itself justify indiscriminate occlusal grinding.

Revise: Volume I — Unit 5 · Diploma LO 2a.15, 2b.3, 2c.10

Return to Question 1283 ↑

Q1284 ADVANCED · Block E

Best answer: C. Assess whether symptoms reflect muscle-related TMD before considering occlusal alteration

Justification: Muscle-related TMD generally starts with reversible, conservative care; extensive irreversible occlusal procedures are not first line.

Why not the closest alternatives: Symptoms alone do not establish that an occlusal change will cure TMD.

Revise: Volume I — Unit 5 · Diploma LO 2a.15, 2b.3, 2c.10

Return to Question 1284 ↑

Q1285 ADVANCED · Block E

Best answer: A. Obtain an adequate clinical periodontal chart and targeted radiographs

Justification: Periodontal examination and indicated radiographs are necessary to assess disease activity and abutment prognosis.

Why not the closest alternatives: The prosthetic design should not precede a complete diagnosis.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.11, 2a.17

Return to Question 1285 ↑

Q1286 ADVANCED · Block E

Best answer: B. Use pulpal sensibility and percussion tests alongside imaging

Justification: Combine sensibility, percussion, history and radiographs to distinguish pulpal/endodontic from mechanical causes.

Why not the closest alternatives: A normal radiograph does not prove pulpal health.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.11, 2a.17

Return to Question 1286 ↑

Q1287 ADVANCED · Block E

Best answer: C. Request appropriate blood tests or medical liaison for suspected systemic disease

Justification: Possible haematinic or systemic issues should be investigated with appropriate medical liaison and relevant tests.

Why not the closest alternatives: Changing denture teeth alone will not address a likely systemic contributor.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.11, 2a.17

Return to Question 1287 ↑

Q1288 ADVANCED · Block E

Best answer: A. Refer an unexplained non-healing oral lesion urgently for assessment

Justification: Persistent high-risk site lesions warrant urgent oral-medicine/maxillofacial assessment.

Why not the closest alternatives: Repeating the adjustment without referral would delay diagnosis.

Revise: Volume I — Unit 5 · Diploma LO 2a.9, 2a.11, 2a.17

Return to Question 1288 ↑

Q1289 ADVANCED · Block E

Best answer: D. Demonstrated chewing impairment and the patient’s preference for replacement

Justification: An actual functional loss is a key reason to discuss replacement; a shortened dental arch is not invariably adequate for everyone.

Why not the closest alternatives: An arbitrary number of missing teeth cannot determine individual benefit.

Revise: Volume I — Unit 5 · Diploma LO 2c.1, 2b.4, 2b.10

Return to Question 1289 ↑

Q1290 ADVANCED · Block E

Best answer: E. Plaque control and periodontal maintenance capacity around a proposed bridge

Justification: The capacity to maintain clean, disease-controlled abutments is fundamental to a predictable fixed prosthesis.

Why not the closest alternatives: Initial desire for a bridge is insufficient when foreseeable maintenance harms are high.

Revise: Volume I — Unit 5 · Diploma LO 2c.1, 2b.4, 2b.10

Return to Question 1290 ↑

Q1291 ADVANCED · Block E

Best answer: D. Aesthetic concern in a visible region despite acceptable mastication

Justification: Appearance and self-perceived impact are legitimate indications for discussing conservative replacement options.

Why not the closest alternatives: Absence of a chewing problem does not make anterior replacement unnecessary for this patient.

Revise: Volume I — Unit 5 · Diploma LO 2c.1, 2b.4, 2b.10

Return to Question 1291 ↑

Q1292 ADVANCED · Block E

Best answer: B. Occlusal space and opposing tooth position after long-standing edentulism

Justification: Available restorative space and opposing tooth position may require preparatory correction before any definitive replacement.

Why not the closest alternatives: Implant planning cannot ignore a mechanically inadequate interarch space.

Revise: Volume I — Unit 5 · Diploma LO 2c.1, 2b.4, 2b.10

Return to Question 1292 ↑

Q1293 ADVANCED · Block E

Best answer: B. Repair the local restoration after addressing the specific failure mechanism

Justification: Repairable material and identifiable correctable causes support conservative local repair rather than immediate full-coverage work.

Why not the closest alternatives: Escalation to a crown after one failure is not automatically justified.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.6, 2c.2

Return to Question 1293 ↑

Q1294 ADVANCED · Block E

Best answer: D. Replace a repeatedly failing restoration with a conservative indirect option when tissue permits

Justification: A conservative bonded indirect restoration may offer a more durable option while preserving sound enamel.

Why not the closest alternatives: Repeated direct replacement without reconsidering restoration type is less appropriate after several mechanism-consistent failures.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.6, 2c.2

Return to Question 1294 ↑

Q1295 ADVANCED · Block E

Best answer: E. Reconsider prosthesis design and strategic abutments before repeating a failed long-span bridge

Justification: Multiple design and biologic factors should be reassessed before repeating the same prosthesis.

Why not the closest alternatives: Recementation alone does not address an overloaded and compromised bridge design.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.6, 2c.2

Return to Question 1295 ↑

Q1296 ADVANCED · Block E

Best answer: D. Stabilise a high-risk oral environment before undertaking definitive rehabilitation

Justification: Risk stabilisation and realistic hygiene and recall capacity must precede definitive reconstruction.

Why not the closest alternatives: More extensive coverage is not protective against ongoing uncontrolled disease.

Revise: Volume I — Unit 5 · Diploma LO 2a.18, 2b.6, 2c.2

Return to Question 1296 ↑

BLOCK F · Removable partial dentures

Q1037 ADVANCED · Block F

Best answer: E. Use mesial rests and an appropriate stress-releasing retainer assembly

Justification: Mesial rest and stress-releasing RPI/RPA principles reduce harmful torque under saddle rotation when anatomy permits.

Why not the closest alternatives: Rigid distal-rest clasp assemblies may transmit greater leverage to the abutments.

Revise: Diploma LO 2c.2, 2c.6, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Distal-extension RPD stress control

Return to Question 1037 ↑

Q1038 ADVANCED · Block F

Best answer: D. Reconsider the design after surveying and evaluating abutment contours

Justification: Survey the cast and reconsider the retainer; an I-bar is not universally applicable, especially across severe tissue undercuts.

Why not the closest alternatives: Blindly fitting a textbook RPI assembly ignores anatomical contraindications.

Revise: Diploma LO 2c.2, 2c.6, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Distal-extension RPD stress control

Return to Question 1038 ↑

Q1039 ADVANCED · Block F

Best answer: B. Reconsider the design after surveying and evaluating abutment contours

Justification: Support and clasp design depend on actual saddle class, survey findings and abutment anatomy, rather than assuming a distal-extension stress-release scheme.

Why not the closest alternatives: The special stress-release principles of a Kennedy I saddle need not dictate all tooth-supported RPD designs.

Revise: Diploma LO 2c.2, 2c.6, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Distal-extension RPD stress control

Return to Question 1039 ↑

Q1040 ADVANCED · Block F

Best answer: B. Use a lingual bar when functional clearance is adequate

Justification: Adequate functional clearance allows a hygienic lingual bar with appropriate gingival distance and depth.

Why not the closest alternatives: A plate covers more dental surfaces unnecessarily when a bar fits.

Revise: Diploma LO 2c.6, 2c.13, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD major connector

Return to Question 1040 ↑

Q1041 ADVANCED · Block F

Best answer: B. Use a lingual plate when clearance is inadequate and appropriate coverage is possible

Justification: A lingual plate may be appropriate when a standard bar lacks space and contacts/contours can be managed.

Why not the closest alternatives: A standard bar placed too close to the gingiva can damage soft tissues.

Revise: Diploma LO 2c.6, 2c.13, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD major connector

Return to Question 1041 ↑

Q1042 ADVANCED · Block F

Best answer: B. Consider a labial bar or surgical correction where lingual tori preclude conventional designs

Justification: A labial bar or consideration of torus removal may be required after careful surveying and shared decision-making.

Why not the closest alternatives: Forcing an unrelieved lingual connector over tori is unsafe.

Revise: Diploma LO 2c.6, 2c.13, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD major connector

Return to Question 1042 ↑

Q1043 ADVANCED · Block F

Best answer: C. Adjust the clasp only after surveying its support, position and fatigue history

Justification: Clasp deformation and metal fatigue require controlled evaluation and an appropriate repair/replacement decision, not patient bending.

Why not the closest alternatives: Replacing the whole RPD without a diagnosis may be unnecessary.

Revise: Diploma LO 2c.6, 2c.20, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD loss of retention

Return to Question 1043 ↑

Q1044 ADVANCED · Block F

Best answer: B. Investigate loss of occlusal support and tissue adaptation before clasp tightening

Justification: Support loss from ridge change causes functional rotation; assess reline/impression and occlusion before tightening a clasp.

Why not the closest alternatives: Retention is not synonymous with saddle support.

Revise: Diploma LO 2c.6, 2c.20, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD loss of retention

Return to Question 1044 ↑

Q1045 ADVANCED · Block F

Best answer: B. Identify a missing or worn guide plane/rest and redesign the deficient component

Justification: An ineffective rest/guide plane may prevent a predictable path and stable seating; correct the support and guiding surfaces.

Why not the closest alternatives: More clasp force alone will not establish proper support and guidance.

Revise: Diploma LO 2c.6, 2c.20, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD loss of retention

Return to Question 1045 ↑

Q1112 ADVANCED · Block F

Best answer: B. Position indirect retention as far as feasible from the fulcrum line

Justification: An appropriately placed indirect retainer with leverage from the fulcrum line helps resist lift of distal-extension saddles.

Why not the closest alternatives: Direct clasp retention alone may not control rotational movement.

Revise: Diploma LO 2c.6, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Indirect retention and bounded saddles

Return to Question 1112 ↑

Q1113 ADVANCED · Block F

Best answer: C. Provide rigid tooth-borne support for a bounded saddle

Justification: A bounded saddle gains predominantly tooth-borne support through rests and rigid connection.

Why not the closest alternatives: A distal-extension altered-cast technique is not routinely required for a tooth-borne saddle.

Revise: Diploma LO 2c.6, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Indirect retention and bounded saddles

Return to Question 1113 ↑

Q1114 ADVANCED · Block F

Best answer: A. Assess the need for altered-cast or functional impression techniques in distal extension

Justification: Differential tooth and mucosal support may justify a functional impression or altered-cast approach.

Why not the closest alternatives: Adding more rigid clasps does not compensate for resilient tissue support.

Revise: Diploma LO 2c.6, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Indirect retention and bounded saddles

Return to Question 1114 ↑

Q1115 ADVANCED · Block F

Best answer: E. Survey path of insertion and evaluate guide planes and retentive undercuts

Justification: Surveying identifies a single feasible path and the need for planned tooth modification or blockout.

Why not the closest alternatives: Unplanned clasp placement risks traumatic insertion and poor seating.

Revise: Diploma LO 2c.6, 2c.20, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: Surveying a partially dentate arch

Return to Question 1115 ↑

Q1116 ADVANCED · Block F

Best answer: A. Modify tooth contours conservatively where necessary for appropriate guide planes

Justification: Conservative enamel contour modification can establish a guide plane when properly planned and consented.

Why not the closest alternatives: A poor path should not be accepted merely to avoid all preparation.

Revise: Diploma LO 2c.6, 2c.20, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: Surveying a partially dentate arch

Return to Question 1116 ↑

Q1117 ADVANCED · Block F

Best answer: C. Survey path of insertion and evaluate guide planes and retentive undercuts

Justification: Surveyed undercuts must be considered together with stable support; restoring the rest seat or contour may be necessary.

Why not the closest alternatives: Retention without support accelerates instability and tissue damage.

Revise: Diploma LO 2c.6, 2c.20, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: Surveying a partially dentate arch

Return to Question 1117 ↑

Q1169 ADVANCED · Block F

Best answer: B. Use the surveyor to locate a single feasible path of insertion

Justification: Guide planes, blockout and rest seat alignment must permit a safe insertion path before retentive force is increased.

Why not the closest alternatives: Forcing seating risks tooth damage and poor stability.

Revise: Diploma LO 2c.6, 2c.20, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD path and clasp choice

Return to Question 1169 ↑

Q1170 ADVANCED · Block F

Best answer: B. Evaluate a wrought-wire retainer where flexibility or adjustment is needed

Justification: Wrought-wire retainer systems may allow flexibility and controlled adjustment in suitable designs.

Why not the closest alternatives: An I-bar crossing high tissue undercuts can traumatise soft tissue.

Revise: Diploma LO 2c.6, 2c.20, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD path and clasp choice

Return to Question 1170 ↑

Q1171 ADVANCED · Block F

Best answer: C. Choose a rigid cast clasp where geometry and support permit predictable deflection

Justification: A properly designed cast clasp may provide retention when support, bracing and survey findings permit.

Why not the closest alternatives: Clasp selection is not about maximum force independent of periodontal support.

Revise: Diploma LO 2c.6, 2c.20, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: RPD path and clasp choice

Return to Question 1171 ↑

Q1297 ADVANCED · Block F

Best answer: C. Use a mesial occlusal rest on a distal-extension terminal abutment in a suitable stress-releasing design

Justification: Mesial rests with suitable guiding planes and stress-release can help manage distal-extension saddle rotation and abutment loading.

Why not the closest alternatives: Simply adding a larger distal clasp can increase leverage on a terminal abutment.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1297 ↑

Q1298 ADVANCED · Block F

Best answer: E. Place an indirect retainer anterior to the fulcrum line on a suitable abutment

Justification: An appropriately placed indirect retainer counters rotation around the fulcrum line.

Why not the closest alternatives: A larger clasp undercut may not efficiently oppose the rotational displacement.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1298 ↑

Q1299 ADVANCED · Block F

Best answer: C. Provide broad denture-base coverage to improve support of a free-end saddle

Justification: Broad functional base coverage distributes distal-extension load and reduces tissue stress.

Why not the closest alternatives: More rigid clasp arms do not replace missing denture-base support.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1299 ↑

Q1300 ADVANCED · Block F

Best answer: A. Survey and establish an appropriate path of insertion before designing clasps

Justification: Surveying and a planned path of insertion are prerequisites to a mechanically consistent RPD design.

Why not the closest alternatives: Changing one clasp without revisiting the framework path can reproduce binding.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1300 ↑

Q1301 ADVANCED · Block F

Best answer: C. Choose a lingual bar where functional depth and gingival clearance are adequate

Justification: A lingual bar is often the hygienic, relatively low-coverage connector when functional clearance is sufficient.

Why not the closest alternatives: A plate would unnecessarily cover additional tooth surfaces in this anatomy.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1301 ↑

Q1302 ADVANCED · Block F

Best answer: B. Consider a lingual plate where clearance is inadequate and tooth contours permit

Justification: A lingual plate may be appropriate when bar clearance cannot be achieved, provided tooth support and hygiene are acceptable.

Why not the closest alternatives: Forcing a bar into the gingival tissues risks inflammation and discomfort.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1302 ↑

Q1303 ADVANCED · Block F

Best answer: E. Choose a palatal strap with adequate width and rigidity for a bounded saddle

Justification: A correctly dimensioned palatal strap can provide rigidity with reasonable patient acceptance.

Why not the closest alternatives: Full palatal coverage would be unnecessary in a well-supported bounded span.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1303 ↑

Q1304 ADVANCED · Block F

Best answer: C. Consider a broad palatal plate for poor support in extensive maxillary distal extensions

Justification: A broad palatal connector can distribute load and supplement limited dental support when appropriately planned.

Why not the closest alternatives: A narrow connector alone may fail to provide adequate rigidity and support.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1304 ↑

Q1305 ADVANCED · Block F

Best answer: C. Use an appropriately designed RPI-type assembly for a suitable distal extension

Justification: An RPI concept can allow favourable release and reduce torque with appropriate rest and proximal plate design.

Why not the closest alternatives: A simple rigid circumferential clasp may load the abutment unfavourably during tissue-ward rotation.

Revise: Volume I — Unit 6 · Diploma LO 2c.3, 2c.13

Return to Question 1305 ↑

Q1306 ADVANCED · Block F

Best answer: B. Use a wrought-wire retentive arm where increased flexibility is needed

Justification: A suitably designed wrought-wire clasp can offer flexibility where a cast rigid arm is unsuitable.

Why not the closest alternatives: Repeatedly bending a cast clasp is not a durable correction.

Revise: Volume I — Unit 6 · Diploma LO 2c.3, 2c.13

Return to Question 1306 ↑

Q1307 ADVANCED · Block F

Best answer: E. Use a cast circumferential clasp for a suitable bounded saddle abutment

Justification: A conventional cast circumferential clasp can provide support and retention in the simpler tooth-supported case.

Why not the closest alternatives: Complex stress-release is not automatically necessary for a bounded saddle.

Revise: Volume I — Unit 6 · Diploma LO 2c.3, 2c.13

Return to Question 1307 ↑

Q1308 ADVANCED · Block F

Best answer: A. Adjust the survey line or guide plane through conservative recontouring where safe

Justification: Conservative recontouring or replacement of the contour can produce a more appropriate survey line and guiding surface.

Why not the closest alternatives: Choosing an extreme undercut without correction creates avoidable stress and insertion problems.

Revise: Volume I — Unit 6 · Diploma LO 2c.3, 2c.13

Return to Question 1308 ↑

Q1309 ADVANCED · Block F

Best answer: C. Correct absent or ineffective occlusal rests on the abutments

Justification: Definitive tooth support depends on correctly located and seated rests; without them tissue loading and movement increase.

Why not the closest alternatives: A tighter clasp cannot substitute for inadequate support.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.4

Return to Question 1309 ↑

Q1310 ADVANCED · Block F

Best answer: E. Record appropriate functional support for a distal-extension base

Justification: A suitable altered-cast or functional-support approach can improve adaptation where indicated for distal extensions.

Why not the closest alternatives: Adding clasp tension without improving saddle support can worsen abutment stress.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.4

Return to Question 1310 ↑

Q1311 ADVANCED · Block F

Best answer: E. Reassess clasp reciprocity and framework fit before tightening retention

Justification: Framework seating, clasp balance and reciprocal contact must be corrected before altering retention.

Why not the closest alternatives: Arbitrary clasp tightening would increase unopposed horizontal forces.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.4

Return to Question 1311 ↑

Q1312 ADVANCED · Block F

Best answer: D. Adjust an occlusal interference causing denture rocking in function

Justification: Correct a demonstrable functional occlusal interference while preserving the intended support and retention.

Why not the closest alternatives: A remake without investigating contacts would miss a simple mechanical cause.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.4

Return to Question 1312 ↑

Q1313 ADVANCED · Block F

Best answer: A. Reduce an excessive distal cantilever of artificial teeth on a free-end saddle

Justification: An excessive distal tooth extension increases the lever arm and tissue loading; limiting the distal occlusal table may improve stability.

Why not the closest alternatives: Simply stiffening clasps can increase terminal-abutment torque.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.20

Return to Question 1313 ↑

Q1314 ADVANCED · Block F

Best answer: B. Use an appropriate guide-plane and rest configuration to control insertion forces

Justification: A properly surveyed insertion path and controlled guide plane reduce unnecessary lateral force on an abutment.

Why not the closest alternatives: More retentive clasp tips will not correct a binding path.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.20

Return to Question 1314 ↑

Q1315 ADVANCED · Block F

Best answer: A. Reassess abutment prognosis and consider alternative support where periodontal disease is active

Justification: Active periodontal prognosis must be reassessed before using the tooth as a strategic RPD support and retainer.

Why not the closest alternatives: An ingenious clasp design cannot compensate for unstable periodontal health.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.20

Return to Question 1315 ↑

Q1316 ADVANCED · Block F

Best answer: C. Reduce plaque traps and improve cleansability around clasp and connector surfaces

Justification: Reduce plaque-retentive coverage where design permits and establish personalised cleaning and review.

Why not the closest alternatives: Unchanged bulky contours may perpetuate disease even after initial professional cleaning.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.20

Return to Question 1316 ↑

Q1317 ADVANCED · Block F

Best answer: D. Reduce excessive occlusal loading on a distal-extension RPD

Justification: Occlusal load distribution is critical when mucosal support differs from tooth support.

Why not the closest alternatives: Changing acrylic colour or thickness alone cannot correct overload.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1317 ↑

Q1318 ADVANCED · Block F

Best answer: B. Design an appropriate indirect retainer for rotation away from tissues

Justification: Correct indirect retention counteracts rotation about the fulcrum when dislodging forces act.

Why not the closest alternatives: Additional base extension alone does not guarantee resistance to lifting.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1318 ↑

Q1319 ADVANCED · Block F

Best answer: C. Investigate a compromised periodontal abutment before using it for support

Justification: Periodontal stability and prognosis precede use of a compromised tooth as a support element.

Why not the closest alternatives: An occlusal rest on an active high-risk abutment may accelerate harm.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1319 ↑

Q1320 ADVANCED · Block F

Best answer: C. Reassess the occlusal plane before arranging artificial teeth in a missing-tooth space

Justification: The opposing tooth position and occlusal plane need evaluation before attempting replacement in a reduced space.

Why not the closest alternatives: Simply setting an artificial tooth high out of occlusion may not fulfil functional aims.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.20

Return to Question 1320 ↑

Q1321 ADVANCED · Block F

Best answer: D. Preserve sound strategically important teeth and plan a cleanable tooth-supported RPD

Justification: A conventional tooth-supported RPD is proportionate and maintainable for bounded saddle defects.

Why not the closest alternatives: Removing sound abutments for complete dentures would sacrifice useful support.

Revise: Volume I — Unit 6 · Diploma LO 2c.4, 2c.13

Return to Question 1321 ↑

Q1322 ADVANCED · Block F

Best answer: C. Use a distal-extension RPD with realistic patient counselling about movement and maintenance

Justification: An appropriately designed free-end RPD remains a legitimate option with honest discussion of support and maintenance.

Why not the closest alternatives: Free-end movement is a design constraint, not an automatic contraindication.

Revise: Volume I — Unit 6 · Diploma LO 2c.4, 2c.13

Return to Question 1322 ↑

Q1323 ADVANCED · Block F

Best answer: A. Consider a shortened dental arch without distal tooth replacement

Justification: Observation and continued preventive care may be better than an unwanted distal extension.

Why not the closest alternatives: Prosthodontic success includes restraint when the patient has adequate function.

Revise: Volume I — Unit 6 · Diploma LO 2c.4, 2c.13

Return to Question 1323 ↑

Q1324 ADVANCED · Block F

Best answer: D. Use an interim acrylic RPD where rapid changes or additions are expected

Justification: A temporary, easily adjustable acrylic RPD can maintain aesthetics through changing dentition and tissue contours.

Why not the closest alternatives: A definitive cobalt-chromium framework is less adaptable during rapid treatment changes.

Revise: Volume I — Unit 6 · Diploma LO 2c.4, 2c.13

Return to Question 1324 ↑

Q1325 ADVANCED · Block F

Best answer: D. Loss of rest support due to a poorly fitting framework

Justification: Loss of correct rest seating changes support and can cause rotation; the framework/crown relationship needs reassessment.

Why not the closest alternatives: Tightening clasps is inappropriate while the rigid framework is displaced.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.21

Return to Question 1325 ↑

Q1326 ADVANCED · Block F

Best answer: C. Excessive posterior flange extension in a distal-extension saddle

Justification: The posterior flange has encroached on functional tissue movement; assess and relieve it appropriately.

Why not the closest alternatives: A material change in the whole denture is unlikely to be necessary.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.21

Return to Question 1326 ↑

Q1327 ADVANCED · Block F

Best answer: A. Clasp deformation from inappropriate patient adjustment

Justification: Unauthorised clasp bending can permanently deform or fracture an alloy; assess fit and repair or replace appropriately.

Why not the closest alternatives: A new reline alone will not restore the lost clasp geometry.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.21

Return to Question 1327 ↑

Q1328 ADVANCED · Block F

Best answer: E. Inflammation caused by plaque-retentive denture contours

Justification: The pattern points toward plaque-associated inflammation from coverage and inadequate hygiene rather than simple traumatic overload.

Why not the closest alternatives: Occlusal adjustments alone will not resolve a plaque-retentive design problem.

Revise: Volume I — Unit 6 · Diploma LO 2c.13, 2c.21

Return to Question 1328 ↑

Q1329 ADVANCED · Block F

Best answer: D. Shift the fulcrum strategy by appropriately located mesial rests and stress release

Justification: Mesial rests and suitable stress-release design may better accommodate distal-extension movement.

Why not the closest alternatives: Simply increasing retention would not remove the lever mechanism.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1329 ↑

Q1330 ADVANCED · Block F

Best answer: C. Use a flexible clasp assembly where a deep undercut demands it

Justification: Material and arm geometry should accommodate the functional undercut while avoiding excessive tooth force.

Why not the closest alternatives: A short cobalt-chromium clasp in an excessive undercut risks overload and fracture.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1330 ↑

Q1331 ADVANCED · Block F

Best answer: A. Provide a guiding plane and reciprocation rather than relying on frictional clasping

Justification: A planned insertion path, guiding plane and reciprocal element are essential to reduce harmful lateral forces.

Why not the closest alternatives: Greater retention in the absence of reciprocity would worsen the problem.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1331 ↑

Q1332 ADVANCED · Block F

Best answer: A. Reduce an over-extended labial/buccal flange that conflicts with muscles

Justification: Adjust the flange to permit function and avoid tissue trauma; no redesign of the clasp system is necessarily needed.

Why not the closest alternatives: A change to the major connector cannot correct a localized labial over-extension.

Revise: Volume I — Unit 6 · Diploma LO 2c.2, 2c.13

Return to Question 1332 ↑

BLOCK G · Complete dentures

Q1046 ADVANCED · Block G

Best answer: C. Check functional peripheral border extension and polished surface contours

Justification: Functional displacement points to lingual border overextension and polished-surface interference.

Why not the closest alternatives: Changing vertical dimension is unlikely to resolve tongue-induced displacement.

Revise: Diploma LO 2c.9, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Diagnosing complete denture instability

Return to Question 1046 ↑

Q1047 ADVANCED · Block G

Best answer: D. Investigate occlusal errors and centric discrepancy

Justification: A closure-related problem with otherwise good retention is consistent with an occlusal error; remount and correct contacts.

Why not the closest alternatives: Adhesive would obscure rather than resolve the cause.

Revise: Diploma LO 2c.9, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Diagnosing complete denture instability

Return to Question 1047 ↑

Q1048 ADVANCED · Block G

Best answer: A. Manage xerostomia and tissue condition before assuming a new denture is required

Justification: A reduced saliva film compromises denture retention; address hyposalivation and mucosal health before remaking.

Why not the closest alternatives: A higher OVD or an adhesive-only strategy fails to address the main change.

Revise: Diploma LO 2c.9, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Diagnosing complete denture instability

Return to Question 1048 ↑

Q1049 ADVANCED · Block G

Best answer: D. Assess freeway space, speech, aesthetics and existing vertical dimension before adjustment

Justification: Multiple functional checks are needed to establish whether OVD is excessive and define a controlled correction.

Why not the closest alternatives: An arbitrary reduction may create new problems.

Revise: Diploma LO 2c.13, 2c.21, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Changing vertical dimension in dentures

Return to Question 1049 ↑

Q1050 ADVANCED · Block G

Best answer: B. Use a copy-denture approach retaining tolerated relationships

Justification: Copy dentures preserve familiar relationships and polished surfaces while allowing necessary controlled changes.

Why not the closest alternatives: Maximum OVD increase is counterproductive for a poor adapter.

Revise: Diploma LO 2c.13, 2c.21, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Changing vertical dimension in dentures

Return to Question 1050 ↑

Q1051 ADVANCED · Block G

Best answer: D. Remount and correct occlusion while preserving an otherwise acceptable OVD

Justification: Correct the demonstrable occlusal discrepancy while retaining a successful OVD.

Why not the closest alternatives: OVD change is unjustified when the fault is a deflective contact.

Revise: Diploma LO 2c.13, 2c.21, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Changing vertical dimension in dentures

Return to Question 1051 ↑

Q1052 ADVANCED · Block G

Best answer: C. Use a window or selective-pressure impression technique

Justification: Window/selective-pressure methods control how the displaceable ridge is recorded while capturing stable supporting surfaces.

Why not the closest alternatives: Uniform compression misrepresents the resting anatomy.

Revise: Diploma LO 2c.6, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Flabby ridge impressions

Return to Question 1052 ↑

Q1053 ADVANCED · Block G

Best answer: E. Use mucostatic registration of the mobile anterior tissues with controlled support elsewhere

Justification: Record mobile tissues without displacement and stable areas with deliberate support, rather than compressing the entire ridge.

Why not the closest alternatives: A nonselective compressive technique can create instability.

Revise: Diploma LO 2c.6, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Flabby ridge impressions

Return to Question 1053 ↑

Q1054 ADVANCED · Block G

Best answer: D. Investigate underlying pathology before assuming the ridge is harmless

Justification: An atypical enlarging, ulcerated lesion needs investigation before prosthetic assumptions.

Why not the closest alternatives: A new impression alone could delay diagnosis of significant disease.

Revise: Diploma LO 2c.6, 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Flabby ridge impressions

Return to Question 1054 ↑

Q1118 ADVANCED · Block G

Best answer: D. Use pressure-indicator paste and functional movement to locate overextension

Justification: Functional marking can locate an overextended flange responsible for pressure during tongue or mylohyoid movement.

Why not the closest alternatives: Broad fitting-surface relief unnecessarily sacrifices support.

Revise: Diploma LO 2c.21, 2c.22, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Denture soreness source localisation

Return to Question 1118 ↑

Q1119 ADVANCED · Block G

Best answer: B. Assess occlusal deflective contacts and correct them after remount

Justification: A deflective occlusal load can traumatically concentrate force despite an adequately fitting base.

Why not the closest alternatives: Reducing broad denture-bearing surfaces does not correct the overload.

Revise: Diploma LO 2c.21, 2c.22, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Denture soreness source localisation

Return to Question 1119 ↑

Q1120 ADVANCED · Block G

Best answer: B. Refer a persistent unexplained mucosal ulcer for investigation

Justification: A non-healing unexplained oral ulcer warrants urgent investigation rather than repeated mechanical adjustments.

Why not the closest alternatives: Assuming all ulcers are traumatic risks missing malignancy.

Revise: Diploma LO 2c.21, 2c.22, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Denture soreness source localisation

Return to Question 1120 ↑

Q1121 ADVANCED · Block G

Best answer: B. Optimise conventional mandibular denture design and tissue condition before escalating

Justification: Correct remediable conventional errors before concluding that implants are required.

Why not the closest alternatives: An implant treatment decision based on a poorly made denture is premature.

Revise: Diploma LO 2c.4, 2c.9, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: Lower complete denture replacement options

Return to Question 1121 ↑

Q1122 ADVANCED · Block G

Best answer: D. Discuss two-implant overdenture retention as a potential option after assessment

Justification: A mandibular implant-retained overdenture is an established option that can improve retention in appropriate cases.

Why not the closest alternatives: Implants remain elective and require individual assessment and maintenance consent.

Revise: Diploma LO 2c.4, 2c.9, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: Lower complete denture replacement options

Return to Question 1122 ↑

Q1123 ADVANCED · Block G

Best answer: A. Refer for complex implant planning where anatomy or systemic risks exceed Level 2

Justification: High anatomical complexity makes specialist planning appropriate to minimise neurovascular risk.

Why not the closest alternatives: A routine implant approach is inappropriate without suitable imaging and experienced assessment.

Revise: Diploma LO 2c.4, 2c.9, 2c.21 · Volume I and Diploma learning outcomes (August 2026) · Theme: Lower complete denture replacement options

Return to Question 1123 ↑

Q1172 ADVANCED · Block G

Best answer: D. Explain adaptation, agree targeted adjustments and set review criteria

Justification: Explain expected adaptation, investigate actual phonetic and sore areas, adjust only where indicated and review progress.

Why not the closest alternatives: An immediate full remake is not evidence-led.

Revise: Diploma LO 2c.9, 2c.21, 2c.26 · Volume I and Diploma learning outcomes (August 2026) · Theme: Denture adaptation and management of complaints

Return to Question 1172 ↑

Q1173 ADVANCED · Block G

Best answer: A. Use a copy-denture technique when established contours aid adaptation

Justification: Copy techniques retain tolerable tooth position and polished surfaces while correcting fit or defects.

Why not the closest alternatives: A completely new design may create avoidable adaptation difficulties.

Revise: Diploma LO 2c.9, 2c.21, 2c.26 · Volume I and Diploma learning outcomes (August 2026) · Theme: Denture adaptation and management of complaints

Return to Question 1173 ↑

Q1174 ADVANCED · Block G

Best answer: A. Modify the denture to improve cleansability where stomatitis is recurrent

Justification: Improve hygiene, cleaning access, fit and appropriate management of denture stomatitis to prevent recurrence.

Why not the closest alternatives: Merely remaking without preventive support risks repeat disease.

Revise: Diploma LO 2c.9, 2c.21, 2c.26 · Volume I and Diploma learning outcomes (August 2026) · Theme: Denture adaptation and management of complaints

Return to Question 1174 ↑

Q1333 ADVANCED · Block G

Best answer: C. Reassess maxillary posterior palatal seal position and function

Justification: An inadequate posterior palatal seal can compromise maxillary denture retention; locate functional anatomy and correct the posterior extension.

Why not the closest alternatives: Altering the lower neutral zone would not remedy an upper posterior seal defect.

Revise: Volume I — Unit 7 · Diploma LO 2c.4, 2c.9, 2c.21

Return to Question 1333 ↑

Q1334 ADVANCED · Block G

Best answer: B. Reduce overextension in the mandibular masseteric notch

Justification: The masseter can displace an overextended mandibular distobuccal flange during clenching.

Why not the closest alternatives: A palatal seal applies to the upper denture, not this lower functional border.

Revise: Volume I — Unit 7 · Diploma LO 2c.4, 2c.9, 2c.21

Return to Question 1334 ↑

Q1335 ADVANCED · Block G

Best answer: C. Re-establish the mandibular neutral-zone contour and tongue space

Justification: A bulky lingual polished surface can invade tongue space, so neutral-zone recontouring is appropriate.

Why not the closest alternatives: More border extension may make this particular instability worse.

Revise: Volume I — Unit 7 · Diploma LO 2c.4, 2c.9, 2c.21

Return to Question 1335 ↑

Q1336 ADVANCED · Block G

Best answer: E. Improve retention by accurately recording functional border depth

Justification: Under-extended borders reduce the peripheral seal; a properly functionally recorded extension can improve stability.

Why not the closest alternatives: Adding adhesive before correcting substantial under-extension treats a symptom, not its cause.

Revise: Volume I — Unit 7 · Diploma LO 2c.4, 2c.9, 2c.21

Return to Question 1336 ↑

Q1337 ADVANCED · Block G

Best answer: D. Reassess OVD using several clinical criteria rather than a single number

Justification: Rest position is variable; combine facial support, speech, aesthetics, function and occlusal records rather than relying on one fixed subtraction.

Why not the closest alternatives: A nominal 3-mm freeway space cannot substitute for full assessment.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1337 ↑

Q1338 ADVANCED · Block G

Best answer: E. Record a reproducible horizontal jaw relation before setting posterior teeth

Justification: A reproducible jaw relation is needed before arranging occlusion and finalising dentures.

Why not the closest alternatives: Proceeding to final processing with inconsistent records risks unstable contacts.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1338 ↑

Q1339 ADVANCED · Block G

Best answer: B. Correct an excessive OVD causing clicking, muscular strain and limited freeway space

Justification: An excessive OVD is plausible; re-evaluate the jaw relation and remount or remake as appropriate.

Why not the closest alternatives: Simply adding denture adhesive will not correct an excessive vertical dimension.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1339 ↑

Q1340 ADVANCED · Block G

Best answer: C. Investigate a reduced OVD associated with poor support and aesthetics

Justification: Assess OVD and aesthetics in relation to worn teeth and ridge changes before prescribing replacement.

Why not the closest alternatives: One photograph or absolute metric alone cannot define the correct new dimension.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1340 ↑

Q1341 ADVANCED · Block G

Best answer: E. Use a selective-pressure technique for a displaceable anterior maxillary ridge

Justification: Selective pressure or a windowed approach avoids recording flabby tissue in a distorted position.

Why not the closest alternatives: A uniformly forceful impression may worsen final denture instability.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1341 ↑

Q1342 ADVANCED · Block G

Best answer: B. Use functional border moulding to record a resorbed mandibular lingual sulcus

Justification: Functional border recording of the lingual and posterior sulci can improve extension without impinging on moving tissues.

Why not the closest alternatives: A purely static impression may underestimate useful lingual extension.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1342 ↑

Q1343 ADVANCED · Block G

Best answer: B. Use a copy-denture approach where adaptation and established contours are key

Justification: A replica/copy technique can retain successful features while restoring fit and function.

Why not the closest alternatives: Radical redesign may be less acceptable even if mechanically ideal.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1343 ↑

Q1344 ADVANCED · Block G

Best answer: B. Provide targeted relief over prominent pressure-sensitive bony anatomy

Justification: Provide appropriate relief over non-load-bearing sensitive bony prominences, while retaining broad support elsewhere.

Why not the closest alternatives: Heavy pressure over a torus risks recurrence of the same ulceration.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.22

Return to Question 1344 ↑

Q1345 ADVANCED · Block G

Best answer: C. Inadequate upper posterior palatal seal or border adaptation

Justification: Failure of the functional posterior seal plausibly reduces maxillary retention, even with good basal adaptation.

Why not the closest alternatives: Occlusal adjustment is unlikely to close the posterior air leak.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.22

Return to Question 1345 ↑

Q1346 ADVANCED · Block G

Best answer: C. Excessive posterior occlusal contact causing tipping

Justification: Premature heavy posterior contacts can tip the denture in function; assess balanced support and contacts.

Why not the closest alternatives: Saliva is not the main issue when instability is tightly linked to biting.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.22

Return to Question 1346 ↑

Q1347 ADVANCED · Block G

Best answer: D. An incorrect neutral-zone contour in a highly resorbed lower arch

Justification: The muscle balance around the lower denture is critical when ridge retention is poor; correct neutral-zone contour.

Why not the closest alternatives: Simply increasing acrylic thickness could aggravate displacement.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.22

Return to Question 1347 ↑

Q1348 ADVANCED · Block G

Best answer: E. Reduced salivary film from xerostomia

Justification: Xerostomia degrades the salivary film required for adhesion and cohesion; assess and manage dryness and the prosthesis together.

Why not the closest alternatives: Repositioning teeth does not correct loss of a functional salivary film.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.22

Return to Question 1348 ↑

Q1349 ADVANCED · Block G

Best answer: E. Use conventional complete dentures with explicit discussion of achievable limitations

Justification: Conventional complete dentures remain an appropriate patient-centred option with favourable anatomy and preferences.

Why not the closest alternatives: Implants are not universally required for adequate function.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.25

Return to Question 1349 ↑

Q1350 ADVANCED · Block G

Best answer: D. Assess a mandibular implant-retained overdenture with ongoing maintenance needs

Justification: Implant-retained overdenture assessment is reasonable after conventional limitations are understood; selection depends on individual risk and anatomy.

Why not the closest alternatives: Continuously remaking identical dentures may not resolve a severely compromised ridge.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.25

Return to Question 1350 ↑

Q1351 ADVANCED · Block G

Best answer: E. Use an interim immediate denture while post-extraction tissues change

Justification: An immediate denture can maintain appearance through the healing phase, with planned relines and subsequent review.

Why not the closest alternatives: A definitive long-term base fit cannot be assumed immediately post-extraction.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.25

Return to Question 1351 ↑

Q1352 ADVANCED · Block G

Best answer: E. Refer for a complex surgical-prosthetic plan when standard anatomy is profoundly altered

Justification: Complex acquired defects can exceed Level 2 care and warrant specialist multidisciplinary planning.

Why not the closest alternatives: Routine complete-denture techniques may be inadequate for major tissue defects.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.25

Return to Question 1352 ↑

Q1353 ADVANCED · Block G

Best answer: E. Adjust a localised traumatic pressure area and review healing promptly

Justification: Correct a confirmed pressure spot selectively and arrange appropriate review.

Why not the closest alternatives: A complete remake is excessive for a single identified adjustment area.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.26

Return to Question 1353 ↑

Q1354 ADVANCED · Block G

Best answer: C. Re-mount and correct demonstrable occlusal errors after processing

Justification: Remounting can identify and correct processing-related occlusal discrepancies in a controlled way.

Why not the closest alternatives: Arbitrarily relieving the fitting surface may sacrifice an otherwise good base adaptation.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.26

Return to Question 1354 ↑

Q1355 ADVANCED · Block G

Best answer: E. Investigate recurrent midline fracture and redesign for the underlying load mechanism

Justification: Recurring fatigue fractures need an assessment of occlusal forces, fit, reinforcement and design, not only another repair.

Why not the closest alternatives: Repeated repair without investigating load predicts further failure.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.26

Return to Question 1355 ↑

Q1356 ADVANCED · Block G

Best answer: E. Reassess altered denture-bearing tissues before undertaking a hard reline

Justification: A well-indicated reline can restore intimate base adaptation when the tooth arrangement remains acceptable.

Why not the closest alternatives: Replacing all dentures may be unnecessary where only the fitting surface has changed.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.26

Return to Question 1356 ↑

Q1357 ADVANCED · Block G

Best answer: A. Assess lip support and anterior tooth display using try-in and photographs

Justification: Anterior tooth position influences lip support and aesthetics; a try-in is the appropriate time to modify it.

Why not the closest alternatives: Posterior occlusal refinement is secondary to the presenting aesthetic concern.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.23

Return to Question 1357 ↑

Q1358 ADVANCED · Block G

Best answer: D. Adjust tongue space and lower posterior tooth position within the neutral zone

Justification: The neutral zone and tongue space guide lower tooth positioning, especially with a resorbed arch.

Why not the closest alternatives: Changing anterior tooth display would not remove a posterior tongue-space conflict.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.23

Return to Question 1358 ↑

Q1359 ADVANCED · Block G

Best answer: A. Evaluate bilateral contacts and occlusal plane to improve functional stability

Justification: Occlusal plane and balanced functional contacts should be reviewed prior to processing.

Why not the closest alternatives: A well-seated static rim does not prove that dynamic occlusion is stable.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.23

Return to Question 1359 ↑

Q1360 ADVANCED · Block G

Best answer: A. Assess phonetics and incisal edge position using speech tests

Justification: Anterior tooth position, incisal edges and vertical relation affect speech; targeted phonetic testing informs adjustment.

Why not the closest alternatives: Increased retention alone will not correct a consistent phonetic error.

Revise: Volume I — Unit 7 · Diploma LO 2c.9, 2c.23

Return to Question 1360 ↑

Q1361 ADVANCED · Block G

Best answer: E. Review tissue health, fit, occlusion and hygiene at planned intervals

Justification: Routine review should assess mucosa, fit, occlusion, hygiene and changing oral/systemic status even when the denture is serviceable.

Why not the closest alternatives: Absence of pain does not ensure absence of mucosal pathology or poor adaptation.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.27

Return to Question 1361 ↑

Q1362 ADVANCED · Block G

Best answer: A. Arrange modification or reline for loss of tissue adaptation despite stable teeth

Justification: A reline may be appropriate if adaptation has changed without more fundamental prosthetic problems.

Why not the closest alternatives: A new set of teeth is not necessarily required for base adaptation alone.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.27

Return to Question 1362 ↑

Q1363 ADVANCED · Block G

Best answer: E. Re-evaluate a change in medical or salivary status affecting comfort

Justification: Review systemic medication effects and dry-mouth prevention before blaming a mechanically sound denture.

Why not the closest alternatives: A reline alone will not replace the lost salivary lubrication.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.27

Return to Question 1363 ↑

Q1364 ADVANCED · Block G

Best answer: C. Reassess prosthesis suitability when manual dexterity declines markedly

Justification: Design and support must accommodate changing functional ability, with carer involvement where appropriate and consented.

Why not the closest alternatives: A technically precise but unusable prosthesis is not clinically successful.

Revise: Volume I — Unit 7 · Diploma LO 2c.21, 2c.27

Return to Question 1364 ↑

BLOCK H · Fixed prosthodontics

Q1055 ADVANCED · Block H

Best answer: E. Use a conservative bonded indirect restoration where substrate and isolation permit

Justification: A partial-coverage indirect bonded restoration can protect the weakened cusp while preserving sound tooth tissue.

Why not the closest alternatives: Full-coverage crowns are not automatically necessary for a single fractured cusp.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Preserving structure in fixed prosthodontics

Return to Question 1055 ↑

Q1056 ADVANCED · Block H

Best answer: B. Re-evaluate restorability and ferrule before preparing the tooth

Justification: Residual tooth structure, ferrule and the feasibility of definitive margins drive restorability.

Why not the closest alternatives: Another post cannot compensate for an un-restorable root without adequate ferrule.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Preserving structure in fixed prosthodontics

Return to Question 1056 ↑

Q1057 ADVANCED · Block H

Best answer: A. Select a resin-bonded bridge rather than full-coverage abutment preparations

Justification: An enamel-bonded resin-retained bridge is a conservative option for a single missing tooth with suitable abutments.

Why not the closest alternatives: Preparing both sound teeth for complete crowns sacrifices unnecessary structure.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Preserving structure in fixed prosthodontics

Return to Question 1057 ↑

Q1058 ADVANCED · Block H

Best answer: D. Investigate and stabilise active periodontal disease before definitive bridgework

Justification: Abutment prognosis cannot be reliably assessed until periodontal disease and maintenance are addressed.

Why not the closest alternatives: Ante’s-law-style calculations do not override active disease.

Revise: Diploma LO 2c.6, 2c.14, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Bridge abutment prognosis

Return to Question 1058 ↑

Q1059 ADVANCED · Block H

Best answer: B. Consider cantilevered adhesive replacement with strict case selection

Justification: A carefully selected conservative cantilever resin-bonded bridge can avoid unnecessary tooth reduction.

Why not the closest alternatives: Full coverage of adjacent teeth is not indicated simply because a tooth is missing.

Revise: Diploma LO 2c.6, 2c.14, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Bridge abutment prognosis

Return to Question 1059 ↑

Q1060 ADVANCED · Block H

Best answer: C. Choose implant replacement after appropriate planning rather than overloading weak abutments

Justification: A properly assessed implant-supported option can avoid obliging a weak tooth to carry an extensive bridge.

Why not the closest alternatives: A fixed bridge built on a doubtful abutment may have a poor long-term prognosis.

Revise: Diploma LO 2c.6, 2c.14, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Bridge abutment prognosis

Return to Question 1060 ↑

Q1061 ADVANCED · Block H

Best answer: D. Assess preparation geometry, height and taper before remaking the crown

Justification: Insufficient resistance and retention form cannot be cured reliably by cement selection alone.

Why not the closest alternatives: Stronger cement is not a substitute for sound preparation design.

Revise: Diploma LO 2c.11, 2c.14, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Repeated crown decementation

Return to Question 1061 ↑

Q1062 ADVANCED · Block H

Best answer: B. Investigate occlusion and parafunction contributing to loss of retention

Justification: Excessive lateral contacts and parafunction may overload cement and restoration despite acceptable preparation geometry.

Why not the closest alternatives: Geometry alone would miss the evident functional loading pattern.

Revise: Diploma LO 2c.11, 2c.14, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Repeated crown decementation

Return to Question 1062 ↑

Q1063 ADVANCED · Block H

Best answer: E. Investigate secondary caries or fracture before making a new core

Justification: The biological integrity of the tooth and possible caries/fracture require diagnosis before recementation.

Why not the closest alternatives: Blind recementation risks retaining active pathology.

Revise: Diploma LO 2c.11, 2c.14, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Repeated crown decementation

Return to Question 1063 ↑

Q1064 ADVANCED · Block H

Best answer: E. Consider a single-retainer cantilever resin-bonded bridge

Justification: A single-retainer cantilever reduces differential mobility and debond risk relative to linked independent abutments.

Why not the closest alternatives: A fixed-fixed two-wing design can debond silently on one wing.

Revise: Diploma LO 2c.6, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Resin-bonded bridge planning

Return to Question 1064 ↑

Q1065 ADVANCED · Block H

Best answer: A. Assess available enamel, occlusal clearance and abutment movement before design

Justification: Adhesive bridges require an occlusal analysis and suitable enamel surface with a feasible path of insertion.

Why not the closest alternatives: Choosing a retainer configuration without assessing clearances is premature.

Revise: Diploma LO 2c.6, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Resin-bonded bridge planning

Return to Question 1065 ↑

Q1066 ADVANCED · Block H

Best answer: A. Investigate implant-supported replacement if the adhesive substrate is inadequate

Justification: Poor enamel substrate reduces the predictability of adhesive retention; alternative fixed options warrant discussion.

Why not the closest alternatives: Repeatedly rebonding into mainly dentine ignores the substrate limitation.

Revise: Diploma LO 2c.6, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Resin-bonded bridge planning

Return to Question 1066 ↑

Q1124 ADVANCED · Block H

Best answer: C. Prefer accessible supragingival margins when clinically feasible

Justification: Supragingival margins facilitate inspection, cleaning and preservation of periodontal tissues when feasible.

Why not the closest alternatives: Deep subgingival margins add biological and technical burdens without benefit here.

Revise: Diploma LO 2c.11, 2c.14, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Margin placement and periodontal health

Return to Question 1124 ↑

Q1125 ADVANCED · Block H

Best answer: C. Assess ferrule and biologic tissues before modifying a deep margin

Justification: Restorability and respect for supracrestal tissue attachment must be evaluated before selecting treatment.

Why not the closest alternatives: Extending a finish line indiscriminately beneath gingiva is not safe.

Revise: Diploma LO 2c.11, 2c.14, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Margin placement and periodontal health

Return to Question 1125 ↑

Q1126 ADVANCED · Block H

Best answer: D. Complete periodontal treatment and plaque control before definitive crownwork

Justification: A stable periodontal environment is important before placing definitive margins and restorations.

Why not the closest alternatives: A crown cannot substitute for active periodontal therapy.

Revise: Diploma LO 2c.11, 2c.14, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Margin placement and periodontal health

Return to Question 1126 ↑

Q1127 ADVANCED · Block H

Best answer: B. Investigate suspected vertical root fracture before committing to further restoration

Justification: The pattern is strongly suspicious for vertical root fracture, which changes prognosis fundamentally.

Why not the closest alternatives: Routine retreatment or a new crown would not correct a split root.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Vertical root fracture versus non-restorable crown

Return to Question 1127 ↑

Q1128 ADVANCED · Block H

Best answer: E. Repair a small accessible marginal ceramic chip when the rest of the crown is sound

Justification: Repair or smoothing may be reasonable for a limited non-structural defect.

Why not the closest alternatives: An immediate remake exposes the tooth to avoidable intervention.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Vertical root fracture versus non-restorable crown

Return to Question 1128 ↑

Q1129 ADVANCED · Block H

Best answer: E. Refer a complex root fracture or uncertain prognosis for specialist evaluation

Justification: A specialist diagnosis is warranted when a complex root fracture or retreatment decision exceeds the available certainty.

Why not the closest alternatives: Empiric repeated restoration is not a sound approach.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Vertical root fracture versus non-restorable crown

Return to Question 1129 ↑

Q1175 ADVANCED · Block H

Best answer: D. Use an additive direct composite where no-preparation reshaping is feasible

Justification: Additive direct composite meets the shape and age-related maintenance needs without irreversible preparation.

Why not the closest alternatives: Ceramic or full-coverage options sacrifice tissue when the case is suited to additive repair.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anterior aesthetic restoration selection

Return to Question 1175 ↑

Q1176 ADVANCED · Block H

Best answer: C. Select a conservative minimal-preparation ceramic veneer where enamel is plentiful and prior composites have failed

Justification: A minimal-preparation indirect labio-incisal ceramic veneer may be justified when repeated composite failure occurs with favourable enamel bonding.

Why not the closest alternatives: A full crown requires much greater preparation and is not the least invasive appropriate option.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anterior aesthetic restoration selection

Return to Question 1176 ↑

Q1177 ADVANCED · Block H

Best answer: B. Reassess the occlusal and parafunctional cause before another definitive restoration

Justification: Address the cause of mechanical overload and consider reversible protection before repeating an aesthetic restoration.

Why not the closest alternatives: Changing material without controlling the cause invites another failure.

Revise: Diploma LO 2c.11, 2c.13, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: Anterior aesthetic restoration selection

Return to Question 1177 ↑

Q1365 ADVANCED · Block H

Best answer: E. Direct composite with a palatal index and conservative enamel margin

Justification: A direct bonded restoration is conservative and repairable for a small first-time defect with adequate enamel.

Why not the closest alternatives: Indirect ceramic may be unnecessary at this stage.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.11, 2c.13

Return to Question 1365 ↑

Q1366 ADVANCED · Block H

Best answer: C. Adhesively bonded labio-incisal glass-ceramic veneer

Justification: A minimal bonded ceramic labio-incisal restoration may provide a durable tissue-conserving solution after repeated direct material failure.

Why not the closest alternatives: A full-coverage crown sacrifices healthy palatal and proximal tissue unnecessarily.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.11, 2c.13

Return to Question 1366 ↑

Q1367 ADVANCED · Block H

Best answer: E. Cuspal-coverage bonded onlay for a heavily undermined posterior cusp

Justification: An appropriately designed adhesive cuspal-coverage onlay can protect compromised cusps without full circumferential preparation.

Why not the closest alternatives: Simple composite replacement may fail to protect a structurally undermined cusp.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.11, 2c.13

Return to Question 1367 ↑

Q1368 ADVANCED · Block H

Best answer: E. Full-coverage crown where remaining geometry supports retentive preparation

Justification: Full coverage with a feasible ferrule and retention form may be appropriate in a substantially compromised tooth.

Why not the closest alternatives: A minimally prepared veneer cannot replace missing axial support.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.11, 2c.13

Return to Question 1368 ↑

Q1369 ADVANCED · Block H

Best answer: C. Increase sound axial wall height and improve preparation resistance form where feasible

Justification: Resistance and retention are limited by wall height and taper; preparation geometry must be reassessed.

Why not the closest alternatives: Stronger luting agent alone cannot rescue grossly inadequate resistance form.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.14

Return to Question 1369 ↑

Q1370 ADVANCED · Block H

Best answer: B. Preserve or develop an adequate ferrule rather than rely on a post alone

Justification: Ferrule is a major determinant of resistance to fracture; a post serves core retention and does not replace missing ferrule.

Why not the closest alternatives: A stronger post does not by itself strengthen a structurally weak root.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.14

Return to Question 1370 ↑

Q1371 ADVANCED · Block H

Best answer: D. Choose an adhesive restoration when enamel and isolation support bonding

Justification: An adhesive partial-coverage strategy may retain more natural tissue than a conventional crown.

Why not the closest alternatives: Preparing circumferential walls for retention may be unnecessary.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.14

Return to Question 1371 ↑

Q1372 ADVANCED · Block H

Best answer: D. Reconsider restorability if sound cervical tooth structure cannot be obtained

Justification: Restorability should be revisited when adequate ferrule and biological conditions cannot realistically be achieved.

Why not the closest alternatives: Post placement should not be used to conceal a hopeless coronal foundation.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.14

Return to Question 1372 ↑

Q1373 ADVANCED · Block H

Best answer: B. Reliable enamel bonding and conservative preparation for an aesthetic indirect restoration

Justification: Enamel-based adhesion and tissue preservation favour an appropriate bonded aesthetic restoration.

Why not the closest alternatives: Conventional crown retention would require needless tooth reduction.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1373 ↑

Q1374 ADVANCED · Block H

Best answer: C. Preparation retention and material requirements for a conventional posterior crown

Justification: Cement and material selection should fit the restoration and preparation geometry, not a blanket demand for resin bonding.

Why not the closest alternatives: Adhesive resin is not mandatory for every retentive full-coverage crown.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1374 ↑

Q1375 ADVANCED · Block H

Best answer: A. Moisture isolation and suitable surface treatment for resin-bonded retention

Justification: Isolation and material-specific surface preparation are crucial to resin-retained prostheses.

Why not the closest alternatives: Changing to a longer-span conventional bridge without addressing bonding may be disproportionate.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1375 ↑

Q1376 ADVANCED · Block H

Best answer: E. Caries activity and cleanability before definitive indirect replacement

Justification: Risk control and accessible contours determine long-term success; material choice alone cannot prevent recurrence.

Why not the closest alternatives: Replacing crowns into an uncontrolled cariogenic environment invites another failure.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1376 ↑

Q1377 ADVANCED · Block H

Best answer: B. A single-wing resin-bonded bridge on a sound enamel-rich abutment

Justification: A suitably designed single-retainer resin-bonded bridge can preserve sound adjacent tooth structure.

Why not the closest alternatives: Conventional full-coverage bridge preparations would be unnecessarily invasive.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.6

Return to Question 1377 ↑

Q1378 ADVANCED · Block H

Best answer: E. A conventional fixed bridge supported by suitable teeth with adequate periodontal reserve

Justification: A conventional bridge may be efficient where abutments independently need crowns and biologic support is adequate.

Why not the closest alternatives: The least invasive option in one situation is not invariably the least invasive across already heavily restored abutments.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.6

Return to Question 1378 ↑

Q1379 ADVANCED · Block H

Best answer: A. A removable replacement when hygiene, cost or future flexibility dominates

Justification: A removable option offers flexibility while prognosis and future changes are clarified.

Why not the closest alternatives: A definitive bridge anchored to doubtful teeth creates avoidable risk.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.6

Return to Question 1379 ↑

Q1380 ADVANCED · Block H

Best answer: E. No replacement with monitoring when function and preferences support it

Justification: Observation is a legitimate, patient-centred choice where replacement offers little demonstrable benefit.

Why not the closest alternatives: A full set of teeth is not a necessary endpoint for every patient.

Revise: Volume I — Unit 8 · Diploma LO 2c.2, 2c.6

Return to Question 1380 ↑

Q1381 ADVANCED · Block H

Best answer: A. Avoid a long unsupported span when flexure and abutment reserve are inadequate

Justification: Long spans flex more and transmit high loads; limited abutment reserve and parafunction challenge prognosis.

Why not the closest alternatives: A stronger ceramic does not eliminate the lever and biologic limitations.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.13

Return to Question 1381 ↑

Q1382 ADVANCED · Block H

Best answer: D. Reassess the connector or fixed-movable design to accommodate differential abutment movement

Justification: Connector strategy and abutment movement must be considered, sometimes requiring a different design or separate prostheses.

Why not the closest alternatives: Forcing a rigid path onto incompatible abutments may compromise fit and stress distribution.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.13

Return to Question 1382 ↑

Q1383 ADVANCED · Block H

Best answer: B. Reduce an excessive cantilever moment arm and occlusal loading

Justification: Cantilever loading produces bending moments; review span, contacts and alternative replacement before repeating the same design.

Why not the closest alternatives: Re-cementation does not remove an excessive distal lever arm.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.13

Return to Question 1383 ↑

Q1384 ADVANCED · Block H

Best answer: E. Use a minimal single-retainer adhesive design where anatomy supports it

Justification: A carefully planned minimal adhesive bridge preserves both intact abutments rather than preparing them for full coverage.

Why not the closest alternatives: More retainers do not automatically produce a more reliable resin-bonded bridge.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.13

Return to Question 1384 ↑

Q1385 ADVANCED · Block H

Best answer: C. Inspect a retainer’s bonding failure mechanism and verify isolation and occlusion

Justification: Bonding conditions and occlusal loading both need review before repeating retention.

Why not the closest alternatives: A clean debond is not proof that every adhesive bridge is unsuitable.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.21

Return to Question 1385 ↑

Q1386 ADVANCED · Block H

Best answer: E. Assess marginal leakage, recurrent caries and pulpal status beneath a failed crown

Justification: Investigate caries extent and pulpal condition; recementation without diagnosis may seal pathology under a defective crown.

Why not the closest alternatives: Cement strength alone is not the reason for every loose crown.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.21

Return to Question 1386 ↑

Q1387 ADVANCED · Block H

Best answer: A. Examine abutment mobility and periodontal support after a bridge failure

Justification: Periodontal deterioration can materially alter the load-bearing capacity of bridge abutments, requiring diagnostic reassessment.

Why not the closest alternatives: Replacing only the connector ignores the changing biologic foundation.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.21

Return to Question 1387 ↑

Q1388 ADVANCED · Block H

Best answer: C. Assess parafunction and unsupported ceramic thickness after chipping

Justification: Load direction, clearance and restoration thickness determine fatigue and fracture risk; review design before replacement.

Why not the closest alternatives: Bond strength is not the sole determinant of a mechanically overloaded ceramic.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.21

Return to Question 1388 ↑

Q1389 ADVANCED · Block H

Best answer: E. Use bonded ceramic for enamel-rich anterior aesthetic restoration

Justification: Enamel-rich labial defects with aesthetic priorities may suit a bonded ceramic veneer after shared planning.

Why not the closest alternatives: A posterior metal-ceramic design would be disproportionate and less aesthetic.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1389 ↑

Q1390 ADVANCED · Block H

Best answer: B. Choose a robust posterior monolithic restoration where load and clearance dominate

Justification: An appropriately engineered monolithic high-strength restoration may better suit the load and thickness limits.

Why not the closest alternatives: A thin aesthetic glass veneer would not address the full occlusal requirement.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1390 ↑

Q1391 ADVANCED · Block H

Best answer: B. Use a conservative direct composite for a small, repairable lesion

Justification: A direct adhesive restoration is proportionate when symptoms and risk justify treatment.

Why not the closest alternatives: Full circumferential preparation would remove unnecessary sound tissue.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1391 ↑

Q1392 ADVANCED · Block H

Best answer: B. Select a suitable cast/onlay restoration for extensive posterior cuspal compromise

Justification: Cuspal protection with a suitable partial-coverage indirect restoration can be preferable to continued large direct replacements.

Why not the closest alternatives: A full crown is not automatically required when bonded partial coverage meets the biomechanical objective.

Revise: Volume I — Unit 8 · Diploma LO 2c.13, 2c.14

Return to Question 1392 ↑

Q1393 ADVANCED · Block H

Best answer: D. Stabilise periodontal and caries risk before evaluating bridge retainers

Justification: Disease stabilisation and abutment prognosis must precede definitive bridge design.

Why not the closest alternatives: The best framework cannot compensate for progressive periodontal breakdown.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2b.4

Return to Question 1393 ↑

Q1394 ADVANCED · Block H

Best answer: D. Perform a diagnostic trial for substantial aesthetic or occlusal alteration

Justification: Diagnostic wax-up and reversible evaluation reduce uncertainty before major elective irreversible changes.

Why not the closest alternatives: Immediate definitive preparations carry avoidable risk.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2b.4

Return to Question 1394 ↑

Q1395 ADVANCED · Block H

Best answer: E. Consider an adhesive single-retainer design to conserve intact adjacent teeth

Justification: An adhesive minimal-preparation option should be part of the conversation before conventional full-coverage bridges.

Why not the closest alternatives: Unnecessary preparation of two sound teeth can impose long-term maintenance burden.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2b.4

Return to Question 1395 ↑

Q1396 ADVANCED · Block H

Best answer: E. Avoid fixed replacement when the patient cannot maintain complex contours

Justification: Maintainability is an essential part of suitability; a simpler accessible solution may give better health outcomes.

Why not the closest alternatives: A technically feasible bridge is not automatically a good patient-centred option.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2b.4

Return to Question 1396 ↑

Q1397 ADVANCED · Block H

Best answer: E. Use direct bonded composite to repair a localised conservative anterior defect

Justification: Repairable bonded composite avoids sacrificing healthy enamel for a first small defect.

Why not the closest alternatives: The more complex restoration is not necessarily the better intervention.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.13

Return to Question 1397 ↑

Q1398 ADVANCED · Block H

Best answer: C. Consider minimal bonded ceramic after repeated cohesive direct-composite fracture

Justification: A conservative bonded veneer may be reasonable when repeated direct material fracture persists in otherwise favourable adhesive anatomy.

Why not the closest alternatives: Simply replacing the same composite without revisiting the restoration design may reproduce failure.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.13

Return to Question 1398 ↑

Q1399 ADVANCED · Block H

Best answer: C. Use cuspal coverage to prevent propagation of a significant posterior cusp crack

Justification: After crack assessment, cuspal coverage can protect a structurally weakened tooth while preserving sound structure.

Why not the closest alternatives: An unprotected direct restoration may permit crack propagation.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.13

Return to Question 1399 ↑

Q1400 ADVANCED · Block H

Best answer: C. Reassess ferrule, root condition and prognosis before crown on a root-filled tooth

Justification: Restorability and ferrule determine whether a post-retained core and crown is prudent; assess structure before execution.

Why not the closest alternatives: A post does not create missing cervical dentine or eliminate fracture risk.

Revise: Volume I — Unit 8 · Diploma LO 2c.11, 2c.13

Return to Question 1400 ↑

Q1401 ADVANCED · Block H

Best answer: E. Evaluate total abutment periodontal support, not merely crown length

Justification: Remaining periodontal support, disease control and mobility matter more than visual crown height alone.

Why not the closest alternatives: An apparently strong crown does not guarantee a sustainable abutment.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.20

Return to Question 1401 ↑

Q1402 ADVANCED · Block H

Best answer: A. Evaluate span length, occlusion and likely flexure of the planned prosthesis

Justification: Span flexure and bending forces can dominate prognosis even when abutments appear healthy.

Why not the closest alternatives: Cement selection alone cannot overcome long-span beam mechanics.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.20

Return to Question 1402 ↑

Q1403 ADVANCED · Block H

Best answer: A. Evaluate hygiene access and expected maintenance burden

Justification: Hygiene and ability to maintain pontic embrasures are major determinants of treatment suitability.

Why not the closest alternatives: Designing only for mechanical stiffness may worsen periodontal risk.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.20

Return to Question 1403 ↑

Q1404 ADVANCED · Block H

Best answer: C. Use sound enamel and appropriate case selection for adhesive retainer design

Justification: A suitably selected resin-bonded design can preserve intact adjacent tooth substance.

Why not the closest alternatives: Routine full-crown preparation is not needed to provide a fixed replacement in every case.

Revise: Volume I — Unit 8 · Diploma LO 2c.6, 2c.20

Return to Question 1404 ↑

BLOCK I · Occlusion and temporomandibular disorders

Q1067 ADVANCED · Block I

Best answer: E. Begin with explanation, self-care, and reversible management

Justification: Most uncomplicated muscular TMD responds to reversible education, self-care and appropriate symptom management.

Why not the closest alternatives: Irreversible occlusal changes lack a justified role as first-line care.

Revise: Diploma LO 2a.20, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: TMD and occlusal intervention

Return to Question 1067 ↑

Q1068 ADVANCED · Block I

Best answer: B. Consider a reversible stabilisation appliance where indicated

Justification: A reversible hard stabilisation appliance may help protection and symptom control alongside behavioural measures.

Why not the closest alternatives: Full-mouth occlusal reconstruction is not warranted simply because bruxism exists.

Revise: Diploma LO 2a.20, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: TMD and occlusal intervention

Return to Question 1068 ↑

Q1069 ADVANCED · Block I

Best answer: D. Assess for red flags and refer if there are concerning systemic or neurological findings

Justification: These features raise a serious systemic vascular differential requiring urgent medical assessment.

Why not the closest alternatives: Routine TMD management or splinting must not delay urgent evaluation.

Revise: Diploma LO 2a.20, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: TMD and occlusal intervention

Return to Question 1069 ↑

Q1130 ADVANCED · Block I

Best answer: A. Make reversible assessment of mandibular position and occlusal stability before extensive reconstruction

Justification: A reproducible mandibular reference and functional assessment are needed before an extensive fixed plan.

Why not the closest alternatives: Creating space by arbitrarily raising OVD does not solve instability.

Revise: Diploma LO 2a.15, 2a.20, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: Occlusal diagnosis and mounting

Return to Question 1130 ↑

Q1131 ADVANCED · Block I

Best answer: E. Use a deprogramming or trial provisional phase when jaw position is uncertain

Justification: A reversible trial phase with stable provisional contacts can help assess adaptation and jaw relation before irreversible reconstruction.

Why not the closest alternatives: Proceeding directly to final crowns is risky when the reference position is uncertain.

Revise: Diploma LO 2a.15, 2a.20, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: Occlusal diagnosis and mounting

Return to Question 1131 ↑

Q1132 ADVANCED · Block I

Best answer: D. Accept a harmless non-working contact without irreversible intervention

Justification: A minor contact in isolation is not a sufficient indication for irreversible enamel alteration.

Why not the closest alternatives: Preventive occlusal equilibration is not justified by a contact alone.

Revise: Diploma LO 2a.15, 2a.20, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: Occlusal diagnosis and mounting

Return to Question 1132 ↑

Q1405 ADVANCED · Block I

Best answer: D. Begin reversible education, self-care and symptom-directed TMD management

Justification: Muscle-related TMD usually merits reversible, conservative care first; the evidence does not justify extensive irreversible occlusal alteration.

Why not the closest alternatives: Changing crowns to cure diffuse myalgia may introduce more risk than benefit.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19

Return to Question 1405 ↑

Q1406 ADVANCED · Block I

Best answer: B. Check contacts and selective adjustment of a new restoration that clearly creates an acute interference

Justification: A verified high point on a new restoration should be corrected carefully; this is different from an unproven general occlusal cause of TMD.

Why not the closest alternatives: A global occlusal scheme change is disproportionate to a discrete post-operative interference.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19

Return to Question 1406 ↑

Q1407 ADVANCED · Block I

Best answer: A. Investigate a suspected disc-related locking problem and refer if persistent or severe

Justification: A possible acute disc displacement without reduction warrants structured assessment and appropriate escalation if persistent or severe.

Why not the closest alternatives: Routine crown adjustment does not treat joint locking.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19

Return to Question 1407 ↑

Q1408 ADVANCED · Block I

Best answer: B. Assess parafunctional loading and protective strategies for repeated restoration failures

Justification: Parafunction and loading should inform design, patient counselling and protective options before replacing restorations.

Why not the closest alternatives: A stronger ceramic without addressing loading may still fail.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19

Return to Question 1408 ↑

Q1409 ADVANCED · Block I

Best answer: B. A reproducible closure slide is a finding to document, not automatic grounds for equilibration

Justification: An occlusal discrepancy should be documented and considered in prosthetic planning but need not be treated in isolation.

Why not the closest alternatives: Automatic full-mouth equilibration is not evidence-based for an asymptomatic slide.

Revise: Volume I — Unit 9 · Diploma LO 2a.15, 2a.20, 2c.19

Return to Question 1409 ↑

Q1410 ADVANCED · Block I

Best answer: C. Conservative TMD care is appropriate before irreversible bite modification

Justification: Reversible self-care and conservative management are preferred before irreversible occlusal interventions.

Why not the closest alternatives: There is no sufficient basis to redesign an otherwise functioning occlusion solely to treat non-specific TMD.

Revise: Volume I — Unit 9 · Diploma LO 2a.15, 2a.20, 2c.19

Return to Question 1410 ↑

Q1411 ADVANCED · Block I

Best answer: C. New symptomatic occlusal contacts should be identified and corrected when causally linked

Justification: A discrete iatrogenic interference can be corrected while preserving general occlusion; it differs from nonspecific TMD.

Why not the closest alternatives: A whole-arch reconstruction is not needed for a single confirmed new high spot.

Revise: Volume I — Unit 9 · Diploma LO 2a.15, 2a.20, 2c.19

Return to Question 1411 ↑

Q1412 ADVANCED · Block I

Best answer: B. A change in OVD requires evaluation of function, speech, comfort and adaptation

Justification: Changing vertical dimension and space relationships should be assessed using reversible or provisional evaluation of comfort, speech and function.

Why not the closest alternatives: Definitive irreversible treatment before testing the proposed relationship increases risk.

Revise: Volume I — Unit 9 · Diploma LO 2a.15, 2a.20, 2c.19

Return to Question 1412 ↑

Q1413 ADVANCED · Block I

Best answer: E. Muscle-related pain: conservative management and reassessment

Justification: Features favour myalgia, for which reversible and behavioural self-care forms the initial pathway.

Why not the closest alternatives: Joint surgery and occlusal reconstruction are not first-line for uncomplicated muscle pain.

Revise: Volume I — Unit 9 · Diploma LO 2c.19, 2a.17

Return to Question 1413 ↑

Q1414 ADVANCED · Block I

Best answer: A. Disc displacement with reduction: reassurance and monitoring if minimally symptomatic

Justification: An asymptomatic reducing disc displacement commonly needs explanation, monitoring and avoidance of unnecessary invasive treatment.

Why not the closest alternatives: A click alone does not justify permanent occlusal modification.

Revise: Volume I — Unit 9 · Diploma LO 2c.19, 2a.17

Return to Question 1414 ↑

Q1415 ADVANCED · Block I

Best answer: B. Disc displacement without reduction: structured assessment of restricted opening

Justification: Closed lock/disc displacement without reduction is a plausible mechanism; assess function and manage or refer according to severity.

Why not the closest alternatives: A stable crown does not explain the characteristic change from click to restriction.

Revise: Volume I — Unit 9 · Diploma LO 2c.19, 2a.17

Return to Question 1415 ↑

Q1416 ADVANCED · Block I

Best answer: C. Possible inflammatory joint disease: medical/specialist evaluation when systemic features exist

Justification: Systemic inflammatory disease can affect the TMJ and requires coordinated assessment beyond routine appliance adjustment.

Why not the closest alternatives: Classifying every painful joint as simple myalgia risks missing systemic pathology.

Revise: Volume I — Unit 9 · Diploma LO 2c.19, 2a.17

Return to Question 1416 ↑

Q1417 ADVANCED · Block I

Best answer: A. Verify centric and eccentric contacts before finalising an extensive posterior restoration

Justification: Functional contact mapping helps avoid unanticipated interferences and damaging restorative loads.

Why not the closest alternatives: Static intercuspation alone may not predict functional loading.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19, 2c.11

Return to Question 1417 ↑

Q1418 ADVANCED · Block I

Best answer: C. Use a protective, reversible option for nocturnal parafunctional overload where indicated

Justification: Protective appliances and behavioural advice may be considered as reversible risk management with ongoing review.

Why not the closest alternatives: Irreversible equilibration does not reliably eliminate sleep bruxism.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19, 2c.11

Return to Question 1418 ↑

Q1419 ADVANCED · Block I

Best answer: C. Avoid occlusal grinding to treat otherwise uncomplicated TMD

Justification: Current management principles favour reversible measures and avoid irreversible occlusal alteration in uncomplicated TMD.

Why not the closest alternatives: Dental grinding cannot be justified by a nonspecific causal assumption.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19, 2c.11

Return to Question 1419 ↑

Q1420 ADVANCED · Block I

Best answer: D. Assess whether a patient has enough restorative space before changing OVD

Justification: Establish the true space and OVD relationship; local additive Dahl-type strategies may be appropriate depending on the patient.

Why not the closest alternatives: Assuming generalized OVD collapse may lead to unnecessary full-mouth work.

Revise: Volume I — Unit 9 · Diploma LO 2a.20, 2c.19, 2c.11

Return to Question 1420 ↑

BLOCK J · Tooth wear, the failing dentition, interfaces and trauma

Q1070 ADVANCED · Block J

Best answer: E. Monitor and prevent before irreversible restoration when disease is stable and asymptomatic

Justification: Stable asymptomatic wear may need surveillance, prevention and photographs rather than immediate restorative treatment.

Why not the closest alternatives: The intervention burden would exceed the clinical benefit.

Revise: Diploma LO 2c.12, 2b.7, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: Tooth wear restorative timing

Return to Question 1070 ↑

Q1071 ADVANCED · Block J

Best answer: A. Use localised additive composite with the Dahl concept after risk control

Justification: An additive composite Dahl approach can create space conservatively in an appropriate localised case.

Why not the closest alternatives: Full-mouth OVD change is disproportionate when posterior support remains stable.

Revise: Diploma LO 2c.12, 2b.7, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: Tooth wear restorative timing

Return to Question 1071 ↑

Q1072 ADVANCED · Block J

Best answer: C. Address uncontrolled acid exposure before definitive restoration

Justification: Ongoing acid exposure and caries risk will undermine complex restoration; a sensitive medical and preventive plan is essential.

Why not the closest alternatives: Extensive irreversible crowns before risk control are not sound first-line care.

Revise: Diploma LO 2c.12, 2b.7, 2c.19 · Volume I and Diploma learning outcomes (August 2026) · Theme: Tooth wear restorative timing

Return to Question 1072 ↑

Q1073 ADVANCED · Block J

Best answer: D. Create a tooth-by-tooth prognosis after controlling disease and testing restorability

Justification: A comprehensive prognosis and disease diagnosis are required before defining the number and value of teeth to retain.

Why not the closest alternatives: Unexamined extraction can sacrifice useful strategic teeth.

Revise: Diploma LO 2c.10, 2c.11, 2c.16 · Volume I and Diploma learning outcomes (August 2026) · Theme: Failing dentition and staged prognosis

Return to Question 1073 ↑

Q1074 ADVANCED · Block J

Best answer: D. Preserve strategic restorable teeth for a phased treatment plan

Justification: Strategic roots/teeth can contribute to support, proprioception and adaptation, when maintenance is feasible.

Why not the closest alternatives: Automatic full clearance is not always the best option.

Revise: Diploma LO 2c.10, 2c.11, 2c.16 · Volume I and Diploma learning outcomes (August 2026) · Theme: Failing dentition and staged prognosis

Return to Question 1074 ↑

Q1075 ADVANCED · Block J

Best answer: B. Use provisional stabilisation and re-evaluation before committing to a final rehabilitation

Justification: A provisional phase demonstrates disease control, maintenance adherence and function before expensive irreversible work.

Why not the closest alternatives: A biologically uncertain situation makes immediate final work high risk.

Revise: Diploma LO 2c.10, 2c.11, 2c.16 · Volume I and Diploma learning outcomes (August 2026) · Theme: Failing dentition and staged prognosis

Return to Question 1075 ↑

Q1133 ADVANCED · Block J

Best answer: B. Address reflux or vomiting and introduce appropriate prevention before definitive coverage

Justification: Controlling the cause and introducing targeted preventive strategies is foundational before definitive restoration.

Why not the closest alternatives: Full coverage does not treat ongoing acid challenges.

Revise: Diploma LO 2c.12, 2c.17, 2c.18 · Volume I and Diploma learning outcomes (August 2026) · Theme: Intrinsic erosion and medical liaison

Return to Question 1133 ↑

Q1134 ADVANCED · Block J

Best answer: A. Coordinate medical assessment of a possible eating disorder sensitively

Justification: Recognise possible eating disorder signs, communicate non-judgmentally and coordinate appropriate medical support with consent.

Why not the closest alternatives: Confrontation or irreversible restorations alone can harm care.

Revise: Diploma LO 2c.12, 2c.17, 2c.18 · Volume I and Diploma learning outcomes (August 2026) · Theme: Intrinsic erosion and medical liaison

Return to Question 1134 ↑

Q1135 ADVANCED · Block J

Best answer: E. Plan a local additive restoration once causes are controlled and symptoms justify it

Justification: A limited additive restoration can be appropriate once risk is controlled and symptoms justify active treatment.

Why not the closest alternatives: A full-mouth crown approach would be unnecessarily invasive.

Revise: Diploma LO 2c.12, 2c.17, 2c.18 · Volume I and Diploma learning outcomes (August 2026) · Theme: Intrinsic erosion and medical liaison

Return to Question 1135 ↑

Q1136 ADVANCED · Block J

Best answer: D. Use serial clinical photographs and appropriate casts or scans to assess progression

Justification: Repeat photographs and 3-D casts or scans can help establish actual interval change.

Why not the closest alternatives: A single cross-sectional score cannot show progression by itself.

Revise: Diploma LO 2a.15, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Documenting progression of tooth wear

Return to Question 1136 ↑

Q1137 ADVANCED · Block J

Best answer: E. Plan preventive and behavioural changes with periodic reassessment

Justification: Target risk factors and monitor rather than undertaking immediate extensive restoration.

Why not the closest alternatives: Any tooth wear score alone does not mandate raising OVD.

Revise: Diploma LO 2a.15, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Documenting progression of tooth wear

Return to Question 1137 ↑

Q1138 ADVANCED · Block J

Best answer: B. Use the Smith and Knight index when detailed surface-by-surface documentation is required

Justification: Smith and Knight scores individual tooth surfaces, whereas BEWE is a sextant-based screening method.

Why not the closest alternatives: A single BEWE summary sacrifices the requested level of detail.

Revise: Diploma LO 2a.15, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Documenting progression of tooth wear

Return to Question 1138 ↑

Q1421 ADVANCED · Block J

Best answer: C. Use additive anterior direct composite with a Dahl-type approach after risk control

Justification: Localised additive direct composite using a Dahl concept can create space while preserving tissue when carefully selected.

Why not the closest alternatives: Full-mouth OVD increase may be unnecessary for an isolated anterior space problem.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.11, 2b.3

Return to Question 1421 ↑

Q1422 ADVANCED · Block J

Best answer: A. Control active acid exposure and monitor before initiating definitive work

Justification: Stabilise the cause and record progression before elective indirect bonding.

Why not the closest alternatives: Veneers cannot reliably outlast uncontrolled acid exposure without preventive change.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.11, 2b.3

Return to Question 1422 ↑

Q1423 ADVANCED · Block J

Best answer: E. Use a local cuspal-coverage strategy for a structurally compromised tooth

Justification: Following appropriate crack assessment, cuspal coverage protects compromised tooth tissue.

Why not the closest alternatives: Replacing the large filling without protecting the cusp may permit progression.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.11, 2b.3

Return to Question 1423 ↑

Q1424 ADVANCED · Block J

Best answer: E. Arrange specialist assessment for severe generalised wear with multiple modifying factors

Justification: Complex interrelated wear, occlusion and periodontal factors warrant coordinated specialist-level assessment.

Why not the closest alternatives: A simple anterior Dahl procedure cannot solve widespread instability.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.11, 2b.3

Return to Question 1424 ↑

Q1425 ADVANCED · Block J

Best answer: C. Smooth cupped surfaces and restorations standing proud suggest erosion

Justification: Chemical dissolution produces characteristic cupping and relatively raised restorations.

Why not the closest alternatives: Bruxism alone typically produces matching contact facets rather than smooth cupping.

Revise: Volume I — Unit 10 · Diploma LO 2a.15, 2c.10

Return to Question 1425 ↑

Q1426 ADVANCED · Block J

Best answer: A. Matched planar facets and hypertrophic elevators suggest attrition

Justification: Tooth-to-tooth mechanical contact explains complementary shiny facets with parafunction.

Why not the closest alternatives: Acid erosion alone would not explain matching opposing tooth contact patterns.

Revise: Volume I — Unit 10 · Diploma LO 2a.15, 2c.10

Return to Question 1426 ↑

Q1427 ADVANCED · Block J

Best answer: C. Sharp notches related to an abrasive habit suggest abrasion

Justification: Mechanical abrasion from the identified habit is a plausible dominant cause and should be addressed.

Why not the closest alternatives: A single wedge-shaped cervical lesion is not proof of a flexural abfraction mechanism.

Revise: Volume I — Unit 10 · Diploma LO 2a.15, 2c.10

Return to Question 1427 ↑

Q1428 ADVANCED · Block J

Best answer: E. Widespread cervical lesions with multiple mechanisms require mixed-aetiology assessment

Justification: More than one mechanism may contribute to non-carious tissue loss; prevention must target all relevant risks.

Why not the closest alternatives: Choosing one label exclusively risks inadequate disease control.

Revise: Volume I — Unit 10 · Diploma LO 2a.15, 2c.10

Return to Question 1428 ↑

Q1429 ADVANCED · Block J

Best answer: E. Establish the periodontal and endodontic prognosis of strategic teeth first

Justification: Strategic tooth prognosis determines feasible fixed and removable plans; investigations come before irreversible abutment commitments.

Why not the closest alternatives: Choosing a final prosthesis before prognosis is established risks avoidable redesign.

Revise: Volume I — Unit 10 · Diploma LO 2c.16, 2c.20, 2b.5

Return to Question 1429 ↑

Q1430 ADVANCED · Block J

Best answer: E. Prioritise plaque and caries control before extensive crowns or dentures

Justification: Controlling disease and securing realistic maintenance is a prerequisite to long-term fixed reconstruction.

Why not the closest alternatives: Ceramic coverage cannot compensate for active untreated disease.

Revise: Volume I — Unit 10 · Diploma LO 2c.16, 2c.20, 2b.5

Return to Question 1430 ↑

Q1431 ADVANCED · Block J

Best answer: D. Preserve strategically useful roots when suitable for an overdenture

Justification: Suitable retained roots can support an overdenture strategy and may help preserve local tissues; selection requires sound prognosis.

Why not the closest alternatives: Extraction solely to simplify a complete denture is not always the most conservative option.

Revise: Volume I — Unit 10 · Diploma LO 2c.16, 2c.20, 2b.5

Return to Question 1431 ↑

Q1432 ADVANCED · Block J

Best answer: B. Use an interim prosthesis when the dentition is changing rapidly

Justification: Interim prosthodontics allows function and appearance while prognosis and tissues evolve.

Why not the closest alternatives: A definitive expensive framework in a rapidly changing arch may be difficult to adapt.

Revise: Volume I — Unit 10 · Diploma LO 2c.16, 2c.20, 2b.5

Return to Question 1432 ↑

Q1433 ADVANCED · Block J

Best answer: A. Assess a suspected vertical root fracture before attempting retreatment or recrowning

Justification: Combined findings strongly raise concern for a vertical root fracture; clarify prognosis before another restoration.

Why not the closest alternatives: Replacing the crown will not correct a structural root fracture.

Revise: Volume I — Unit 10 · Diploma LO 2a.17, 2c.18, 2c.20

Return to Question 1433 ↑

Q1434 ADVANCED · Block J

Best answer: B. Investigate a possible combined endodontic-periodontal lesion with relevant testing

Justification: The endodontic and periodontal contributions must be distinguished to plan treatment and assess prognosis.

Why not the closest alternatives: Assuming irreversible periodontal loss without endodontic testing may be misleading.

Revise: Volume I — Unit 10 · Diploma LO 2a.17, 2c.18, 2c.20

Return to Question 1434 ↑

Q1435 ADVANCED · Block J

Best answer: C. Stabilise a traumatised vital tooth and monitor pulpal outcome as indicated

Justification: Appropriate baseline records, stabilization where indicated and follow-up testing are necessary because pulpal response may evolve.

Why not the closest alternatives: Immediate prophylactic root treatment without an indication may be overtreatment.

Revise: Volume I — Unit 10 · Diploma LO 2a.17, 2c.18, 2c.20

Return to Question 1435 ↑

Q1436 ADVANCED · Block J

Best answer: D. Refer a complex luxation or avulsion injury according to current trauma guidance

Justification: Complex traumatic displacement requires appropriate urgent trauma-led management and may exceed routine Level 2 care.

Why not the closest alternatives: Routine composite repair alone cannot address intrusive injury and alveolar involvement.

Revise: Volume I — Unit 10 · Diploma LO 2a.17, 2c.18, 2c.20

Return to Question 1436 ↑

Q1437 ADVANCED · Block J

Best answer: B. Control gastro-oesophageal reflux risk and reduce acid challenges before restoring

Justification: Medical management and prevention of intrinsic acid exposure are necessary before definitive restoration.

Why not the closest alternatives: Crowns alone cannot address ongoing regurgitation and acid challenges.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.12, 2c.21

Return to Question 1437 ↑

Q1438 ADVANCED · Block J

Best answer: B. Use additive restorations where local wear is symptomatic and tissue-preserving space creation is feasible

Justification: An additive Dahl-type approach may restore anatomy with limited tooth preparation and monitored occlusal re-establishment.

Why not the closest alternatives: Full-mouth crowns are more invasive than necessary in this localised case.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.12, 2c.21

Return to Question 1438 ↑

Q1439 ADVANCED · Block J

Best answer: A. Modify an occlusal overload before repeating a fractured restoration

Justification: A demonstrable overload should be corrected as part of definitive repair or replacement.

Why not the closest alternatives: A stronger ceramic without contact review may fail again.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.12, 2c.21

Return to Question 1439 ↑

Q1440 ADVANCED · Block J

Best answer: D. Assess possible eating-disorder-related acid exposure with sensitive referral

Justification: Offer a confidential, non-judgemental assessment and appropriate medical liaison while undertaking dental prevention.

Why not the closest alternatives: Immediate irreversible restoration without addressing the cause may fail and harm trust.

Revise: Volume I — Unit 10 · Diploma LO 2c.10, 2c.12, 2c.21

Return to Question 1440 ↑

Q1441 ADVANCED · Block J

Best answer: B. Conservative repair of an isolated marginal direct-composite defect with sound tooth

Justification: A local bonded repair is proportionate when most of the restoration and tooth remain serviceable.

Why not the closest alternatives: Replacing the entire restoration can unnecessarily sacrifice additional tissue.

Revise: Volume I — Unit 10 · Diploma LO 2c.11, 2b.6, 2c.20

Return to Question 1441 ↑

Q1442 ADVANCED · Block J

Best answer: C. Indirect cuspal coverage for repeated posterior cusp fracture with adequate structure

Justification: A conservative cuspal-covering indirect restoration can address the repeated structural failure mechanism.

Why not the closest alternatives: Replacing the MOD composite again without cusp protection risks recurrence.

Revise: Volume I — Unit 10 · Diploma LO 2c.11, 2b.6, 2c.20

Return to Question 1442 ↑

Q1443 ADVANCED · Block J

Best answer: C. Assess restorability and ferrule for a compromised root-treated tooth

Justification: Determine whether sound ferrule and a viable root can be achieved before any further crown or post.

Why not the closest alternatives: Making a larger post will not replace missing cervical tooth structure.

Revise: Volume I — Unit 10 · Diploma LO 2c.11, 2b.6, 2c.20

Return to Question 1443 ↑

Q1444 ADVANCED · Block J

Best answer: E. Remove active disease or unstable caries risk before replacing a large restoration

Justification: Disease stabilization and accessible hygiene must precede elective full-mouth recrowning.

Why not the closest alternatives: Repeated crowns into high-risk conditions invite further marginal failure.

Revise: Volume I — Unit 10 · Diploma LO 2c.11, 2b.6, 2c.20

Return to Question 1444 ↑

BLOCK K · Implants and digital prosthodontics

Q1076 ADVANCED · Block K

Best answer: E. Diagnose peri-implant mucositis and intensify plaque control with prosthesis access review

Justification: Inflammation without progressive supporting bone loss is consistent with peri-implant mucositis.

Why not the closest alternatives: Implantitis and implant removal are not automatically justified by bleeding.

Revise: Diploma LO 2a.19, 2c.20, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Peri-implant complication classification

Return to Question 1076 ↑

Q1077 ADVANCED · Block K

Best answer: D. Suspect peri-implantitis and arrange appropriate disease management or specialist referral

Justification: Progressive bone loss plus inflammation suggests peri-implantitis and requires severity assessment and often specialist care.

Why not the closest alternatives: A simple screw check cannot explain the inflammatory bone-loss pattern.

Revise: Diploma LO 2a.19, 2c.20, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Peri-implant complication classification

Return to Question 1077 ↑

Q1078 ADVANCED · Block K

Best answer: B. Investigate excess subgingival cement and correct the prosthetic cause

Justification: Excess cement can provoke peri-implant inflammation; identify and remove the cause while managing tissues.

Why not the closest alternatives: Neither immediate removal nor assuming a loose screw addresses the likely trigger.

Revise: Diploma LO 2a.19, 2c.20, 2c.22 · Volume I and Diploma learning outcomes (August 2026) · Theme: Peri-implant complication classification

Return to Question 1078 ↑

Q1079 ADVANCED · Block K

Best answer: E. Use a conventional impression or adjunct records where scanning cannot capture the needed anatomy

Justification: A functional impression or conventional adjunct may still be needed where a static optical scan lacks tissue-displacement information.

Why not the closest alternatives: Digital capture of teeth does not automatically capture functional mucosal support.

Revise: Diploma LO 2c.5, 1.9, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Digital workflow limitations

Return to Question 1079 ↑

Q1080 ADVANCED · Block K

Best answer: C. Confirm scan completeness, bite registration and laboratory verification before manufacture

Justification: Digital errors can propagate into restorations; verify scan completeness and occlusal record before fabrication.

Why not the closest alternatives: CBCT is not a substitute for precise occlusal and surface scanning.

Revise: Diploma LO 2c.5, 1.9, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Digital workflow limitations

Return to Question 1080 ↑

Q1081 ADVANCED · Block K

Best answer: C. Digitally design the prosthesis while explaining that biology and occlusion still govern the outcome

Justification: Digital efficiency does not supersede diagnosis, preparation, material limits, occlusal adjustment and maintenance.

Why not the closest alternatives: A digital label confers neither biological success nor unlimited accuracy.

Revise: Diploma LO 2c.5, 1.9, 2b.10 · Volume I and Diploma learning outcomes (August 2026) · Theme: Digital workflow limitations

Return to Question 1081 ↑

Q1139 ADVANCED · Block K

Best answer: C. Check and correct a loose abutment screw and evaluate occlusion and components

Justification: Prosthetic component loosening requires evaluation of the screw connection, components and loading.

Why not the closest alternatives: The findings do not show fixture loss of osseointegration.

Revise: Diploma LO 2a.19, 2c.5, 2c.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Implant crown maintenance decision

Return to Question 1139 ↑

Q1140 ADVANCED · Block K

Best answer: A. Assess screw-retained retrieval where recurrent cement-associated inflammation occurs

Justification: Retrievability and correction of cement-related inflammation are central when excess cement is suspected.

Why not the closest alternatives: Simply tightening a screw will not resolve excess cement.

Revise: Diploma LO 2a.19, 2c.5, 2c.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Implant crown maintenance decision

Return to Question 1140 ↑

Q1141 ADVANCED · Block K

Best answer: D. Refer when true fixture mobility or significant progressive bone loss is detected

Justification: Fixture movement is a serious failure indicator requiring experienced assessment and management.

Why not the closest alternatives: This is not a simple abutment screw problem.

Revise: Diploma LO 2a.19, 2c.5, 2c.13 · Volume I and Diploma learning outcomes (August 2026) · Theme: Implant crown maintenance decision

Return to Question 1141 ↑

Q1142 ADVANCED · Block K

Best answer: B. Use appropriate 3-D imaging only after identifying a clear clinical question and justified need

Justification: A justified small-field CBCT may provide necessary cross-sectional anatomy for a risk-bearing decision.

Why not the closest alternatives: Routine CBCT for all restorations is not an acceptable principle.

Revise: Diploma LO 2a.9, 2c.5, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: CBCT and digital implant planning

Return to Question 1142 ↑

Q1143 ADVANCED · Block K

Best answer: E. Recognise that metal artefacts and segmentation errors can affect digital planning

Justification: Metal artefact can reduce accuracy of anatomy and digital segmentation; critical distances need corroboration and cautious interpretation.

Why not the closest alternatives: Software planning is not automatically precise simply because it is three-dimensional.

Revise: Diploma LO 2a.9, 2c.5, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: CBCT and digital implant planning

Return to Question 1143 ↑

Q1144 ADVANCED · Block K

Best answer: A. Review prosthesis-driven position, restorative space and nerve proximity before surgery

Justification: Implant placement must be prosthetically driven and account for the restorative envelope and adjacent anatomy.

Why not the closest alternatives: A centrally located fixture on CBCT is not necessarily restoratively acceptable.

Revise: Diploma LO 2a.9, 2c.5, 2c.14 · Volume I and Diploma learning outcomes (August 2026) · Theme: CBCT and digital implant planning

Return to Question 1144 ↑

Q1445 ADVANCED · Block K

Best answer: E. Peri-implant mucositis: address plaque and access for cleaning without assuming bone loss

Justification: Inflammation without progressive bone loss is consistent with peri-implant mucositis; hygiene and accessibility are core.

Why not the closest alternatives: Bleeding alone does not establish peri-implantitis.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5

Return to Question 1445 ↑

Q1446 ADVANCED · Block K

Best answer: E. Suspected peri-implantitis: assess progressive bone loss and arrange appropriate management/referral

Justification: Progressive supporting bone loss with inflammation raises concern for peri-implantitis and needs appropriate investigation and treatment/referral.

Why not the closest alternatives: Mucositis alone would not account for new radiographic bone loss.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5

Return to Question 1446 ↑

Q1447 ADVANCED · Block K

Best answer: B. Mechanical screw-related complication: verify prosthetic components and connection

Justification: A prosthetic connection or screw complication is plausible; assess components using the appropriate system and repair protocol.

Why not the closest alternatives: Antibiotics for presumed infection would not correct an unstable connection.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5

Return to Question 1447 ↑

Q1448 ADVANCED · Block K

Best answer: A. Excess cement-related inflammation: investigate and remove the causative excess where indicated

Justification: Residual cement is a recognized local irritant; investigate its removal and ensure the restoration remains retrievable and maintainable.

Why not the closest alternatives: Plaque control alone may fail if a local cement nidus remains.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5

Return to Question 1448 ↑

Q1449 ADVANCED · Block K

Best answer: E. Obtain a clinically appropriate periapical baseline for monitoring bone levels around an implant

Justification: A reproducible appropriate periapical baseline assists future bone-level interpretation.

Why not the closest alternatives: Routine CBCT is not required for every uncomplicated maintenance visit.

Revise: Volume I — Unit 11 · Diploma LO 2a.9, 2a.19, 2c.5

Return to Question 1449 ↑

Q1450 ADVANCED · Block K

Best answer: B. Use a small-field CBCT where a 3-D anatomical question will change implant planning

Justification: CBCT can be justified for specific three-dimensional anatomical decisions when benefits outweigh dose and change management.

Why not the closest alternatives: A panoramic alone may not clarify the critical three-dimensional constraint.

Revise: Volume I — Unit 11 · Diploma LO 2a.9, 2a.19, 2c.5

Return to Question 1450 ↑

Q1451 ADVANCED · Block K

Best answer: B. Reassess occlusal and prosthetic space using clinical and diagnostic records

Justification: Prosthetically driven planning must establish vertical and prosthetic space before implant positioning.

Why not the closest alternatives: Placing implants before considering the restorative envelope may compromise the final result.

Revise: Volume I — Unit 11 · Diploma LO 2a.9, 2a.19, 2c.5

Return to Question 1451 ↑

Q1452 ADVANCED · Block K

Best answer: A. Refer when anatomy or augmentation complexity exceeds the operator’s scope

Justification: Complex anatomy and augmentation may exceed Level 2 care; referral is appropriate before irreversible planning.

Why not the closest alternatives: A digital guide does not remove surgical complexity or practitioner limitations.

Revise: Volume I — Unit 11 · Diploma LO 2a.9, 2a.19, 2c.5

Return to Question 1452 ↑

Q1453 ADVANCED · Block K

Best answer: C. Establish accessible cleaning and a patient-specific peri-implant recall pathway

Justification: Supportive maintenance and hygiene access are central to preventing progression from soft tissue inflammation.

Why not the closest alternatives: Re-making an otherwise sound overdenture without changing maintenance may not help.

Revise: Volume I — Unit 11 · Diploma LO 2c.5, 2c.21

Return to Question 1453 ↑

Q1454 ADVANCED · Block K

Best answer: A. Review parafunction and prosthetic load before replacing repeatedly fractured components

Justification: Occlusal and material design should be reviewed for excessive loading before another identical replacement.

Why not the closest alternatives: Antibiotics are not indicated for a purely mechanical failure.

Revise: Volume I — Unit 11 · Diploma LO 2c.5, 2c.21

Return to Question 1454 ↑

Q1455 ADVANCED · Block K

Best answer: D. Correct a poorly cleansable emergence profile before recurrent inflammation worsens

Justification: Prosthetic cleansability and emergence contour can be major contributors to inflammation, requiring restoration review.

Why not the closest alternatives: Merely increasing recall frequency without correcting an inaccessible contour may fail.

Revise: Volume I — Unit 11 · Diploma LO 2c.5, 2c.21

Return to Question 1455 ↑

Q1456 ADVANCED · Block K

Best answer: C. Verify retrievability and system compatibility before servicing an implant crown

Justification: Implant system identification and correct tools are essential for safe retrievable maintenance.

Why not the closest alternatives: Forcing an unknown component risks damaging the implant connection.

Revise: Volume I — Unit 11 · Diploma LO 2c.5, 2c.21

Return to Question 1456 ↑

Q1457 ADVANCED · Block K

Best answer: C. Verify marginal capture and scan accuracy before accepting a digital impression

Justification: Reliable digital impressions still require exposed, clean, accurately captured margins.

Why not the closest alternatives: Software smoothing cannot reconstruct clinically unseen margins with certainty.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5

Return to Question 1457 ↑

Q1458 ADVANCED · Block K

Best answer: B. Use conventional or combined records where extensive edentulous mobile tissues are inadequately scanned

Justification: Digital capture may be unreliable in poorly landmarked mobile tissues; conventional or hybrid techniques can be more accurate.

Why not the closest alternatives: Repeating the same poor scan alone does not resolve fundamental registration difficulty.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5

Return to Question 1458 ↑

Q1459 ADVANCED · Block K

Best answer: E. Assess digital articulation and occlusal registration before final milling

Justification: Accurate jaw registration and functional verification are still necessary in digital workflows.

Why not the closest alternatives: A precisely milled restoration can embody an incorrect bite record.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5

Return to Question 1459 ↑

Q1460 ADVANCED · Block K

Best answer: C. Check support, accuracy and manufacturing tolerance of a surgical guide

Justification: Guide support and seating accuracy must be verified; an unstable guide cannot safely transfer the intended position.

Why not the closest alternatives: Digital planning does not guarantee physical guide fit.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5

Return to Question 1460 ↑

Q1461 ADVANCED · Block K

Best answer: B. Consider a conventional denture when function is satisfactory and surgery is declined

Justification: Do not impose implant treatment when function and preferences are already satisfied.

Why not the closest alternatives: Implants add cost, surgery and long-term maintenance burdens.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5, 2b.4

Return to Question 1461 ↑

Q1462 ADVANCED · Block K

Best answer: B. Assess a mandibular implant-retained overdenture for difficult conventional retention

Justification: Mandibular implant-overdenture assessment is appropriate where a conventional lower denture remains severely compromised.

Why not the closest alternatives: Repeated conventional remakes may not overcome extreme mandibular ridge resorption.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5, 2b.4

Return to Question 1462 ↑

Q1463 ADVANCED · Block K

Best answer: D. Plan retrievability and hygiene access for a proposed implant restoration

Justification: Restorative design should address retrievability, loading and hygiene before completion.

Why not the closest alternatives: Aesthetics alone should not dictate a design that cannot be maintained.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5, 2b.4

Return to Question 1463 ↑

Q1464 ADVANCED · Block K

Best answer: D. Refer for assessment where severe ridge deficiency or medical complexity alters implant risk

Justification: This exceeds routine Level 2 complexity and should be assessed through appropriate specialist referral.

Why not the closest alternatives: A short training course or digital plan does not make high-risk augmentation low complexity.

Revise: Volume I — Unit 11 · Diploma LO 2a.19, 2c.5, 2b.4

Return to Question 1464 ↑

Q1465 ADVANCED · Block K

Best answer: B. Stitching and registration error in a long-span intraoral scan

Justification: Long-span stitching can introduce cumulative registration error; validate the impression or alternative record approach.

Why not the closest alternatives: A technically beautiful CAD design cannot correct an inaccurate scan.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5, 2c.21

Return to Question 1465 ↑

Q1466 ADVANCED · Block K

Best answer: C. Lack of adequate functional soft-tissue information in a scan of a complete denture arch

Justification: Soft-tissue mobility and functional extensions may not be captured by a static scan alone.

Why not the closest alternatives: High print resolution does not compensate for missing functional anatomy.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5, 2c.21

Return to Question 1466 ↑

Q1467 ADVANCED · Block K

Best answer: D. Incorrect digital jaw relation despite accurately scanned teeth

Justification: An incorrect jaw relation can be faithfully reproduced by accurate manufacturing.

Why not the closest alternatives: Redoing the milling without correcting the bite record may reproduce the problem.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5, 2c.21

Return to Question 1467 ↑

Q1468 ADVANCED · Block K

Best answer: E. Manufacturing tolerance or distortion after milling/printing

Justification: Review fabrication shrinkage, orientation, calibration and post-processing tolerances when digital records are sound.

Why not the closest alternatives: Digital design accuracy and manufacturing accuracy are separate concerns.

Revise: Volume I — Unit 11 · Diploma LO 1.9, 2c.5, 2c.21

Return to Question 1468 ↑

BLOCK L · Complications, failures and maintenance

Q1082 ADVANCED · Block L

Best answer: D. Diagnose biological and mechanical causes before replacing the restoration

Justification: Repeated failure requires a diagnosis of biological risks and mechanical design rather than repetition.

Why not the closest alternatives: A stronger cement cannot control active periodontal disease.

Revise: Diploma LO 2c.21, 2c.22, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Finding causes of failures

Return to Question 1082 ↑

Q1083 ADVANCED · Block L

Best answer: C. Assess tissue support, occlusion and hygiene before denture remake

Justification: Examine adaptation, tissue condition, occlusion and patient function to decide reline, repair or remake.

Why not the closest alternatives: Replacement without analysis may recreate the same difficulty.

Revise: Diploma LO 2c.21, 2c.22, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Finding causes of failures

Return to Question 1083 ↑

Q1084 ADVANCED · Block L

Best answer: E. Refer a recurrent high-complexity failure for specialist assessment

Justification: Repeated high-complexity failure merits detailed specialist mechanical, occlusal and laboratory assessment.

Why not the closest alternatives: A third like-for-like remake without identifying loading will likely fail.

Revise: Diploma LO 2c.21, 2c.22, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Finding causes of failures

Return to Question 1084 ↑

Q1145 ADVANCED · Block L

Best answer: E. Assess fit, flexure and occlusal loading before making a replacement

Justification: Determine the bending, support and loading cause; repeating the geometry with a new alloy may reproduce the failure.

Why not the closest alternatives: Material substitution alone is not a comprehensive failure analysis.

Revise: Diploma LO 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Framework fracture investigations

Return to Question 1145 ↑

Q1146 ADVANCED · Block L

Best answer: B. Repair a minor removable component with a technically suitable method where function is sound

Justification: A suitable local repair can preserve a functional prosthesis when fracture and structural integrity allow.

Why not the closest alternatives: Full remake is not mandatory for every local defect.

Revise: Diploma LO 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Framework fracture investigations

Return to Question 1146 ↑

Q1147 ADVANCED · Block L

Best answer: A. Investigate abutment support and implant components in a recurrent fracture

Justification: Cantilever biomechanics, connection integrity, framework fit and occlusion influence recurrent component fracture.

Why not the closest alternatives: Simply tightening replacement screws may perpetuate the same overload.

Revise: Diploma LO 2c.21, 2c.25 · Volume I and Diploma learning outcomes (August 2026) · Theme: Framework fracture investigations

Return to Question 1147 ↑

Q1469 ADVANCED · Block L

Best answer: D. Relieve a confirmed pressure area after identifying local contact

Justification: Targeted adjustment and review are proportionate for a clearly local mechanical pressure injury.

Why not the closest alternatives: Routine antifungal treatment is not appropriate without an infective diagnosis.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.22, 2c.26

Return to Question 1469 ↑

Q1470 ADVANCED · Block L

Best answer: C. Repair a small isolated fracture when design and support are otherwise sound

Justification: A local repair is reasonable when the cause is isolated trauma and the underlying prosthesis remains sound.

Why not the closest alternatives: A complete remake may sacrifice a functioning framework unnecessarily.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.22, 2c.26

Return to Question 1470 ↑

Q1471 ADVANCED · Block L

Best answer: B. Re-evaluate denture support and occlusion after recurrent fracture

Justification: Recurrent fracture requires a root-cause review of load, geometry, adaptation and reinforcement.

Why not the closest alternatives: Another simple glue repair without redesign is likely to fail.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.22, 2c.26

Return to Question 1471 ↑

Q1472 ADVANCED · Block L

Best answer: B. Reline a stable prosthesis whose fitting surface no longer adapts

Justification: A suitable reline can restore adaptation when support changed but other prosthesis features remain appropriate.

Why not the closest alternatives: Replacing the entire denture may not be necessary.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.22, 2c.26

Return to Question 1472 ↑

Q1473 ADVANCED · Block L

Best answer: C. Reinforce prevention and scheduled recall for a high-caries-risk RPD wearer

Justification: Preventive risk management is needed to protect abutments; replacing a sound RPD without controlling caries is inadequate.

Why not the closest alternatives: Ignoring the salivary and dietary changes risks loss of key teeth.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1473 ↑

Q1474 ADVANCED · Block L

Best answer: E. Review periodontal support around strategic abutments before considering another bridge

Justification: Abutment periodontium determines the prognosis of the whole bridge; diagnose and manage disease before prosthetic repair.

Why not the closest alternatives: A new crown margin alone cannot restore lost attachment.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1474 ↑

Q1475 ADVANCED · Block L

Best answer: B. Investigate progressive peri-implant bone loss with appropriate referral

Justification: This may represent peri-implantitis and needs appropriate assessment and disease management/referral.

Why not the closest alternatives: Polishing cannot be the sole response to progressive supporting-bone loss.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1475 ↑

Q1476 ADVANCED · Block L

Best answer: B. Manage denture-related mucosal inflammation and cleaning practices

Justification: Denture hygiene, overnight removal, fit and contributing host factors should be addressed.

Why not the closest alternatives: Empirical antifungals alone may not correct the underlying conditions.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1476 ↑

Q1477 ADVANCED · Block L

Best answer: E. Assess and treat active marginal caries before replacing a loose crown

Justification: Recurrent decay and restorability must be assessed before definitive recementation.

Why not the closest alternatives: Recementing over active caries is not a sound long-term plan.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.26

Return to Question 1477 ↑

Q1478 ADVANCED · Block L

Best answer: E. Investigate occlusal load and connector dimensions after recurrent bridge fracture

Justification: Mechanical load and connector design are central to this failure and need reassessment.

Why not the closest alternatives: Simply changing cement is unlikely to prevent connector fracture.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.26

Return to Question 1478 ↑

Q1479 ADVANCED · Block L

Best answer: D. Check a resin-bonded framework for contamination and design failure before rebonding

Justification: Review the entire bonding protocol and load before deciding whether rebonding is appropriate.

Why not the closest alternatives: Rebonding under identical contamination and overload may fail again.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.26

Return to Question 1479 ↑

Q1480 ADVANCED · Block L

Best answer: D. Assess pulp and root prognosis following repeated unexplained tenderness

Justification: Root condition and possible fracture or combined pathology must be investigated before prosthetic intervention.

Why not the closest alternatives: Changing the crown without clarifying the source of tenderness risks unnecessary care.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.26

Return to Question 1480 ↑

Q1481 ADVANCED · Block L

Best answer: C. Routine periodic review of a stable prosthesis and oral tissues

Justification: Maintenance includes periodic examination of oral tissues, prosthesis fit and occlusion even when the patient has no symptoms.

Why not the closest alternatives: A symptom-free prosthesis does not mean the tissues can be ignored indefinitely.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1481 ↑

Q1482 ADVANCED · Block L

Best answer: B. Early review after a new adjustment to confirm mucosal healing and function

Justification: Timely review confirms healing and the adequacy of adjustment; unresolved lesions need escalation.

Why not the closest alternatives: A one-year delay would be inappropriate after a known injury.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1482 ↑

Q1483 ADVANCED · Block L

Best answer: B. Frequent risk-based review while active disease is being brought under control

Justification: Review interval should reflect active risk and response to prevention, not an arbitrary standard annual recall.

Why not the closest alternatives: A routine long recall may allow further damage in an unstable high-risk patient.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1483 ↑

Q1484 ADVANCED · Block L

Best answer: C. Urgent escalation for red-flag pathology detected during prosthetic review

Justification: Suspicious persistent pathology warrants urgent assessment, overriding routine denture follow-up.

Why not the closest alternatives: Further minor adjustments must not delay referral.

Revise: Volume I — Unit 12 · Diploma LO 2c.21, 2c.27

Return to Question 1484 ↑

BLOCK M · Governance, law and communication

Q1085 ADVANCED · Block M

Best answer: A. Present risks, alternatives including no treatment, and the patient-specific implications

Justification: Valid consent requires material patient-specific risks and reasonable alternatives including no treatment.

Why not the closest alternatives: Financial preference cannot substitute for informed patient choice.

Revise: Diploma LO 3a.1, 3a.5, 3b.1, 3b.4 · Volume I and Diploma learning outcomes (August 2026) · Theme: Consent and patient autonomy

Return to Question 1085 ↑

Q1086 ADVANCED · Block M

Best answer: D. Confirm understanding and capacity and offer supported decision-making

Justification: Capacity is decision- and time-specific and should be supported and assessed before elective consent.

Why not the closest alternatives: The mere presence of age or a diagnosis does not automatically remove capacity.

Revise: Diploma LO 3a.1, 3a.5, 3b.1, 3b.4 · Volume I and Diploma learning outcomes (August 2026) · Theme: Consent and patient autonomy

Return to Question 1086 ↑

Q1087 ADVANCED · Block M

Best answer: B. Document the complication candidly and follow duty-of-candour processes

Justification: Be open about the adverse outcome, explain remediation and document according to professional candour duties.

Why not the closest alternatives: Concealment because the error was from a laboratory is unacceptable.

Revise: Diploma LO 3a.1, 3a.5, 3b.1, 3b.4 · Volume I and Diploma learning outcomes (August 2026) · Theme: Consent and patient autonomy

Return to Question 1087 ↑

Q1148 ADVANCED · Block M

Best answer: E. Document the diagnostic reasoning, alternatives, patient goals and agreed risks

Justification: Decision-making and consent records must capture material risks, alternatives and the patient’s priorities.

Why not the closest alternatives: A treatment code and signature alone cannot demonstrate adequate communication.

Revise: Diploma LO 3a.1, 3a.3, 3b.5 · Volume I and Diploma learning outcomes (August 2026) · Theme: Records, communication and handover

Return to Question 1148 ↑

Q1149 ADVANCED · Block M

Best answer: C. Communicate a clear written maintenance and referral plan at handover

Justification: A handover should define the treatment performed, risks, complications, maintenance interval and responsibility for follow-up.

Why not the closest alternatives: An unsupported verbal instruction risks fragmented care.

Revise: Diploma LO 3a.1, 3a.3, 3b.5 · Volume I and Diploma learning outcomes (August 2026) · Theme: Records, communication and handover

Return to Question 1149 ↑

Q1150 ADVANCED · Block M

Best answer: B. Disclose a clinical error and arrange corrective care with candid documentation

Justification: Explain the event honestly, correct it, document the discussion and follow applicable candour processes.

Why not the closest alternatives: Hiding the mistake undermines autonomy and safety.

Revise: Diploma LO 3a.1, 3a.3, 3b.5 · Volume I and Diploma learning outcomes (August 2026) · Theme: Records, communication and handover

Return to Question 1150 ↑

Q1485 ADVANCED · Block M

Best answer: E. Explain a complication openly and arrange appropriate follow-up in accordance with duty of candour

Justification: Explain the event and corrective plan transparently, provide care and document appropriately under professional duties.

Why not the closest alternatives: Concealing a preventable injury would violate patient trust and governance.

Revise: Volume I — Unit 13 · Diploma LO 3a.2, 3a.8, 3b.3

Return to Question 1485 ↑

Q1486 ADVANCED · Block M

Best answer: A. Document a near miss and analyse the process to reduce recurrence

Justification: A near miss merits documentation and systems improvement, not merely relief that no harm occurred.

Why not the closest alternatives: An audit is valuable precisely because near misses reveal latent risk.

Revise: Volume I — Unit 13 · Diploma LO 3a.2, 3a.8, 3b.3

Return to Question 1486 ↑

Q1487 ADVANCED · Block M

Best answer: B. Respect and record an informed refusal after discussing options and consequences

Justification: Respect informed refusal and keep a clear record of alternatives and risk discussions.

Why not the closest alternatives: Clinician preference does not override a valid patient decision.

Revise: Volume I — Unit 13 · Diploma LO 3a.2, 3a.8, 3b.3

Return to Question 1487 ↑

Q1488 ADVANCED · Block M

Best answer: B. Refer beyond personal competence even if the patient strongly prefers treatment locally

Justification: Scope of competence and patient safety require referral or a safe alternative, irrespective of financial inducement.

Why not the closest alternatives: Consent cannot make inadequate competence acceptable.

Revise: Volume I — Unit 13 · Diploma LO 3a.2, 3a.8, 3b.3

Return to Question 1488 ↑

Q1489 ADVANCED · Block M

Best answer: B. Document baseline findings, risks, options, consent and treatment discussions

Justification: A contemporaneous record should document the substance of shared decision-making, not only a consent label.

Why not the closest alternatives: A generic consent form cannot substitute for an individual discussion.

Revise: Volume I — Unit 13 · Diploma LO 3a.5, 3b.1, 3b.5

Return to Question 1489 ↑

Q1490 ADVANCED · Block M

Best answer: E. Use a structured clinical audit to measure and improve recall or complication outcomes

Justification: An audit with a defined standard, measurable outcomes and re-audit is appropriate for a recurring service-quality problem.

Why not the closest alternatives: Anecdotal changes cannot reliably show improvement.

Revise: Volume I — Unit 13 · Diploma LO 3a.5, 3b.1, 3b.5

Return to Question 1490 ↑

Q1491 ADVANCED · Block M

Best answer: C. Ensure radiation justification and clinical evaluation are recorded

Justification: Imaging must be justified and appropriately evaluated/documented; audit and training may follow if the process failed.

Why not the closest alternatives: A radiograph is not justified simply because the equipment was available.

Revise: Volume I — Unit 13 · Diploma LO 3a.5, 3b.1, 3b.5

Return to Question 1491 ↑

Q1492 ADVANCED · Block M

Best answer: E. Provide a clear referral with urgency, clinical findings and relevant investigations

Justification: A useful referral includes the concern, urgency, timeline, prior records, relevant imaging, implant details and current management.

Why not the closest alternatives: A bare “please see” letter can delay safe decision-making.

Revise: Volume I — Unit 13 · Diploma LO 3a.5, 3b.1, 3b.5

Return to Question 1492 ↑

Q1493 ADVANCED · Block M

Best answer: D. Use a qualified interpreter or reliable supported communication for complex consent

Justification: Complex choices require reliable communication and a chance to express personal preferences independently.

Why not the closest alternatives: Nodding or family assent alone is not evidence of informed consent.

Revise: Volume I — Unit 13 · Diploma LO 3a.3, 3b.2, 3b.3

Return to Question 1493 ↑

Q1494 ADVANCED · Block M

Best answer: C. Check decision-specific capacity and optimise the person’s ability to decide

Justification: Capacity is decision-specific and support should be offered before concluding a person cannot decide.

Why not the closest alternatives: Diagnostic labels alone cannot replace an individual capacity assessment.

Revise: Volume I — Unit 13 · Diploma LO 3a.3, 3b.2, 3b.3

Return to Question 1494 ↑

Q1495 ADVANCED · Block M

Best answer: D. Discuss foreseeable long-term maintenance and alternatives in a balanced way

Justification: Correct unrealistic beliefs with clear discussion of biologic risks, complications, cleaning and maintenance alternatives.

Why not the closest alternatives: A successful initial implant restoration still needs lifelong care.

Revise: Volume I — Unit 13 · Diploma LO 3a.3, 3b.2, 3b.3

Return to Question 1495 ↑

Q1496 ADVANCED · Block M

Best answer: B. Document a significant treatment change and seek renewed informed consent

Justification: A material change in proposed treatment requires a renewed discussion and valid consent before proceeding.

Why not the closest alternatives: Consent to one bridge design is not blanket consent to unforeseen extractions or different irreversible care.

Revise: Volume I — Unit 13 · Diploma LO 3a.3, 3b.2, 3b.3

Return to Question 1496 ↑

BLOCK N · Integrated cases

Q1151 ADVANCED · Block N

Best answer: A. Stabilise disease and prepare a phased risk-based plan

Justification: Active disease and behavioural factors must be controlled before selecting definitive restorations.

Why not the closest alternatives: Expensive ceramics are likely to fail while primary risks remain active.

Revise: Diploma LO 2a.17, 2b.6, 2c.10, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Cross-domain sequence in combined disease

Return to Question 1151 ↑

Q1152 ADVANCED · Block N

Best answer: B. Arrange appropriate multidisciplinary assessment of complexity

Justification: Multiple interacting complex factors call for specialist multidisciplinary diagnosis and staged planning.

Why not the closest alternatives: A single-stage irreversible approach is not suitable for high-complexity uncertainty.

Revise: Diploma LO 2a.17, 2b.6, 2c.10, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Cross-domain sequence in combined disease

Return to Question 1152 ↑

Q1153 ADVANCED · Block N

Best answer: A. Choose local reversible repair after risk control where appropriate

Justification: Limited additive repair can match the complaint once causal risk is controlled.

Why not the closest alternatives: Full-mouth ceramics or complete clearance would be disproportionate.

Revise: Diploma LO 2a.17, 2b.6, 2c.10, 2c.12 · Volume I and Diploma learning outcomes (August 2026) · Theme: Cross-domain sequence in combined disease

Return to Question 1153 ↑

Q1154 ADVANCED · Block N

Best answer: D. Complete periodontal treatment and reassess abutments before long-span bridgework

Justification: Active periodontal disease and uncertain abutments preclude reliable long-span planning.

Why not the closest alternatives: A larger bridge will not stabilise untreated attachment loss.

Revise: Diploma LO 2a.19, 2b.7, 2c.13, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Integrated periodontal and prosthetic choices

Return to Question 1154 ↑

Q1155 ADVANCED · Block N

Best answer: E. Choose a hygienic, maintainable RPD when fixed work is high-risk and patient preference supports it

Justification: A carefully designed and maintainable RPD can match both functional needs and the patient's preferences.

Why not the closest alternatives: Surgical implant replacement is not obligatory.

Revise: Diploma LO 2a.19, 2b.7, 2c.13, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Integrated periodontal and prosthetic choices

Return to Question 1155 ↑

Q1156 ADVANCED · Block N

Best answer: A. Refer severe progressive peri-implant bone loss for specialist management

Justification: Progressive peri-implant disease may require specialist management beyond routine prosthetic adjustments.

Why not the closest alternatives: An adhesive or clasp design cannot resolve implant-associated bone loss.

Revise: Diploma LO 2a.19, 2b.7, 2c.13, 2c.20 · Volume I and Diploma learning outcomes (August 2026) · Theme: Integrated periodontal and prosthetic choices

Return to Question 1156 ↑

Q1178 ADVANCED · Block N

Best answer: E. Adopt a staged plan with disease stabilisation, provisionalisation and reassessment

Justification: Long-term success depends on disease and behaviour control, a prognosis-based plan and confirmation of readiness before definitive treatment.

Why not the closest alternatives: A technically impressive immediate reconstruction may have low sustainability.

Revise: Diploma LO 2a.18, 2b.6, 2c.11, 3b.1 · Volume I and Diploma learning outcomes (August 2026) · Theme: Competing reasonable options in complex treatment planning

Return to Question 1178 ↑

Q1179 ADVANCED · Block N

Best answer: D. Offer a conservative local adhesive restoration where prognosis is favourable

Justification: A well-selected partial-coverage adhesive restoration can preserve tissue in a favourable local case.

Why not the closest alternatives: Specialist full-mouth care or irreversible crowns are excessive.

Revise: Diploma LO 2a.18, 2b.6, 2c.11, 3b.1 · Volume I and Diploma learning outcomes (August 2026) · Theme: Competing reasonable options in complex treatment planning

Return to Question 1179 ↑

Q1180 ADVANCED · Block N

Best answer: D. Refer severe multidisciplinary complexity for specialist-led planning

Justification: The interactions among anatomy, occlusion, systemic and prosthetic issues justify specialist-led multidisciplinary planning.

Why not the closest alternatives: Immediate routine fixed rehabilitation is beyond a typical enhanced-skills scope.

Revise: Diploma LO 2a.18, 2b.6, 2c.11, 3b.1 · Volume I and Diploma learning outcomes (August 2026) · Theme: Competing reasonable options in complex treatment planning

Return to Question 1180 ↑

Q1497 ADVANCED · Block N

Best answer: E. Stabilise active disease, clarify abutment prognosis and phase conservative options

Justification: Risk control and tooth prognosis must precede a definitive bridge; saliva, caries and periodontal issues materially alter expected survival.

Why not the closest alternatives: A mechanically feasible bridge can fail early if multiple biologic risks remain uncontrolled.

Revise: Volume I — Unit All · Diploma LO 2a.1–2a.21, 2b.1–2b.12, 2c.1–2c.27, 3a.1–3b.5

Return to Question 1497 ↑

Q1498 ADVANCED · Block N

Best answer: D. Offer a provisional removable solution while complex diagnostic issues are addressed

Justification: A provisional removable option preserves aesthetics while pulp, periodontal and tissue prognosis are established.

Why not the closest alternatives: Immediate definitive implants or bridges may pre-empt unresolved post-trauma biology.

Revise: Volume I — Unit All · Diploma LO 2a.1–2a.21, 2b.1–2b.12, 2c.1–2c.27, 3a.1–3b.5

Return to Question 1498 ↑

Q1499 ADVANCED · Block N

Best answer: E. Use a conservative adhesive replacement after confirming stable disease and favourable anatomy

Justification: A suitable conservative adhesive replacement may fulfil aesthetics and function while protecting healthy abutments.

Why not the closest alternatives: Preparing both healthy teeth for full coverage would be unnecessarily invasive.

Revise: Volume I — Unit All · Diploma LO 2a.1–2a.21, 2b.1–2b.12, 2c.1–2c.27, 3a.1–3b.5

Return to Question 1499 ↑

Q1500 ADVANCED · Block N

Best answer: D. Refer for integrated specialist planning of major acquired or developmental defects

Justification: Complex developmental rehabilitation with multiple changing factors requires multidisciplinary specialist planning.

Why not the closest alternatives: An isolated bridge design cannot responsibly resolve the whole clinical picture.

Revise: Volume I — Unit All · Diploma LO 2a.1–2a.21, 2b.1–2b.12, 2c.1–2c.27, 3a.1–3b.5

Return to Question 1500 ↑

APPENDIX / EDITORIAL AND CLINICAL SCOPE

What is included

1,500 five-option questions with one keyed response: 270 Foundation, 730 Intermediate and 500 Advanced. Every source question retains its numeric ID (1–1500), its tier, stem, options, keyed answer, rationale and notebook references when supplied.

What has been checked

The most recent website data underwent a structural audit for question counts, uniqueness, answer-key validity, option completeness and identified answer cues. Some items were rewritten, but this edition has not been independently reviewed question-by-question by qualified examination specialists.

Why some answer notes are brief

The source data contain uneven explanations. Where a separate analysis of the distractors is missing, that limitation is displayed explicitly. We have not fabricated new clinical justifications in order to make the printed edition look more complete.

Medical and clinical guidance

Clinical recommendations, threshold values, drug doses, restorative dimensions, and laboratory material specifications can change with jurisdiction and updates to professional guidance. Check high-stakes clinical decisions against current authoritative standards before applying them.

Examination status

The Diploma learning outcomes are used for broad scope. The original question bank and updated website describe 150 SBAs in 180 minutes; candidates should verify the present examination format with the college. The specialist DSFE sample is used only as a reasoning benchmark, not an official source of Diploma questions.

Attribution and non-endorsement

Author: Dr. Kam O Man, Owen. Independent revision material. It is not an official examination publication and is not endorsed by RCSEd, the GDC, NHS England or DSFE.

Primary study companion: Diploma in Prosthodontics — Volume I, Integrated Revision Notebook (Final). Source data: November 2026 GitHub Pages revised SBA bank.